DBT: Deb's Board Training Domain Packet

# Area I: Values and Ethics

402 original practice questions, grouped by sub-area. Questions come first; answers and rationales for every option are in a separate section at the back, so no answer appears on the same page as its question.

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All questions are original practice items written for this study set; they are not ASWB questions. The ID beside each answer lets you report a problem with a question in the app.

# I.A Ethical Principles and Responsibilities

**1.** During a session at a community mental health agency, a client says they have saved a supply of pills and plan to take them tonight. The client then leaves the office and does not answer phone calls. The social worker calls emergency services. What information is **BEST** for the social worker to share with the dispatcher?

**A.** A general description of the risk without the client's name

**B.** The client's name, likely location and stated overdose plan

**C.** The client's diagnosis, treatment history and recent session notes

**2.** A social worker in private practice receives a subpoena from the attorney for a client's former partner in a custody case. It asks for the client's full treatment record. The client has not signed a release, and no judge has ordered the records. What should the social worker do **FIRST**?

**A.** Send the full record to the attorney by the stated deadline

**B.** Disregard the subpoena since it lacks a judge's signature

**C.** Contact the client to discuss the subpoena and their wishes

**D.** Send the attorney a letter asserting privilege for the client

**3.** A client who arrived in the United States as a refugee 4 months ago comes to a community agency for help with sleep problems. At intake, the client hesitates to sign forms and asks whether the information will go to immigration officials. What should the social worker do **FIRST**?

**A.** Explain what stays confidential and the limits that apply

**B.** Promise the client that their records will be kept private

**C.** Complete the trauma history while the client is present

**4.** A social worker at a program known in the community for treating substance use disorders gets a phone call from a client's employer. The caller asks whether the client has been attending sessions this month. The client has signed no release for the employer. How should the social worker respond?

**A.** Confirm attendance but decline to give clinical details

**B.** Neither confirm nor deny that the person is a client

**C.** Ask the employer to send the request in writing

**5.** Which term names a client's legal right to block a social worker's testimony about their sessions in court?

**A.** Privileged communication
**B.** Confidentiality
**C.** Privacy
**D.** Informed consent

**6.** A social worker in private practice receives a release, signed by a client, from the client's attorney. The client has sued another driver after a car crash and claims emotional distress. The release covers treatment records from the past year. Those records describe a past affair that the client may not want raised in court. What is the **MOST** appropriate response?

**A.** Assert privilege until a judge orders release of the records
**B.** Withhold the pages about the affair to protect the client
**C.** Send the records the release covers to the attorney today
**D.** Review the records' contents with the client, then release them

**7.** A social worker at a family services agency receives a court order signed by a judge in a child dependency case. It directs release of a parent's complete treatment record. The record includes the parent's detailed account of childhood sexual abuse, which has no bearing on the current case. What is the **BEST** response?

**A.** Send the complete record by the court's stated deadline
**B.** Decline to follow the order to protect the parent
**C.** Ask the court to limit the order or seal the record
**D.** Send a brief summary in place of the full record

**8.** A social worker at a community mental health center receives a subpoena from an insurance company's attorney in a disability lawsuit brought by a client. The social worker calls the client, who says firmly that they do not want the records released. The response date is 10 days away. What should the social worker do **NEXT**?

**A.** Get legal advice on asserting the client's privilege
**B.** Send the records, since the subpoena is a legal demand
**C.** Discuss with the client why they oppose the release
**D.** Ask the client to contact the attorney who issued it

**9.** A client at an outpatient behavioral health clinic has signed the clinic's standard consent for sharing information with other treating providers. The client's new psychiatrist at another practice asks the social worker for the process notes that the social worker keeps separately from the clinic record. What should the social worker do?

**A.** Send the notes, since the request is for treatment

**B.** Give the psychiatrist a verbal summary of the notes

**C.** Get the client's specific written authorization for them

**10.** A social worker in private practice bills a client's health plan with the client's written authorization. During a utilization review, the plan's reviewer asks for copies of all session notes to decide whether to approve more sessions. What is the **BEST** way for the social worker to respond?

**A.** Send copies of every session note the reviewer has requested

**B.** Send the treatment plan and progress the decision requires

**C.** Send the notes with identifying details removed

**11.** At the first meeting of an intensive outpatient group for adults recovering from alcohol use disorder, a member asks the social worker, 'Can you promise that nobody here will repeat what I say?' What is the **BEST** response?

**A.** Explain the group's privacy agreement and the limits on enforcing it

**B.** Assure the member that group rules keep everything private

**C.** Suggest the member keep sensitive details out of group

**12.** At the start of couples work by video, a social worker explained a policy that information shared privately by one partner may be raised in joint sessions. Before the fourth session, one partner emails the social worker about a recent affair and asks that it stay secret. What is the **BEST** response?

**A.** Keep the information private to protect the alliance

**B.** Disclose the affair to the other partner in the next session

**C.** End joint work and refer each partner for individual help

**D.** Remind the partner of the agreed rule and urge disclosure

**13.** A hospice social worker worked with a patient who died last month. The patient's adult child calls and asks for copies of the social worker's records, saying the family wants to understand the patient's last weeks. What should the social worker do **FIRST**?

**A.** Send the records, since the caller is the patient's next of kin

**B.** Explain that records stay sealed after a patient's death

**C.** Ask for documents showing legal authority for the estate

**14.** A child welfare social worker returns to their car after a home visit and finds that the agency laptop has been stolen. The laptop holds case files on several families and lacks encryption. What should the social worker do **FIRST**?

**A.** List which families' files were stored on the laptop

**B.** Report the theft to the privacy officer

**C.** Call each affected family to warn them of the breach

**D.** File a police report and wait to see if it is recovered

**15.** A social worker at an opioid treatment program gets a call from an emergency department physician. A program client was brought in unresponsive, and the physician asks for the client's current medications to guide treatment. The client has signed no consent for the hospital. What is the **BEST** response?

**A.** Provide the information needed for emergency care

**B.** Decline until the client can sign a written consent

**C.** Confirm that the person is enrolled, but share nothing more

**D.** Ask the client's emergency contact to give consent

**16.** A social worker meets weekly with a 15-year-old client in outpatient therapy. At intake, the parent agreed that session details would stay private unless a safety concern arose. A month later the parent calls, worried, and asks what the teen has been talking about. What is the **MOST** appropriate response?

**A.** Tell the parent nothing can be shared without the teen's consent

**B.** Give general progress and revisit the confidentiality agreement

**C.** Suggest that the parent ask the teen directly

**17.** Records kept by a social worker employed by a public school district are generally governed by which federal privacy law?

**A.** The HIPAA Privacy Rule

**B.** 42 CFR Part 2

**C.** FERPA

**18.** A state law limits disclosure of client information more than HIPAA does. Which rule generally applies?

**A.** The stricter state law

**B.** HIPAA, since federal law takes priority

**C.** Whichever law the state licensing board adopts

**19.** A social worker in a rural county wants advice from an out-of-town peer consultation group about a client with complex grief. The client is the town's only veterinarian, and the case involves a well-known local accident. How can the social worker **MOST** effectively protect the client's identity while consulting?

**A.** Refer to the client by initials during the consultation

**B.** Omit or change details that could reveal who the client is

**C.** Wait to consult until the client agrees to the consultation

**D.** Share full details, since consultants also owe confidentiality

**20.** During a video session, a client joins from a shared break room at work, and coworkers can be heard nearby. When the social worker asks whether the client can talk freely, the client says, 'It's fine, nobody pays attention.' The client then begins to describe a recent panic attack. What is the **MOST** helpful response?

**A.** Continue the session, since the client accepts the risk

**B.** End the session and require a private space next time

**C.** Explain the risk of being overheard and offer options

**21.** A client at a community counseling center signed a release last month allowing the social worker to send monthly progress updates to the client's employee assistance program. Today the client hands the social worker a written statement revoking that release. What should the social worker do **FIRST**?

**A.** Ask the client to explain the reasons for revoking

**B.** Send one final update to close out the arrangement

**C.** Ask the program to return the updates already sent

**D.** Stop further updates and note the revocation in the record

**22.** An adult patient on an inpatient psychiatric unit has declined to let staff share any information with family. The patient's parent calls the social worker and wants to describe changes in the patient's behavior in the weeks before admission. What is the **BEST** response?

**A.** Hear the parent's account but disclose no patient details

**B.** Decline the call, since the patient has refused family contact

**C.** Give a brief update so the parent feels less worried

**23.** A client who has finished treatment at a community agency asks the social worker for a full copy of their record. The social worker worries the client will be upset by the frank descriptions of the client's anger outbursts in the progress notes. What is the **BEST** response?

**A.** Release the copy after a meeting to review it together

**B.** Provide the copy and offer to go over it together

**C.** Give a written summary instead of the requested copy

**24.** An outpatient social worker has seen an adult client for depression for 2 months. The client's primary care physician calls before a routine visit next week and asks for a summary of therapy progress. The client has not signed a release for this physician and has a session with the social worker in 2 days. What is the **BEST** response to the physician?

**A.** Share a brief progress summary, since this is for treatment

**B.** Say that consent is needed before sharing, then follow up

**C.** Confirm the client is in treatment but share no clinical details

**D.** Ask the physician to send a written records request

**25.** A community mental health center's director wants to feature a current client's recovery story, with a photo and first name, in the agency's annual fundraising report. In a session, the client says they would be proud to take part and that telling their story supports their recovery. What is the **BEST** way for the social worker to proceed?

**A.** Decline, since featuring current clients in publicity is improper

**B.** Include the story and note the client's agreement in the chart

**C.** Obtain written authorization after explaining that saying no is fine

**26.** A client of a community mental health agency died in an incident covered widely in the news. A local story claims the agency had refused the client services. A reporter calls the social worker who served the client and asks for comment, and the agency director wants the record set straight. How should the social worker respond to the reporter?

**A.** Decline to discuss whether or how the person received care

**B.** Share the dates of service to correct the false claim

**C.** Decline to give details but say the story's claim is untrue

**27.** A client with PTSD asks the social worker to support a request to the local housing authority for a ground-floor unit. The client signs a release. The housing authority's verification form asks for the client's diagnosis, medications and full treatment history. What is the **MOST** appropriate way to complete the request?

**A.** Complete every item on the form, since the client signed a release

**B.** Decline, since housing verification falls outside the treatment role

**C.** Ask the client's physician to supply the diagnosis instead

**D.** Verify the disability and the need without listing the diagnosis

**28.** A client attends court-ordered outpatient therapy after an assault conviction. The client signed a release allowing the social worker to report attendance and participation to the probation officer. The officer calls and asks what the client has said in sessions about a new dating relationship. What is the **MOST** appropriate response?

**A.** Describe what the client said, since the court ordered treatment

**B.** Report attendance and participation, and decline to share more

**C.** Decline to report attendance or participation until a subpoena arrives

**29.** At a clinic intake, a client explains that their partner monitors the home phone and mail and becomes violent over medical bills. The client asks the clinic to contact them only by work email. The front desk says its system sends automatic reminder calls and letters to the home. What should the social worker do **FIRST**?

**A.** Have the client's contact method changed before any reminder goes out

**B.** Develop a safety plan for what to do if the partner finds a letter

**C.** Explain the reminder system and ask the client to accept home contact

**30.** Under HIPAA, what type of permitted disclosure is it when a practice sends a former client's outstanding bill to a collection agency?

**A.** Health care operations

**B.** Treatment

**C.** Payment

**D.** Required by law

**31.** Under 42 CFR Part 2, when state law lets a teen consent alone to substance use treatment, whose written consent is needed to share a progress update with the teen's parent?

**A.** The teen's consent alone

**B.** The parent's consent alone

**C.** Both teen and parent consent

**D.** The program director's approval

**32.** A hospital social work department receives a disability insurer's request for a former patient's psychosocial assessment. The attached release is signed and dated by the patient, but the lines for the purpose of the disclosure and the expiration date are blank. What should the social worker do **FIRST**?

**A.** Send the assessment, since the patient signed the release

**B.** Contact the patient to complete a valid authorization

**C.** Send a brief summary instead of the full assessment

**D.** Tell the insurer that the records require a subpoena

**33.** An adult client asks a community agency for a copy of their record. During treatment, the client's sibling met once with the social worker to share family history, after the worker agreed to keep the sibling's comments private. Those comments appear in the record and would identify the sibling. What is the **BEST** response to the client's request?

**A.** Release the full record, including the sibling's comments

**B.** Offer a verbal summary of the record instead of copies

**C.** Release the record with the sibling's comments withheld

**34.** A hospital social worker notices that a colleague on the team has smelled of alcohol at 2 morning meetings and missed a patient appointment last week. The colleague is off today, and no patient has been harmed. What should the social worker do **FIRST**?

**A.** Talk privately with the colleague about the observed concerns

**B.** File a complaint about the colleague with the licensing board

**C.** Keep watching the colleague until the pattern is clearer

**35.** On an inpatient psychiatric unit, a voluntary patient with schizophrenia who has decision-making capacity refuses a new antipsychotic. The psychiatrist proposes mixing it into the patient's juice without telling the patient. In the team meeting, the social worker objects, but the psychiatrist and nurse manager plan to start tomorrow. What should the social worker do **NEXT**?

**A.** Have the psychiatrist walk through the plan once more

**B.** Record the objection and follow the team's plan

**C.** Take the concern to the hospital ethics committee today

**D.** Tell the patient about the plan before tomorrow

**36.** A social worker co-leads a skills group with a newly hired colleague. In the last two sessions, the colleague taught distress-tolerance skills inaccurately, and members seemed confused. The colleague mentions never having received DBT training and says feedback is welcome. What should the social worker do **FIRST**?

**A.** Talk with the colleague and share DBT training resources

**B.** Tell the program manager the colleague lacks DBT training

**C.** Correct the colleague's teaching during the next group

**37.** A new client in private practice says that a social worker at another agency, who treated the client last year, borrowed $2,000 during treatment and never repaid it. The client feels embarrassed and asks whether that was allowed. What is the **BEST** response?

**A.** File a complaint about the former provider with the board

**B.** Say it was improper and describe complaint options

**C.** Call the former provider to ask about the loan

**D.** Suggest setting it aside to focus on current goals

**38.** During a hospital case review after a patient's suicide attempt, two staff members say a social work colleague ignored the patient's call the day before. The social worker covered that unit and had read the record, which shows the colleague returned the call within an hour and documented a safety check. What is the **MOST** appropriate action?

**A.** Wait to raise the documented facts until the review asks

**B.** Tell the colleague privately about the comments

**C.** Give the review leader the documented call details

**39.** A colleague at a community agency is contesting a disciplinary action by management. The colleague asks the social worker to collect signatures from the social worker's clients on a petition supporting the colleague. How should the social worker respond to the request?

**A.** Decline to involve clients in the dispute

**B.** Leave the petition in the waiting room without comment

**C.** Ask the clients who know the colleague well

**40.** Two social workers co-lead a 12-week outpatient group for adults in early recovery from substance use. At week 6, they begin a romantic relationship; neither supervises the other. Members have said that having both leaders matters to them. What is the **MOST** appropriate step for the co-leaders?

**A.** Keep the relationship private and continue co-leading

**B.** Tell the group about the relationship at the next session

**C.** Discuss with their supervisor whether to change co-leaders

**D.** Have one co-leader leave the group before next week

**41.** Social workers at a community mental health center have voted to strike in 10 days over unsafe caseloads, and the employer has refused further talks. A social worker plans to join the strike. The worker's caseload includes two clients with suicide attempts in the past 3 months and several clients who rely on weekly sessions. What should the social worker do **FIRST**?

**A.** Tell clients the strike date at their next sessions

**B.** Plan interim coverage and crisis access for each client

**C.** Withdraw from the strike because of the high-risk clients

**42.** A wildfire destroys hundreds of homes in a neighboring state, and thousands of residents move into evacuation shelters. A licensed clinical social worker with crisis training wants to help and plans to drive there this weekend to offer counseling at the shelters. What is the **BEST** way for the social worker to act on this goal?

**A.** Volunteer through an organized disaster mental health response

**B.** Go to the shelters and offer crisis counseling on arrival

**C.** Wait to help until licensed in the affected state

**D.** Offer free video counseling to evacuees from home

**43.** A social worker learns that a case manager at the same agency, who has a bachelor's degree in psychology and no license, sees private psychotherapy clients on weekends and advertises as a 'clinical social worker.' When the social worker raises the concern privately, the case manager says the title is harmless and declines to change the ads. What should the social worker do **NEXT**?

**A.** Report the case manager to the state licensing board

**B.** File a complaint about the case manager with NASW

**C.** Tell the agency director about the case manager's ads

**44.** At a first meeting, an older adult tells the social worker, a newly assigned case manager at a senior center, that the previous case manager 'never returned a single call' and 'made me feel like a bother.' The client then asks whether that worker was 'just lazy.' What is the **BEST** response by the social worker?

**A.** Agree that the previous worker handled the case poorly

**B.** Ask what the client needs to feel supported this time

**C.** Explain that heavy caseloads can delay returned calls

**D.** Offer to help file a complaint about the previous worker

**45.** Months ago, a coworker privately told the social worker about a past psychiatric hospitalization. Now their shared supervisor, concerned about the coworker's late reports, asks the social worker, 'Is something going on with them that I should know about?' Nothing the social worker has seen points to risk for clients. What is the **BEST** response?

**A.** Share the history to help explain the late reports

**B.** Urge the coworker to tell the supervisor about the hospitalization

**C.** Advise the supervisor to raise the late reports with the coworker

**46.** A social worker at a county child welfare agency drafts a letter to the local newspaper opposing a proposed state cut to foster care funding. The letter draws on the social worker's general experience and names no families. The draft is signed with the social worker's name, job title, and agency. The agency has not taken a position on the cut. What is the **BEST** action for the social worker?

**A.** Sign as a private citizen and note the views are personal

**B.** Withhold the letter, since county employees should stay neutral

**C.** Ask foster parents on the caseload to co-sign the letter

**D.** Send the letter as drafted, since advocacy is an ethical duty

**47.** A prospective client calls a group practice asking to start weekly individual therapy with the social worker. During the call, the client mentions seeing another mental health provider weekly for the same concerns and adds, 'I haven't told them I'm looking around.' What should the social worker do **FIRST**?

**A.** Schedule an intake appointment for this week as the client requested

**B.** Decline until the client ends the current therapy

**C.** Discuss how adding a provider could affect current care

**D.** Ask permission to contact the current provider

**48.** A colleague at a family service agency has filed a formal grievance over a demotion. The director tells the social worker that the colleague's former supervisory post is open and asks whether the social worker knows 'anything that would help the agency's case.' The social worker wants the post. Which response is **MOST** consistent with the social worker's ethical responsibilities?

**A.** Limit any comments to facts the social worker saw firsthand

**B.** Apply on own merits without commenting on the dispute

**C.** Withdraw from consideration out of loyalty to the colleague

**49.** A hospital social worker is asked to help with an older adult patient who was just diagnosed with advanced cancer. Before the physician meets with the patient, the patient's adult children ask the team not to share the diagnosis. They explain that in their family's culture, elders are protected from bad news. What should the social worker do **FIRST**?

**A.** Tell the adult children that the patient has a legal right to know

**B.** Agree to the family's request out of respect for their cultural values

**C.** Refer the disagreement to the hospital ethics committee for a decision

**D.** Ask the patient how much they want to know and who should be told

**50.** A transitional housing program requires staff to discharge any resident who misses curfew 3 times. A resident has missed curfew 3 times because a new job ends after curfew. The resident has no other place to live. What should the social worker do **FIRST**?

**A.** Ask agency leaders whether the rule allows a work exception

**B.** Begin discharge planning as the written policy requires

**C.** Encourage the resident to look for a job with earlier hours

**D.** Request an ethics committee review of the curfew policy

**51.** During a home visit, a 79-year-old client who lives alone says the adult children want an in-home aide after two minor falls this year, neither causing injury. The client understands the risks, describes a plan to wear a medical alert pendant, and declines the aide. Under the Ethical Principles Screen, which principle takes priority in this situation?

**A.** Least harm

**B.** Self-determination

**C.** Quality of life

**D.** Protection of life

**52.** A resident of a memory care unit who has major neurocognitive disorder asks staff every afternoon where their spouse is. The spouse died 2 years ago. Each time staff explain the death, the resident grieves as if hearing it for the first time. Staff ask the social worker how to answer. What is the **BEST** guidance?

**A.** Gently restate the fact of the death each time

**B.** Say the spouse is at work and will visit later

**C.** Ask the family to decide what staff should say

**D.** Acknowledge the longing and invite shared memories

**53.** A foster parent tells a child welfare social worker that a 14-year-old in care has been meeting a birth parent at a mall on weekends. The court allows only supervised visits. The foster parent adds that the youth asked them not to tell anyone. The social worker has a scheduled meeting with the youth this afternoon. What should the social worker do **FIRST**?

**A.** Promise the youth secrecy if the meetings stop

**B.** Notify the court that the visit order was violated

**C.** Ask the youth about the meetings and how safe they felt

**D.** Report the foster parent's account to the supervisor

**54.** A community mental health agency accepts a state grant that requires entering each client's name and diagnosis into a shared state database. The social worker believes this conflicts with clients' privacy and raises it with the supervisor. The supervisor agrees but says the program cannot change the grant terms. Enrollment begins next month. What should the social worker do **NEXT**?

**A.** Ask the ethics committee to review the requirement

**B.** Let each client decide whether to be entered

**C.** Enter client data as the grant requires

**D.** Leave client names out of the database entries

**55.** A client at a community agency receives disability benefits that limit how much the client may earn. In session, the client mentions doing cash yard work most weekends. The client then asks the social worker to sign a letter to the benefits office stating that the client has no income. What is the **MOST** appropriate response?

**A.** Sign a letter worded as the client's own report of no income

**B.** Decline to sign while keeping the disclosure private

**C.** Report the unreported earnings to the benefits office

**56.** A hospital social worker has one remaining charity-funded home health slot this month, and two patients being discharged qualify. The program's written criteria rank applicants by medical risk at home. A nurse suggests giving the slot to the patient who asked about it first. What is the **BEST** way for the social worker to decide?

**A.** Use the established risk ranking for both patients

**B.** Give the slot to the patient who asked first

**C.** Ask the program director to choose between the patients

**57.** Which situation is an ethical dilemma rather than a clear-cut ethical violation?

**A.** A supervisor asks a worker to backdate a missing progress note

**B.** A capable client refuses a shelter bed on a freezing night

**C.** An agency bills insurance for canceled group sessions

**D.** A worker invests in a current client's small business

**58.** At an adult day program, a client with mild neurocognitive disorder wants to keep paying their own bills. After weighing options and consulting a supervisor, the social worker supports the plan, and the client agrees to a weekly check-in with a trusted friend. The reasoning is documented. What should the social worker do **NEXT**?

**A.** Request a formal capacity evaluation of the client

**B.** Treat the question as resolved and close it

**C.** Set a date to review how the plan is working

**59.** A hospice social worker visits a family at home. The parent who is dying has asked that their 9-year-old not be told about the prognosis yet. While drawing with the social worker, the child asks directly whether the parent is going to die. What is the **BEST** response?

**A.** Validate the worry and bring the question to the parent

**B.** Answer honestly that the parent is expected to die soon

**C.** Say the parent's doctors will help them feel better

**60.** When values in the NASW Code of Ethics conflict, the Code asks social workers to use their informed judgment. According to the Code, how else should such a decision be evaluated?

**A.** By whether the client prefers the outcome

**B.** By the employing agency's written policies

**C.** By the Ethical Principles Screen's ranking

**D.** By how a peer review process would view it

**61.** A parent working toward reunification tells the child welfare social worker about drinking heavily one night last weekend while the children were at the foster home. The parent has met every other case plan goal and asks the worker to leave the drinking out of next week's court report. What is the **MOST** appropriate response?

**A.** Omit it, since the children were elsewhere

**B.** Include it in the report without telling the parent

**C.** Tell the parent the report will include it, with the progress

**62.** A social worker staffs a tenant association that is developing a cooperative housing building. At a meeting, members vote to bar anyone with a criminal record from applying. Several members have relatives returning from prison who had hoped to apply. What is the **BEST** response by the social worker?

**A.** Help members write the vote into the building's rules

**B.** Raise how the rule excludes people and discuss alternatives

**C.** Tell members the ban is unjust and ask them to repeal it

**63.** A parole officer asks a reentry program social worker how a client is 'doing overall.' The client's release covers only attendance at mandated treatment, and the client also attends an optional peer group. After reviewing the release and the Code and talking with a supervisor, the social worker tells the officer that the client attends all mandated sessions and explains this choice to the client. What should the social worker do **NEXT**?

**A.** Ask a second colleague whether they agree

**B.** Record the options weighed, consultation and reasoning

**C.** Set a date to review how the decision works out

**64.** A social worker at a nonprofit counseling agency meets with a client who works nights and can attend only evening sessions, which the agency does not offer. A free clinic nearby runs evening groups for the client's concern. The agency director has told staff not to mention that clinic because funding depends on client numbers. Which action is **MOST** consistent with the social worker's ethical duties?

**A.** Tell the client about the clinic's evening services

**B.** Help the client ask the employer for a daytime shift

**C.** Ask the director to approve mentioning the evening clinic

**65.** A new state law requires mental health providers to notify a parent before a minor's fourth counseling session. A social worker in a school-based counseling program believes the law will keep some teens from seeking help. The program's attorney has confirmed that the law applies, and several students are already in counseling. What is the **BEST** action for the social worker to take?

**A.** Keep the current practice until a court rules on the law

**B.** Stop taking new student clients until the law changes

**C.** Comply with the law and advocate for changing it

**D.** Ask an ethics committee whether to follow the law

**66.** An ethics committee debates whether a hospital social worker may share a patient's history with a landlord to save the patient's housing. One member grants that this single disclosure might help. The member argues that keeping patient information private, case after case, yields the best results for patients overall, so the history should stay private. Which ethical theory is this member **MOST** likely using?

**A.** Rule utilitarianism

**B.** Act utilitarianism

**C.** Deontology

**67.** A hospital patient with cancer and full decision-making capacity is visited by an adult child, who does most of the talking. When the social worker mentions the upcoming scan, the patient says, 'Just give the results to my child. I don't want to hear them.' The oncologist's usual practice is to give results directly to patients. What should the social worker do **FIRST**?

**A.** Encourage the patient to hear the results from the oncologist

**B.** Request an ethics consultation on keeping results from a patient

**C.** Meet privately with the patient to confirm the wish

**D.** Help the team record the wish and give results to the adult child

**68.** A hospice patient with full decision-making capacity wants to ride 300 miles by car to see a sibling one last time. The nurse has told the patient that the drive will be exhausting and could worsen symptoms. The team asks the social worker, who has not yet met the patient, to talk the patient out of it. What should the social worker do **FIRST**?

**A.** Suggest a video visit instead, as a safer way to see the sibling

**B.** Help the patient plan the trip with the team's precautions

**C.** Explore with the patient what the visit means to them

**69.** At intake at a community mental health agency, a client says they do not read well and asks the social worker to 'just show me where to sign' the consent form. The client speaks English fluently. What is the **MOST** effective way for the social worker to obtain informed consent?

**A.** Read the form aloud, then have the client sign it

**B.** Explain each part plainly and check understanding

**C.** Point to the signature line, as the client requested

**70.** A supervisor at a family service agency asks a social worker to video-record sessions with an adult client for use in supervision. The client has not yet heard about the request. Before the next session, what is the **MOST** appropriate way for the social worker to proceed?

**A.** Tell the client the agency requires recording for supervision

**B.** Start recording and tell the client they may ask to stop

**C.** Explain the recording's purpose and use and ask for consent

**71.** A resident of a skilled nursing facility has major neurocognitive disorder and has been found to lack capacity to make health care decisions. The resident's adult child, the legally appointed health care agent, has consented to the resident joining a weekly reminiscence group. What should the social worker do **NEXT**?

**A.** Enroll the resident now, since the agent has already consented

**B.** Have the resident sign the group consent form too

**C.** Explain the group simply and ask the resident's assent

**D.** Postpone the group until capacity improves

**72.** A social worker trained in prolonged exposure therapy has provided supportive therapy for 4 months to an adult client with posttraumatic stress disorder. The social worker now believes prolonged exposure would help and wants to begin it. What should the social worker do **FIRST**?

**A.** Discuss the approach, its risks and alternatives with the client

**B.** Have the client sign an updated treatment consent form

**C.** Begin with a short trial exposure to gauge the response

**73.** A client ordered by a court to attend a driving-under-the-influence program asks the social worker at intake, 'What happens if I don't want to do this?' What is the **MOST** appropriate response?

**A.** Tell the client the court order removes the right to refuse

**B.** Explain the right to decline and what is reported

**C.** Suggest the client ask their attorney before going further

**74.** A parent brings an 8-year-old to an outpatient clinic for anxiety treatment. The parent says the child's other parent lives across town after a recent divorce and 'doesn't need to know about this.' The parent is unsure what the custody order says about medical decisions. What should the social worker do **FIRST**?

**A.** Begin treatment with the consent of the parent who is present

**B.** Call the other parent to ask for permission to treat

**C.** Review the custody order to confirm who may consent

**D.** Require both parents' signatures before any sessions

**75.** An adult client at a group private practice asks to use their employer health plan to pay for counseling. The plan requires a diagnosis on every claim and reviews progress after 12 sessions before approving more. The client is about to sign the consent form. What is **MOST** important for the social worker to explain before services begin?

**A.** What the plan will receive and how it may limit sessions

**B.** That session content will stay between the client and worker

**C.** That the client may drop the plan and self-pay later

**76.** A 16-year-old comes to a community health clinic without a parent and asks the social worker for help with daily drinking. The teen does not want a parent contacted and is ready to start today. What should the social worker do **FIRST**?

**A.** Call a parent to get consent before any counseling

**B.** Determine whether state law lets the teen consent alone

**C.** Begin counseling, since the teen is asking for help

**77.** A 28-year-old client with moderate intellectual developmental disorder lives in supported housing. The client's legal guardian, a sibling, has signed consent for the client to attend a sheltered day program five days a week. In planning meetings, which the guardian has not attended, the client says again and again that they want to keep their part-time grocery job instead. What should the social worker do **FIRST**?

**A.** Start the day program, since the guardian has consented

**B.** Treat the client's preference as overriding the guardian

**C.** Petition the court to review the guardianship

**D.** Bring the client's stated wishes to the guardian

**78.** Before a first session at an outpatient clinic, a social worker's colleague suggests looking up the new client's public social media accounts 'to get the full picture.' The referral describes mild anxiety about a job change. How should the social worker handle the suggestion?

**A.** Look at public posts, which anyone can already see

**B.** Search now, then tell the client what turned up

**C.** Seek the client's consent before doing any search

**79.** A couple receives services at a university training clinic. At intake, both partners consented in writing to a faculty team observing sessions through a one-way mirror. Midway through the fifth session, one partner says they no longer want the team watching. What should the social worker do **FIRST**?

**A.** Continue, since both partners consented in writing at intake

**B.** Stop the team's observation from this point on

**C.** Ask the team lead to explain why observation helps

**D.** Explore the partner's concerns before making any change

**80.** A client comes to an outpatient substance use program's walk-in hours asking to start treatment today. The client smells of alcohol, slurs some words, says they had 'a few drinks' an hour ago, and walks steadily. The intake packet includes the consent to treatment. What is the **BEST** way for the social worker to proceed?

**A.** Offer brief support now and hold consent until sober

**B.** Complete the consent forms while motivation is high

**C.** Ask the client to return another day when sober

**81.** When a parent gives legal permission for a 10-year-old's treatment, what term describes the child's own agreement to take part?

**A.** Proxy consent

**B.** Implied consent

**C.** Assent

**D.** Substituted judgment

**82.** A community mental health agency has started offering text-message check-ins between sessions. An adult client in the depression program asks to begin receiving them this week. What should the social worker do **FIRST**?

**A.** Text the client the policy and start once the client replies OK

**B.** Review the texting policy and its risks, then get consent

**C.** Agree, and invite the client to text during any crisis

**83.** A patient on an inpatient psychiatric unit has schizophrenia and is preparing for discharge. The team suggests that the patient's parent sign consent for a supported housing program. The patient has no guardian. In a capacity evaluation, the psychiatrist documents that the patient can describe the program, weigh it against living alone and state a choice. What should the social worker do **NEXT**?

**A.** Ask the parent to sign the program consent as the team suggested

**B.** Review the program with the patient and obtain their own consent

**C.** Ask the patient and parent to sign the consent together

**84.** Shelter staff tell a new resident that their bed will be given to someone else unless they sign up for the agency's counseling program today. The resident accurately explains the program's purpose, risks and alternatives, then signs the consent form. Which element of valid consent is **MOST** likely missing?

**A.** Voluntariness

**B.** Disclosure

**C.** Comprehension

**85.** A 17-year-old requests counseling at a community clinic for panic attacks. The client lives independently and gives intake a copy of a court order of emancipation. The intake coordinator says a parent must still sign the treatment consent because the client is under 18. What is the **MOST** appropriate way for the social worker to proceed?

**A.** Have the client sign the treatment consent themselves

**B.** Contact a parent to sign the consent before treatment begins

**C.** Have the client sign, then let a parent know care has begun

**86.** A child welfare court orders a parenting capacity assessment, and a social worker is assigned to complete it. At the first meeting, the parent says, 'I'm so glad I finally have someone to talk to about everything.' What should the social worker do **FIRST**?

**A.** Begin the interview while the parent is open and engaged

**B.** Offer supportive counseling sessions alongside the assessment

**C.** Explain the evaluator role and who will receive the report

**87.** A new client has filed a lawsuit against a former employer, claiming emotional distress from workplace harassment. The client wants therapy focused on the harassment and its effects on sleep and mood. At intake, the client asks what the social worker will write down. What is the **BEST** response?

**A.** Explain that the lawsuit may lead to requests for the records

**B.** Recommend waiting to start therapy until the lawsuit is resolved

**C.** Agree to keep notes brief and leave out harassment details

**D.** Assure the client that therapy records are privileged in court

**88.** Police bring an adult to the emergency department on an emergency hold after the person threatened to jump from a bridge. The person refuses to sign any forms and says, 'I'm not agreeing to anything.' The social worker is assigned to complete the psychosocial assessment. What should the social worker do **FIRST**?

**A.** Start the assessment interview and return to the forms later

**B.** Explain the purpose of the assessment and what may be shared

**C.** Wait to begin until the person agrees to sign the forms

**89.** A clinic's waitlist for therapy intake has grown to about 6 weeks. Before a meeting with hospital discharge planners, the clinic director tells the outreach social worker to keep promising 'appointments within the week' so referrals and funding stay steady. Several patients referred last month waited weeks without care. What should the social worker do **FIRST**?

**A.** Describe the program to planners as the director instructed

**B.** Tell the director that planners need the actual wait time

**C.** Report the director's instruction to the clinic's board

**D.** Give planners the actual wait without telling the director

**90.** A hospital social worker helped a patient begin a disability benefits appeal and said the filing deadline was 30 days away. A week later, rereading the denial letter, the social worker sees the deadline is actually 3 days away. The patient was discharged home yesterday and has a working phone. What should the social worker do **FIRST**?

**A.** Tell the supervisor and wait for direction on the patient

**B.** Correct the deadline date in the patient's chart

**C.** Call today with the correct date and help submit it

**91.** A social worker in a group practice completes a 2-day workshop in a trauma treatment model. The practice's marketing coordinator then updates the website to call the social worker a 'certified trauma specialist.' Certification in the model requires supervised cases that the social worker has not yet begun. What should the social worker do?

**A.** Keep the description while starting the certification cases

**B.** Have the profile describe the training actually received

**C.** Tell new clients at intake that certification is pending

**92.** A client with panic disorder asks a social worker to use a 'one-session trauma release' technique that online videos say ends panic in a single visit. The social worker, who is trained in cognitive-behavioral therapy for panic, finds no published research supporting the technique. The client has not had any prior therapy for panic and says quick relief is the goal. What is the **BEST** response?

**A.** Ask what draws the client to this technique

**B.** Try the technique once, as the client prefers

**C.** Refer the client to a practitioner who offers the technique

**D.** Explain the evidence and offer an established treatment

**93.** Residents of a public housing complex tell a community social worker that the city plans to close the neighborhood's only public health clinic. Several residents want to speak at next month's city council hearing but have never addressed public officials. What is the **MOST** appropriate role for the social worker?

**A.** Help residents prepare and deliver their own testimony

**B.** Testify at the hearing on the residents' behalf

**C.** Stay neutral because the closure is a political matter

**94.** A client in treatment for depression works at a warehouse. The client describes unpaid overtime, a paycheck $600 short, and a supervisor who threatens to cut hours if workers complain. The social worker has explored how the job affects the client's mood, and the client says recovering the wages would ease both the bills and the hopelessness. What should the social worker do **NEXT**?

**A.** Focus sessions on coping skills for workplace stress

**B.** Explore further what the lost wages mean to the client

**C.** Connect the client with a free workers' rights legal clinic

**D.** Call the employer to request the unpaid wages

**95.** A family shelter starts requiring residents to wear colored wristbands in the dining hall that show whether they receive public benefits, so staff can track meal funding. Residents tell a social worker they feel marked in front of other families. Which core value of the profession does this practice **MOST** directly conflict with?

**A.** Social justice

**B.** Dignity and worth of the person

**C.** Importance of human relationships

**96.** A child protective services social worker arrives with a court order and a police officer to remove a 6-year-old after substantiated severe neglect. The parent is crying but calm and cooperative and asks for a few minutes to say goodbye and to pack the child's stuffed animal and pajamas. What is the **BEST** response?

**A.** Complete the removal quickly to limit the child's distress

**B.** Let the officer decide whether a goodbye is safe

**C.** Permit a brief supervised farewell and the comfort items

**97.** Which core value in the NASW Code asks social workers to promote clients' right to make their own choices?

**A.** Social justice

**B.** Integrity

**C.** Service

**D.** Dignity and worth

**98.** A client who has met with the social worker twice about job stress asks for a letter stating a need for an emotional support dog. The landlord will begin eviction over the unapproved dog in 10 days. Neither session has touched on the dog or the client's housing. What should the social worker do **FIRST**?

**A.** Assess whether a disability-related need for the animal exists

**B.** Refer the client to a provider who evaluates support animal requests

**C.** Write the requested letter so the eviction can be stopped

**99.** A hospital social worker notices that uninsured patients discharged to shelters rarely leave with a follow-up appointment, while insured patients almost always do. For 3 months, the social worker has arranged appointments case by case and tracked the gap, which has not narrowed. What should the social worker do **NEXT**?

**A.** Keep arranging follow-up appointments for each affected patient

**B.** File a complaint about the hospital with a federal civil rights office

**C.** Present the findings to leadership with proposed changes

**100.** At a community mental health agency, a psychiatrist asks the social worker on a 9-year-old's treatment team to give the child a standardized intelligence test to help clarify a diagnosis. The social worker has never been trained to administer or interpret such tests. A psychologist on staff provides cognitive testing. What is the **MOST** appropriate response by the social worker?

**A.** Refer the child to the psychologist for testing

**B.** Give the test after studying its administration manual closely

**C.** Do the testing and have the psychologist score it

**D.** Describe the child's abilities from interview observations

**101.** A social worker has treated an asylum seeker for PTSD for 4 months. In earlier sessions the client described being beaten in detention in the home country. The client's immigration attorney asks the social worker to write a forensic evaluation stating whether the client's symptoms are consistent with torture, for the asylum hearing. The social worker has never conducted forensic evaluations. What is the **BEST** response?

**A.** Write the asylum evaluation from the 4 months of session notes

**B.** Write the evaluation after consulting a forensic evaluator

**C.** Ask the client to describe the abuse again in full detail

**D.** Refer the client to an evaluator trained in asylum cases

**102.** A social worker hears many clients describe abuse and violence. Over time, the social worker develops intrusive images, avoidance and hyperarousal. Which term describes this pattern?

**A.** Secondary traumatic stress

**B.** Occupational burnout

**C.** Countertransference reaction

**103.** A child welfare social worker has carried a caseload far above agency limits for a year. The social worker feels drained before each workday, has grown cynical about the families served and doubts that the work makes a difference. At night, the social worker lies awake thinking about overdue paperwork. Which condition is **MOST** likely?

**A.** Secondary traumatic stress

**B.** Vicarious trauma

**C.** Countertransference

**D.** Occupational burnout

**104.** A social worker has counseled adult survivors of sexual assault for 8 years. Over time, the worker has come to assume most strangers have harmful motives, has stopped letting their teenager ride public transit, and tells a colleague that the world now feels unsafe in a way it never did. The worker says these views developed gradually. Which concept **MOST** likely explains these changes?

**A.** Secondary traumatic stress

**B.** Burnout

**C.** Vicarious trauma

**D.** Countertransference

**105.** What term names the good feelings helpers get from doing trauma work well?

**A.** Compassion satisfaction

**B.** Vicarious resilience

**C.** Posttraumatic growth

**106.** A social worker conducts forensic interviews at a child advocacy center. In recent weeks, the worker has begun cutting interviews short when children start giving details, startles at small noises, and lies awake replaying children's statements. Two interviews this month had to be repeated. What should the social worker do **FIRST**?

**A.** Complete a self-screening measure to gauge the symptoms

**B.** Tell the supervisor how the reactions affect interviews

**C.** Request a transfer out of forensic interviewing

**D.** Begin a daily mindfulness practice to ease symptoms

**107.** After staff cuts, social workers on a hospital's medical units have handled nearly double their usual discharges for a year. A social worker notices that several colleagues, like the worker, feel drained, have grown detached from patients, and doubt their work makes a difference. Management's response so far has been an optional lunchtime wellness webinar. What is the **BEST** action for the social worker?

**A.** Encourage colleagues to attend the wellness webinars

**B.** Build a personal routine of exercise, sleep and breaks

**C.** Propose workload relief to leaders using discharge data

**108.** Two weeks after a child on the unit's caseload died from abuse, a child welfare supervisor notices that a supervisee who worked the case startles easily, avoids that neighborhood, and snaps at coworkers. The supervisee has not mentioned any of this. What should the supervisor do **FIRST**?

**A.** Refer the supervisee to the employee assistance program for help

**B.** Meet privately to ask how the case is affecting the supervisee

**C.** Reassign the supervisee's cases in that neighborhood

**109.** A hospital discharge planner whose caseload has doubled has started calling patients by bed number and diagnosis in team meetings. When a family calls in tears about a delayed discharge, the planner answers curtly and feels little concern. Which component of burnout is **MOST** likely present?

**A.** Emotional exhaustion

**B.** Reduced personal accomplishment

**C.** Depersonalization

**110.** In 2021, the NASW Code of Ethics added a sentence on caring for one's own well-being. Which ethical principle now holds it?

**A.** Competence

**B.** Service

**C.** Dignity and worth of the person

**D.** Integrity

**111.** A community mental health social worker learns in the morning that a long-term client died by suicide overnight. The supervisor offers to have colleagues cover the worker's afternoon, which includes two new crisis evaluations, and mentions a support meeting for staff. The worker feels numb and would rather stay busy. What is the **MOST** appropriate response for the worker?

**A.** Keep the afternoon's crisis evaluations to stay in routine

**B.** Accept coverage for this afternoon's crisis evaluations

**C.** Ask a colleague to sit in on the crisis evaluations

**112.** A social worker in solo private practice gave clients a personal cell number when the practice opened and now answers their texts most evenings and weekends. The worker sleeps poorly, feels irritable in sessions, and recently answered a client's late-night scheduling text sharply. What is the **MOST** effective way to address the problem?

**A.** Set after-hours limits and explain crisis options to clients

**B.** Begin personal counseling for the stress and poor sleep

**C.** Stop replying to evening texts without telling clients

**113.** After a flood, a county mental health agency asks staff to add evening shifts at evacuation shelters. A social worker whose own home flooded is staying with relatives, sleeping poorly, and handling insurance claims. The social worker wants to help and has not told the supervisor about the damage. What is the **BEST** action for the social worker?

**A.** Tell the supervisor about the losses and agree on limited shifts

**B.** Work the shifts and seek supervision if stress becomes a problem

**C.** Ask the supervisor to excuse the social worker from shelter work

**114.** A social worker newly assigned to a sexual assault program finds that clients' accounts stir memories of an assault the social worker experienced years ago. The social worker has discussed this in supervision, and the supervisor suggested personal counseling. The social worker worries that seeking counseling would look like incompetence. Which step is **MOST** consistent with ethical self-care?

**A.** Ask the supervisor to provide personal counseling in supervision

**B.** Begin personal counseling while continuing in supervision

**C.** Request a transfer away from the sexual assault program

**115.** A program director asks a social worker to start offering eye movement desensitization and reprocessing (EMDR) to clients next month. The social worker already treats several clients with trauma using cognitive behavioral methods but has had no EMDR training. A trained EMDR provider works at a clinic across town. What is the **MOST** appropriate response?

**A.** Begin EMDR using an online training video as a guide

**B.** Transfer current trauma clients to the trained EMDR provider

**C.** Explain the need for formal training and supervision first

**116.** On Monday morning, a licensed clinical social worker in solo private practice checks the state board's website and finds that the license expired 10 days ago because required continuing education hours were never reported. Eight clients are scheduled this week. What should the social worker do?

**A.** Pause client sessions until the license is reinstated

**B.** Finish the missing hours online and keep this week's sessions

**C.** See current clients while the license renewal is pending

**117.** The only social worker at a rural health clinic has a new client with gambling disorder who wants treatment. The nearest gambling specialist is 3 hours away, and the client cannot travel or use video. The social worker has general addiction training but has never treated gambling disorder. What should the social worker do **FIRST**?

**A.** Make a referral to the distant specialist

**B.** Arrange consultation with a gambling expert

**C.** Begin weekly gambling-focused sessions with the client

**118.** For years, an employee assistance social worker has responded to workplace deaths at a large company with a required group meeting within 48 hours, in which each employee describes the event and their reactions. While updating training, the social worker reads a systematic review finding that single-session debriefing does not prevent PTSD and may increase symptoms for some people. What is the **BEST** response?

**A.** Switch to psychological first aid with follow-up

**B.** Keep the meetings but make attendance voluntary

**C.** Continue the meetings since employees rate them highly

**119.** A pre-licensure associate social worker at an outpatient clinic learns that the clinical supervisor will be on medical leave for 6 weeks. The supervisor suggests the associate keep seeing clients and save questions until the leave ends. The associate replies that two clients have recent self-harm and waiting is not workable, but the supervisor repeats the suggestion. What should the associate do **NEXT**?

**A.** Raise the concern with the supervisor once more

**B.** Continue sessions and keep a list of questions

**C.** Pause client sessions until the supervisor returns

**D.** Ask the program director to arrange interim supervision

**120.** A licensed clinical social worker kept the license current through continuing education during a 5-year break from practice. Before the break, the worker provided school-based counseling to children. A community clinic now offers the worker a full caseload of adults with serious mental illness, starting in 2 weeks. What is the **MOST** appropriate way for the worker to proceed?

**A.** Accept the full caseload after a weekend training on serious mental illness

**B.** Decline the position because the population is new

**C.** Accept with gradual intake, training and supervision

**121.** What is the main purpose of the continuing education that state boards require for social work license renewal?

**A.** Raising the profession's standing among other disciplines

**B.** Protecting the public by keeping practitioners competent

**C.** Preparing licensees for specialty certification

**122.** At a training, a social worker at an opioid treatment program learns that the federal confidentiality rule for substance use disorder records was revised in 2024. The program's consent forms date from 2019, and the social worker will ask new clients to sign them this week. The program has a compliance officer who oversees confidentiality policies. What should the social worker do **FIRST**?

**A.** Continue using the forms until the program issues new ones

**B.** Ask the compliance officer whether the forms meet the current rule

**C.** Revise the consent form using a summary of the new rule

**D.** Pause new consents until the compliance officer approves new forms

**123.** After a weekend continuing education workshop, a social worker is eager to try a new brief technique for trauma. The presenter supported it with case stories and client testimonials. The social worker has several clients with trauma histories and a supervisor available for consultation. Which action is **BEST** before the social worker uses the technique with clients?

**A.** Try it with consenting clients while getting supervision

**B.** Ask the presenter for more detailed case examples

**C.** Search for published outcome studies of the method

**124.** A licensed social worker opens a solo practice and no longer has a supervisor. Which habit is **MOST** appropriate for keeping clinical judgment sound over time?

**A.** Meeting regularly with a consultation group to review cases

**B.** Completing the continuing education hours required for renewal

**C.** Reviewing past supervision notes when a case feels difficult

## I.B Ethical Service Delivery

**125.** An adult client with schizophrenia has been stable for a year on an antipsychotic medication. The client tells the social worker that weight gain from the medication is unbearable and that they plan to stop taking it. The client is coherent and understands the risk of relapse. How should the social worker respond?

**A.** Tell the client that stopping the medication would be a mistake

**B.** Ask the client's family to encourage continued medication use

**C.** Explore the concerns and link the client with the prescriber

**D.** Suggest that the client lower the dose gradually before stopping

**126.** An adult client with bipolar I disorder left a psychiatric unit 3 weeks ago. The client now declines the mood stabilizer the psychiatrist prescribed, preferring to manage symptoms with sleep routines and a peer support group. The client sleeps normally, is not manic and denies suicidal thoughts. What is the social worker's **BEST** response?

**A.** Respect the choice and build a plan for early warning signs

**B.** Ask the psychiatrist to seek a court order for the medication

**C.** Tell the client that relapse is likely without the medication

**127.** During a home visit, an assertive community treatment social worker finds that a client with schizophrenia has refused all food and water for 3 days, believing it is poisoned. The client is dizzy, confused and unsteady when standing, and refuses to go to the hospital. What should the social worker do **FIRST**?

**A.** Explore the client's reasons for refusing the hospital

**B.** Respect the refusal and return tomorrow with sealed food

**C.** Seek emergency evaluation despite the client's refusal

**128.** In an integrated primary care clinic, a client who left home last month after a partner pushed them down the stairs says they have decided to move back in. The couple has no children. The client asks the social worker for help 'being smart about it.' What is the **BEST** response?

**A.** Urge a shelter stay until the partner gets help

**B.** Explore the client's mixed feelings about going back

**C.** Develop a safety plan with the client for living at home

**D.** Contact the partner to set conditions for the return

**129.** A client with bipolar I disorder has been stable for a year at a community mental health center. The client says that during past manic episodes, hospital staff gave medications that caused severe side effects and ignored the client's wishes. The client wants future care to reflect those preferences. What is the **BEST** way for the social worker to help?

**A.** Ask the psychiatrist to add the preferences to the chart

**B.** Help the client complete a psychiatric advance directive

**C.** Suggest the client tell a relative which medications to refuse

**D.** Reassure the client that hospitals follow stated wishes

**130.** An adult with an intellectual disability chooses trusted people who help them understand options and communicate choices, while the adult keeps the final say. Which arrangement does this describe?

**A.** Supported decision-making

**B.** Limited guardianship

**C.** Substituted judgment

**D.** Health care power of attorney

**131.** A 22-year-old client at a community counseling center has a job offer in another city. The client tells the social worker, 'In my family, big decisions are made together. I want my parents to decide this with me.' What is the **BEST** response?

**A.** Encourage the client to make this choice independently

**B.** Help the client plan how to include the parents

**C.** Explore whether the parents are pressuring the client

**132.** A client who entered an inpatient psychiatric unit voluntarily for depression with psychotic features asks to leave in 2 days, sooner than the team advises, to keep a new job. The psychosis has cleared, and the psychiatrist's assessment this morning found no suicidal thoughts or intent. The team asks the social worker to help. What is the **MOST** appropriate action?

**A.** Plan the client's discharge with follow-up care arranged

**B.** Repeat a full suicide risk assessment before planning

**C.** Ask the psychiatrist to consider a hold to finish treatment

**133.** A child protective services investigation closed with no findings, and the agency offered the family voluntary in-home parenting support. At the first home visit, before the social worker can describe the program, the parent says sharply, 'We were cleared. We don't need anybody else checking up on us.' What should the social worker do **FIRST**?

**A.** Acknowledge the frustration behind the comment

**B.** Notify the investigator that the family refused services

**C.** Accept the refusal and explain how to return later

**D.** Explain that the service is voluntary and what it offers

**134.** A 15-year-old in foster care asks to attend the upcoming meeting where the team will discuss changing the permanency goal from reunification to adoption. The foster parent suggests keeping the youth away because the discussion may be upsetting. What is the **MOST** appropriate response by the child welfare social worker?

**A.** Include the youth after preparing them for the meeting

**B.** Hold the meeting without the youth and report back

**C.** Ask the youth to send written views instead

**135.** A hospital oncology social worker meets for the first time with a client newly diagnosed with early-stage colon cancer. The oncologist has twice explained that surgery offers a strong chance of cure, and the client can restate the risks accurately. The client wants to try herbal treatment from a trusted practitioner for 3 months first. The oncologist asks the social worker to help. What should the social worker do **FIRST**?

**A.** Request a capacity evaluation before honoring the choice

**B.** Respect the choice and keep follow-up care open

**C.** Ask the client's family to encourage the surgery

**D.** Ask about the client's reasons and concerns

**136.** A client with schizophrenia lives in a Housing First apartment program. The client wants to stop attending the program's weekly skills group and use that time at a peer-run drop-in center, explaining that peer staff there helped them find a part-time job and that the extra hours would help them keep it. A case manager warns that skipping the group could put the client's apartment at risk. Which response by the social worker is **MOST** helpful?

**A.** Encourage attending both until the client seems more stable

**B.** Support the change and revise the plan together

**C.** Explain that the apartment depends on attending the group

**D.** Explore what the client hopes to gain at the drop-in center

**137.** Which term describes overriding a competent client's choice, or withholding information from the client, on the grounds that doing so is for the client's own good?

**A.** Paternalism

**B.** Beneficence

**C.** Nonmaleficence

**138.** A client with bipolar I disorder arrives at an outpatient session talking rapidly and says they slept 3 hours in two nights. The client plans to withdraw their entire retirement savings tomorrow to fund a business idea from a late-night video. On a call from the session, the prescriber judges that a visit first thing tomorrow morning is soon enough. What should the social worker do **NEXT**?

**A.** Accept the withdrawal as the client's own financial choice

**B.** Send the client for a same-day crisis evaluation

**C.** Ask the client to delay the withdrawal until after the appointment

**139.** A nursing home resident in their 80s recently chaired the residents' council and manages their own finances. The resident has begun a romantic relationship with another resident. The resident's adult children call the social worker, upset, ask what staff know about the relationship, and demand that staff keep the two apart. What should the social worker do **FIRST**?

**A.** Assure the children that staff will limit the residents' time alone

**B.** Ask the resident how they want staff to respond to the children

**C.** Explain to the children that the relationship is the resident's choice

**140.** A client with schizophrenia has had a representative payee for Social Security benefits since a hospital stay 5 years ago. For the past 18 months, the client has worked part-time, paid rent from wages on time and kept a written budget. The client tells the social worker, 'I want my check sent to me again.' What is the **MOST** appropriate response?

**A.** Help the client gather evidence and request direct payment

**B.** Ask the payee to give the client a larger weekly allowance

**C.** Advise the client to keep the payee until the next yearly review

**D.** Arrange a formal assessment of the client's money skills first

**141.** A 20-year-old comes to a first appointment at a community clinic because the parents insisted. The social worker explains that services are voluntary and the client may decline them. The client folds their arms and says, 'They think I'm depressed. I'm just tired of them running my life.' What should the social worker do **NEXT**?

**A.** Ask what the client would want to change, if anything

**B.** End the meeting, since the client is declining services

**C.** Give a brief depression screen to check the parents' concern

**142.** A 79-year-old hospital patient recovering from a hip fracture wants to go home alone instead of to a rehabilitation facility. A psychiatric consultant found the patient able to make this decision, and the patient knows another fall could happen. Worried, the team asks the social worker to hold off on discharge planning until the patient agrees to rehab. What should the social worker do **NEXT**?

**A.** Request a second capacity evaluation from another consultant

**B.** Ask the patient's adult child to urge the move to rehab

**C.** Have the patient sign a discharge-against-advice form first

**D.** Plan a home discharge with services the patient accepts

**143.** A client with bipolar I disorder has been stable on medication for 3 years and works full-time. The client tells the community mental health social worker that they and their partner plan to try for a baby next year. At a team meeting, a colleague suggests that the worker urge the client to reconsider. What is the **MOST** appropriate response?

**A.** Encourage the client to wait for 5 years of stability

**B.** Share the team's concern that pregnancy may trigger relapse

**C.** Help the client plan pregnancy care with the prescriber

**144.** A tenant in supportive housing keeps stacks of newspapers and boxes that block the apartment's only exit path. After an inspection, the fire marshal gives the tenant 14 days to clear the path or the lease will be ended. The tenant tells the social worker, 'It's my home. I decide what stays.' What should the social worker do **FIRST**?

**A.** Explain the deadline and what is at stake if it passes

**B.** Arrange a crew to clear the path while the tenant is out

**C.** Ask the property manager to respect the tenant's choice

**D.** Refer the tenant to a treatment group for hoarding

**145.** A firefighter with PTSD after a fatal blaze has had 4 sessions of prolonged exposure therapy. The client says, 'I can't keep retelling that night. Can we just talk instead?' The client still has nightmares and wants help with them. What should the social worker do **FIRST**?

**A.** Ask what has made these sessions feel so hard

**B.** Describe other effective treatments and let the client choose

**C.** Shift to the supportive talk sessions the client asked for

**146.** A group home for adults with serious mental illness supports a resident's wish to ride the city bus alone to a new job. A staff member asks what happens if the resident gets lost. The team leader says an occasional wrong turn is part of ordinary adult life. The team arranges travel training and lets the resident ride alone. Which principle is the team **MOST** likely applying?

**A.** Least restrictive alternative

**B.** Dignity of risk

**C.** Harm reduction

**D.** Beneficence

**147.** A social worker provides weekly video sessions to an adult client with depression. At the start of a session, the client mentions that they are calling from a vacation rental in another state where the social worker is not licensed. What should the social worker do **FIRST**?

**A.** Determine whether practice is permitted in that state

**B.** Continue the session because the client is already established

**C.** Identify emergency resources in the client's current state

**D.** End the session and terminate services with the client

**148.** A social worker sees an adult client with generalized anxiety disorder by video each week. The client will spend the next month at a relative's home in another state. The social worker confirms that this state's rules permit the sessions to continue. What should the social worker do **NEXT**?

**A.** Identify emergency services near where the client will stay

**B.** Continue weekly sessions as usual under the current safety plan

**C.** Ask the client to pause sessions until returning home

**149.** A client in an intensive outpatient program for alcohol use sends a friend request to the social worker's personal social media account. The program gave each client written social media rules at intake. What is the **MOST** appropriate response?

**A.** Leave it unaccepted and review the policy at the next session

**B.** Send the client a private message explaining the policy

**C.** Delete the request without mentioning it to the client

**150.** A former client posts an online review of a social worker's private practice. The review claims the social worker 'never returned calls' and mentions the client's divorce. Phone logs show the calls were returned the same day. What is the **BEST** way for the social worker to handle the review?

**A.** Reply with the call dates without naming the client

**B.** Call the former client and ask to have the review removed

**C.** Refrain from any reply that confirms a client relationship

**151.** At 10 p.m., a social worker scrolling a public community page sees a post made 15 minutes earlier by a current adult client. It shows a pile of pill bottles with the words 'Done fighting. Goodbye.' The social worker calls the client's phone and reaches voicemail. What should the social worker do **NEXT**?

**A.** Text the client to ask about current safety

**B.** Request an emergency welfare check at the client's home

**C.** Message the client on the platform asking for a call back

**D.** Discuss at the next session how the post was found

**152.** A client receiving video sessions for depression from home says, in a low voice, that their partner works from home, listens at the door and checks the client's phone. The client wants to keep meeting and works in an office two blocks from the clinic. What is the **BEST** way for the social worker to proceed?

**A.** Continue video sessions using headphones and a code word

**B.** Assess the client's comfort with the clinic's video platform

**C.** Switch to secure text messaging between sessions

**D.** Offer in-person sessions at the clinic during the workday

**153.** A hospital social worker's work phone breaks, and a replacement will take a week. Several discharged patients need follow-up calls about appointments. A coworker suggests using a personal cell phone in the meantime. What is the **BEST** way to handle patient contact this week?

**A.** Use the office landline and secure messaging system

**B.** Text patients from the personal phone without names

**C.** Use the personal phone and delete messages each day

**154.** During a video session, a client tearfully discusses the anniversary of a sibling's death and says, 'Sometimes I wonder what the point is.' Before the social worker can ask more, the video freezes and the call drops. Reconnecting fails. At intake, they agreed the social worker would phone the client if video failed. What should the social worker do **FIRST**?

**A.** Ask local police to check on the client at home

**B.** Send a secure message offering to finish next week

**C.** Call the client's number on file

**155.** A family caregiver support program moves to remote-only services. An 80-year-old caregiver in a rural area has a basic cell phone and no home internet, and wants to keep meeting with the social worker. Which response is **MOST** helpful?

**A.** Lend a tablet and teach the caregiver to use video

**B.** Hold the sessions as regular voice calls

**C.** Refer the caregiver to an in-person group 60 miles away

**156.** A client with low vision asks the agency to email a copy of their record to a personal email account. The agency usually uses an encrypted portal, but the client finds it hard to use with screen-reading software. What is the **BEST** response?

**A.** Require the client to use the encrypted portal for access

**B.** Mail a printed copy to the client's home address instead

**C.** Warn of email risks, then send it if the client accepts

**D.** Email a brief summary rather than the complete record

**157.** A social worker leads a weekly video support group for adults with chronic pain. Members agreed to join from private spaces. Midway through, one member speaks up from a crowded coffee shop, with nearby voices clearly audible, just as another member begins describing a recent hospitalization. What should the social worker do **FIRST**?

**A.** Pause and ask the member to move somewhere quiet or wear headphones

**B.** Remind everyone of the group's confidentiality agreement and continue

**C.** Mute the member's microphone and continue the discussion

**158.** A client logs on for a scheduled video session during a lunch break. The social worker sees that the client is driving, with the phone mounted on the dashboard. The client says it is fine to talk because the drive is 20 minutes. What should the social worker do **FIRST**?

**A.** Switch to audio so the client can watch the road

**B.** Ask the client to park safely or reschedule the session

**C.** Keep the session to a short check-in during the drive

**159.** An outpatient substance use program sends automatic text reminders before appointments. A new client says they often forget appointments and would like reminders. However, everyone in the household shares one phone, and relatives read the messages on it. Which plan for reminders is **MOST** appropriate?

**A.** Send the standard reminder, since the client asked for one

**B.** Skip reminders to keep the treatment private

**C.** Mail written reminders to the client's home instead

**D.** Agree with the client on a discreet reminder method

**160.** A client living with bipolar I disorder tells the social worker about a public video they posted describing their first year back at work after a long hospitalization. The client says they would like the social worker to watch it and talk about it at the next session. What is the **MOST** appropriate response?

**A.** Explain that the Code bars viewing a client's online videos

**B.** Follow the client's channel to show support

**C.** View the video as invited and discuss it next session

**161.** While scrolling a community events page on a personal account, a social worker sees a friend's recent party photo that shows a client holding a drink. The client is in treatment for alcohol use disorder and said at the last session that they have been sober for 4 months. What is the **BEST** action for the social worker?

**A.** Tell the client what was seen and discuss it

**B.** Record a possible relapse in the client's chart

**C.** Keep it private to protect the client's trust

**162.** When should a social worker who uses social media professionally review their social media policy with clients?

**A.** When a client first sends a connection request

**B.** At the first interview, updating it as needed

**C.** Once a working alliance has formed

**163.** A social worker at a group practice learns that the new video platform automatically records every telehealth session and stores the files for quality review. The consent forms do not mention recording, and clients have not been told. Only the practice's owners can change the platform's settings. The office manager says the vendor contract requires it. What should the social worker do **FIRST**?

**A.** Raise the recording problem with the practice's leadership

**B.** Tell each of their clients about the recordings at the next session

**C.** Continue sessions as usual since the contract requires recording

**D.** Report the practice to the state licensing board

**164.** Which approach to recruiting current clients for an agency research study is **MOST** appropriate?

**A.** Assure clients that declining has no effect on services

**B.** Offer participants priority placement on the agency's waitlist

**C.** Ask each client's own social worker to invite them

**165.** An outpatient clinic ran a quality-improvement project tracking depression scores for clients in a new group program. The results look strong, and the program director asks the social worker to prepare the client data for a journal article. The clinic has a research partnership with a nearby university. What should the social worker do **FIRST**?

**A.** Submit the project for IRB review before using the data

**B.** Remove client identifiers from the data set

**C.** Ask former group members for permission to publish

**D.** Draft the article, since the project used routine care data

**166.** A university study will interview youth in residential treatment about their placement experiences. A 15-year-old's guardian has signed permission. The social worker helping with recruitment explains the study to the youth, who listens, asks one question, and then says they do not want to take part. What is the **MOST** appropriate response?

**A.** Enroll the youth, since the guardian gave written permission

**B.** Review the study's benefits again and encourage joining

**C.** Accept the decision and note the refusal in the study log

**167.** A social worker conducts interviews for an IRB-approved study of job-training outcomes. A question about past jobs leads one participant to describe a workplace assault and become very distressed. The social worker stops the interview, stays with the participant until calm, and, with consent, links them to a counseling service. What should the social worker do **NEXT**?

**A.** Call the participant's emergency contact about the distress

**B.** Report the event to the IRB promptly

**C.** Describe the event at the IRB's annual continuing review

**168.** A researcher designs an online survey that collects no names, contact details, IP addresses, or linking codes. Which protection does this design provide?

**A.** Anonymity

**B.** Confidentiality

**C.** De-identification

**169.** A social worker at a family shelter is helping a university team design a study of residents' health needs. To boost enrollment, a team member proposes giving each participant priority on the shelter's waitlist for permanent housing. What is the **BEST** way for the social worker to respond to the proposal?

**A.** Offer participants added case management hours instead

**B.** Recommend offering no compensation for taking part

**C.** Suggest a modest cash payment separate from services

**170.** Under the NASW Code of Ethics, what must social workers tell research participants about withdrawing from a study?

**A.** They may withdraw before data collection begins

**B.** They may withdraw if the review board approves

**C.** They may withdraw at any time without penalty

**171.** A social worker leads the evaluation of a parenting program that runs at four sites. Outcomes improved at three sites but not at the fourth, which had heavy staff turnover during the year. Worried about renewal, the agency director asks the social worker to leave the fourth site out of the report to the funder. What is the **BEST** response?

**A.** Report every site and explain the fourth site's context

**B.** Drop the fourth site from the report as asked

**C.** Combine the four sites into one overall average

**172.** A social worker is conducting an IRB-approved research interview with an older adult about caring for a spouse who died last year. Partway through, the participant begins crying while describing the final weeks of the spouse's illness. What should the social worker do **FIRST**?

**A.** Continue with the protocol so the data stay consistent

**B.** Pause and acknowledge how painful this is to recall

**C.** End the interview and give a list of grief resources

**173.** A social worker is the only mental health provider in a small rural town. A new client's child plays on the same youth soccer team as the social worker's child, so they will run into each other at weekly games. The nearest other provider is 2 hours away. What is the **MOST** appropriate action?

**A.** Refer the client to the provider who is 2 hours away

**B.** Continue treatment without raising the overlap with the client

**C.** Agree with the client on how to handle outside contact

**174.** In a remote town with one mental health provider, a social worker treats a client who owns the only hair salon. At intake, the two agreed on how to handle contact at the salon. During the social worker's next haircut, the client refuses payment, saying, 'You've helped me so much.' What is the social worker's **BEST** response?

**A.** Accept the free haircut as a sign of the client's gratitude

**B.** Pay the usual price and explore the gesture at the next session

**C.** Use a salon in another town for the rest of treatment

**175.** During a home visit, an 84-year-old client who lives alone on a fixed income hands the social worker a holiday card with $150 in cash inside. The client says the social worker has been 'like family' this year and would be hurt by a refusal. What is the social worker's **BEST** response?

**A.** Accept the cash and give it to the agency's client fund

**B.** Decline the cash warmly and talk about what the gift means

**C.** Accept the cash to spare the client's feelings

**D.** Return the cash later by mail with a policy note

**176.** At the last of 10 sessions at a community clinic, a client gives the social worker a small jar of homemade jam. The client explains that in their family, sharing food is how people show gratitude. What is the **MOST** appropriate response?

**A.** Accept the jam and express appreciation

**B.** Decline the jam, explaining that gifts blur roles

**C.** Suggest the client share the jam with clinic staff

**177.** A social worker who is going through a divorce reviews notes from 4 recent sessions with a client whose marriage is ending. In each session, the social worker spent much of the time describing their own separation. The client recently said, 'It helps to know you get it.' What should the social worker do **FIRST**?

**A.** Keep sharing, since the client finds it helpful

**B.** Transfer the client to a colleague without a similar history

**C.** Tell the client that future sessions will focus on them

**D.** Seek supervision about the pattern of self-disclosure

**178.** In an intensive outpatient program for alcohol use disorder, a client says nobody really stays sober and asks the social worker, 'Have you ever been through this?' The social worker has been in recovery for 12 years, and program policy allows staff to share their recovery status. What is the **BEST** response?

**A.** Describe the social worker's own path to sobriety in detail

**B.** Explain that the social worker's personal life stays private

**C.** Confirm it briefly, then turn back to the client's doubts

**179.** At a neighborhood street fair, a social worker runs into a former client whose services ended 3 years ago. After a friendly conversation, the former client asks the social worker out on a date. How should the social worker respond?

**A.** Accept, since over 2 years have passed

**B.** Decline, citing the lasting professional duty

**C.** Offer to meet as friends instead of dating

**180.** A social worker is providing family therapy for a 19-year-old client. The client's 27-year-old sibling attended 3 of the sessions. Between sessions, the sibling messages the social worker to ask for a dinner date, adding, 'I'm technically outside the treatment.' What is the **MOST** ethical response?

**A.** Decline the invitation and keep the professional role

**B.** Accept, since the sibling is outside the client role

**C.** Accept after the family's treatment has ended

**D.** Decline and transfer the family to a colleague

**181.** An employee assistance program social worker reviews the next day's schedule and sees that a newly assigned client is someone the social worker had a brief sexual relationship with 10 years ago. The two have had no contact since. The program has 4 other staff social workers. What should the social worker do?

**A.** Begin services, since the relationship ended long ago

**B.** Meet once to learn whether the history concerns the client

**C.** Ask that the case be reassigned to a colleague

**D.** Disclose the history and let the client decide

**182.** A hospital social worker is helping a 79-year-old patient choose a skilled nursing facility for rehabilitation after a stroke. The social worker's partner is the admissions director at one of the 5 facilities with open beds. The patient asks which facility the social worker would pick. What is the **BEST** response?

**A.** Recommend the partner's facility, which has strong ratings

**B.** Leave the partner's facility off the list of options

**C.** Disclose the connection and review each option neutrally

**183.** A social worker in private practice often refers clients with eating disorders to residential care. A new residential program's marketing director offers the social worker $500 for each referred client who enrolls. The social worker would provide no services to those clients after the referral. What should the social worker do **FIRST**?

**A.** Decline the payment and base choices on clinical fit

**B.** Ask the program to donate the fees to a client scholarship fund

**C.** Ask for the program's outcome data before referring

**184.** A social worker at a community agency is planning a kitchen remodel. The social worker's partner collected bids, and the lowest bid comes from a contractor who is a current client of the social worker. The client does not yet know the social worker is the homeowner. What is the **MOST** appropriate action?

**A.** Hire the client and pay the full bid amount

**B.** Accept one of the other bids instead

**C.** Ask the client in session how the job would feel

**185.** At a community mental health center, a client tells the social worker that their prescription copay went up and they are $40 short this month. The client asks to borrow the money until payday, promising to repay it in 2 weeks. The center has an emergency assistance fund. What is the **BEST** response?

**A.** Lend the money with a written repayment date

**B.** Give the client $40 as a one-time gift

**C.** Suggest the client ask relatives for emergency help

**D.** Help the client apply to the emergency fund

**186.** A 14-year-old in residential treatment, who has a history of sexual abuse, greets the social worker with long, full-body hugs before each session. The client says the hugs make them feel safe. Other staff have started to comment on the greetings. What is the social worker's **BEST** response?

**A.** Offer a side hug or handshake and discuss the change

**B.** Keep the hugs but make each one brief

**C.** Step back from hugs and avoid raising the topic

**187.** A social worker in a state prison provides counseling to an incarcerated client. Near the end of a session, the client hands the social worker a sealed letter and asks the worker to mail it to the client's adult child from outside the facility. The client says the letter holds family news. What is the **MOST** appropriate response?

**A.** Mail the letter as a one-time favor to the client

**B.** Explain how to send it through the facility's mail

**C.** Read the letter first, then mail it if it is harmless

**D.** Report the request to facility security as misconduct

**188.** A social worker in private practice is asked by a close friend to provide therapy for the friend's 15-year-old, who has been anxious and refusing school. The friend says they trust no one else with their child. Several qualified child and adolescent providers practice nearby. What is the **BEST** response?

**A.** Agree, and keep the friendship separate from sessions

**B.** Agree once the teen and friend sign informed consent

**C.** Decline and give names of other local providers

**D.** Assess the teen once, then refer to another provider

**189.** A social worker provided couples therapy to two partners for 8 months. The partners have since separated and are disputing custody of their children. One former partner asks the social worker to write a letter to the court describing that partner as the better parent. What is the **MOST** appropriate response?

**A.** Write a letter describing that partner's parenting strengths

**B.** Explain why the past role rules out taking a side

**C.** Write the letter once the other partner also consents

**190.** At intake, a social worker learns that a new client lives 3 houses away on the same block. The two have only exchanged casual greetings. The social worker discusses the overlap with the client, who wants to continue, and the supervisor agrees the risk is low. What should the social worker do **NEXT**?

**A.** Transfer the client to a colleague to be safe

**B.** Wait to record the overlap until a problem arises

**C.** Document the discussion and the boundary plan

**191.** On the day of discharge from an inpatient psychiatric unit, a patient thanks the social worker and asks for the worker's personal cell phone number, saying they fear no one will answer at night 'if things get bad again.' The patient starts outpatient care next week. What is the social worker's **BEST** response?

**A.** Share the number for after-hours emergencies

**B.** Provide clinic and 24-hour crisis contacts

**C.** Record the request in the discharge note

**D.** Explain the policy against sharing personal numbers

**192.** Which approach to client testimonials is consistent with the NASW Code of Ethics?

**A.** Avoid soliciting testimonials from current clients

**B.** Request testimonials from clients who are doing well

**C.** Ask for reviews during a client's final session

**193.** Under the NASW Code of Ethics, which condition must be present before a social worker accepts goods or services from a client instead of a fee?

**A.** The goods match the fee's market value

**B.** The client's insurance coverage has lapsed

**C.** A supervisor approves the plan in writing

**D.** The client proposes it with informed consent

**194.** After discussing the request with the hospice team, a social worker accepts a family's invitation to a patient's memorial service. The social worker sits near the back, offers brief condolences to the family and leaves before the reception. Which concept does the social worker's attendance **MOST** likely illustrate?

**A.** Boundary crossing

**B.** Boundary violation

**C.** Dual relationship

**D.** Countertransference

**195.** A social worker in long-term recovery attends a weekly community 12-step meeting, which is the worker's main recovery support. One evening, a current outpatient client walks in, sees the social worker and sits down. The social worker is unsure whether to keep attending. The client's next session is in 4 days. What should the social worker do **FIRST**?

**A.** Discuss the encounter with the client at the next session

**B.** Find a different meeting to attend from now on

**C.** Talk through the overlap with a supervisor or consultant

**196.** On a pediatric intensive care unit, a patient's parent has just learned that the child died. The hospital social worker has supported the family through the 2-week stay. Sobbing, the parent turns and reaches out to hug the social worker. What is the **BEST** response?

**A.** Offer the parent a hand on the shoulder instead

**B.** Accept the brief hug and stay with the parent

**C.** Step back gently and offer a chair and tissues

**197.** A social worker in a housing program volunteers for a ballot campaign to fund affordable housing, a measure that would help many clients. The campaign asks volunteers to share flyers urging a yes vote. The social worker plans to hand them out on home visits and tell clients that support is optional. Which action is **MOST** consistent with the NASW Code of Ethics?

**A.** Hand out the flyers, stressing that support is optional

**B.** Keep the election out of home-visit conversations

**C.** Offer nonpartisan voting information in place of the flyers

**D.** Give campaign flyers to clients who ask about the measure

**198.** A hospital social worker serves on the ethics committee. A case comes to the committee in which an ICU patient's adult child, the health care agent, disagrees with the medical team. The social worker recognizes the agent as a former client from 3 years ago. The committee meets in an hour. What is the **BEST** action?

**A.** Tell the chair the agent is a former client and step aside

**B.** Step aside, saying a conflict exists without naming it

**C.** Take part, since services ended 3 years ago

**D.** Ask the agent for permission before taking part

**199.** A client in outpatient treatment arrives upset. The client found the social worker's personal social media account and saw a public post supporting a cause the client strongly opposes. 'I don't know if I can trust you now,' the client says. What should the social worker do **FIRST**?

**A.** Invite the client to describe how the post affected the relationship

**B.** Make the account private and remove the post

**C.** Explain that personal views stay separate from the client's care

**200.** A community mental health agency hires a former client as a peer support specialist. The new hire finished treatment with a staff social worker 2 years ago and did well. The program director asks that social worker to be the new hire's direct supervisor, citing their good rapport. What is the **MOST** appropriate response from the social worker?

**A.** Accept, setting clear rules that keep supervision work-focused

**B.** Ask that another staff member supervise the new hire

**C.** Object to the agency hiring its former clients as staff

**201.** In a group practice, a social worker learns at the second session that a new client manages the department where the social worker's adult child works. The client is deciding on layoffs in that department. After the social worker explains the conflict, the client says, 'It doesn't bother me. I'd rather stay with you.' What should the social worker do **NEXT**?

**A.** Arrange for a colleague to take over the client's care

**B.** Continue, with a written agreement on outside contact

**C.** Ask how the client feels about the overlap

**202.** A social worker begins voluntary in-home parenting support with a family. At each visit, the parent sets out tea and a small plate of homemade pastries, explaining that in their culture a guest is always served. The family has offered nothing else. What is the **MOST** appropriate response?

**A.** Decline politely, explaining that workers cannot accept gifts

**B.** Accept the tea and bring a small treat to the next visit

**C.** Accept the refreshments graciously as hospitality

**203.** A nursing home resident with no living family has worked with the facility social worker for 3 years. The resident, who has full decision-making capacity, asks the social worker to serve as health care agent, saying, 'You know what I want better than anyone.' What is the **BEST** response?

**A.** Agree, since no family is available to serve

**B.** Suggest that a court appoint a guardian for the resident

**C.** Decline, and help the resident consider other possible agents

**D.** Ask the administrator whether staff may serve as agents

**204.** Under the NASW Code of Ethics, when should a social worker avoid a dual relationship with a client or former client?

**A.** When it carries a risk of exploitation or harm to the client

**B.** When it begins less than 2 years after services have ended

**C.** When it involves social contact outside the office

**205.** A 9-year-old client in a school-based program tells the social worker that a parent hit them with a belt last night. The client shows the social worker red marks on the back of their legs. What should the social worker do **FIRST**?

**A.** Make a report to child protective services

**B.** Call the parent to ask about the marks

**C.** Interview the child in detail about other incidents

**206.** During a session, an adult client who is angry about a recent layoff says, 'My manager is going to get what's coming.' The client has no known history of violence and quickly changes the subject. What should the social worker do **FIRST**?

**A.** Notify the manager and local police of a possible threat

**B.** Ask the client directly what the statement meant

**C.** Document the statement and revisit it next session

**D.** Consult a supervisor about the duty to warn

**207.** On an inpatient psychiatric unit, a patient scheduled for discharge this afternoon tells the social worker they plan to go to their former partner's home tonight and shoot the partner with the handgun kept in their closet. The patient says the decision is made. What should the social worker do **FIRST**?

**A.** Have the team hold the discharge for reevaluation

**B.** Warn the former partner and proceed with the discharge

**C.** Ask more about how firmly the patient intends to act

**D.** Notify the outpatient provider to follow up this week

**208.** A client who gave birth 6 weeks ago tells the social worker at an outpatient perinatal mental health clinic about sudden, unwanted images of the baby slipping under the water during baths. The client cries, calls the images 'horrifying,' and now asks a partner to give all baths. The social worker finds the client's thinking clear and organized, and the client wants help. What is the **BEST** response?

**A.** Notify the child abuse hotline about the disclosure

**B.** Send the client to the emergency department for evaluation

**C.** Tell the client's partner to oversee every feeding and bath

**D.** Continue treatment and record the risk assessment

**209.** A social worker at a community youth center learns from a 7-year-old that a parent's partner burned the child's arm with a lighter, and sees a fresh round burn. The social worker informs the center director, who says staff will talk with the parent at pickup tomorrow and asks the social worker to hold off. What should the social worker do **NEXT**?

**A.** Wait until the director has spoken with the parent

**B.** Ask the child more questions to firm up the details

**C.** Call the child abuse hotline today

**210.** In outpatient treatment for depression, an adult client says they left their 5-year-old alone overnight twice last month while working night shifts. The social worker determines that a report is mandated by law. The client has no history of violence, and the child is staying with a grandparent this week. What is the **BEST** way to proceed?

**A.** File the report without telling the client, to avoid conflict

**B.** Tell the client about the report, then make it

**C.** Give the client a week to arrange child care first

**D.** Make the report, then transfer the client to a colleague

**211.** At a community health center, a 34-year-old client says their partner shoved them into a wall during an argument last week. The client, who lives alone and works full time, asks the social worker not to involve the police. What should the social worker do?

**A.** Work out a plan with the client for staying safe

**B.** Report the assault to adult protective services

**C.** Urge the client to seek a protective order this week

**212.** At a residential program for adolescents, a social worker arriving for an evening group sees a colleague clocking in for the overnight shift. The colleague smells strongly of alcohol, slurs words, and will be the only staff member awake with 12 residents. What should the social worker do **FIRST**?

**A.** Speak with the colleague one-on-one about the alcohol smell

**B.** Stay late to cover and raise it with the colleague tomorrow

**C.** Notify the on-call supervisor right away

**213.** What threshold of belief triggers a mandated reporter's legal duty to report possible child abuse or neglect?

**A.** Probable cause

**B.** Clear and convincing evidence

**C.** Preponderance of the evidence

**D.** Reasonable suspicion

**214.** In an outpatient session, a client says they will stab a former business partner tonight, gives the partner's home address, and shows a folding knife. The social worker urges voluntary hospitalization, but the client refuses and walks out. What should the social worker do **NEXT**?

**A.** Call the client tomorrow to reassess the threat

**B.** Notify law enforcement and warn the intended victim

**C.** Raise the case at the next team consultation

**215.** At an opioid treatment program, a client says their partner burned their 6-year-old with a cigarette this morning, and the client saw the mark. The social worker knows federal rules strictly protect substance use treatment records and is unsure whether a report is allowed. What is the **BEST** action?

**A.** Ask the client to sign a release before reporting

**B.** Get guidance from the program's attorney before acting

**C.** Make the abuse report with just the facts it requires

**D.** Make the report and send the client's treatment file

**216.** An adult client in outpatient therapy discloses being sexually abused by a family friend from ages 8 to 11. The client recently learned that this person now coaches a youth soccer team. The client does not want to be part of any report. The social worker is unsure whether the law requires a report in this situation. What is the **BEST** way to proceed?

**A.** Seek consultation about the applicable reporting law

**B.** Report the family friend to child protective services now

**C.** Encourage the client to make the report personally

**D.** Explain that abuse of an adult in childhood cannot be reported

**217.** A hospice social worker noticed a colleague nodding off in team meetings and slurring words on a call. Two weeks ago, the social worker raised this privately, and the colleague denied any problem. This week the colleague missed two patient visits and seemed sedated at a family meeting. What should the social worker do **NEXT**?

**A.** Meet with the colleague a second time to urge help

**B.** Keep a log of incidents for a few more weeks

**C.** Take the concerns to the colleague's supervisor

**218.** In a caregiver support program, a client caring for a parent with major neurocognitive disorder admits shoving the parent twice this week when the parent resisted bathing. The parent has bruises on both upper arms. The client is tearful and asks for help managing stress. What should the social worker do **FIRST**?

**A.** Arrange respite care to ease the caregiver's strain

**B.** Contact adult protective services about the incidents

**C.** Explore the client's guilt about the shoving

**D.** Teach the client calmer ways to handle bathing

**219.** A staff member at an after-school program tells the program's social worker that an 8-year-old has lingered at pickup each Friday this month and once asked to stay late. The staff member describes the child as otherwise cheerful and engaged. What should the social worker do **FIRST**?

**A.** Talk with the child using open, non-leading questions

**B.** Report the staff member's concern to child protective services

**C.** Ask the parent at pickup whether anything is wrong at home

**220.** A social worker makes an honest report of suspected child abuse that is later not confirmed. What legal protection generally applies?

**A.** Psychotherapist-patient privilege

**B.** Statutory immunity

**C.** Sovereign immunity

**221.** An adult client seen in outpatient care after a painful divorce describes recurring thoughts of hurting the former spouse that 'come out of nowhere' at night and frighten the client. The client has given their handgun to a sibling for safekeeping and is eager to keep working on the anger. What is the **BEST** response?

**A.** Warn the former spouse about the client's thoughts

**B.** Arrange a hospital evaluation of dangerousness today

**C.** Intensify treatment with a written safety plan

**222.** At a group home for adults with intellectual developmental disorder, a resident tells the social worker that a night staff member slapped them, and the social worker sees a red mark on the resident's cheek. The social worker informs the house manager, who says the resident 'makes things up' and plans an internal review next week. What should the social worker do **NEXT**?

**A.** Report the incident to the designated outside agency

**B.** Wait for the results of the internal review

**C.** Ask the staff member for an account of the night

**223.** A hospital social worker plans discharge for an 82-year-old patient with moderate major neurocognitive disorder. The patient's grandchild holds power of attorney. The billing office says the patient's nursing home bills are 5 months unpaid, and bank statements show $40,000 moved to the grandchild's account. When asked, the patient cannot say where the money went. What should the social worker do **FIRST**?

**A.** Meet with the grandchild to request an accounting of the funds

**B.** Ask the patient more about their finances and wishes

**C.** Refer the family to an elder law attorney for advice

**D.** Report suspected financial exploitation to adult protective services

**224.** During a video session, a 15-year-old client says they want to die. When asked directly, the teen says they plan to take a parent's sleeping pills tonight and knows where they are kept. The teen begs the social worker not to tell anyone. A parent is at home in another room. What should the social worker do **NEXT**?

**A.** Complete a safety plan with the teen before involving anyone else

**B.** Have emergency responders sent before telling the parent

**C.** Bring the parent into the session to address safety now

**225.** At a community after-school program, a 10-year-old shows the social worker a bruise on their side and says a parent's partner punched them last night. The social worker calls the child abuse hotline, and the intake worker accepts the report. What should the social worker do **NEXT**?

**A.** Document when, to whom and what was reported

**B.** Ask the child more questions to add detail for investigators

**C.** Call the parent to explain that a report has been made

**D.** Wait to document until child protective services shares findings

**226.** In a private conversation, a colleague at a family services agency tells a social worker about completing treatment for alcohol use disorder 8 months ago and attending weekly recovery meetings. The colleague's supervisor rates their work highly, and their clients are making progress. A coworker who overheard urges a report to the licensing board. What is the **MOST** appropriate action?

**A.** Report the colleague's history to the state licensing board

**B.** Respect the colleague's privacy and make no report

**C.** Tell the supervisor so the agency can monitor the colleague

**227.** A social worker in a family support program reported suspected neglect after finding a client's 4-year-old alone at home, and explained the report to the client beforehand. At the next home visit, the client says, 'I trusted you, and you called CPS on me. Why should I keep seeing you?' What is the **BEST** response?

**A.** Offer to transfer the client to another worker in the program

**B.** Explain that the law required the report and move to today's plan

**C.** Invite the client to talk about feeling betrayed by the report

**D.** Promise to talk with the client before making any future report

**228.** Two social workers share an office suite but run separate solo practices. One has noticed the other arriving smelling of alcohol several times, and a client in the waiting room says the colleague fell asleep during a session. Last month, the social worker raised the concern privately and urged the colleague to get help. The colleague denied any problem, and the incidents have continued. What should the social worker do **NEXT**?

**A.** Give the colleague a final month to seek treatment

**B.** File a complaint with the colleague's licensing board

**C.** Suggest that the colleague's clients seek care elsewhere

**D.** Get legal advice about liability before acting

**229.** In the fifth session, a client being treated for depression says that 6 years ago they took about $3,000 from a former employer's cash register and were never caught. The client has had no contact with that employer since. Looking anxious, the client asks whether the social worker now has to call the police. What should the social worker do **FIRST**?

**A.** Explore the guilt the client has carried about the theft

**B.** Ask a supervisor whether the theft is reportable

**C.** Explain that confidentiality covers this past theft

**230.** A parent in a contested custody case tells the social worker that their 6-year-old came back from the other parent's home with a burn on one hand. The parent says the child described the other parent holding the hand on a hot stove, and shows a dated photo of the burn. The two parents have accused each other of lying in court filings. The parent asks for a letter for the custody hearing. What should the social worker do **FIRST**?

**A.** Notify protective services about the burn

**B.** Write the letter describing what the parent reported

**C.** Ask to meet with the child to hear the account directly

**D.** Coach the parent on filing the hotline call themselves

**231.** How should a social worker fix a mistake discovered in a completed progress note?

**A.** Replace the note with a corrected version

**B.** Mention the error in the next progress note entry

**C.** Add a dated, signed addendum that corrects it

**232.** During an internal records audit at a community agency, a social worker discovers that notes for two counseling sessions held last month were never written. The social worker has detailed appointment logs and remembers both sessions clearly. What is the **BEST** way to handle the missing notes?

**A.** Write the notes now and date them on the session days

**B.** Write them now, dated today and labeled as late entries

**C.** Leave the gap rather than change the record afterward

**233.** At intake in an outpatient clinic, a client discloses a suicide attempt 8 years ago. The client then asks the social worker to keep it out of the record and falls silent, looking at the intake form. What should the social worker do **FIRST**?

**A.** Ask what worries the client about having it recorded

**B.** Leave it out of the record and keep a private memo

**C.** Record the attempt and explain who can access the record

**D.** Describe the attempt vaguely as past difficulties

**234.** A supervisor reviews a supervisee's unsigned draft progress note for a hospital patient awaiting discharge. The note calls the patient 'manipulative and drug-seeking.' It also records that the patient asked for pain medication 4 times in an hour and refused a walk with physical therapy. What should the supervisor do **FIRST**?

**A.** Have the supervisee revise the draft to describe observed behavior

**B.** Co-sign the draft, then discuss wording in the next supervision

**C.** Have the supervisee add an addendum explaining the terms

**D.** Revise the note personally so it is accurate before co-signing

**235.** A social worker is leaving a community agency in 3 weeks to open a private practice. Several clients will move to colleagues at the agency, and two have said they may follow the social worker later. What is the **BEST** way for the social worker to handle the client records before leaving?

**A.** Take a list of client contact details for future follow-up

**B.** Leave the records at the agency without adding notes

**C.** Leave the records with the agency and write transfer summaries

**236.** Under the HIPAA Privacy Rule, which records must a covered entity keep for 6 years?

**A.** Clinical progress notes for each client

**B.** Written policies and signed authorizations

**C.** Psychotherapy notes kept apart from the record

**D.** Records of clients who have died

**237.** After reviewing a copy of their record, a client in an outpatient substance use program asks in writing that a note be deleted. The note states that the client arrived for group smelling of alcohol and slurring words, which the social worker observed directly. The client says the note is unfair. What is the **BEST** response?

**A.** Explain the client's right to add a written statement of disagreement

**B.** Amend the note, since clients have a right to request amendment

**C.** Explain that signed notes are final and cannot be amended

**238.** A social worker in a home-based family program carries a large caseload. The agency's electronic record lets staff copy the previous visit's note into a new note. A colleague suggests copying notes forward to keep up with documentation. How should the social worker use this feature?

**A.** Copy each note forward and update the visit date

**B.** Edit each copied note to reflect that visit

**C.** Write one summary note every two weeks instead

**239.** Under the HIPAA Privacy Rule, which content can be kept in psychotherapy notes, apart from the main record?

**A.** Session start and stop times

**B.** The treatment plan and progress summary

**C.** Current diagnosis and symptoms

**D.** The worker's analysis of session conversation

**240.** When writing progress notes for members of a therapy group, how should a social worker handle information about the other group members?

**A.** Refer to other members by their initials

**B.** Keep one shared note in a group file

**C.** List every attendee in each member's note

**D.** Leave other members' identities out of each note

**241.** At the final session of a planned termination, a client who completed treatment for an eating disorder asks the social worker to shred their whole record that day. The client wants a 'clean slate' and worries that a future employer might see it. The agency must keep records for a set number of years. What is the **BEST** response?

**A.** Explain the retention rule and how the record is protected

**B.** Remove the most sensitive notes from the record

**C.** Agree to shred it once the client signs a request

**242.** A program director at a family counseling agency tells staff to keep notes for clients in custody disputes 'as thin as possible,' recording only the date and 'attended,' so the notes cannot hurt clients if subpoenaed. A social worker sees several of these clients, including one who recently self-harmed. Which documentation approach is **MOST** appropriate?

**A.** Follow the director's instruction for these clients

**B.** Keep thin notes but document risk events in full

**C.** Write sufficient notes limited to service-relevant facts

**D.** Keep detailed notes in a separate personal file

**243.** Members of a hospital patient's family disagree about whether to continue the patient's dialysis. The social worker consults the hospital's ethics consultant, who suggests a family meeting focused on the patient's previously stated wishes. The social worker schedules the meeting for later in the week. What is **MOST** important to document about the consultation now?

**A.** A brief entry noting that a consultation occurred

**B.** When it occurred, the advice given and the action taken

**C.** A summary in a separate file kept out of the chart

**D.** The family meeting's outcome, once the meeting has been held

**244.** After an assessment, a social worker diagnoses a client with adjustment disorder. The client's insurance plan pays for only a few sessions for that diagnosis, so the client asks the social worker to record major depressive disorder instead. The client does not meet criteria for major depressive disorder. What is the **BEST** response?

**A.** Record the requested diagnosis to keep the client in care

**B.** Refer the client to a provider who accepts self-pay clients

**C.** Explain that billing a false diagnosis is insurance fraud

**D.** Keep the accurate diagnosis and discuss payment options

**245.** An associate social worker at a community clinic notices that claims for the associate's therapy sessions list the licensed supervisor as the provider who delivered them. The health plan's contract requires claims to name the person who actually provided the service. The associate raised this with the supervisor, who replied, 'Everyone does it.' What should the associate do **NEXT**?

**A.** Report the billing practice to the health plan's fraud unit

**B.** Ask the supervisor to co-sign each note to support the claims

**C.** Keep providing sessions and document the objection in a memo

**D.** Take the concern to the organization's compliance officer

**246.** A client in a private practice signed a fee agreement at intake stating that sessions missed without 24 hours' notice cost $40. The client misses a session without calling. The office manager suggests billing the client's health plan for the missed hour instead. How should the social worker handle the missed session?

**A.** Bill the health plan for the missed session

**B.** Bill the plan at a shorter-session rate instead

**C.** Charge the client the agreed missed-session fee

**247.** A clinic social worker spends 30 minutes on the phone helping a client complete a housing application. The client's health plan covers psychotherapy but not case management. A colleague notes that the call could be billed as psychotherapy because the client seemed calmer afterward. What is the **BEST** way for the social worker to handle the call?

**A.** Record it as case management with no therapy claim

**B.** Bill it as therapy rather than case management

**C.** Skip documenting the call because the plan won't pay

**248.** A client at a community mental health center learns that extra weekly sessions there have a 6-week waitlist. The client asks to see the social worker at the social worker's evening private practice and offers to pay a reduced cash fee. What is the **MOST** appropriate response?

**A.** Agree to see the client privately at the reduced fee

**B.** Decline and pursue added services through the center

**C.** See the client outside the center in the evenings for free

**249.** A group practice bills a client's health plan for weekly sessions. The client begins addressing gambling losses and asks that those sessions be kept from the plan, paying the full fee for each one at the visit. The social worker has confirmed that the client understands the cost. How should the social worker handle billing for those sessions?

**A.** Bill the plan but leave gambling off the claim

**B.** Explain that practice policy bills every session to the plan

**C.** Mark those sessions so nothing about them goes to the plan

**250.** In Medicare billing, what does 'incident-to' billing refer to?

**A.** Billing staff services under a supervising practitioner's number

**B.** Billing an emergency service code after a critical incident

**C.** Billing services under the staff member's own provider number

**D.** Billing a liability insurer for care after an accident

**251.** A social worker in private practice sees a 74-year-old client with Medicare for depression. The client's only income is a small pension. After a large dental bill, the client says they may have to stop sessions because of the copayment and asks whether it could be waived for a while. What is the **MOST** appropriate response?

**A.** Explain that Medicare rules bar waiving copays

**B.** Waive copays for every client living on a fixed income

**C.** Waive it after documenting the client's hardship

**252.** A social worker in private practice plans to raise session fees by 20%. One client, who pays out of pocket and lives on a fixed income, is midway through a planned course of trauma treatment. How should the social worker **BEST** handle the change with this client?

**A.** Give early notice and discuss a reduced fee if needed

**B.** Announce the new fee to clients with 30 days' notice

**C.** Refer the client to a lower-cost clinic before the increase

**253.** A client arrives 25 minutes late for a scheduled 50-minute psychotherapy session at a group practice, and the session lasts 25 minutes. The office manager tells the social worker to bill the client's insurer for the full 50 minutes, since the client chose to come late. How should the social worker bill the session?

**A.** Bill the full scheduled session, as directed

**B.** Bill for the time actually provided

**C.** Submit no claim, since the session was incomplete

**254.** A social worker at a community clinic treats an adult with PTSD whose Medicaid managed care plan reviews progress every 8 sessions. Symptom scores have improved modestly but remain clinically significant. On a review call, the plan's reviewer remarks that more sessions are approved more easily when scores show a client 'getting worse.' What is the **BEST** response?

**A.** Ask the client to give worst-day scores on the next measure

**B.** Submit the actual scores with a rationale for more care

**C.** Accept the likely denial and begin planning termination

**255.** During a home visit, a hospice patient with advanced lung disease tells the social worker, 'I've been saving my pain pills. I'm going to take them all tonight so my family can stop taking care of me.' The patient shows the social worker a full bottle hidden in a drawer. What should the social worker do **FIRST**?

**A.** Stay with the patient and get the care team's help now

**B.** Explore the patient's feeling of being a burden to the family

**C.** Explain how legal aid in dying differs from a private overdose

**256.** A hospital patient with advanced major neurocognitive disorder can no longer communicate. The patient's living will declines a feeding tube. The patient's health care agent, an adult child, insists on placing one, saying, 'I can't let my parent starve.' The team met with the agent yesterday to review the directive, but the agent's position has not changed, and the agent wants the tube placed this week. What should the social worker do **NEXT**?

**A.** Follow the agent's instructions as the legal decision-maker

**B.** Schedule another meeting to review the directive with the agent

**C.** Explore the agent's grief about the parent's decline

**D.** Request an ethics consultation on the conflict

**257.** A surrogate makes a choice for a patient who can no longer decide. The surrogate picks what the patient would have wanted, based on the patient's values and past words. Which standard is this?

**A.** Best interests standard

**B.** Substituted judgment standard

**C.** Reasonable person standard

**258.** Before elective surgery, a nurse tells an adult patient that the hospital requires an advance directive on file. The upset patient tells the hospital social worker they do not want to make one and asks whether the surgery can go ahead without it. What should the social worker do **FIRST**?

**A.** Help the patient complete a brief directive

**B.** Clarify the hospital's duties with the nurse

**C.** Inform the patient that a directive is voluntary

**259.** An 82-year-old patient on dialysis for 6 years tells the dialysis social worker they want to stop treatment and enter hospice. Over 3 weeks the patient has given the same reasons, and the nephrologist and a psychiatrist have confirmed decision-making capacity. The patient hopes the family will come to accept the decision. The patient's adult children ask the social worker to talk the patient out of it. What is the **BEST** response?

**A.** Offer the patient a family meeting to explain the choice

**B.** Ask the patient to continue until the family agrees

**C.** Request another psychiatric evaluation first

**D.** Remind the children the patient may legally decide

**260.** In a state where medical aid in dying is legal and the hospital permits it, a patient with terminal pancreatic cancer asks the palliative care social worker how to request it. The patient is calm and describes long-held views about dying on their own terms. What is the **MOST** appropriate response?

**A.** Refer the question to the hospital chaplain

**B.** Request an urgent suicide risk evaluation

**C.** Encourage the patient to focus on comfort care instead

**D.** Discuss the request and involve the physician

**261.** An ICU patient who cannot communicate after a stroke has no advance directive and no named agent. The patient's legal spouse, separated for 3 years, wants aggressive treatment. Two adult children want comfort care. The physician asks the social worker to help resolve the conflict. What should the social worker do **FIRST**?

**A.** Hold a family meeting to seek agreement on goals of care

**B.** Follow the adult children, who have been at the bedside

**C.** Ask the physician to decide based on the patient's interests

**D.** Find out whom the state's surrogate law authorizes to decide

**262.** Under U.S. law, how is a capable patient's request to have a ventilator removed generally classified?

**A.** Medical aid in dying

**B.** Voluntary active euthanasia

**C.** Refusal of medical treatment

**263.** In a state where medical aid in dying is lawful, a hospice patient with terminal cancer asks the social worker for help understanding the request process and talking with family. The social worker objects to aid in dying on religious grounds and, after supervision, concludes they cannot discuss it without their views showing. What is the **BEST** action?

**A.** Tell the patient about the objection so the patient understands

**B.** Suggest that the patient take the questions to the physician

**C.** Continue with the patient while keeping personal views private

**D.** Arrange promptly for a colleague to help the patient

**264.** A social worker supervises a newly licensed social worker whose recent divorce is affecting their work. The supervisee has been late to sessions and tearful with clients. In supervision, the supervisee asks the supervisor to provide personal therapy for a few weeks, saying 'You already know my situation.' What is the supervisor's **BEST** response?

**A.** Agree to provide short-term therapy during supervision

**B.** Decline to provide therapy and address the impact on clients

**C.** Report the supervisee's impairment to the state licensing board

**265.** A social worker supervises a staff member who is working toward clinical licensure. The supervisee invites the supervisor to invest in a weekend wellness business the supervisee is starting. The supervisor will submit an evaluation of the supervisee's hours to the licensing board next month. What should the supervisor do **FIRST**?

**A.** Invest a small amount after submitting the evaluation

**B.** Decline and explain the conflict with the supervisory role

**C.** Record the invitation in the supervision notes

**266.** During weekly chart review, a clinical supervisor at a community agency reads a supervisee's note from 4 days ago. The client said, 'Sometimes I think everyone would be better off without me.' The note shows no follow-up questions, and the client's next appointment is in 10 days. What should the supervisor do **FIRST**?

**A.** Ask the supervisee why the remark went unexplored

**B.** Arrange suicide risk assessment training for the supervisee

**C.** Note the lapse in the supervisee's performance record

**D.** Make sure the client is contacted today to assess risk

**267.** A supervisee in a group practice misses clear signs that a client's condition is worsening, and the client is harmed. The supervisor had met with the supervisee weekly and reviewed cases as agreed. The client's lawsuit names both the supervisee and the supervisor. Which statement about the supervisor's legal exposure is accurate?

**A.** Regular weekly review shields the supervisor from liability

**B.** Liability requires proof that guidance to the supervisee was faulty

**C.** The supervisor may share liability for the supervisee's acts

**268.** A supervisee at a youth services agency invites their clinical supervisor to the public commencement where the supervisee will receive an MSW. The invitation also includes a family dinner at the supervisee's home afterward. Supervision will continue for another year. What is the **MOST** appropriate response by the supervisor?

**A.** Politely decline both the public ceremony and the family dinner

**B.** Attend the public ceremony and decline the family dinner

**C.** Attend the public ceremony and the family dinner

**269.** A hospital opens an outpatient eating disorders program and asks an experienced clinical supervisor to supervise its 4 newly hired social workers. The supervisor has strong skills with mood and anxiety disorders but no training in eating disorders. Several program clients have serious medical complications. What is the **BEST** response by the supervisor?

**A.** Accept and learn the specialty alongside the new staff

**B.** Ask program leaders to secure an eating disorders specialist to supervise

**C.** Accept and meet monthly with an outside eating disorders expert

**D.** Decline and let the program find someone else to supervise

**270.** A clinical supervisor in a group practice is asked to sign a former supervisee's licensure verification form. The form lists 24 months of supervision, but no supervision took place during 3 of those months while the supervisor was on medical leave. The supervisee needs the full total to apply this year. What is the **BEST** action by the supervisor?

**A.** Sign for the months in which supervision occurred

**B.** Sign for the 24 months of supervision and attach a note about the leave

**C.** Decline to sign any part of the verification form

**271.** A field instructor at a middle school will complete an MSW intern's final evaluation in 3 weeks. For several months, the intern's case notes have been late and incomplete, but the field instructor has never raised the issue. The evaluation form includes documentation as a rated skill. What should the field instructor do **FIRST**?

**A.** Rate the intern's documentation as unsatisfactory on the evaluation

**B.** Email the intern a list of the late and incomplete notes

**C.** Meet with the intern now to review documentation against the criteria

**272.** The clinical director of a psychiatric hospital learns that a unit supervisor has begun a romantic relationship with a social worker the supervisor directly supervises. Both say the relationship is consensual and does not affect their work. What should the director do **FIRST**?

**A.** Ask both to keep the relationship separate from work

**B.** Refer the supervisor for a human resources conduct review

**C.** Move the supervisee to a different supervisor right away

**D.** Wait unless a complaint or performance problem arises

**273.** According to the NASW Code of Ethics, what should field instructors ensure when social work students provide services to clients?

**A.** Students' work is reviewed before each session begins

**B.** Clients are routinely told that a student is serving them

**C.** Clients sign a separate consent form for student services

**274.** A program manager at an outpatient clinic tells a clinical supervisor to drop suicide screening at intake to shorten wait times. The supervisor raised the safety risk with the manager in writing and proposed a shorter screening tool, but the manager repeated the directive. What should the supervisor do **NEXT**?

**A.** Have clinical staff keep screening without telling the manager

**B.** Follow the directive while keeping a record of the objection

**C.** Propose a trial of the shorter tool to the manager again

**D.** Take the concern to the clinical director or compliance office

**275.** Two weeks before completing a supervisee's final evaluation for licensure, a clinical supervisor at a family agency receives a gift from the supervisee: concert tickets worth about $200. The card thanks the supervisor for 2 years of guidance. Agency policy says nothing about gifts between staff. What should the supervisor do?

**A.** Return the tickets and explain the conflict with the evaluation

**B.** Hold the tickets until the evaluation has been submitted

**C.** Accept the tickets and thank the supervisee for the gesture

**276.** A former supervisee asks a clinical supervisor for a reference letter for a hospital job. During supervision, the supervisee was warm with patients but often missed documentation deadlines, and two written improvement plans addressed it. The supervisee asks the supervisor to 'focus on the positives.' Which response is **MOST** consistent with ethical standards?

**A.** Write a letter on strengths alone, leaving out the deadline issues

**B.** Write a balanced letter and send it straight to the hospital

**C.** Explain that an honest letter would include the deadline issues

**277.** A hospital field instructor has given an MSW intern weekly feedback and a written learning plan for missed notes and late arrivals. At midterm, the intern still falls short on several competencies. The intern then discloses a learning disability, says the school has never been told, and asks the instructor not to give a failing rating. What should the field instructor do **NEXT**?

**A.** Give a passing midterm rating while the intern seeks help

**B.** Arrange accommodations with the intern without involving the school

**C.** End the field placement, since the learning plan has failed

**D.** Involve the school's field liaison in reviewing the plan

**278.** A clinical supervisor at the only behavioral health agency in a rural county learns that a new supervisee will coach the supervisor's 10-year-old child's soccer team this season. Contact would be limited to weekend practices and games. The agency has no other supervisor with the supervisee's specialty. What is the supervisor's **MOST** appropriate action?

**A.** Arrange video supervision for the supervisee from another county

**B.** Discuss the overlap with the supervisee and agree on clear boundaries

**C.** Continue as usual, since the contact happens outside work

**279.** A child welfare social worker meets with a parent whose 2 children entered foster care 4 months ago. The parent is starting substance use treatment and believes there is no deadline for reunification. What is the **MOST** appropriate way to respond to the parent's belief?

**A.** Explain the permanency time frames and how they shape the plan

**B.** Refer the parent to their attorney to explain the time frames

**C.** Tell the parent that rights end automatically after 15 months

**280.** A client in an intensive outpatient program for alcohol use disorder lost Medicaid coverage 3 weeks ago. The state mailed the renewal form to an old address, and the client never received it. The client is doing well and wants to continue. Several other clients have lost coverage the same way. What should the social worker do **FIRST**?

**A.** Propose a fix for renewal mailings to state officials

**B.** Ask the client why the address was left outdated

**C.** Help the client submit the renewal paperwork this week

**D.** Discharge the client with referrals to free treatment programs

**281.** A city has begun clearing homeless encampments every month under a new ordinance. During each sweep, clients of a street outreach team lose medications, IDs and benefit papers. The team has replaced these items after every sweep, and the losses keep recurring. What is the **BEST** action for the team at this point?

**A.** Keep replacing lost items for clients after each sweep

**B.** Ask police to tell the team before each sweep happens

**C.** Encourage clients to move their camps outside city limits

**D.** Present documented harms to city leaders with proposed changes

**282.** A client with major depressive disorder has employer-sponsored health insurance through a large company. The plan denied payment for therapy after 12 visits this year, citing a visit cap. The social worker reviewed the plan documents with the client and found no visit cap on outpatient medical care. What should the social worker do **NEXT**?

**A.** Help the client appeal the decision under federal parity rules

**B.** Switch the client to a reduced self-pay fee starting right away

**C.** Tell the client the plan broke the law and should be sued

**283.** A parent tells a school social worker that their 8-year-old is struggling with reading and asks for a special education evaluation. The school's practice is to complete 12 weeks of classroom reading intervention before considering a referral, and 4 weeks remain. What is the **MOST** helpful response by the social worker?

**A.** Tell the parent an evaluation can be requested once intervention ends

**B.** Help the parent submit a written request for an evaluation now

**C.** Suggest a private evaluation to avoid waiting for the school

**D.** Ask the teacher to increase the intensity of reading support

**284.** A 45-year-old client with no dependents receives a notice that food benefits will stop after 3 months because the client is not working or in a work program 80 hours a month. The client has severe major depressive disorder, sleeps most of the day and has not held a job in a year. The client's psychiatrist confirms the client cannot work now. What should the social worker do **FIRST**?

**A.** Help the client search for part-time jobs that meet the hour rule

**B.** Help the client request a medical exemption from the time limit

**C.** Write to state legislators about the harm of the work rule

**D.** Refer the client to food pantries to replace the lost benefits

**285.** Under the McKinney-Vento Act, what may a student do when the family becomes homeless and moves to a shelter in another district?

**A.** Remain in the school of origin, with transportation provided

**B.** Enroll in the new district once school records are transferred

**C.** Stay at the original school until the semester ends, then transfer

**286.** A social worker accepts a job at another agency and will leave in 6 weeks. The supervisor has approved a plan for telling clients this week. One client has worked on trauma symptoms with the social worker for 8 months. What should the social worker do **NEXT** with this client?

**A.** Refer the client to a trauma specialist outside the agency

**B.** Wait until the last 2 sessions to tell the client of the move

**C.** Tell the client about the move and discuss options for care

**287.** A social worker in private practice has seen a client for depression for 4 months. The depression has resolved, but the client now spends hours on checking rituals. The social worker is not trained in exposure and response prevention, and two consultations confirmed the client needs it. The client wants to keep meeting with the social worker. What should the social worker do **NEXT**?

**A.** Attend a weekend workshop and then start the treatment

**B.** Continue supportive counseling, as the client prefers

**C.** Help the client transfer to a provider of that treatment

**288.** At a community mental health agency, a client became enraged when a housing letter was delayed and told the social worker, 'I know where you park. You'll regret this.' The next day the client left a similar voicemail. When the supervisor called the client to discuss the threats, the client repeated them. The agency has reported the threats under its safety policy. What is the **BEST** way to handle the clinical relationship?

**A.** Continue sessions in a room close to security staff

**B.** Address the threats with the client at the next session

**C.** End services and arrange care with another provider

**D.** Close the case by letter and decline further contact

**289.** A client in a group practice has 2 sessions left before insurance benefits run out for the year. The client still has frequent panic attacks and cannot afford to pay out of pocket. An appeal to the insurer was denied. What should the social worker do **FIRST**?

**A.** End services after the last covered session, with referrals

**B.** Discuss continuation options, such as a reduced fee

**C.** Keep meeting weekly and let the unpaid balance build

**290.** A client receiving depression treatment at a community mental health clinic has missed 3 sessions in a row. The social worker has called twice and left voicemails, but the client has not replied. The treatment goals are not yet met. What should the social worker do **NEXT**?

**A.** Close the case and record the client's lack of contact

**B.** Send a letter offering a closing session and referrals

**C.** Keep the case open and hold the client's weekly time

**291.** In the fourth session at an outpatient clinic, a client voices strong contempt for people who receive public assistance. The social worker grew up in a family that relied on public assistance, finds the remarks upsetting and intends to refer the client elsewhere. The client is making progress on work-related anxiety. What should the social worker do **FIRST**?

**A.** Seek supervision about the reaction before changing the plan

**B.** Refer the client to a colleague with a different background

**C.** Tell the client how the remarks affect the social worker

**D.** Challenge the client's views on public assistance in session

**292.** A private practice client owes for 6 sessions despite a written fee agreement and two talks about the balance. The social worker had planned to end services for nonpayment at the end of this month. Today the client reports thoughts of suicide most days since losing a job last week. How should the social worker handle the planned termination?

**A.** Postpone ending services while the risk is addressed

**B.** End services as planned and provide crisis line numbers

**C.** Refer the client to a sliding-scale clinic's waitlist

**293.** After a client shouts during a video session, the social worker sends an email that evening saying services are over. The client still has active panic symptoms. The email offers no referral and no final session. Which term **MOST** likely describes the social worker's action?

**A.** Interruption of services

**B.** Boundary violation

**C.** Client abandonment

**294.** A senior center's counseling program for older adults will lose its grant in 60 days. The board has appealed the funder's decision, and a ruling is expected in about 6 weeks. Program staff worry that telling clients now will cause some to drop out early. What is the **BEST** way for the social worker to proceed with current clients?

**A.** Wait for the appeal ruling before saying anything to clients

**B.** Tell clients now and begin planning options with each one

**C.** Give every client a list of community agencies at the last session

**295.** What is the **MOST** appropriate way for a social worker in solo private practice to prepare for sudden death or incapacity?

**A.** Ask a family member to contact clients if needed

**B.** Tell clients at intake to seek another provider if needed

**C.** Name a colleague in a written plan to notify and refer clients

**296.** In the tenth session of trauma-focused work at a private practice, a client sits quietly for several minutes, then says, 'I think I'm done coming here,' and looks away. Sleep and work have improved. The previous session focused on the most painful memory so far. What should the social worker do **FIRST**?

**A.** Invite the client to say more about wanting to stop

**B.** Agree to end and plan a final session to review gains

**C.** Recommend continuing until the trauma processing is finished

**297.** A community agency will close its counseling program in 8 weeks. The director tells staff to wait until the final week to inform clients, so they do not leave early. A social worker meets with the director and explains that clients need time to plan, but the director refuses to change course. The agency's ethics committee meets in 2 weeks. What should the social worker do **NEXT**?

**A.** Tell clients now rather than wait for the committee

**B.** Follow the director's timeline and plan referrals quietly

**C.** Raise the director's timeline with the committee

**298.** A social worker at a community mental health agency learns they need surgery in 3 weeks, followed by a recovery period that could last several months. The worker carries 26 clients, several with recurrent depression, and wants to keep the medical details private. Colleagues at the agency have openings. How should the social worker **BEST** handle the coming absence with clients?

**A.** Wait to tell clients until the surgeon gives a firm medical timeline

**B.** Tell clients soon about a medical leave and review options with each

**C.** Share the diagnosis so clients understand why the worker will be away

**D.** Transfer every client to a colleague now, before the surgery date

**299.** A client in private practice met every treatment goal for generalized anxiety 4 months ago, and symptom ratings have stayed low since. Recent sessions consist of friendly talk about the client's week. The client pays out of pocket and has explained, 'I just like having this hour.' The social worker's practice income has dropped this year. What should the social worker do **FIRST**?

**A.** Pick a date for the last session and inform the client

**B.** Continue weekly sessions, since the client values them

**C.** Raise ending with the client and review the gains made

**300.** An outpatient client with bipolar I disorder is admitted to a state psychiatric hospital and is expected to stay about 6 more weeks. The client has signed a release for the inpatient team. Agency policy automatically closes cases after 30 days without contact, and day 30 is next week. What is the social worker's **MOST** appropriate action?

**A.** Close the case under the policy and mail a letter inviting return

**B.** Ask for an exception that keeps the case open through discharge

**C.** Call the client weekly to keep the case from closing

## I.C Diversity and Social Justice

**301.** An agency begins requiring government-issued photo identification at intake. Over the next month, a social worker notices that several unhoused clients and clients who recently immigrated leave without being served. What should the social worker do **FIRST**?

**A.** Waive the requirement for affected clients without asking approval

**B.** Refer clients without identification to another agency

**C.** Document who is turned away and raise it with agency leaders

**302.** An outpatient clinic closes the case of any client who misses 2 appointments in a row. A social worker reviews closure data and finds that most closed cases involve Black clients from a neighborhood whose bus route was recently cut. What is the **BEST** way for the social worker to address this pattern?

**A.** Help each affected client appeal the closure of their case

**B.** Keep affected clients' cases open without telling managers

**C.** Present the data to leaders and propose changing the rule

**D.** Complete training on implicit bias in clinical decisions

**303.** A social worker chairs the quality committee of a community crisis service. A chart audit shows that Black youth receive police-involved crisis responses about twice as often as white youth with similar risk ratings. Staff say each decision followed protocol. What is the **BEST** response by the committee chair?

**A.** Lead a review of the dispatch process with staff and youth families

**B.** Require every staff member to complete implicit bias training

**C.** Gather another year of data before drawing any conclusions

**D.** Remind staff that the protocol applies equally to every youth

**304.** A social worker links each client's presenting problem to structural power, shares decisions with clients, and examines how the worker's own social position shapes the work. Which framework does this approach reflect?

**A.** Strengths perspective

**B.** Ecological systems perspective

**C.** Anti-oppressive practice

**D.** Person-centered approach

**305.** During a child welfare team meeting, staff are deciding whether a family can keep a safety plan at home or needs a placement. A colleague says, 'Families from that housing complex never follow through.' Others nod, and the supervisor moves toward a vote. The family's record shows they have met each plan task so far. What should the social worker do **FIRST**?

**A.** Name the generalization and redirect to the case facts

**B.** Speak with the colleague privately after the meeting

**C.** Report the colleague's remark to the program director

**306.** In supervision, a supervisee calls a client who has missed four group sessions 'noncompliant' and proposes closing the case. The client works night shifts, relies on two buses, and asked last month whether the group could meet at another time. What is the **BEST** action by the supervising social worker?

**A.** Approve closing the case under the attendance policy

**B.** Guide the supervisee to plan access options with the client

**C.** Suggest the supervisee assess the client's readiness to change

**307.** A social worker at a regional agency is asked to lead a needs assessment for a neighborhood the agency has served poorly. Past agency programs there drew few participants, and local leaders have told the agency, 'You always decide for us.' Agency leadership wants results within 6 months. What is the **BEST** way for the social worker to design the assessment?

**A.** Use a validated national survey to ensure comparable data

**B.** Analyze the agency's service records to find unmet needs

**C.** Hire an outside research firm to run neutral focus groups

**D.** Form a community advisory group to shape questions and methods

**308.** A social worker runs a reentry group at a county jail using a curriculum built around personal choices. In one session, members say that heavy policing of their neighborhoods and lack of jobs also led to their arrests. Several members stop taking part when the social worker returns to the workbook. What is the **BEST** response by the social worker?

**A.** Discuss structural factors alongside personal choices in the session

**B.** Validate the members' points, then return to the planned module

**C.** Record the concerns to share with curriculum developers later

**D.** Keep the focus on personal choices members control after release

**309.** At a domestic violence program, a social worker is safety planning with an adult client whose partner has made threats. The client says calling the police is off the table because officers injured a relative during a past call. The client names a cousin, a neighbor and a church member who would help. What is the **MOST** appropriate action by the social worker?

**A.** Encourage the client to call police in an emergency despite past harm

**B.** Explore the client's feelings about the relative's injury further

**C.** Refer the client to a program with police liaison advocates

**D.** Build the plan around the people and options the client trusts

**310.** A hospice social worker meets with the adult children of a Black patient with advanced cancer. The family declines a hospice admission, saying doctors 'gave up on' a relative decades ago because of race. A team member describes the family as 'in denial.' What is the **MOST** helpful response by the social worker?

**A.** Review hospice benefits and outcome data with the family

**B.** Document the family's denial and revisit hospice next week

**C.** Acknowledge the past harm and ask what would build trust

**311.** A hospital social worker tracks discharges for a year and finds that older patients from a mostly Black zip code go to lower-rated nursing facilities more often than other patients with the same insurance. The social worker shares the data with the unit manager, who says to drop it. What is the **MOST** effective way for the social worker to address the pattern?

**A.** Steer each of the worker's own patients to higher-rated facilities

**B.** Present the findings to the quality improvement committee

**C.** File a complaint with the state civil rights office now

**312.** A social worker who manages an outpatient program reviews caseloads and finds that the program's two Black staff social workers carry nearly all of the Black clients. They are also asked to lead every diversity training, with no change in workload. One of them recently said, 'I'm running on empty.' What should the manager do **FIRST**?

**A.** Meet with the two social workers about the pattern and their workload

**B.** Spread new Black clients evenly across the whole staff

**C.** Add a stipend for the extra diversity training workload

**D.** Suggest the tired staff member use the employee assistance program

**313.** After a client complains about hearing staff speak Spanish in the break room, a clinic director issues a rule that staff must speak only English at all times on the premises, including breaks. Several bilingual staff tell a social worker on the clinic's equity committee that they feel targeted. What is the social worker's **MOST** appropriate action?

**A.** Urge the director to replace the rule with one tied to client needs

**B.** Remind bilingual staff to follow the director's rule during review

**C.** File a discrimination complaint with a federal agency right away

**314.** A Black engineer in CBT for depression is the only Black member of a work team. The client has documented being passed over for two promotions and hearing a manager use a racial slur. The client's thought record reads: 'They'll never promote someone like me, so I'm worthless.' How should the social worker **BEST** approach this thought?

**A.** Challenge the promotion prediction as a fortune-telling distortion

**B.** Shift the sessions toward filing a formal discrimination complaint

**C.** Accept the bias as real and examine the self-worth conclusion

**D.** Set the thought record aside until the workplace problem resolves

**315.** Which term names the process in which oppressed people come to see how social and economic forces shape their lives, and then act together to change them?

**A.** Empowerment

**B.** Critical consciousness

**C.** Cultural humility

**316.** A community mental health agency receives 15 tablets with a year of free data service to lend to clients for video sessions. About 60 clients ask for one. Staff propose a first-come, first-served sign-up form on the agency website. The social worker on the planning group knows that many clients who lack internet at home have been missing sessions. Which way of allocating the tablets is **MOST** fair?

**A.** Offer them through the website form in sign-up order

**B.** Hold a random drawing among the clients who ask

**C.** Give priority to clients without a device or data plan

**D.** Let each worker choose the clients who seem most deserving

**317.** A school district gives every student the same laptop. It also gives home hotspots and extra tech support to students whose families lack internet service. Which justice principle does the added support for some students reflect?

**A.** Equality

**B.** Universal design

**C.** Equity

**318.** After a budget cut eliminated a paid peer specialist position, a recovery agency asks 3 clients in long-term recovery to serve as volunteer peer mentors for 20 hours a week each. They would do the duties the peer specialist once did. The clients are eager to help. The social worker sits on the planning committee. What is the **BEST** recommendation?

**A.** Seek funds to pay the mentors a fair wage

**B.** Accept the clients' offer to serve as volunteers

**C.** Thank the mentors with gift cards each month

**319.** A city plans a new diesel bus depot next to a low-income neighborhood. The only public hearing is held at 10 a.m. on a weekday, in English, with notice posted only on the city website. Residents say they learned of the plan after the vote. Which aspect of environmental justice does the hearing arrangement fail?

**A.** Distributive justice

**B.** Restorative justice

**C.** Intergenerational justice

**D.** Procedural justice

**320.** At a pediatric clinic, a social worker helps a parent whose 7-year-old's asthma keeps worsening. The apartment has visible mold and roaches. With the social worker's help, the parent sent the landlord a written repair request and a doctor's letter 6 weeks ago. The landlord has not responded and has hinted that the lease may not be renewed. What should the social worker do **NEXT**?

**A.** Ask the doctor to strengthen the child's asthma treatment plan

**B.** Help the parent search for a different apartment

**C.** Connect the parent with a tenant legal aid program

**D.** Organize other tenants to confront the landlord together

**321.** A family services agency director says, 'We treat everyone the same, so we don't collect race or ethnicity data. Asking would only make clients uncomfortable.' The social worker leads the agency's quality improvement work. What is the **MOST** appropriate response?

**A.** Offer staff implicit bias training instead of collecting data

**B.** Collect race data just from clients who report unfair treatment

**C.** Propose collecting self-reported race data to find gaps

**322.** A social worker at a housing program learns that a landlord refused to show a vacant apartment to a client after hearing the client has three children, saying the unit 'isn't right for kids.' The client is upset that other families may be turned away too, but does not want to file a complaint, saying the housing search is exhausting enough. Which response is **BEST**?

**A.** Respect the decision and focus on the housing search

**B.** File a fair housing complaint on the client's behalf

**C.** Encourage the client to reconsider filing a complaint

**D.** With permission, report the practice to a fair housing group

**323.** At a family services agency, three bilingual case managers who completed the agency's interpreter training are often pulled from their own caseloads to interpret for colleagues. They carry full caseloads with no added pay, and one was marked down for late notes. They say they want to keep interpreting for families they know but need the work recognized. The social worker chairs the equity committee. What is the **BEST** action?

**A.** Propose crediting interpreting time in workload and pay

**B.** Ask the supervisor to revise the late-notes rating

**C.** Arrange contract interpreters so bilingual staff stop interpreting

**D.** Encourage the case managers to file a grievance themselves

**324.** Which policy approach sets one shared goal for an entire population and then designs different strategies for each group based on the barriers it faces?

**A.** Universal provision

**B.** Targeted universalism

**C.** Means-tested targeting

**325.** A client works 38 hours a week at two jobs, supports two children, and falls behind on rent each winter. The social worker and client have reviewed the household budget together and found spending limited to basics. The client has never filed a tax return because the income seemed too low to matter. Which step is **MOST** helpful?

**A.** Help the client file to claim refundable tax credits

**B.** Help the client apply for emergency rental assistance

**C.** Review the client's spending for further savings

**326.** A 34-year-old bisexual client in a small town meets with a social worker about low mood and poor sleep. The client keeps a same-sex relationship hidden from coworkers, plans each conversation to avoid slips, and expects to be shunned if found out. The client adds, 'Part of me still thinks something is wrong with me.' Which concept **MOST** likely explains this pattern?

**A.** Acculturative stress

**B.** Stereotype threat

**C.** Minority stress

**D.** Identity foreclosure

**327.** A 79-year-old resident of an assisted living facility tells the social worker that the 'friend' who visits daily is the resident's same-sex partner of 35 years. Staff have left the partner out of care-planning meetings because the partner is not family. The resident says, 'When I was young, people lost their jobs over this.' What should the social worker do **FIRST**?

**A.** Ask the resident how to include the partner and what to share

**B.** Tell staff about the relationship so the partner is included

**C.** Help the resident name the partner as health care agent

**D.** Encourage the resident to tell staff for better support

**328.** A 27-year-old client in private practice says, 'I'm gay, but my faith is everything to me. I want you to help me get rid of these same-sex attractions.' The client reports guilt and growing isolation from both church friends and gay friends. What is the **MOST** appropriate response by the social worker?

**A.** Agree to work with the client on reducing same-sex attractions

**B.** Decline change work and offer to explore faith and identity

**C.** Encourage the client to leave the faith community for now

**329.** A college student meets with a campus social worker about stress over choosing a major. The student wants to study art, but the parents favor nursing. The student says, 'In my family, big decisions are made together. Going against my parents would bring shame on all of us, and I don't want that.' Which approach is **BEST**?

**A.** Help the student separate personal wishes from parents' wishes

**B.** Invite the family to a session to negotiate the choice

**C.** Help the student weigh options in light of family values

**330.** Six months ago, a store guard falsely accused a 16-year-old Black student of theft. Since then, the student, who attends a mostly white high school, has joined the Black Student Union, reads Black history late into the night, wears African-print clothing, and calls longtime white friends 'part of the problem.' The parent asks the school social worker what is happening. In Cross's model, which stage **MOST** likely fits?

**A.** Encounter

**B.** Immersion-Emersion

**C.** Internalization

**D.** Pre-encounter

**331.** In a rural primary care clinic, a 52-year-old male farmer is referred to the social worker after telling the physician he is 'just worn out.' He has been irritable, drinking more beer at night and snapping at his family. When the social worker mentions depression, he says, 'I'm not some weak guy who needs to cry about it.' Which response is **MOST** helpful?

**A.** Talk about the problems in his terms, such as stress and sleep

**B.** Explain that depression is a medical illness like any other

**C.** Refer him to a substance use program for the drinking

**332.** Which term says that race, gender, class and other identities combine to shape unique experiences of privilege and oppression?

**A.** Double jeopardy

**B.** Cultural pluralism

**C.** Minority stress

**D.** Intersectionality

**333.** A 15-year-old came out as gay to the parents a month ago and is now withdrawn at home; a screening last week found no suicidal thoughts. In a family session, the parents say, 'We love our child, but our faith teaches this is wrong. We won't let these friends visit, and we pray for a change.' The teen looks down and says nothing. Which approach is **BEST**?

**A.** Show how specific accepting behaviors protect the teen's health

**B.** Ask the parents to put their religious beliefs aside for the teen's sake

**C.** See the teen individually and pause the family sessions

**334.** In a pediatric clinic, a Black parent tells the social worker about recent talks with an 11-year-old. The parent explained that some people may treat the child unfairly because of race, then practiced with the child how to stay calm and respond if stopped by police or followed in a store. Which ethnic-racial socialization practice is the parent **MOST** likely using?

**A.** Promotion of mistrust

**B.** Cultural socialization

**C.** Preparation for bias

**D.** Egalitarianism

**335.** A 19-year-old college student tells a campus social worker, 'I'm probably gay, though part of me wishes I weren't.' The student has started going to a campus LGBTQ meeting, mainly to feel less alone, and finds some members welcoming and others distant. The student has told no one else. Using Cass's model, which stage **MOST** likely fits this student?

**A.** Identity comparison

**B.** Identity acceptance

**C.** Identity tolerance

**D.** Identity pride

**336.** In a children's hospital, Deaf parents who use American Sign Language learn that their 10-month-old has profound hearing loss. After a full discussion with the audiologist, they decline cochlear implant surgery. They plan to raise the child in ASL within the Deaf community and have joined a family sign program. The surgeon asks the social worker to 'help them see what is best' and mentions neglect. What is the **MOST** appropriate response?

**A.** Encourage the parents to reconsider, citing spoken-language outcomes

**B.** Support the parents' decision and their plan for early ASL

**C.** Request an ethics consultation to resolve the disagreement

**D.** File a medical neglect report with the child welfare agency

**337.** A 26-year-old gay Muslim client tells a social worker that the local LGBTQ center's events, often held in bars, feel foreign to their faith, while sermons at their mosque make them hide their sexuality. The client says clearly that they want to keep both their faith and their relationships, and asks where they might find belonging. The social worker had suggested the center's support group. What should the social worker do **NEXT**?

**A.** Urge the client to give the LGBTQ group a few more tries

**B.** Explore whether the client's faith reflects internalized homophobia

**C.** Help the client find LGBTQ groups within their faith, including online

**338.** A 15-year-old with one Black parent and one white parent tells the school social worker that classmates keep saying the teen 'has to pick a side.' The teen calls themself mixed and feels close to both families' traditions. A teacher has suggested the teen would fit in better by identifying as Black, since that is how others see them. What is the **MOST** appropriate response?

**A.** Encourage the teen to identify as others perceive them

**B.** Explore which side of the family feels most like home

**C.** Affirm the teen's right to define their own identity

**339.** In a primary care clinic, a client with type 2 diabetes who uses insulin mentions a plan to fast during Ramadan next month. The physician, concerned about low blood sugar, asks the social worker to 'talk them out of it.' The social worker is meeting the client for the first time. What should the social worker do **FIRST**?

**A.** Ask the client to describe what fasting means to them

**B.** Arrange a pre-Ramadan medical visit to plan safer fasting

**C.** Help the client plan Ramadan meals before dawn and after sunset

**D.** Explain that the faith excuses people who are ill from fasting

**340.** A client with limited English proficiency arrives for a mental health intake with their 15-year-old child, who offers to interpret. The client says they would prefer that the child interpret. What is the **MOST** appropriate action?

**A.** Proceed with the child interpreting, as the client prefers

**B.** Use a qualified telephone or video interpreter for the intake

**C.** Ask a bilingual receptionist to interpret during the intake session

**341.** In a hospital emergency department, a social worker is asked to assess a patient with limited English who took extra pills. The patient's adult sibling, who is fluent in English, offers to interpret. A bilingual nurse whose language skills have not been assessed is on shift. A telephone interpreter is available at once, and an in-person interpreter can arrive in 2 hours. What should the social worker do **FIRST**?

**A.** Let the sibling interpret so the assessment can begin

**B.** Start the assessment with the phone interpreter

**C.** Wait for the in-person interpreter before assessing

**D.** Ask the bilingual nurse to interpret the interview

**342.** A home health social worker visits an older client who speaks Vietnamese to set up a schedule for home-delivered meals. The client's adult child offers to interpret. With the child out of the room, the social worker uses a telephone interpreter to explain that a free interpreter is available. The client clearly prefers the child, and the child agrees. What should the social worker do **NEXT**?

**A.** Proceed as the client chose and document the choice

**B.** Keep the telephone interpreter on the line for the visit

**C.** Explain again why a trained interpreter is better

**343.** A Deaf adult who grew up in another country begins services at a community mental health center. A qualified ASL interpreter attends the intake but reports that the client uses many home signs and signs from another country, and much is being missed. Twenty minutes of slower pacing and simpler questions have not closed the gap. What should the social worker do **NEXT**?

**A.** Add a Certified Deaf Interpreter to the team

**B.** Replace the interpreter with written notes in English

**C.** Invite a family member who knows the client's signs

**D.** Refer the client to a provider who signs fluently

**344.** An autistic 26-year-old client at a community agency joins a job-search support group. After 2 sessions, the client tells the social worker that the bright lights and overlapping voices in the group room become overwhelming, so the client leaves early. The client still wants to be in the group. What is the **BEST** response by the social worker?

**A.** Coach the client to ask the group for lighting and noise changes

**B.** Arrange sensory changes, such as softer lighting and a break space

**C.** Offer individual sessions in place of the group

**345.** At a hospital discharge meeting, a 40-year-old patient with cerebral palsy communicates by typing on a speech-generating device. Each time the team asks about plans for home, members turn to the patient's caregiver for the answer. The patient starts typing an answer but stops. What should the social worker do **FIRST**?

**A.** Ask the patient the question and wait for the typed reply

**B.** Ask the caregiver to answer, then check with the patient

**C.** Switch to yes-or-no questions to speed up the meeting

**346.** A caregiver support group at a nonprofit agency meets in a second-floor room with no elevator. A new member who uses a wheelchair wants to join. The program director suggests that the social worker offer this member individual sessions instead. A ground-floor conference room is free at the group's meeting time. What should the social worker do?

**A.** Offer the member individual sessions as the director suggests

**B.** Move the group to the accessible downstairs room

**C.** Have the member join the group by video from downstairs

**347.** During a child welfare meeting with a parent who speaks Haitian Creole, the social worker notices that the agency interpreter is giving long replies the parent did not say. The interpreter then adds, in English, 'I told the parent to be grateful for the help.' What should the social worker do **FIRST**?

**A.** Ask a bilingual coworker to take over interpreting now

**B.** Report the interpreter to the language service afterward

**C.** Pause and restate the interpreter's role right away

**348.** A Deaf client at a nonprofit counseling agency asks for an ASL interpreter for weekly sessions. The program manager tells the social worker to use written notes because interpreters are costly. When the social worker lays out the client's request and the access concern, the manager repeats the decision. What should the social worker do **NEXT**?

**A.** Use written notes as directed and record the client's request

**B.** Ask the client to bring a relative who signs to sessions

**C.** Seek guidance from the agency's director or compliance officer

**D.** Refer the client to an agency that provides interpreters

**349.** A client arrives at a private nonprofit counseling center with a dog that wears no vest. The client explains that the dog is trained to nudge them and lead them outside when a dissociative episode begins. The receptionist says animals cannot come in unless there is a doctor's letter and asks the social worker how to proceed. What should the social worker tell the receptionist?

**A.** Admit the dog without requiring any paperwork

**B.** Admit the dog once a provider letter is on file

**C.** Admit the dog but keep it in the waiting area during sessions

**350.** An autistic adult starts counseling at an agency whose secure online system supports both video and live written messaging. The client explains that speaking aloud under stress makes them lose words and that video eye contact leads to shutdown, but they write clearly and quickly. The supervisor prefers video so the social worker can observe affect. What is the **BEST** way to provide services?

**A.** Hold sessions through the system's text chat

**B.** Hold video sessions on the system with the camera off

**C.** Refer the client to an autism specialty program

**351.** A parent's child welfare case plan requires a 10-week parenting class run by a private nonprofit. The parent has a documented reading disability and turns in blank worksheets but takes an active part in discussions. The facilitator plans to report the parent as noncompliant, and the parent asks the caseworker, a social worker, for help. What should the social worker do **FIRST**?

**A.** Help the parent complete the worksheets after each class

**B.** Request oral or recorded worksheet responses

**C.** Find a different parenting program with less written work

**352.** A 70-year-old client with macular degeneration receives telehealth counseling from an agency. The client uses a screen reader but cannot read the agency's scanned consent forms and treatment plans, so an adult child reads them aloud. The client says they want to review these papers privately and at their own pace, and that Word documents work well with the screen reader. What is the **MOST** appropriate response?

**A.** Read each document aloud to the client during sessions

**B.** Email the scanned forms so the client can enlarge them

**C.** Send the documents as accessible electronic files

**353.** A child welfare social worker is building a case plan with a parent whose children are in foster care. The parent agrees to every proposed goal without comment. Earlier, the parent described distrust of the agency after relatives lost custody of their children. What is the **MOST** appropriate response by the social worker?

**A.** Finalize the plan, since the parent agreed to each goal

**B.** Acknowledge the agency's authority and ask the parent's own goals

**C.** Tell the parent they seem to be holding back opinions

**D.** Reassure the parent that the agency's aim is reunification

**354.** A Black client in outpatient treatment for depression asks the white social worker about working with a Black social worker at the clinic instead. Invited to say more, the client says the work has helped but that they want to talk about racism at work without explaining it first. A Black colleague has openings. What should the social worker do **NEXT**?

**A.** Explore whether the request reflects discomfort with the current work

**B.** Arrange the transfer and offer a joint handoff session

**C.** Propose a few more sessions to work through the concern together

**D.** Reassure the client that the worker is willing to learn

**355.** A 16-year-old Mexican American client tells a social worker that darker-skinned relatives are 'less smart' and that lighter skin makes people 'more respectable.' The client avoids the sun to stay light and calls classmates with Indigenous features 'low class.' These views echo messages the client has heard from family and media. Which concept **MOST** likely explains the client's beliefs?

**A.** Internalized racial oppression

**B.** Acculturative stress

**C.** Assimilation

**D.** Stereotype threat

**356.** At a first session in private practice, a Black client describes tension with neighbors since moving to a mostly white suburb. The client pauses, looks at the white social worker, and says, 'I don't know if you'd get this part.' What is the social worker's **BEST** response?

**A.** Assure the client of the worker's training in diverse cultures

**B.** Note the comment and return to it once rapport is stronger

**C.** Name the racial difference and ask how it may affect the work

**357.** A social worker leads a support group for older adult caregivers at a senior center. In the third meeting, white members do most of the talking, and one member twice talks over a Black member to correct that member's story about a clinic visit. The Black member and two Latino members have gone quiet. What is the **MOST** appropriate action by the social worker?

**A.** Check in privately with the quieter members after the meeting

**B.** Name the pattern and set a norm that each member is heard

**C.** Let the group develop its own norms over the next sessions

**358.** A white social worker goes with a Black client to a benefits office to appeal a denial. The caseworker there speaks to the social worker alone and brushes off the client's answers. The client prepared the appeal and wants to present it. What is the **BEST** action by the social worker?

**A.** Present the appeal so the caseworker will listen and decide

**B.** File a complaint about the caseworker's conduct after the visit

**C.** Turn each question back to the client and stay present

**359.** A white supervisee describes a Black client who wants help finding a better-paying job. The supervisee keeps steering sessions toward anger management, saying, 'I know what this client really needs.' The client has missed the last two sessions. What is the **MOST** appropriate response by the supervising social worker?

**A.** Reassign the client to a social worker with more experience

**B.** Help the supervisee reflect on privilege and power in this pattern

**C.** Direct the supervisee to follow the client's stated goals

**360.** A white clinical supervisor notices that a Black supervisee agrees with every plan in supervision but later handles several family cases differently, with good results. When the supervisor asks about it, the supervisee says, 'I didn't think disagreeing with you was an option here.' What is the supervisor's **BEST** response?

**A.** Remind the supervisee to voice disagreement instead of bypassing plans

**B.** Offer to arrange supervision with a Black colleague instead

**C.** Name the power and racial difference and invite open disagreement

**361.** After one session with a Black social worker, a white client at a community mental health center asks the intake supervisor for a new worker. The client says, 'Nothing personal, but I'd do better with someone more like me.' The supervisor has already talked with the social worker, who is comfortable continuing. What is the supervisor's **BEST** response?

**A.** Reassign the client to a white social worker as requested

**B.** Meet with the client about the request and affirm the assignment

**C.** Ask the social worker again how continuing would feel

**362.** In an employee assistance session, a white client says a workplace diversity training called them 'privileged.' The client says angrily, 'I grew up poor and worked for everything I have. Nobody handed me anything.' This is the first time the client has raised the topic. What is the social worker's **MOST** appropriate response?

**A.** Explore what the training's label stirred up for the client

**B.** Explain that racial privilege can exist alongside economic hardship

**C.** Agree that the training should have focused on class instead

**363.** A client who resettled in the United States 8 months ago brings a small gift to every home visit. The client presses each gift into the social worker's hands at the door and seems anxious when the worker hesitates. The two have not yet talked about the gifts. What response is **MOST** helpful?

**A.** Explain that services continue with or without gifts

**B.** Accept each gift to honor the client's custom

**C.** Ask the client what the gifts are meant to express

**D.** Decline the gifts, citing the agency's no-gifts policy

**364.** In the third session, a Black client tells the social worker that an earlier comment, 'You're so articulate,' felt like a racial stereotype. The social worker listens without interrupting until the client finishes. What should the social worker do **NEXT**?

**A.** Acknowledge the impact of the comment and apologize

**B.** Explain that the comment was meant as a compliment

**C.** Offer to transfer the client to a different social worker

**365.** At intake, a social worker asked a client with a Spanish surname whether they needed an interpreter. The client, who grew up in the United States and speaks only English, said no and was quiet for the rest of the session. In supervision afterward, the social worker identifies the question as an assumption based on the client's name. What is the social worker's **BEST** action?

**A.** Offer the client a transfer to a bilingual colleague at the agency

**B.** Wait to see if the client brings up the question

**C.** Acknowledge the question's impact with the client next session

**366.** A hospital social worker meets with a Black adult patient admitted during a sickle cell pain crisis. The patient says pain medicine has been delayed and that a nurse called the patient 'drug-seeking.' The chart shows pain scores at the top of the scale for 6 hours. What should the social worker do **FIRST**?

**A.** Help the patient file a grievance with patient relations

**B.** Explain how the unit decides when to give pain medicine

**C.** Coach the patient to describe the pain calmly to staff

**D.** Bring the untreated pain to the care team now

**367.** In a telehealth session, a Black client describes months of coworkers touching the client's hair, asking whether the client 'got the job through a program,' and joking about the client's 'attitude.' The client now sleeps poorly and dreads Mondays. The client asks, 'Am I being too sensitive?' What is the **MOST** appropriate response by the social worker?

**A.** Help the client test whether the coworkers really meant harm

**B.** Encourage the client to report each incident to human resources

**C.** Affirm that repeated slights can cause real stress and harm

**368.** A client tells a social worker about being the sole Asian American employee on a team and being left out of meetings. A friend responded, 'I don't see race. People are just people.' Under Sue and colleagues' taxonomy, the friend's comment is **MOST** likely which type of microaggression?

**A.** Microinvalidation

**B.** Microinsult

**C.** Microassault

**369.** A school social worker reviews a year of the worker's own referral records. Black students were referred for behavior evaluations at about twice the rate of white students with similar incident reports. The worker has already flagged the five most recent referrals. What should the social worker do **NEXT**?

**A.** Re-review the flagged referrals alone and withdraw biased ones

**B.** Take an online implicit association test to gauge the bias

**C.** Review the referrals and the pattern with a supervisor

**D.** Raise the referral threshold for Black students going forward

**370.** In a residential treatment group for adolescents, one member says to an Asian American member, 'Your English is really good for someone like you.' The Asian American member laughs and looks down. Other members glance at the social worker leading the group. What is the social worker's **BEST** response?

**A.** Ask the Asian American member to share how the comment felt

**B.** Pause the group and name the comment as a stereotype

**C.** Talk with the member who made the comment after group

**D.** Continue the discussion, since the member laughed it off

**371.** On an inpatient psychiatric unit, a Black adult patient is admitted with psychotic symptoms after a week without sleep. The patient is guarded with staff and tells the social worker about being restrained during a past emergency visit while a white patient who was shouting was not. In rounds, the team cites the patient's distrust of staff as paranoia that supports a schizophrenia diagnosis. What should the social worker do **FIRST**?

**A.** Point out that the distrust may reflect experiences of racism

**B.** Record the restraint account in the psychosocial assessment

**C.** Question the patient further to see if the distrust is a fixed belief

**372.** Which statement about implicit bias is **MOST** accurate?

**A.** It can shape judgments even in people with egalitarian beliefs

**B.** It is prejudice that people knowingly hide from others

**C.** It can be lastingly reduced by a brief training session

**373.** A 68-year-old transgender woman on a hospital medical unit has stopped eating her meals and refuses physical therapy. She tells the social worker that staff keep calling her 'sir' and using her former name, which is printed on her wristband. She asks the social worker for help and agrees that her care team can be told. What should the social worker do **NEXT**?

**A.** Ask the patient to describe how the comments affect her

**B.** Request a psychiatric consult for her refusals and poor intake

**C.** Coach the patient to correct staff each time it happens

**D.** Ask the unit to update her name and pronouns and brief staff

**374.** At the social worker's invitation, a 15-year-old Black adoptee in a family session describes being followed through a store by a security guard. The white adoptive parent responds, 'You're overthinking it. In this family, we don't see color.' The teen looks away and stops talking. What should the social worker do **NEXT**?

**A.** Meet with the parent alone later to address the color-blind view

**B.** Help the teen plan ways to respond calmly to store security

**C.** Slow the exchange and help the parent hear the teen's experience

**375.** A first-generation college student meets with a campus social worker after failing a math placement exam. The student earned A's in high school math and scores well on practice tests at home. Just before the exam, an advisor remarked that students 'from your neighborhood' usually need remedial math. The student says their mind went blank. Which concept **MOST** likely explains the poor score?

**A.** Impostor phenomenon

**B.** Learned helplessness

**C.** Social anxiety disorder

**D.** Stereotype threat

**376.** A social worker notices that the clinic's intake coordinator routes every Spanish-speaking caller to case management only, noting 'not suited for therapy,' even when callers ask for counseling. The clinic has bilingual therapists with openings. The social worker raised the pattern privately with the coordinator, who replied, 'It's what works for those clients.' What should the social worker do **NEXT**?

**A.** Talk with the coordinator again about the routing pattern

**B.** Contact the affected callers to offer clinic therapy directly

**C.** Bring the routing pattern to the clinic's program director

**377.** A school social worker notices that a 9-year-old who recently arrived from Nigeria has started asking classmates to call them 'Sam' instead of their Yoruba name. Since the first week, the teacher has shortened the name, saying it is 'too hard to say.' The child seems quieter in class. What should the social worker do **FIRST**?

**A.** Ask the teacher to use the child's full Yoruba name

**B.** Lead a class lesson on names and their meanings

**C.** Ask the child privately about the name they want and why

**D.** Encourage the child to correct the teacher's pronunciation

**378.** At a drop-in center for youth experiencing homelessness, a 16-year-old who left home after coming out as gay tells the social worker about staying with adults who offer a bed in exchange for sex. The youth says, 'Nobody makes me. It's my choice.' No third party arranges the meetings. Under federal law, how is this situation classified?

**A.** Survival sex, since no one else arranges or profits

**B.** Sex trafficking of a minor

**C.** Trafficking if force or coercion can be shown

**379.** A social worker at an urban domestic violence program completes intake with a client who identifies as Diné (Navajo) and as Mexican American. The agency database allows more than one race, but staff usually copy the single race on the police report, which lists 'White.' The program director is preparing a report on violence against Indigenous clients. What is the **BEST** way to complete the race entry?

**A.** Copy the race listed on the police report

**B.** Enter each race the client names, plus tribal nation

**C.** Record the Native identity alone so the report counts the client

**380.** A university team plans to study blood samples that members of a tribal community gave years ago for diabetes research. The new study would analyze the samples for population ancestry. The samples are de-identified, and the university's review board has ruled that new consent is not required. The social worker on the team knows the community has had samples used in ways members never agreed to. What is the **MOST** appropriate step?

**A.** Proceed, as the review board has approved the plan

**B.** Contact the original donors to ask for individual consent

**C.** Share study results with tribal leaders once analysis ends

**D.** Seek the tribal government's approval before any reuse

**381.** At an urban Indian health center, a family tells the social worker that their 19-year-old relative has not been seen or heard from in 2 days, which is out of character. When they called police, an officer said young adults often leave on their own and told them to call back in 3 days. The family asks for help. What should the social worker do **FIRST**?

**A.** Help the family file a missing person report today

**B.** Help the family post a missing notice on social media

**C.** Advise the family to wait 3 days while calling friends

**D.** Help the family call area hospitals and jails

**382.** A social worker at a residential substance use program is writing an intake summary for a 24-year-old client. The client describes being made to sell sex by an older partner starting at age 16. An old court record lists 'juvenile prostitution arrest, age 16.' What is the **MOST** appropriate way to document this history?

**A.** Describe it as sexual exploitation that began at age 16

**B.** Copy the court record's wording for consistency

**C.** Leave the history out to protect the client from future stigma

**383.** A school social worker meets with a 16-year-old who arrived in the country alone and now lives with an adult sponsor. The teen falls asleep in class. After gentle questions, the teen explains working nights at a factory 6 days a week to repay a large travel debt to the sponsor, who keeps the teen's identity papers. The teen adds, 'In my country, young people help the family.' What should the social worker do **NEXT**?

**A.** Contact the child abuse hotline about suspected trafficking

**B.** Meet with the sponsor to discuss cutting the work hours

**C.** Ask the teen more about family expectations back home

**D.** Treat the work as a cultural norm and adjust school supports

**384.** A community health center serving mainly Black, Latina and Native clients on Medicaid sets a quarterly target for long-acting contraceptive placements, such as implants and IUDs. Staff who meet it are praised at meetings. A client tells the social worker that a staff member kept pushing an implant after the client asked for pills and explained that a grandparent was sterilized without consent. The client now has the pills. What is the **BEST** action for the social worker?

**A.** Report the staff member's conduct to the clinic manager

**B.** Ask clinic leaders to drop the target for these methods

**C.** Train staff to counsel neutrally while keeping the target

**385.** A residential substance use program serves several Native clients. A non-Native staff member began leading smudging and a 'spirit animal' exercise for all residents after watching online videos. Native residents tell the director the activities mock their ceremonies, though two of them want traditional healing as part of treatment. The director stops the activities and asks the social worker for advice. What should the social worker recommend **FIRST**?

**A.** Ban Native cultural practices from the program

**B.** Ask the Native residents to lead smudging for the whole group

**C.** Seek guidance from local tribal leaders on these practices

**D.** Have staff complete training on Native spiritual traditions

**386.** The Missing and Murdered Indigenous Women and Relatives (MMIW) movement chiefly calls attention to which problem?

**A.** Loss of Native languages caused by past boarding school policies

**B.** Disappearances and killings met by weak official response

**C.** Overrepresentation of Native children in foster care

**387.** A middle school uses restorative circles after harm. A 13-year-old painted insults over a classmate's mural and has admitted it. The classmate tells the school social worker, 'I don't want to sit in a room with them.' The principal says the circle is school policy and should go ahead. What is the **MOST** appropriate response?

**A.** Meet with the classmate alone to learn what repair they want

**B.** Encourage the classmate to attend, as meeting aids healing

**C.** Hold the circle with a staff member speaking for the classmate

**388.** Which question is central to a restorative justice response to wrongdoing?

**A.** Who was harmed, and what do they need?

**B.** Which rule was broken, and what penalty fits?

**C.** How can both sides compromise and move on?

**389.** A juvenile diversion program refers a 15-year-old for a restorative conference with a store owner whose shop window was broken. At the preparation meeting, the youth tells the social worker, 'I wasn't even there. They just blame me because of my friends.' The store owner is eager to meet. What is the **BEST** way to proceed?

**A.** Hold the conference so the youth hears the owner's losses

**B.** Put the conference on hold while the youth denies involvement

**C.** Let the store owner decide whether the conference goes ahead

**D.** Urge the youth to admit the act to stay in diversion

**390.** A county child welfare agency and two tribal nations agree to a joint truth-and-reconciliation effort about decades of Native children placed in non-Native foster homes. Agency leaders want to open with a formal apology and a list of reforms drafted by agency staff. The social worker on the planning group is asked for input. Which opening step is **MOST** consistent with this kind of effort?

**A.** Release the agency's apology and reform list at the first session

**B.** Gather statements from affected families, with tribes co-leading

**C.** Have agency staff review old case files to find errors

**391.** A 68-year-old client at a county behavioral health clinic says that, as a child, county caseworkers removed the client from a Native family and placed the client with strangers, and the client's siblings were never found. The client has described this in detail over two sessions. Today the client asks the social worker, 'Does the county admit it was wrong?' What should the social worker do **FIRST**?

**A.** Explain how child welfare policies have changed since that time

**B.** Acknowledge that removals like this caused deep, lasting harm

**C.** Invite the client to say more about what the removal cost

**D.** Refer the question to county leaders for an official reply

**392.** At a county behavioral health clinic, a staff member made a mocking comment about a colleague's tribal ceremony during a team meeting. The staff member has admitted it and wants to make amends, and the colleague has agreed to a restorative circle. The clinic asks the social worker, a trained facilitator, to lead it. What should the social worker do **FIRST**?

**A.** Convene the circle and open with shared ground rules

**B.** Ask the staff member to write an apology to the colleague

**C.** Meet separately with each person to prepare

**393.** In a youth diversion conference, a 16-year-old who vandalized a community garden hears from gardeners about the damage. Elders and the youth's coach say the act was wrong and hurtful, then describe the youth's strengths and invite the youth to help replant. The gardeners then thank the youth and ask them to join the next community workday. Which concept **MOST** likely describes this approach?

**A.** Reintegrative shaming

**B.** Stigmatizing shaming

**C.** Labeling

**D.** Deterrence

**394.** At a family shelter, a client who fled political violence and arrived in the country 10 months ago tells the social worker, 'A friend said there is a time limit to ask for protection here. Should I apply for asylum?' The client has not met with any lawyer. What should the social worker do **FIRST**?

**A.** Explore the violence the client faced before leaving

**B.** Help the client write a statement for the asylum form

**C.** Arrange a prompt meeting with an immigration legal provider

**395.** At a community health center, a social worker completes an intake with an adult client who was resettled as a refugee 3 months ago after 6 years in a refugee camp. The client reports poor sleep, headaches and worry about a sibling still abroad. What is the **MOST** appropriate way for the social worker to structure the assessment?

**A.** Focus on current resettlement needs such as housing and work

**B.** Begin with detailed processing of the most painful war memories

**C.** Ask about life before leaving, in transit and since arriving

**396.** A 14-year-old joined a parent in the United States 4 months ago after living 7 years with a grandparent in the home country while the parent worked here. The school social worker learns the teen is withdrawn, cries about the grandparent and refuses to follow the parent's rules. The parent calls the teen ungrateful. Which approach is **BEST**?

**A.** Help the parent set firmer rules with clear consequences

**B.** Help both see the conflict as grief and strain from long separation

**C.** Refer the teen for evaluation of oppositional defiant disorder

**397.** At a refugee resettlement agency, a client who arrived 8 months ago is behind on the electric bill. While going over the household budget, the client mentions sending about a third of each paycheck to parents and siblings in a refugee camp, then stops and watches the social worker's reaction. What should the social worker do **FIRST**?

**A.** Ask the client about the money sent home and what it means

**B.** Advise pausing the transfers until the electric bill is current

**C.** Build a budget that treats the transfers as a fixed expense

**398.** A community counseling agency's intake form asks every new client for immigration status. No funder or program rule requires it, and staff say it is just for agency records. Since local enforcement actions made the news, several clients have left the question blank or ended intake early. What is the **BEST** recommendation for the social worker on the quality committee?

**A.** Keep the question but label it optional

**B.** Add a note on the form assuring clients of privacy

**C.** Take the question off the form

**399.** Which term describes a person who entered the United States on a student visa and is now applying for protection from persecution in the home country?

**A.** Refugee

**B.** Asylum seeker

**C.** Temporary protected status holder

**400.** At a school team meeting, a teacher proposes testing a 12-year-old refugee student for intellectual disability because the student reads at an early-elementary level in English. Records show about 3 years of schooling in a refugee camp, taught in Arabic. The student solves math puzzles quickly and teaches classmates complex games. What is the **BEST** recommendation for the school social worker to make?

**A.** Proceed with the school's standard intelligence tests in English

**B.** Delay any evaluation until the student is fluent in English

**C.** Place the student in a self-contained class while testing proceeds

**D.** Seek native-language testing that accounts for lost instruction

**401.** A 68-year-old refugee from a rural farming region has lived in the United States for 3 years. The client works part-time and enjoys time with grandchildren. The client mourns that the family can no longer hold seasonal ceremonies for ancestors at the village shrine and dreams that the ancestors are hungry and calling. The client says, 'I left them with no one to care for them.' Which concept **MOST** likely describes this distress?

**A.** Prolonged grief disorder

**B.** Posttraumatic stress disorder

**C.** Acculturative stress

**D.** Cultural bereavement

**402.** At a community health center, an undocumented parent says a neighbor was just detained by immigration agents. The parent's children, ages 6 and 9, are U.S. citizens. After spending most of the session on the arrest, the sleepless nights since and a call that morning to the family's immigration attorney, the parent asks, 'If they take me, who will pick up my kids?' What should the social worker do **NEXT**?

**A.** Help the parent make a written plan naming a caregiver

**B.** Invite the parent to say more about the fear of detention

**C.** Reassure the parent that parents of citizen children are rarely detained

# Answers and rationales

## I.A Ethical Principles and Responsibilities

### 1. ETH-0001 (rev 1) · Best answer: B

During a session at a community mental health agency, a client says they have saved a supply of pills and plan to take them tonight. The client then leaves the office and does not answer phone calls. The social worker calls emergency services. What information is **BEST** for the social worker to share with the dispatcher?

**A.** A general description of the risk without the client's name
**B.** The client's name, likely location and stated overdose plan
**C.** The client's diagnosis, treatment history and recent session notes

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). Leaving out the client's name protects privacy, but responders cannot find or help a person they cannot identify. When a threat to life is imminent, the Code permits disclosing identifying information to those who can prevent the harm.

**B** **Correct.** Confidentiality gives way when disclosure is needed to prevent serious, foreseeable and imminent harm, but only the information needed for that purpose may be shared (NASW Code 1.07(c)). Responders need to know who the client is, where to look and what the danger is, so B is the minimum disclosure that can still protect the client. It beats C because diagnosis, history and session content do not help responders find the client. Sharing them breaches confidentiality with no safety benefit. HIPAA likewise permits a good-faith disclosure to people able to prevent a serious and imminent threat (45 CFR 164.512(j)).

**C** **Incorrect** (trap: Misreads law or ethics). Diagnosis, treatment history and session notes go beyond what responders need to locate the client and respond. Sharing them breaks the minimum-necessary limit on safety disclosures, even though some disclosure is justified here.

**Takeaway:** In an emergency disclosure, share only what responders need to prevent the harm: who, where and what danger.

**Sources:** NASW Code of Ethics (2021), 1.07(c); HIPAA Privacy Rule, 45 CFR 164.512(j)(1)(i)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

### 2. ETH-0100 (rev 1) · Best answer: C

A social worker in private practice receives a subpoena from the attorney for a client's former partner in a custody case. It asks for the client's full treatment record. The client has not signed a release, and no judge has ordered the records. What should the social worker do **FIRST**?

**A.** Send the full record to the attorney by the stated deadline
**B.** Disregard the subpoena since it lacks a judge's signature
**C.** Contact the client to discuss the subpoena and their wishes
**D.** Send the attorney a letter asserting privilege for the client

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Releasing the whole file on an attorney's subpoena treats a request as if it were a court order. Without the client's authorization or a judge's order, this breaches confidentiality and privilege. Even with an order, only what is required is disclosed.

**B** **Incorrect** (trap: Misreads law or ethics). A subpoena is a legal document that needs a timely response, even without a judge's signature. Ignoring it can bring legal penalties. The right course is to respond through proper steps, neither disregarding it nor complying automatically.

**C** **Correct.** A subpoena from an attorney is not a court order, and the privilege belongs to the client, not the social worker. Records go out only with the client's written authorization or a court order (NASW Code 1.07(b), 1.07(j)). Contacting the client first tells them about the request and lets them decide whether to authorize release or have their own attorney object. It beats D because asserting privilege is the right move once the client declines to authorize release; the client, as holder, decides first. HIPAA likewise bars release on a bare subpoena without notice or a protective order (45 CFR 164.512(e)(1)(ii)).

**D** **Incorrect** (trap: Right action, wrong time). Asserting privilege on the client's behalf is appropriate when the client does not authorize release. Doing it before speaking with the client skips the holder's decision, since the client may choose to release the records or have counsel object.

**Takeaway:** A subpoena is not a court order: notify the client first, and release records only with written authorization or a judge's order.

**Sources:** NASW Code of Ethics (2021), 1.07(j); NASW Code of Ethics (2021), 1.07(b); HIPAA Privacy Rule, 45 CFR 164.512(e)(1)(ii)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

### 3. ETH-0119 (rev 2) · Best answer: A

A client who arrived in the United States as a refugee 4 months ago comes to a community agency for help with sleep problems. At intake, the client hesitates to sign forms and asks whether the information will go to immigration officials. What should the social worker do **FIRST**?

**A.** Explain what stays confidential and the limits that apply
**B.** Promise the client that their records will be kept private
**C.** Complete the trauma history while the client is present

**Why each option is right or wrong**

**A** **Correct.** Clients should learn the nature and limits of confidentiality early in the relationship (NASW Code 1.07(e)). A refugee's fear of authorities is a reasonable response to past experience, and clear, honest information builds the trust needed to engage. Explaining who can see the record, and the narrow exceptions such as safety and abuse reporting, answers the client's question directly. It beats B because a blanket promise of privacy is false and would break trust when a limit applies; trustworthiness and transparency are core trauma-informed principles (SAMHSA, 2014).

**B** **Incorrect** (trap: Misreads law or ethics). A guarantee of total privacy ignores real limits such as imminent danger, abuse reporting and court orders. A false promise may feel reassuring now but damages trust if a limit ever applies.

**C** **Incorrect** (trap: Right action, wrong time). A detailed trauma history is part of assessment, but pressing for it before the client's question is answered and trust is built can feel like interrogation. It belongs later, once consent and safety are established.

**Takeaway:** When a client fears how information will be used, explain confidentiality and its limits honestly before gathering sensitive history.

**Sources:** NASW Code of Ethics (2021), 1.07(e); SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), six key principles: trustworthiness and transparency
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 4. ETH-1000 (rev 1) · Best answer: B

A social worker at a program known in the community for treating substance use disorders gets a phone call from a client's employer. The caller asks whether the client has been attending sessions this month. The client has signed no release for the employer. How should the social worker respond?

**A.** Confirm attendance but decline to give clinical details
**B.** Neither confirm nor deny that the person is a client
**C.** Ask the employer to send the request in writing

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Withholding clinical details sounds careful, but confirming attendance already tells the employer that the person is in substance use treatment. That fact is itself protected. With a signed release naming the employer, sharing attendance would be appropriate.

**B** **Correct.** Under 42 CFR Part 2, even acknowledging that someone is a patient of a program known for substance use treatment is a protected disclosure. It is allowed only with the patient's written consent or a qualifying court order (42 CFR 2.13(c)). The Code also requires valid consent before any disclosure (NASW Code 1.07(b)). With no release on file, the social worker neither confirms nor denies. B beats A because confirming attendance reveals the very fact the rule protects: that the person is in substance use treatment.

**C** **Incorrect** (trap: Misuses the technique). A written request changes nothing. Without the client's written consent or a court order, the program still cannot disclose, and asking for paperwork hints that a record exists. The program may say only that federal rules restrict disclosure.

**Takeaway:** Without written consent, a substance use program neither confirms nor denies that a person is a patient.

**Sources:** 42 CFR Part 2, 42 CFR 2.13(c); NASW Code of Ethics (2021), 1.07(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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### 5. ETH-1001 (rev 1) · Best answer: A

Which term names a client's legal right to block a social worker's testimony about their sessions in court?

**A.** Privileged communication
**B.** Confidentiality
**C.** Privacy
**D.** Informed consent

#### Why each option is right or wrong

**A** **Correct.** Privileged communication is a legal rule of evidence that protects what a client says in treatment from compelled disclosure in legal proceedings. The privilege belongs to the client, who may assert or waive it, and federal courts recognize it for licensed clinical social workers (Jaffee v. Redmond, 1996). It beats B because confidentiality is the professional and ethical duty not to disclose, which by itself does not stop a court from compelling testimony (NASW Code 1.07(j)).

**B** **Incorrect** (trap: Wrong model or concept). Confidentiality is the social worker's ethical duty to keep client information private. It governs everyday disclosures, but a court can still compel testimony unless a legal privilege applies.

**C** **Incorrect** (trap: Wrong model or concept). Privacy is the client's broader right to control access to personal information, including what they share at all. It is a different idea from the rule of evidence that applies in court.

**D** **Incorrect** (trap: Wrong model or concept). Informed consent concerns agreeing to services after understanding their purpose, risks and alternatives. It does not decide whether session content can be compelled in court.

**Takeaway:** Privilege is the client's legal right to keep session content out of court; confidentiality is the worker's ethical duty.

**Sources:** Jaffee v. Redmond, 518 U.S. 1 (1996); NASW Code of Ethics (2021), 1.07(j)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 6. ETH-1002 (rev 1) · Best answer: D

A social worker in private practice receives a release, signed by a client, from the client's attorney. The client has sued another driver after a car crash and claims emotional distress. The release covers treatment records from the past year. Those records describe a past affair that the client may not want raised in court. What is the **MOST** appropriate response?

**A.** Assert privilege until a judge orders release of the records
**B.** Withhold the pages about the affair to protect the client
**C.** Send the records the release covers to the attorney today
**D.** Review the records' contents with the client, then release them

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Asserting privilege is right when no authorization exists. Here the holder of the privilege has waived it in writing for these records, so the social worker cannot assert it against the client's wishes.

**B** **Incorrect** (trap: Overrides the client's choice). Removing pages the client authorized overrides the client's decision and alters what is produced. If the client wants a narrower release after reviewing the content, the client can change it; the social worker cannot decide that alone.

**C** **Incorrect** (trap: Good but not best). The release is valid, so sending the records is lawful. It is weaker because the client may not realize the records include the affair, and the Code asks workers to explain disclosures and their consequences first when feasible.

**D** **Correct.** The client, not the social worker, holds privilege, and a valid signed release waives it for the records it covers (Jaffee v. Redmond, 1996; NASW Code 1.07(b)). The worker honors that choice but, when feasible, tells the client about the disclosure and its possible consequences beforehand (NASW Code 1.07(d)). Reviewing the contents lets the client decide with full knowledge, including whether to narrow the release. D beats C because sending at once is lawful but skips the step that lets the client weigh the sensitive content.

**Takeaway:** When a client waives privilege, honor the waiver, but first help the client understand what the released records contain.

**Sources:** Jaffee v. Redmond, 518 U.S. 1 (1996); NASW Code of Ethics (2021), 1.07(b); NASW Code of Ethics (2021), 1.07(d)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 7. ETH-1003 (rev 1) · Best answer: C

A social worker at a family services agency receives a court order signed by a judge in a child dependency case. It directs release of a parent's complete treatment record. The record includes the parent's detailed account of childhood sexual abuse, which has no bearing on the current case. What is the **BEST** response?

**A.** Send the complete record by the court's stated deadline
**B.** Decline to follow the order to protect the parent
**C.** Ask the court to limit the order or seal the record
**D.** Send a brief summary in place of the full record

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Complying is lawful, but sending everything without first asking the court for protection exposes sensitive history unrelated to the case. If the court declines to narrow the order, full compliance is then correct.

**B** **Incorrect** (trap: Misreads law or ethics). A judge's order is legally binding. Refusing to comply risks contempt and does little to protect the parent; the lawful path is to ask the court itself to limit or seal the disclosure.

**C** **Correct.** A court order, unlike an attorney's subpoena, carries the court's authority and must be followed. Still, the Code directs social workers to protect confidentiality as far as the law permits: when ordered disclosure could harm the client, the worker asks the court to withdraw or narrow the order or keep the records under seal (NASW Code 1.07(j)). HIPAA likewise limits disclosure to what the order expressly authorizes (45 CFR 164.512(e)(1)(i)). C beats A because full compliance is lawful but exposes unrelated trauma history without first seeking protection.

**D** **Incorrect** (trap: Misreads law or ethics). Substituting a summary changes what the court ordered without its permission. The court, not the social worker, decides what is produced; a summary is acceptable only if the court agrees to modify the order.

**Takeaway:** Obey a court order, but when disclosure could harm the client, ask the court to narrow the order or seal the records.

**Sources:** NASW Code of Ethics (2021), 1.07(j); HIPAA Privacy Rule, 45 CFR 164.512(e)(1)(i)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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### 8. ETH-1004 (rev 2) · Best answer: A

A social worker at a community mental health center receives a subpoena from an insurance company's attorney in a disability lawsuit brought by a client. The social worker calls the client, who says firmly that they do not want the records released. The response date is 10 days away. What should the social worker do **NEXT**?

**A.** Get legal advice on asserting the client's privilege
**B.** Send the records, since the subpoena is a legal demand
**C.** Discuss with the client why they oppose the release
**D.** Ask the client to contact the attorney who issued it

**Why each option is right or wrong**

**A** **Correct.** A subpoena must be answered, but it is not a court order and does not by itself authorize release. Once the client refuses consent, the worker asserts privilege on the client's behalf and gets legal advice on how to respond, such as objecting or seeking to quash, before the deadline (NASW Code 1.07(j); 45 CFR 164.512(e)(1)(ii)). A beats C because the client's decision is already clear; more discussion does not answer the subpoena, and the deadline keeps running.

**B** **Incorrect** (trap: Misreads law or ethics). An attorney's subpoena is a demand to respond, not a judge's order to disclose. Releasing records over the client's refusal, without a court order or the assurances HIPAA requires, breaches confidentiality.

**C** **Incorrect** (trap: Good but not best). Understanding the client's reasons can help the relationship, and it would be the right move if the client were unsure. Here the client has clearly refused, so the pressing task is a lawful response to the subpoena before the deadline.

**D** **Incorrect** (trap: Hands off too soon). The subpoena was served on the social worker, who must respond in time. Sending the client to the attorney who issued it hands off the worker's own duty, and that attorney represents the insurer, not the client.

**Takeaway:** When a client refuses to authorize release under subpoena, assert privilege and get legal advice before the response deadline.

**Sources:** NASW Code of Ethics (2021), 1.07(j); HIPAA Privacy Rule, 45 CFR 164.512(e)(1)(ii)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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## 9. ETH-1005 (rev 1) · Best answer: C

A client at an outpatient behavioral health clinic has signed the clinic's standard consent for sharing information with other treating providers. The client's new psychiatrist at another practice asks the social worker for the process notes that the social worker keeps separately from the clinic record. What should the social worker do?

**A.** Send the notes, since the request is for treatment
**B.** Give the psychiatrist a verbal summary of the notes
**C.** Get the client's specific written authorization for them

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). HIPAA lets providers share most of the record for treatment without authorization, which makes this tempting. Psychotherapy notes are the exception: they need a specific authorization, even when another treating provider asks.

**B** **Incorrect** (trap: Misuses the technique). Summarizing the notes aloud still discloses their content. Changing the format from paper to speech does not remove the need for the client's specific authorization.

**C** **Correct.** Notes that analyze session conversation and are kept apart from the medical record are psychotherapy notes under HIPAA. Unlike the rest of the record, they need the client's specific authorization even for disclosure to another treating provider, and a general consent for sharing records does not cover them (45 CFR 164.508(a)(2)). The

Code also requires valid consent for disclosure (NASW Code 1.07(b)). C beats A because the usual treatment exception applies to the clinic record, not to separately kept psychotherapy notes.

**Takeaway:** Separately kept psychotherapy notes need the client's specific authorization, even when another treating provider requests them.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.508(a)(2); NASW Code of Ethics (2021), 1.07(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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## 10. ETH-1006 (rev 2) · Best answer: B

A social worker in private practice bills a client's health plan with the client's written authorization. During a utilization review, the plan's reviewer asks for copies of all session notes to decide whether to approve more sessions. What is the **BEST** way for the social worker to respond?

**A.** Send copies of every session note the reviewer has requested
**B.** Send the treatment plan and progress the decision requires
**C.** Send the notes with identifying details removed

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Sending every note exceeds what the reviewer needs to approve sessions and exposes details the payment decision does not require. Full notes would be appropriate only if the client specifically authorized them for a stated reason.

**B** **Correct.** Disclosures to a payer require the client's authorization (NASW Code 1.07(h)), and even an authorized disclosure is limited to what its purpose requires (NASW Code 1.07(c)). HIPAA likewise applies a minimum-necessary standard to disclosures for payment (45 CFR 164.502(b)). The reviewer needs the diagnosis, treatment plan and progress toward goals, not session-by-session detail. B beats A because full notes go beyond what the approval decision requires.

**C** **Incorrect** (trap: Misuses the technique). Removing identifiers does not work for a coverage decision, because the plan must know whose care it is approving. It also still sends far more clinical detail than the decision needs. De-identification suits consultation or teaching, not billing.

**Takeaway:** Share with a payer just the information its decision needs, even when the client has authorized disclosure.

**Sources:** NASW Code of Ethics (2021), 1.07(h); NASW Code of Ethics (2021), 1.07(c); HIPAA Privacy Rule, 45 CFR 164.502(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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### 11. ETH-1007 (rev 1) · Best answer: A

At the first meeting of an intensive outpatient group for adults recovering from alcohol use disorder, a member asks the social worker, 'Can you promise that nobody here will repeat what I say?' What is the **BEST** response?

**A.** Explain the group's privacy agreement and the limits on enforcing it
**B.** Assure the member that group rules keep everything private
**C.** Suggest the member keep sensitive details out of group

**Why each option is right or wrong**

**A** **Correct.** In group work, the social worker seeks agreement among members to keep shared information confidential and tells them that the worker cannot guarantee every member will honor it (NASW Code 1.07(f)). The worker also explains the worker's own legal limits on confidentiality (NASW Code 1.07(e)). An honest answer lets each member decide what to share. A beats C because advising silence about sensitive topics undercuts the purpose of the group, while an honest explanation protects members and still invites participation.

**B** **Incorrect** (trap: Misreads law or ethics). This promise cannot be kept, because group members are not bound the way professionals are. A false guarantee misleads the member and damages trust if someone later repeats what was said.

**C** **Incorrect** (trap: Good but not best). Members may choose what to share, but steering them away from sensitive material weakens the group. Explaining the agreement and its limits lets each member make an informed choice instead.

**Takeaway:** In groups, ask members to agree to confidentiality, and say honestly that the social worker cannot guarantee others will comply.

**Sources:** NASW Code of Ethics (2021), 1.07(f); NASW Code of Ethics (2021), 1.07(e)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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### 12. ETH-1008 (rev 1) · Best answer: D

At the start of couples work by video, a social worker explained a policy that information shared privately by one partner may be raised in joint sessions. Before the fourth session, one partner emails the social worker about a recent affair and asks that it stay secret. What is the **BEST** response?

**A.** Keep the information private to protect the alliance
**B.** Disclose the affair to the other partner in the next session
**C.** End joint work and refer each partner for individual help
**D.** Remind the partner of the agreed rule and urge disclosure

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Keeping the secret breaks the policy both partners accepted and makes the social worker an ally of one partner. It would be appropriate only if a different policy, such as keeping individual disclosures private, had been agreed at the start.

**B** **Incorrect** (trap: Right action, wrong time). The policy allows the information to come out, but announcing it without warning skips the step of letting the partner disclose it. The worker should first remind the partner of the policy and encourage self-disclosure.

**C** **Incorrect** (trap: Hands off too soon). Ending couples work hands off a situation the policy was designed to handle. Referral might be considered later if the couple's goals change, but the agreed policy already gives a path forward.

**D** **Correct.** When work involves more than one person, the social worker tells clients at the outset how information shared among participants will be handled (NASW Code 1.07(g)). Having set a policy that private disclosures may be raised jointly, the worker applies it. The fair first move is to remind the partner of the policy and encourage them to disclose in session. D beats B because disclosure by the worker removes the partner's chance to share it themselves; the worker raises it only if the partner declines.

**Takeaway:** Apply the information-sharing policy set at the start of couples or family work, giving the disclosing person a chance to share first.

**Sources:** NASW Code of Ethics (2021), 1.07(g)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 13. ETH-1009 (rev 1) · Best answer: C

A hospice social worker worked with a patient who died last month. The patient's adult child calls and asks for copies of the social worker's records, saying the family wants to understand the patient's last weeks. What should the social worker do **FIRST**?

**A.** Send the records, since the caller is the patient's next of kin
**B.** Explain that records stay sealed after a patient's death
**C.** Ask for documents showing legal authority for the estate

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Releasing the records may be the right step later, if the caller shows legal authority such as appointment as executor. Being next of kin does not by itself confer that authority, so verification comes first.

**B** **Incorrect** (trap: Misreads law or ethics). Confidentiality survives death, but records are not permanently closed. A legally authorized personal representative can access them, so turning the family away misstates the rule.

**C** **Correct.** Confidentiality continues after a client dies (NASW Code 1.07(w)). Under HIPAA, a person with legal authority to act for the deceased or the estate, such as an executor or administrator, is treated as the personal representative (45 CFR 164.502(g)(4)). The social worker first asks the caller for documents showing that authority. C beats A because being next of kin does not by itself give legal authority over the records; release can follow once authority is shown.

**Takeaway:** Confidentiality survives death; release a deceased client's records to a person with legal authority, such as the estate's representative.

**Sources:** NASW Code of Ethics (2021), 1.07(w); HIPAA Privacy Rule, 45 CFR 164.502(g)(4)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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### 14. ETH-1010 (rev 2) · Best answer: B

A child welfare social worker returns to their car after a home visit and finds that the agency laptop has been stolen. The laptop holds case files on several families and lacks encryption. What should the social worker do **FIRST**?

**A.** List which families' files were stored on the laptop
**B.** Report the theft to the privacy officer
**C.** Call each affected family to warn them of the breach
**D.** File a police report and wait to see if it is recovered

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Identifying the affected files matters, and the agency will need that list. It should not delay the report, though: agency records can show which files were stored, and the privacy officer can start that review at once.

**B** **Correct.** A possible breach of client records is reported at once through the agency's breach procedures, so the agency can assess the risk and notify affected people as the law requires (NASW Code 1.07(o); 45 CFR 164.404). The privacy officer coordinates notification, police contact and mitigation. B beats A because listing the files is useful, but the agency can do it from its own records, and waiting slows a response that is time-limited by law.

**C** **Incorrect** (trap: Right action, wrong time). Affected families may need to be told, but notification follows the agency's breach assessment and legal requirements. Calling families personally before the agency acts can give them inconsistent or incomplete information.

**D** **Incorrect** (trap: Ignores risk). A police report is reasonable, and the agency may make one, but waiting for recovery delays the breach response. Unencrypted files may already be exposed, so the agency must be told right away.

**Takeaway:** Report a suspected breach of client records to the agency's privacy officer at once; the agency assesses and notifies affected clients.

**Sources:** NASW Code of Ethics (2021), 1.07(o); HIPAA Breach Notification Rule, 45 CFR 164.404
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 15. ETH-1011 (rev 2) · Best answer: A

A social worker at an opioid treatment program gets a call from an emergency department physician. A program client was brought in unresponsive, and the physician asks for the client's current medications to guide treatment. The client has signed no consent for the hospital. What is the **BEST** response?

**A.** Provide the information needed for emergency care
**B.** Decline until the client can sign a written consent
**C.** Confirm that the person is enrolled, but share nothing more
**D.** Ask the client's emergency contact to give consent

**Why each option is right or wrong**

**A** **Correct.** 42 CFR Part 2 lets a program disclose patient information without consent to medical personnel as needed to meet a bona fide medical emergency in which the patient cannot consent (42 CFR 2.51). An unresponsive client in an emergency department meets that standard, and the physician needs current medications to treat safely. The program then documents the disclosure. A beats B because waiting for consent from someone who cannot give it risks the client's life. The Code also permits disclosure to prevent serious, imminent harm (NASW Code 1.07(c)).

**B** **Incorrect** (trap: Misreads law or ethics). Part 2 usually requires written consent, which makes this tempting. The medical emergency exception exists for exactly this case: the client cannot consent, and treatment depends on the information.

**C** **Incorrect** (trap: Good but not best). Confirming enrollment discloses protected information without helping the physician treat. When disclosure is justified, it should include what the emergency requires, such as current medications.

**D** **Incorrect** (trap: Ignores risk). An emergency contact usually has no legal authority to consent for an adult, and searching for one delays care. The emergency exception allows direct disclosure to the treating physician.

**Takeaway:** In a bona fide medical emergency, a substance use program may share needed information with treating medical staff without consent, then document it.

**Sources:** 42 CFR Part 2, 42 CFR 2.51; NASW Code of Ethics (2021), 1.07(c)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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## 16. ETH-1012 (rev 2) · Best answer: B

A social worker meets weekly with a 15-year-old client in outpatient therapy. At intake, the parent agreed that session details would stay private unless a safety concern arose. A month later the parent calls, worried, and asks what the teen has been talking about. What is the **MOST** appropriate response?

**A.** Tell the parent nothing can be shared without the teen's consent
**B.** Give general progress and revisit the confidentiality agreement
**C.** Suggest that the parent ask the teen directly

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Protecting the teen's privacy is right, which makes this tempting. The intake agreement kept session details private, not all information, so a flat refusal shuts out a worried parent, overstates the agreement and may weaken the parent's support for treatment.

**B** **Correct.** When a parent agrees to a confidential relationship between a provider and a minor, HIPAA does not treat the parent as the minor's personal representative for that care (45 CFR 164.502(g)(3)(i)(C)). The Code calls for discussing the limits of confidentiality early and as needed (NASW Code 1.07(e)). Sharing general progress and revisiting the agreement respects the parent's concern while protecting the teen's trust. B beats C because redirecting the parent is reasonable but leaves the worry unanswered and the agreement unexplained.

**C** **Incorrect** (trap: Good but not best). Encouraging parent-teen communication is useful, but by itself it sidesteps the parent's worry. The social worker can offer general progress and restate the agreement, and may also suggest a family conversation.

**Takeaway:** Honor a confidentiality agreement made with a minor and parent at intake; offer general progress and restate its limits.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.502(g)(3)(i)(C); NASW Code of Ethics (2021), 1.07(e)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 17. ETH-1013 (rev 1) · Best answer: C

Records kept by a social worker employed by a public school district are generally governed by which federal privacy law?

**A.** The HIPAA Privacy Rule
**B.** 42 CFR Part 2
**C.** FERPA

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). HIPAA governs most health care providers, which makes it the usual answer. Records that a public school keeps as education records are excluded from HIPAA's protected health information, so FERPA applies instead.

**B** **Incorrect** (trap: Wrong model or concept). 42 CFR Part 2 protects records of federally assisted substance use disorder programs. It does not govern a school social worker's general student records.

**C** **Correct.** Health records kept by school employees, or by providers acting for a public school, are generally education records under FERPA, the Family Educational Rights and Privacy Act. Most elementary and secondary schools are not HIPAA covered entities, and education records are excluded from HIPAA's definition of protected health information (Joint Guidance, HHS and U.S. Department of Education, 2019). C beats A because HIPAA governs most health care settings, but not education records kept by a school.

**Takeaway:** A public school social worker's records are usually FERPA education records, not HIPAA protected health information.

**Sources:** U.S. Department of Health and Human Services & U.S. Department of Education (2019). Joint Guidance on the Application of FERPA and HIPAA to Student Health Records, Education records and the HIPAA definition of PHI; Family Educational Rights and Privacy Act (FERPA), 20 U.S.C. 1232g; 34 CFR Part 99
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 18. ETH-1014 (rev 3) · Best answer: A

A state law limits disclosure of client information more than HIPAA does. Which rule generally applies?

**A.** The stricter state law
**B.** HIPAA, since federal law takes priority
**C.** Whichever law the state licensing board adopts

**Why each option is right or wrong**

**A** **Correct.** HIPAA sets a federal floor for privacy. A state law that is more stringent, meaning it gives individuals greater privacy protection or greater access to their own records, is not preempted and governs (45 CFR 160.203(b)). Social workers therefore follow whichever rule protects the client more. A beats B because federal law usually controls conflicts, but HIPAA itself expressly leaves stricter state privacy laws in place.

**B** **Incorrect** (trap: Misreads law or ethics). Federal law often overrides state law, which makes this tempting. HIPAA is different: it preempts state laws that are less protective and leaves more stringent ones in force.

**C** **Incorrect** (trap: Misreads law or ethics). Licensing boards enforce practice rules, but they do not decide whether HIPAA preempts a state law. HIPAA's own preemption rule settles it: a more stringent state privacy law stays in force and applies.

**Takeaway:** HIPAA is a floor: a more stringent state privacy law governs.

**Sources:** HIPAA Privacy Rule, 45 CFR 160.203(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.1 Confidentiality law and ethics, its limits, HIPAA/PHI privacy · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 19. ETH-1015 (rev 3) · Best answer: B

A social worker in a rural county wants advice from an out-of-town peer consultation group about a client with complex grief. The client is the town's only veterinarian, and the case involves a well-known local accident. How can the social worker **MOST** effectively protect the client's identity while consulting?

**A.** Refer to the client by initials during the consultation
**B.** Omit or change details that could reveal who the client is
**C.** Wait to consult until the client agrees to the consultation
**D.** Share full details, since consultants also owe confidentiality

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Initials are a common shortcut, but they hide just the name. A unique occupation and a widely reported local accident can still identify the client, so the identifying details themselves must be removed.

**B** **Correct.** When consulting, social workers share the least information needed and avoid revealing a client's identity unless the client consents or there is a compelling need (NASW Code 1.07(v); NASW Code 2.05(c)). In a small community, a job and a well-known event can identify someone even without a name, so the worker removes or alters such details. B beats A because initials hide the name but leave the unique job and local accident that could identify the client.

**C** **Incorrect** (trap: Good but not best). Consent would allow sharing identifying details, but it is unnecessary for de-identified consultation and delays help the case may need now. It also does nothing to protect identity, which is the stated goal.

**D** **Incorrect** (trap: Misreads law or ethics). Consultants do owe confidentiality, but the Code still limits what is shared to what the consultation needs. Full details expose identifying facts that the consultation does not require.

**Takeaway:** In consultation, remove every detail that could identify the client, not just the name, especially in small communities.

**Sources:** NASW Code of Ethics (2021), 1.07(v); NASW Code of Ethics (2021), 2.05(c)
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### 20. ETH-1016 (rev 3) · Best answer: C

During a video session, a client joins from a shared break room at work, and coworkers can be heard nearby. When the social worker asks whether the client can talk freely, the client says, 'It's fine, nobody pays attention.' The client then begins to describe a recent panic attack. What is the **MOST** helpful response?

**A.** Continue the session, since the client accepts the risk
**B.** End the session and require a private space next time
**C.** Explain the risk of being overheard and offer options

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Respecting the client's choice matters, but the choice is not yet informed: coworkers nearby could overhear a sensitive disclosure. Continuing would fit once the client understands the risk and still chooses to go on.

**B** **Incorrect** (trap: Overrides the client's choice). Ending the session and setting a rule overrides the client and withholds help in the moment. Offering options lets the client protect their privacy while keeping control of the decision.

**C** **Correct.** Confidential matters should be discussed only where privacy can be ensured, electronically or in person (NASW Code 1.07(i)), and remote services call for checking that the client understands their risks and limits (NASW Code 1.03(g)). A quick reassurance does not show that the client understands the risk, so the worker explains how the client could be overheard and offers choices, such as moving, using text chat or rescheduling, while leaving the decision with the client. C beats A because continuing honors the client's wish without making sure the choice is informed.

**Takeaway:** In remote sessions, make sure the client understands privacy risks in their setting and offer options before sensitive work continues.

**Sources:** NASW Code of Ethics (2021), 1.07(i); NASW Code of Ethics (2021), 1.03(g)
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### 21. ETH-1017 (rev 1) · Best answer: D

A client at a community counseling center signed a release last month allowing the social worker to send monthly progress updates to the client's employee assistance program. Today the client hands the social worker a written statement revoking that release. What should the social worker do **FIRST**?

**A.** Ask the client to explain the reasons for revoking
**B.** Send one final update to close out the arrangement
**C.** Ask the program to return the updates already sent
**D.** Stop further updates and note the revocation in the record

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Talking about the change may matter clinically, and the worker can invite that discussion afterward. Making the revocation wait on an explanation treats a right as something the client must justify.

**B** **Incorrect** (trap: Misreads law or ethics). Once revoked, the release no longer authorizes disclosures. Sending another update, even to close out the arrangement, would be an unauthorized disclosure.

**C** **Incorrect** (trap: Good but not best). A revocation applies to future disclosures. Updates sent while the release was valid were lawful and cannot be recalled; the task is to stop new disclosures.

**D** **Correct.** Clients may revoke an authorization at any time in writing. The revocation applies to future disclosures but cannot undo actions already taken in reliance on it (45 CFR 164.508(b)(5)). Disclosure requires valid, current consent (NASW Code 1.07(b)), so the worker stops sending updates and documents the revocation. D beats A because asking about the reasons may help later, but the client's right to revoke does not depend on giving reasons, and disclosures must stop now.

**Takeaway:** A written revocation of a release takes effect at once for future disclosures; stop sharing and document it.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.508(b)(5); NASW Code of Ethics (2021), 1.07(b)
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## 22. ETH-1018 (rev 2) · Best answer: A

An adult patient on an inpatient psychiatric unit has declined to let staff share any information with family. The patient's parent calls the social worker and wants to describe changes in the patient's behavior in the weeks before admission. What is the **BEST** response?

**A.** Hear the parent's account but disclose no patient details
**B.** Decline the call, since the patient has refused family contact
**C.** Give a brief update so the parent feels less worried

**Why each option is right or wrong**

**A** **Correct.** Confidentiality limits what the social worker discloses, not what the worker may hear. Because the patient declined disclosure to family, the worker shares nothing about the patient's care (NASW Code 1.07(b); 45 CFR 164.510(b)). The worker may still listen to what the parent offers, which can matter for safety and treatment; HHS guidance on HIPAA and mental health says providers may listen to family members. A beats B because refusing the call protects nothing and loses information the team may need.

**B** **Incorrect** (trap: Good but not best). The patient's refusal limits what staff disclose, not what they receive. Ending the call discards information that may bear on safety and treatment, without adding any protection for the patient.

**C** **Incorrect** (trap: Misreads law or ethics). The patient declined all sharing with family, so even a brief update would disclose information without consent. An update would be appropriate if the patient agreed or an emergency required it.

**Takeaway:** A client's refusal to allow disclosure limits what the social worker shares, not what the worker may hear from family.

**Sources:** NASW Code of Ethics (2021), 1.07(b); HIPAA Privacy Rule, 45 CFR 164.510(b); U.S. Department of Health and Human Services. HIPAA Privacy Rule and Sharing Information Related to Mental Health, Guidance on listening to family members

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### 23. ETH-1019 (rev 3) · Best answer: B

A client who has finished treatment at a community agency asks the social worker for a full copy of their record. The social worker worries the client will be upset by the frank descriptions of the client's anger outbursts in the progress notes. What is the **BEST** response?

**A.** Release the copy after a meeting to review it together

**B.** Provide the copy and offer to go over it together

**C.** Give a written summary instead of the requested copy

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Reviewing the record together can help, so this is tempting. Requiring a meeting before release, though, puts a condition on a right of access the client already holds; the client decides whether to go over the record with the worker (NASW Code 1.08(a); 45 CFR 164.524).

**B** **Correct.** Clients have a right to reasonable access to their records, and the social worker helps them understand what the records say (NASW Code 1.08(a)). Access may be limited only in exceptional cases with compelling evidence of serious harm. Likely upset does not meet that bar, and HIPAA gives a similar right of access (45 CFR 164.524). Offering to review the record together supports the client in reading frank notes. B beats C because a summary falls short of the full copy the client asked for and is entitled to.

**C** **Incorrect** (trap: Good but not best). A summary can be offered as an extra if the client wants one, but the client asked for the full record and has a right to it. Substituting a summary limits access without a valid reason.

**Takeaway:** Give clients access to their records and help them understand the content; limit access just for compelling evidence of serious harm.

**Sources:** NASW Code of Ethics (2021), 1.08(a); HIPAA Privacy Rule, 45 CFR 164.524

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### 24. ETH-1125 (rev 2) · Best answer: B

An outpatient social worker has seen an adult client for depression for 2 months. The client's primary care physician calls before a routine visit next week and asks for a summary of therapy progress. The client has not signed a release for this physician and has a session with the social worker in 2 days. What is the **BEST** response to the physician?

**A.** Share a brief progress summary, since this is for treatment
**B.** Say that consent is needed before sharing, then follow up
**C.** Confirm the client is in treatment but share no clinical details
**D.** Ask the physician to send a written records request

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). HIPAA does permit treatment disclosures without authorization, and coordinating with the physician has real value. But the Code expects consent when it can readily be obtained, and nothing here is urgent. Sharing now would fit an emergency or a client who had already agreed to coordination.

**B** **Correct.** Confidential information is shared with valid consent unless there is a compelling professional reason (NASW Code 1.07(b), 1.07(c)). HIPAA permits a provider to disclose information for another provider's treatment (45 CFR 164.506(c)(2)), but a permission is not a requirement, and the Code sets the higher bar. Nothing here is urgent: the visit is next week and the client will be seen in 2 days, so asking first costs little and respects the client's control. B beats A because A treats a HIPAA permission as if it settled the ethical question when the client can easily be asked.

**C** **Incorrect** (trap: Misreads law or ethics). Confirming that the person is in treatment is itself a disclosure, made here without the client's agreement. A neutral reply that sharing needs consent avoids confirming anything, and the client can be asked in 2 days.

**D** **Incorrect** (trap: Good but not best). A written request adds paperwork but does not solve the core problem: the client has not agreed. Whatever form the request takes, the social worker still needs the client's consent before releasing a summary.

**Takeaway:** HIPAA permission to share for treatment is not a duty to share; when time allows, get the client's consent first.

**Sources:** NASW Code of Ethics (2021), 1.07(b); NASW Code of Ethics (2021), 1.07(c); HIPAA Privacy Rule, 45 CFR 164.506(c)(2)
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### 25. ETH-1126 (rev 1) · Best answer: C

A community mental health center's director wants to feature a current client's recovery story, with a photo and first name, in the agency's annual fundraising report. In a session, the client says they would be proud to take part and that telling their story supports their recovery. What is the **BEST** way for the social worker to proceed?

**A.** Decline, since featuring current clients in publicity is improper
**B.** Include the story and note the client's agreement in the chart
**C.** Obtain written authorization after explaining that saying no is fine

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Featuring a current client does carry a risk of pressure, so caution is right. But a competent adult may choose to share their own story. Refusing outright decides for the client instead of making sure the choice is free and informed.

**B** **Incorrect** (trap: Misreads law or ethics). A verbal yes noted in the chart is not a valid authorization to publish a photo and name. A written authorization should state what will be shared, where and for how long, and the client may revoke it.

**C** **Correct.** Client information is disclosed only with valid consent (NASW Code 1.07(b)), and HIPAA requires a written authorization for a use like publicity (45 CFR 164.508(a)(1)). Because the request comes from the agency serving the client, the social worker makes sure the client knows that declining will not affect services, so the agency does not take unfair advantage of the relationship (1.06(b)). C beats A because the client is a competent adult who wants to share the story; a blanket refusal overrides that choice when a free, informed authorization can protect the client.

**Takeaway:** A client may share their story publicly, but only through a free, informed, written authorization naming what will be used.

**Sources:** NASW Code of Ethics (2021), 1.07(b); NASW Code of Ethics (2021), 1.06(b); HIPAA Privacy Rule, 45 CFR 164.508(a)(1)

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## 26. ETH-1127 (rev 2) · Best answer: A

A client of a community mental health agency died in an incident covered widely in the news. A local story claims the agency had refused the client services. A reporter calls the social worker who served the client and asks for comment, and the agency director wants the record set straight. How should the social worker respond to the reporter?

**A.** Decline to discuss whether or how the person received care

**B.** Share the dates of service to correct the false claim

**C.** Decline to give details but say the story's claim is untrue

**Why each option is right or wrong**

**A** **Correct.** Confidentiality continues after a client dies (NASW Code 1.07(w)), and social workers protect it when the media ask questions (1.07(k)). A false story that harms the agency does not create a compelling reason to disclose; the agency can speak generally about its intake policies without referring to any person. A beats B because dates of service, though limited, confirm that the person was a client and reveal details of care, which is exactly what the Code protects.

**B** **Incorrect** (trap: Misreads law or ethics). Wanting to correct a false claim is understandable, especially with the agency's reputation at stake. But dates of service confirm that the person was a client. Defending the agency is not a compelling reason to disclose.

**C** **Incorrect** (trap: Misreads law or ethics). Withholding details feels careful, but calling the claim untrue tells the reporter the agency knows this person's service history,

which confirms a relationship. Any agency response must stay general, about intake policy, without reference to the individual.

**Takeaway:** Confidentiality survives death and covers media requests; agency or worker reputation is not a compelling reason to disclose.

**Sources:** NASW Code of Ethics (2021), 1.07(w); NASW Code of Ethics (2021), 1.07(k)
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## 27. ETH-1128 (rev 1) · Best answer: D

A client with PTSD asks the social worker to support a request to the local housing authority for a ground-floor unit. The client signs a release. The housing authority's verification form asks for the client's diagnosis, medications and full treatment history. What is the **MOST** appropriate way to complete the request?

**A.** Complete every item on the form, since the client signed a release
**B.** Decline, since housing verification falls outside the treatment role
**C.** Ask the client's physician to supply the diagnosis instead
**D.** Verify the disability and the need without listing the diagnosis

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). A release makes disclosure lawful, but it does not make every requested detail necessary. The form asks for more than the housing authority needs, and listing the diagnosis and history exposes the client without helping the decision.

**B** **Incorrect** (trap: Good but not best). Verification letters are a common way to support a client's access to housing. The social worker knows the client's needs and is a reliable third party, so declining leaves the client without help the worker can properly give.

**C** **Incorrect** (trap: Hands off too soon). The social worker is already well placed to verify the need, so sending the client elsewhere delays the request. A physician would face the same question of how much to disclose.

**D** **Correct.** In all instances, social workers disclose the least information needed for the purpose (NASW Code 1.07(c)). HUD guidance says housing authorities may request only what is needed to evaluate a disability-related need and must not ask about the nature or severity of a disability (HUD, 2025). A letter confirming that the client has a disability and needs a ground-floor unit meets the purpose. D beats A because a signed release permits disclosure but does not call for sharing more than the request requires.

**Takeaway:** A release permits disclosure but does not require sharing more than the purpose needs; verify a need without unnecessary diagnostic detail.

**Sources:** NASW Code of Ethics (2021), 1.07(c); HUD Housing Choice Voucher Program Guidebook (2025), Fair Housing and Nondiscrimination Requirements: verifying disability-related need
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### 28. ETH-1129 (rev 1) · Best answer: B

A client attends court-ordered outpatient therapy after an assault conviction. The client signed a release allowing the social worker to report attendance and participation to the probation officer. The officer calls and asks what the client has said in sessions about a new dating relationship. What is the **MOST** appropriate response?

**A.** Describe what the client said, since the court ordered treatment
**B.** Report attendance and participation, and decline to share more
**C.** Decline to report attendance or participation until a subpoena arrives

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). A court order to attend treatment is not consent to reveal what is said in sessions. The client authorized attendance and participation reports only, and this request goes well past that.

**B** **Correct.** A release defines what may be shared, and disclosure stays within that scope and the least amount needed (NASW Code 1.07(b), 1.07(c)). This client authorized reports of attendance and participation only; what the client said about a relationship falls outside it. The officer can seek more through the client's consent or the court. B beats C because the client did authorize some disclosure, and refusing all of it could hurt the client's standing with probation for no reason.

**C** **Incorrect** (trap: Good but not best). The release is valid and covers attendance and participation, so the officer is entitled to that much. Holding it back until a subpoena arrives withholds what the client agreed to share and may count against the client.

**Takeaway:** Share what the release covers and no more; a court-ordered referral does not widen the client's consent.

**Sources:** NASW Code of Ethics (2021), 1.07(b); NASW Code of Ethics (2021), 1.07(c)
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### 29. ETH-1130 (rev 1) · Best answer: A

At a clinic intake, a client explains that their partner monitors the home phone and mail and becomes violent over medical bills. The client asks the clinic to contact them only by work email. The front desk says its system sends automatic reminder calls and letters to the home. What should the social worker do **FIRST**?

**A.** Have the client's contact method changed before any reminder goes out
**B.** Develop a safety plan for what to do if the partner finds a letter
**C.** Explain the reminder system and ask the client to accept home contact

**Why each option is right or wrong**

**A** **Correct.** HIPAA requires health care providers to accommodate reasonable requests to receive communications by other means or at other locations, and they may not require an explanation (45 CFR 164.522(b)). A reminder call or letter could reach the partner and put the client in danger, so the contact change must come before anything else. A beats B because safety planning is valuable but comes later; preventing the risky contact in the first place protects the client now.

**B** **Incorrect** (trap: Right action, wrong time). Safety planning belongs in the work with this client and should follow soon. Doing it first, while reminders are still set to go to the home, plans around a danger the clinic can simply remove.

**C** **Incorrect** (trap: Ignores risk). A clinic's system settings do not override the client's right to a reasonable request for confidential contact. Asking the client to accept home reminders could expose them to violence.

**Takeaway:** Providers must honor reasonable requests for confidential contact methods; change the settings before any communication goes out.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.522(b)
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### 30. ETH-1131 (rev 1) · Best answer: C

Under HIPAA, what type of permitted disclosure is it when a practice sends a former client's outstanding bill to a collection agency?

**A.** Health care operations
**B.** Treatment
**C.** Payment
**D.** Required by law

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Health care operations covers activities such as quality review, auditing, training and business management. Collection activities are named in the payment definition, so this disclosure is classified as payment.

**B** **Incorrect** (trap: Wrong model or concept). Treatment means providing, coordinating or managing care among providers. Sending a balance to a collection agency involves no care, so it is not a treatment disclosure.

**C** **Correct.** HIPAA's definition of payment includes billing and collection activities (45 CFR 164.501), and a covered entity may disclose information for its own payment without authorization (164.506(c)(1)). The minimum necessary rule still applies, so only identifying, contact and balance details go to the agency, never clinical content (164.502(b); NASW Code 1.07(c)). C beats A because, although health care operations includes general business management, HIPAA names collection activities in the payment definition.

**D** **Incorrect** (trap: Wrong model or concept). Required-by-law disclosures are those a statute or court compels, such as certain abuse reports. No law requires sending a debt to collections; the practice chooses to do it under the payment provision.

**Takeaway:** HIPAA classes collection activity as payment; disclose only identifying, contact and balance details, never clinical information.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.501 (definition: payment); HIPAA Privacy Rule, 45 CFR 164.506(c)(1); HIPAA Privacy Rule, 45 CFR 164.502(b); NASW Code of Ethics (2021), 1.07(c)
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### 31. ETH-1132 (rev 1) · Best answer: A

Under 42 CFR Part 2, when state law lets a teen consent alone to substance use treatment, whose written consent is needed to share a progress update with the teen's parent?

**A.** The teen's consent alone
**B.** The parent's consent alone
**C.** Both teen and parent consent
**D.** The program director's approval

**Why each option is right or wrong**

**A** **Correct.** Under 42 CFR Part 2, when state law lets a minor obtain substance use treatment without a parent, only the minor may give written consent to disclose, including to a parent (42 CFR 2.14(a)). Here the teen may consent to treatment alone, so the update needs the teen's consent. A beats C because joint consent from teen and parent is the rule only where state law requires a parent's consent to treatment (2.14(b)), which is not the case here.

**B** **Incorrect** (trap: Misreads law or ethics). A parent's consent controls disclosure only when the parent had authority over the treatment decision. Here state law lets the teen consent alone, so the parent cannot authorize this disclosure.

**C** **Incorrect** (trap: Misreads law or ethics). Joint consent applies where state law requires a parent's consent to treatment (2.14(b)). This state lets the teen consent alone, so the teen alone controls disclosure.

**D** **Incorrect** (trap: Misreads law or ethics). A program director's judgment matters only in a narrow exception: a minor who lacks capacity and faces a substantial threat to life or physical well-being (2.14(c)). That exception is not the issue in a routine progress request.

**Takeaway:** Under Part 2, when state law lets a minor consent to SUD treatment alone, only the minor can authorize disclosure, even to a parent.

**Sources:** 42 CFR Part 2, 42 CFR 2.14(a); 42 CFR Part 2, 42 CFR 2.14(b); 42 CFR Part 2, 42 CFR 2.14(c)
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### 32. ETH-1133 (rev 2) · Best answer: B

A hospital social work department receives a disability insurer's request for a former patient's psychosocial assessment. The attached release is signed and dated by the patient, but the lines for the purpose of the disclosure and the expiration date are blank. What should the social worker do **FIRST**?

**A.** Send the assessment, since the patient signed the release
**B.** Contact the patient to complete a valid authorization
**C.** Send a brief summary instead of the full assessment
**D.** Tell the insurer that the records require a subpoena

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Sending the assessment becomes appropriate once the patient completes a valid authorization. With the purpose and expiration blank, the form is defective, so releasing records now is not yet authorized.

**B** **Correct.** HIPAA treats an authorization as defective when a required core element, such as the purpose or an expiration date or event, is left blank (45 CFR 164.508(b)(2), 164.508(c)(1)). A defective form is not valid consent (NASW Code 1.07(b)). Contacting the patient lets them decide what to release, to whom and for how long. B beats A because a signature alone does not make an incomplete form valid; sending now would be an unauthorized disclosure.

**C** **Incorrect** (trap: Misuses the technique). Sending less is the right instinct about minimum disclosure, but it does not cure the problem. Any disclosure, even a summary, needs a valid authorization, and this one is missing required elements.

**D** **Incorrect** (trap: Misreads law or ethics). Insisting on a subpoena misreads the rules: a valid authorization from the patient is enough for this disclosure. Telling the insurer otherwise delays a claim the patient likely wants processed.

**Takeaway:** A signed release missing required elements is not valid; have the client complete it before disclosing anything.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.508(b)(2); HIPAA Privacy Rule, 45 CFR 164.508(c)(1); NASW Code of Ethics (2021), 1.07(b)

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### 33. ETH-1134 (rev 1) · Best answer: C

An adult client asks a community agency for a copy of their record. During treatment, the client's sibling met once with the social worker to share family history, after the worker agreed to keep the sibling's comments private. Those comments appear in the record and would identify the sibling. What is the **BEST** response to the client's request?

**A.** Release the full record, including the sibling's comments

**B.** Offer a verbal summary of the record instead of copies

**C.** Release the record with the sibling's comments withheld

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). The client's right of access is strong, and most of the record should be released. But information a non-provider gave under a promise of confidentiality may be withheld when it would reveal the source, and the Code asks workers to protect others named in records.

**B** **Incorrect** (trap: Good but not best). A summary falls short of the access the client is entitled to. Only the sibling's comments may be withheld; the rest of the record must be provided as requested.

**C** **Correct.** Clients have a strong right to see their records (NASW Code 1.08(a)), and social workers protect the confidentiality of others named in them (1.08(c)). HIPAA allows denial of information obtained from a non-provider under a promise of confidentiality when access would reveal the source, but requires access to the rest of the record (45 CFR 164.524(a)(2)(v), 164.524(d)(1)). C honors both duties. C beats A because releasing everything breaks the promise made to the sibling, which the rules let the worker keep.

**Takeaway:** Give clients their records, withholding only third-party information shared under a promise of confidentiality that would reveal its source.

**Sources:** NASW Code of Ethics (2021), 1.08(a); NASW Code of Ethics (2021), 1.08(c); HIPAA Privacy Rule, 45 CFR 164.524(a)(2)(v); HIPAA Privacy Rule, 45 CFR 164.524(d)(1)
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## 34. ETH-0103 (rev 1) · Best answer: A

A hospital social worker notices that a colleague on the team has smelled of alcohol at 2 morning meetings and missed a patient appointment last week. The colleague is off today, and no patient has been harmed. What should the social worker do **FIRST**?

**A.** Talk privately with the colleague about the observed concerns
**B.** File a complaint about the colleague with the licensing board
**C.** Keep watching the colleague until the pattern is clearer

#### Why each option is right or wrong

**A** **Correct.** When a social worker has direct knowledge that a colleague's impairment is affecting practice, the Code's first step is to consult the colleague, when feasible, and help them take remedial action (NASW Code 2.08(a)). Here the social worker observed the signs personally, and no patient faces immediate danger, so a private conversation is feasible. It beats B because formal reporting comes later, if the colleague does not take adequate steps (NASW Code 2.08(b)). If a colleague arrived intoxicated to see patients, protecting patients through the supervisor would come first.

**B** **Incorrect** (trap: Right action, wrong time). A board complaint is an appropriate channel if the colleague does not address the problem after being approached. Going there first skips the direct conversation the Code calls for when no one faces immediate danger.

**C** **Incorrect** (trap: Ignores risk). The signs are already firsthand and affecting work, since an appointment was missed. Waiting for more evidence lets the problem grow and puts patients at risk. Observation without action does not meet the duty to colleagues and patients.

**Takeaway:** With direct knowledge of colleague impairment and no imminent danger, talk with the colleague first; use formal channels if that fails.

**Sources:** NASW Code of Ethics (2021), 2.08(a); NASW Code of Ethics (2021), 2.08(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 35. ETH-1093 (rev 1) · Best answer: C

On an inpatient psychiatric unit, a voluntary patient with schizophrenia who has decision-making capacity refuses a new antipsychotic. The psychiatrist proposes mixing it into the patient's juice without telling the patient. In the team meeting, the social worker objects, but the psychiatrist and nurse manager plan to start tomorrow. What should the social worker do **NEXT**?

**A.** Have the psychiatrist walk through the plan once more
**B.** Record the objection and follow the team's plan
**C.** Take the concern to the hospital ethics committee today
**D.** Tell the patient about the plan before tomorrow

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Asking for the rationale fits when a concern is first raised. The objection has already been heard and the plan is set, so repeating the conversation only delays action before tomorrow.

**B** **Incorrect** (trap: Misreads law or ethics). Documenting disagreement while going along with covert medication would make the social worker part of deceiving the patient. The Code calls for pursuing the concern, not only recording it.

**C** **Correct.** When a team decision raises ethical concerns, social workers first try to resolve them through appropriate channels and then pursue other avenues consistent with client well-being (NASW Code 2.03(b)). Giving medication covertly to a capable voluntary patient overrides informed consent (1.03(a)), and objecting within the team has failed with a start date set. An ethics consultation is the hospital's next channel. C beats D because the formal channel can stop the plan without bypassing the team; telling the patient becomes an option if that channel fails.

**D** **Incorrect** (trap: Right action, wrong time). The patient's right to know matters, but going around the team before using the formal channel can fracture care and the patient's trust in staff. It may be warranted if the ethics consultation fails.

**Takeaway:** When a team proceeds with an ethically troubling plan after objection, use the next formal channel, such as an ethics consultation.

**Sources:** NASW Code of Ethics (2021), 2.03(b); NASW Code of Ethics (2021), 1.03(a)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 36. ETH-1094 (rev 1) · Best answer: A

A social worker co-leads a skills group with a newly hired colleague. In the last two sessions, the colleague taught distress-tolerance skills inaccurately, and members seemed confused. The colleague mentions never having received DBT training and says feedback is welcome. What should the social worker do **FIRST**?

**A.** Talk with the colleague and share DBT training resources
**B.** Tell the program manager the colleague lacks DBT training
**C.** Correct the colleague's teaching during the next group

**Why each option is right or wrong**

**A** **Correct.** With direct knowledge of a colleague's incompetence, social workers consult that colleague when feasible and help them take remedial action (NASW Code 2.09(a)). The colleague is open to feedback and the problem is a training gap, so a direct conversation with resources is feasible and likely to work. A beats B because the manager becomes the next step only if the colleague does not take adequate steps to address the problem (2.09(b)).

**B** **Incorrect** (trap: Right action, wrong time). Going through appropriate channels is the second step, used when the colleague does not act. This colleague welcomes feedback, so starting with the manager skips the collegial step.

**C** **Incorrect** (trap: Misuses the technique). Members need accurate skills, but correcting a co-leader in front of the group undermines the colleague and can confuse members about who leads. Feedback belongs in a private conversation first, offered with respect (2.01(a)).

**Takeaway:** For a colleague's correctable incompetence, start with a direct conversation and help, before going to a manager.

**Sources:** NASW Code of Ethics (2021), 2.09(a); NASW Code of Ethics (2021), 2.09(b); NASW Code of Ethics (2021), 2.01(a)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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## 37. ETH-1095 (rev 2) · Best answer: B

A new client in private practice says that a social worker at another agency, who treated the client last year, borrowed $2,000 during treatment and never repaid it. The client feels embarrassed and asks whether that was allowed. What is the **BEST** response?

**A.** File a complaint about the former provider with the board
**B.** Say it was improper and describe complaint options
**C.** Call the former provider to ask about the loan
**D.** Suggest setting it aside to focus on current goals

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Exposing unethical conduct matters, but this information came from the client in confidence. A complaint should be filed by the client or with the client's consent, which has not been given.

**B** **Correct.** Borrowing from a client exploits the professional relationship (NASW Code 1.06(b)), so answering honestly is not unwarranted criticism of a colleague. Social workers act to discourage and correct unethical conduct (2.10(a)), but this account is the client's confidential information. Explaining complaint options and supporting the client's choice addresses the misconduct while leaving the decision with the client. B beats A because filing without the client's consent would disclose confidential information the client has not authorized (1.07(b)).

**C** **Incorrect** (trap: Misreads law or ethics). Contacting the former provider reveals the client's disclosure without consent and places the social worker in the middle of the client's dispute. It also cannot resolve the ethical issue.

**D** **Incorrect** (trap: Good but not best). Current goals matter, but the client asked a direct question about possible exploitation. Deflecting leaves the client without accurate information about their rights and options.

**Takeaway:** When a client reports a prior provider's misconduct, answer honestly and explain complaint options; act on it only with consent.

**Sources:** NASW Code of Ethics (2021), 1.06(b); NASW Code of Ethics (2021), 2.10(a); NASW Code of Ethics (2021), 1.07(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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## 38. ETH-1096 (rev 1) · Best answer: C

During a hospital case review after a patient's suicide attempt, two staff members say a social work colleague ignored the patient's call the day before. The social worker covered that unit and had read the record, which shows the colleague returned the call within an hour and documented a safety check. What is the **MOST** appropriate action?

**A.** Wait to raise the documented facts until the review asks
**B.** Tell the colleague privately about the comments
**C.** Give the review leader the documented call details

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Waiting may seem neutral, but the review could reach conclusions based on a false account. The duty to defend a wrongly accused colleague calls for offering the facts, not waiting to be asked.

**B** **Incorrect** (trap: Good but not best). Letting the colleague know is supportive, but it leaves the review relying on inaccurate information, and the colleague may not be present to respond.

**C** **Correct.** Social workers should defend and assist colleagues who are unjustly charged with unethical conduct (NASW Code 2.10(e)). The social worker has direct knowledge, from the record, that the accusation is false, and the review is forming conclusions now. Giving the documented facts to the person leading the review corrects the account through the proper channel. C beats B because warning the colleague helps them personally but leaves the false account standing in the review.

**Takeaway:** When a colleague is wrongly accused and you have direct knowledge, bring the facts to the proper review process.

**Sources:** NASW Code of Ethics (2021), 2.10(e)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 39. ETH-1098 (rev 1) · Best answer: A

A colleague at a community agency is contesting a disciplinary action by management. The colleague asks the social worker to collect signatures from the social worker's clients on a petition supporting the colleague. How should the social worker respond to the request?

**A.** Decline to involve clients in the dispute
**B.** Leave the petition in the waiting room without comment
**C.** Ask the clients who know the colleague well

**Why each option is right or wrong**

**A** **Correct.** Social workers should not exploit clients in disputes with colleagues or engage clients in inappropriate discussion of those conflicts (NASW Code 2.04(b)). Clients may feel unable to refuse a request connected to their own worker, and the dispute has nothing to do with their care. A beats B because a petition placed in the waiting room still draws clients into a staff conflict, only less visibly.

**B** **Incorrect** (trap: Boundary slip or collusion). Passive placement feels less coercive, but it still uses the agency's service space to recruit clients into a staff dispute, and clients may assume their worker expects them to sign.

**C** **Incorrect** (trap: Boundary slip or collusion). Limiting the request to clients who know the colleague does not change the problem. Those clients may feel the most pressure, and the dispute is still unrelated to their services.

**Takeaway:** Keep clients out of disputes between colleagues and their employers.

**Sources:** NASW Code of Ethics (2021), 2.04(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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### 40. ETH-1099 (rev 1) · Best answer: C

Two social workers co-lead a 12-week outpatient group for adults in early recovery from substance use. At week 6, they begin a romantic relationship; neither supervises the other. Members have said that having both leaders matters to them. What is the **MOST** appropriate step for the co-leaders?

**A.** Keep the relationship private and continue co-leading
**B.** Tell the group about the relationship at the next session
**C.** Discuss with their supervisor whether to change co-leaders
**D.** Have one co-leader leave the group before next week

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). The relationship is personal, but co-leaders whose relationship could shape their group decisions face a possible conflict of interest. Continuing without review skips the check 2.06(b) calls for.

**B** **Incorrect** (trap: Boundary slip or collusion). Telling members shifts the leaders' issue onto clients in early recovery and invites distraction. Whether a conflict exists should be settled by the leaders with supervision, not by the group.

**C** **Correct.** Social workers should avoid sexual relationships with colleagues when a conflict of interest is possible, and those involved should transfer professional responsibilities when necessary (NASW Code 2.06(b)). A co-leaders' relationship can

shape group decisions and dynamics, so whether a change is needed deserves an objective look, while members' attachment to both leaders makes an abrupt exit risky. Consulting serves the clients' interests (2.05(a)). C beats D because a sudden departure mid-cycle may disrupt the group more than a planned transition the supervisor helps design.

**D** **Incorrect** (trap: Good but not best). Transferring responsibilities may prove necessary, and a planned change could be the outcome. Leaving abruptly mid-cycle, without preparing members who value both leaders, risks harming the group.

**Takeaway:** When a relationship with a colleague may create a conflict of interest, consult on whether to transfer responsibilities and plan any change.

**Sources:** NASW Code of Ethics (2021), 2.06(b); NASW Code of Ethics (2021), 2.05(a)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 41. ETH-1100 (rev 2) · Best answer: B

Social workers at a community mental health center have voted to strike in 10 days over unsafe caseloads, and the employer has refused further talks. A social worker plans to join the strike. The worker's caseload includes two clients with suicide attempts in the past 3 months and several clients who rely on weekly sessions. What should the social worker do **FIRST**?

**A.** Tell clients the strike date at their next sessions
**B.** Plan interim coverage and crisis access for each client
**C.** Withdraw from the strike because of the high-risk clients

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Clients should hear about the interruption promptly, but announcing a date with nothing in place leaves high-risk clients without a way to get care. Notice fits right after coverage and crisis access are arranged, so it can include them.

**B** **Correct.** Social workers may take part in organized labor action, but the Code asks them to weigh the possible impact on clients before acting (NASW Code 3.10(a), 3.10(b)). Clients who still need services must not be abandoned, and an anticipated interruption calls for prompt transfer or continuation planning (1.17(b), 1.17(e)). With two recent suicide attempts on the caseload, the worker first arranges who will see each client and how each can reach crisis help. B beats A because telling clients the date is useful only once there is a plan to share with them.

**C** **Incorrect** (trap: Good but not best). The Code permits organized action and notes that reasonable social workers disagree about striking (3.10). Recent attempts make planning essential, but they do not by themselves bar participation; deciding to withdraw before exploring coverage treats the conflict as unresolvable when it can be addressed.

**Takeaway:** Before joining a strike or other job action, arrange coverage and crisis access so clients who still need services are not abandoned.

**Sources:** NASW Code of Ethics (2021), 3.10(a); NASW Code of Ethics (2021), 3.10(b); NASW Code of Ethics (2021), 1.17(b); NASW Code of Ethics (2021), 1.17(e)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 1 of 3

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## 42. ETH-1101 (rev 1) · Best answer: A

A wildfire destroys hundreds of homes in a neighboring state, and thousands of residents move into evacuation shelters. A licensed clinical social worker with crisis training wants to help and plans to drive there this weekend to offer counseling at the shelters. What is the **BEST** way for the social worker to act on this goal?

**A.** Volunteer through an organized disaster mental health response
**B.** Go to the shelters and offer crisis counseling on arrival
**C.** Wait to help until licensed in the affected state
**D.** Offer free video counseling to evacuees from home

#### Why each option is right or wrong

**A** **Correct.** The Code asks social workers to provide appropriate professional services in public emergencies to the fullest extent possible (NASW Code 6.03). Disaster mental health guidance tells providers to operate only within an authorized response system, which assigns roles, screens credentials and coordinates with local authorities (Brymer et al., 2006). Joining an organized response lets the worker help soon without duplicating services or practicing outside the host state's rules. A beats B because arriving alone bypasses incident command and the host state's rules on who may provide counseling.

**B** **Incorrect** (trap: Misreads law or ethics). Wanting to help quickly is admirable, but self-deployed providers can strain shelter operations, duplicate services and practice without authorization in another state. Psychological first aid guidance calls for working inside an authorized response system.

**C** **Incorrect** (trap: Good but not best). Full licensure in another state can take months, while the Code encourages service in emergencies. Organized responses screen volunteers' credentials and coordinate with host-state authorities, so waiting is unnecessary for someone willing to join one.

**D** **Incorrect** (trap: Misreads law or ethics). Video sessions with evacuees are still practice where the clients are located, which means following that jurisdiction's laws (1.04(e)). Offering them independently also bypasses the coordination that disaster responses depend on.

**Takeaway:** In public emergencies, offer professional help through an authorized, organized response rather than self-deploying.

**Sources:** NASW Code of Ethics (2021), 6.03; NASW Code of Ethics (2021), 1.04(e); Brymer, M., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Introduction and Overview: Professional Behavior
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 43. ETH-1102 (rev 2) · Best answer: A

A social worker learns that a case manager at the same agency, who has a bachelor's degree in psychology and no license, sees private psychotherapy clients on weekends and advertises as a 'clinical social worker.' When the social worker raises the concern privately, the case manager says the title is harmless and declines to change the ads. What should the social worker do **NEXT**?

**A.** Report the case manager to the state licensing board
**B.** File a complaint about the case manager with NASW
**C.** Tell the agency director about the case manager's ads

**Why each option is right or wrong**

**A** **Correct.** Social workers should act to prevent unauthorized and unqualified practice of social work (NASW Code 5.01(e)). When a direct conversation does not resolve a colleague's unethical conduct, the Code points to formal channels such as a state licensing board (2.10(c), 2.10(d)). Licensing boards enforce practice acts, including who may use protected titles and offer licensed services, so the board has authority over this conduct. A beats C because the weekend practice happens outside the agency, and the director cannot enforce the practice act there; the board can.

**B** **Incorrect** (trap: Misreads law or ethics). NASW adjudicates ethics complaints against its members, and a person who is not a social worker falls outside that process. Enforcement of protected titles and licensed practice belongs to the state.

**C** **Incorrect** (trap: Good but not best). The director would need to know if agency clients or resources were involved, but the stem describes private weekend practice. The agency has no authority over practice outside the job, while the licensing board enforces the practice act.

**Takeaway:** Unlicensed use of a protected social work title goes to the state licensing board, which enforces practice acts.

**Sources:** NASW Code of Ethics (2021), 5.01(e); NASW Code of Ethics (2021), 2.10(c); NASW Code of Ethics (2021), 2.10(d)
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### 44. ETH-1179 (rev 2) · Best answer: B

At a first meeting, an older adult tells the social worker, a newly assigned case manager at a senior center, that the previous case manager 'never returned a single call' and 'made me feel like a bother.' The client then asks whether that worker was 'just lazy.' What is the **BEST** response by the social worker?

**A.** Agree that the previous worker handled the case poorly
**B.** Ask what the client needs to feel supported this time
**C.** Explain that heavy caseloads can delay returned calls
**D.** Offer to help file a complaint about the previous worker

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Agreeing passes judgment on a colleague's conduct without any facts, which is the unwarranted criticism the Code warns against. It may feel validating, but it shifts the focus to blame rather than the client's current needs.

**B** **Correct.** The Code asks social workers to avoid unwarranted negative criticism of colleagues in communications with clients (2.01(b)) while keeping the client's well-being primary (1.01). The new case manager has no facts about what happened, so judging the previous worker either way would be unfounded. Inviting the client to describe what support should look like now honors the frustration and builds the new relationship. It beats C because explaining caseload pressures defends the colleague at the cost of brushing aside the client's experience.

**C** **Incorrect** (trap: Good but not best). Offering an excuse for the previous worker can sound like taking the colleague's side. The client hears that the complaint is being explained away, which weakens trust at a first meeting.

**D** **Incorrect** (trap: Skips assessment). Complaint options matter if the client wants to pursue one, and the social worker can share them later. Proposing a complaint now assumes misconduct the social worker cannot confirm and steers the client toward a step the client did not request.

**Takeaway:** When clients criticize a former provider, avoid judging the colleague either way; focus on what the client needs from the current relationship.

**Sources:** NASW Code of Ethics (2021), 2.01(b); NASW Code of Ethics (2021), 1.01
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## 45. ETH-1180 (rev 1) · Best answer: C

Months ago, a coworker privately told the social worker about a past psychiatric hospitalization. Now their shared supervisor, concerned about the coworker's late reports, asks the social worker, 'Is something going on with them that I should know about?' Nothing the social worker has seen points to risk for clients. What is the **BEST** response?

**A.** Share the history to help explain the late reports
**B.** Urge the coworker to tell the supervisor about the hospitalization
**C.** Advise the supervisor to raise the late reports with the coworker

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Wanting the coworker to get support is understandable, but the history was shared in confidence. Disclosing it without the coworker's permission breaches 2.02 and could expose the coworker to stigma at work.

**B** **Incorrect** (trap: Good but not best). Whether to tell a supervisor about a health history is the coworker's decision. Pressing the coworker to disclose it also assumes the hospitalization explains the late reports, which the social worker does not know.

**C** **Correct.** Social workers should respect confidential information shared by colleagues (2.02). The coworker disclosed the hospitalization privately, and nothing suggests client risk, so nothing overrides that confidence. The supervisor's legitimate concern is

performance, which the supervisor can raise directly with the coworker, who then decides what to share. It beats A because a supportive motive does not make disclosing a colleague's health history appropriate. If impairment were harming clients, the impairment standard (2.08) would guide a different path.

**Takeaway:** Keep colleagues' confidences; redirect performance concerns to the colleague unless client safety is at risk.

**Sources:** NASW Code of Ethics (2021), 2.02; NASW Code of Ethics (2021), 2.08
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### 46. ETH-1181 (rev 1) · Best answer: A

A social worker at a county child welfare agency drafts a letter to the local newspaper opposing a proposed state cut to foster care funding. The letter draws on the social worker's general experience and names no families. The draft is signed with the social worker's name, job title, and agency. The agency has not taken a position on the cut. What is the **BEST** action for the social worker?

**A.** Sign as a private citizen and note the views are personal
**B.** Withhold the letter, since county employees should stay neutral
**C.** Ask foster parents on the caseload to co-sign the letter
**D.** Send the letter as drafted, since advocacy is an ethical duty

#### Why each option is right or wrong

**A** **Correct.** The Code encourages social and political action and advocacy for policy changes that meet basic needs (6.04(a)), and it asks social workers to distinguish statements made as private individuals from those made for their employer (4.06(a)). Signing with the agency name implies an agency position the agency has not taken. Signing as a private citizen keeps the advocacy and removes the misrepresentation. It beats D because the duty to advocate does not include speaking for an employer without authorization.

**B** **Incorrect** (trap: Good but not best). Public employees generally may speak on public issues as private citizens, and the Code encourages policy advocacy. Dropping the letter gives up legitimate advocacy when a simple change in how it is signed solves the problem.

**C** **Incorrect** (trap: Boundary slip or collusion). Recruiting people the social worker serves to back the worker's cause uses the professional relationship for the worker's goals. Foster parents may feel unable to refuse someone with influence over their case.

**D** **Incorrect** (trap: Misreads law or ethics). Advocacy is encouraged, but signing with the job title and agency name implies the agency endorses the letter. The Code asks social workers to make clear when they speak only for themselves.

**Takeaway:** Advocate on policy freely, but make clear when you speak as a private individual rather than for your employer or the profession.

**Sources:** NASW Code of Ethics (2021), 6.04(a); NASW Code of Ethics (2021), 4.06(a)
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### 47. ETH-1182 (rev 1) · Best answer: C

A prospective client calls a group practice asking to start weekly individual therapy with the social worker. During the call, the client mentions seeing another mental health provider weekly for the same concerns and adds, 'I haven't told them I'm looking around.' What should the social worker do **FIRST**?

**A.** Schedule an intake appointment for this week as the client requested
**B.** Decline until the client ends the current therapy
**C.** Discuss how adding a provider could affect current care
**D.** Ask permission to contact the current provider

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Starting services without discussing the existing therapy risks parallel treatment with conflicting approaches. The Code asks the social worker to consider the client's needs carefully before agreeing to provide services.

**B** **Incorrect** (trap: Overrides the client's choice). Requiring the client to end current therapy first imposes a decision that belongs to the client and may cut off support. The client may have good reasons for seeking a change, which the social worker has not yet heard.

**C** **Correct.** When someone receiving services from a colleague seeks help, the Code asks the social worker to consider the client's needs carefully and to discuss the current relationship and the benefits and risks of starting with a new provider (3.06(a)). Two concurrent therapies for the same concerns can work at cross-purposes, so this conversation comes before any other step. It beats D because contacting the current provider is a possible later step that depends on what the client wants after this discussion, and on the client's consent.

**D** **Incorrect** (trap: Right action, wrong time). Coordinating with the current provider may become appropriate, with consent, but it is premature before the social worker understands why the client is seeking a second provider and what the client wants.

**Takeaway:** When a prospective client is already in treatment elsewhere, first discuss that relationship and the risks and benefits of adding a provider.

**Sources:** NASW Code of Ethics (2021), 3.06(a)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.2 Legal-ethical duties to clients, colleagues, profession, and society · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 48. ETH-1183 (rev 2) · Best answer: B

A colleague at a family service agency has filed a formal grievance over a demotion. The director tells the social worker that the colleague's former supervisory post is open and asks whether the social worker knows 'anything that would help the agency's case.' The social worker wants the post. Which response is **MOST** consistent with the social worker's ethical responsibilities?

**A.** Limit any comments to facts the social worker saw firsthand
**B.** Apply on own merits without commenting on the dispute
**C.** Withdraw from consideration out of loyalty to the colleague

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Sticking to firsthand facts sounds fair and cooperative. Offered to the decision maker in a job conversation while the social worker wants the post, it still trades on the dispute (2.04(a)) and can shade into unwarranted criticism of a colleague (2.01(b)). If called as a witness in the formal grievance, truthful testimony is appropriate.

**B** **Correct.** The Code states that social workers should not take advantage of a dispute between a colleague and an employer to obtain a position or advance their own interests (2.04(a)). Applying for an open post is legitimate; supplying damaging information to the side that controls the post, while wanting it, is the exploitation the standard describes. Applying on merit and staying out of the grievance meets both interests. It beats A because even firsthand facts, handed to the person who controls the post while the worker seeks it, use the dispute for advantage; facts the grievance needs can come through its formal process.

**C** **Incorrect** (trap: Good but not best). Loyalty is admirable, but the Code does not bar applying for an open position. Withdrawing is unnecessary when the social worker can pursue the job fairly and simply decline to take part in the dispute.

**Takeaway:** Pursue opportunities on your own merits; never use a colleague's dispute with an employer to advance your own interests.

**Sources:** NASW Code of Ethics (2021), 2.04(a); NASW Code of Ethics (2021), 2.01(b)
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## 49. ETH-0002 (rev 1) · Best answer: D

A hospital social worker is asked to help with an older adult patient who was just diagnosed with advanced cancer. Before the physician meets with the patient, the patient's adult children ask the team not to share the diagnosis. They explain that in their family's culture, elders are protected from bad news. What should the social worker do **FIRST**?

**A.** Tell the adult children that the patient has a legal right to know
**B.** Agree to the family's request out of respect for their cultural values
**C.** Refer the disagreement to the hospital ethics committee for a decision
**D.** Ask the patient how much they want to know and who should be told

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). The patient does hold a right to the information, but announcing it to the family first is confrontational. It also skips the deciding question, because the patient may choose to have family receive the news. Rights are clarified once the patient's preference is known.

**B** **Incorrect** (trap: Overrides the client's choice). Agreeing lets the family decide for a competent adult. Respecting culture means asking the patient how they want their culture honored, rather than assuming the family speaks for them (NASW Code 1.05(c)).

**C** **Incorrect** (trap: Hands off too soon). An ethics consultation can help if the conflict continues after the patient's wishes are known. Referring now hands off a question that a direct conversation with the patient may settle.

**D** **Correct.** When values conflict and no one is in immediate danger, the first step is to clarify the facts. The key fact here is the patient's own wishes. Asking how much the patient wants to know, and who should be told, respects self-determination (NASW Code 1.02) and leaves room for a culturally grounded choice to defer to family (1.05). It beats A because asserting a right to know before learning the patient's wishes could force unwanted information on someone who would choose family-centered disclosure. Reamer's decision-making framework likewise starts by identifying the issues and the people affected.

**Takeaway:** In a values conflict without imminent danger, first clarify the facts, starting with the competent client's own wishes.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.05(c); Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 50. ETH-0101 (rev 1) · Best answer: A

A transitional housing program requires staff to discharge any resident who misses curfew 3 times. A resident has missed curfew 3 times because a new job ends after curfew. The resident has no other place to live. What should the social worker do **FIRST**?

**A.** Ask agency leaders whether the rule allows a work exception
**B.** Begin discharge planning as the written policy requires
**C.** Encourage the resident to look for a job with earlier hours
**D.** Request an ethics committee review of the curfew policy

#### Why each option is right or wrong

**A** **Correct.** When an agency policy conflicts with a client's welfare, the Code directs a responsible effort to resolve the conflict, starting with clarifying the facts and options (NASW Code, Purpose; 3.09(d)). Finding out whether the rule has an exception or appeal process is that first step, and it may solve the problem through ordinary channels. It beats D because an ethics committee review is a later step, used if leaders cannot or will not resolve the conflict. Reamer's decision-making framework likewise begins by identifying the issues and options before escalating (Reamer, 2018).

**B** **Incorrect** (trap: Misreads law or ethics). Discharging the resident to homelessness puts the written rule ahead of serious harm to the client without any effort to resolve the conflict. Following policy is right when the policy is ethical as applied, which is the question still open here.

**C** **Incorrect** (trap: Overrides the client's choice). Steering the resident toward different work asks the client to give up a job they chose in order to fit a rule. The conflict lies in the policy as applied, so the social worker addresses the rule before asking the client to change.

**D** **Incorrect** (trap: Right action, wrong time). Ethics committee review is a reasonable escalation if program leaders refuse an exception or the conflict cannot be resolved. Going there first skips a simpler step that may settle the issue quickly through ordinary channels.

**Takeaway:** When policy and client welfare conflict, first seek resolution through the agency's channels, then consult or escalate if that fails.

**Sources:** NASW Code of Ethics (2021), Purpose; NASW Code of Ethics (2021), 3.09(d); Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 51. ETH-1080 (rev 4) · Best answer: B

During a home visit, a 79-year-old client who lives alone says the adult children want an in-home aide after two minor falls this year, neither causing injury. The client understands the risks, describes a plan to wear a medical alert pendant, and declines the aide. Under the Ethical Principles Screen, which principle takes priority in this situation?

**A.** Least harm
**B.** Self-determination
**C.** Quality of life
**D.** Protection of life

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Least harm matters, and the family's worry reflects it. In the screen, though, it ranks below self-determination, so it governs only if the client's choice were not informed and voluntary.

**B** **Correct.** The Ethical Principles Screen ranks protection of life first, then social justice, then self-determination, autonomy and freedom, ahead of least harm and quality of life (Dolgoff et al., 2012). Minor falls and an informed safety plan do not pose an immediate threat to life, so the client's autonomy ranks highest among the principles in play. This fits the Code, which allows limiting self-determination when a client's actions pose a serious, foreseeable and imminent risk (NASW Code 1.02). B beats A because least harm ranks below self-determination when life is not at stake.

**C** **Incorrect** (trap: Wrong model or concept). Quality of life ranks fifth in the screen, below both self-determination and least harm. It may shape how services are offered but does not outweigh an informed choice.

**D** **Incorrect** (trap: Wrong model or concept). Protection of life ranks first, but the screen ranks only the principles actually in conflict, and this one applies when a choice directly threatens life. Two minor falls without injury and a working alert plan do not show that level of danger.

**Takeaway:** In the Ethical Principles Screen, an informed choice outranks least harm and quality of life unless life itself is threatened.

**Sources:** Dolgoff, R., Harrington, D., & Loewenberg, F. M. (2012). Ethical Decisions for Social Work Practice (9th ed.). Brooks/Cole Cengage, Ethical Principles Screen; NASW Code of Ethics (2021), 1.02
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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### 52. ETH-1081 (rev 1) · Best answer: D

A resident of a memory care unit who has major neurocognitive disorder asks staff every afternoon where their spouse is. The spouse died 2 years ago. Each time staff explain the death, the resident grieves as if hearing it for the first time. Staff ask the social worker how to answer. What is the **BEST** guidance?

**A.** Gently restate the fact of the death each time
**B.** Say the spouse is at work and will visit later
**C.** Ask the family to decide what staff should say
**D.** Acknowledge the longing and invite shared memories

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Truthfulness matters, but each disclosure here produces new grief the resident cannot retain. Repeating it ranks accuracy above harm, the reverse of the screen's order, without helping the resident.

**B** **Incorrect** (trap: Misreads law or ethics). A made-up story avoids distress but is deception, which 4.04 prohibits. It can also backfire when the promised visit never happens, adding to agitation.

**C** **Incorrect** (trap: Hands off too soon). Family knowledge of the resident's history is valuable, but the question is how to resolve a professional ethical conflict. Handing that decision to relatives sidesteps the social worker's own responsibility.

**D** **Correct.** This is a conflict between truthfulness and least harm. The Ethical Principles Screen ranks least harm above truthfulness (Dolgoff et al., 2012), and the stem shows that repeating the death causes fresh grief each time. Yet the Code bars taking part in deception (NASW Code 4.04). Responding to the feeling behind the question and inviting memories honors both: no false statement and no repeated injury. D beats A because restating the death serves accuracy while ignoring documented harm to a resident who cannot retain the information.

**Takeaway:** When telling the truth causes repeated harm a person cannot process, respond to the underlying feeling without deceiving.

**Sources:** Dolgoff, R., Harrington, D., & Loewenberg, F. M. (2012). Ethical Decisions for Social Work Practice (9th ed.). Brooks/Cole Cengage, Ethical Principles Screen: least harm; truthfulness; NASW Code of Ethics (2021), 4.04
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### 53. ETH-1082 (rev 1) · Best answer: C

A foster parent tells a child welfare social worker that a 14-year-old in care has been meeting a birth parent at a mall on weekends. The court allows only supervised visits. The foster parent adds that the youth asked them not to tell anyone. The social worker has a scheduled meeting with the youth this afternoon. What should the social worker do **FIRST**?

**A.** Promise the youth secrecy if the meetings stop
**B.** Notify the court that the visit order was violated
**C.** Ask the youth about the meetings and how safe they felt
**D.** Report the foster parent's account to the supervisor

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). A promise of secrecy cannot be kept, because the worker has duties to the agency and the court. Clients should hear the real limits of confidentiality, not a conditional promise (NASW Code 1.07(e)).

**B** **Incorrect** (trap: Skips assessment). The court may need to know, but filing a notice on a secondhand report skips fact-finding and the agency's own process. It is premature at this point.

**C** **Correct.** Working through a dilemma starts with clarifying the facts and the issues at stake (Reamer, 2018). This report is secondhand, and the court's supervision order signals a possible safety concern, so the youth's own account and sense of safety come first, and the meeting is only hours away. C beats D because the supervisor needs to know, but a same-day conversation gives the supervisor verified facts and safety information to act on instead of an untested account.

**D** **Incorrect** (trap: Right action, wrong time). Supervisor involvement is expected for a possible court-order violation, and it should follow quickly. Passing on an unverified account before speaking with the youth, who will be seen today, gives the supervisor less to act on.

**Takeaway:** When a dilemma arises from secondhand information, clarify the facts and any safety concern before choosing or reporting a course of action.

**Sources:** Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework; NASW Code of Ethics (2021), 1.07(e)
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## 54. ETH-1083 (rev 1) · Best answer: A

A community mental health agency accepts a state grant that requires entering each client's name and diagnosis into a shared state database. The social worker believes this conflicts with clients' privacy and raises it with the supervisor. The supervisor agrees but says the program cannot change the grant terms. Enrollment begins next month. What should the social worker do **NEXT**?

**A.** Ask the ethics committee to review the requirement
**B.** Let each client decide whether to be entered
**C.** Enter client data as the grant requires
**D.** Leave client names out of the database entries

**Why each option is right or wrong**

**A** **Correct.** When agency policy and the Code conflict, social workers make a good-faith effort to resolve it; if no reasonable resolution appears possible, they seek proper consultation, such as an ethics committee or legal counsel, before deciding (NASW Code, Purpose). The supervisor step has failed and a month remains, so formal consultation is next. A beats B because clients must learn about the database at consent (1.03(a)), but an opt-out the grant does not allow would be a promise the agency cannot keep.

**B** **Incorrect** (trap: Misreads law or ethics). Clients do need to hear about the database before services begin (1.03(a)). Offering each one a choice the grant terms do not permit misleads them and leaves the conflict unresolved.

**C** **Incorrect** (trap: Right action, wrong time). Complying may turn out to be the outcome, but doing it before consultation treats the funder's rule as settling the ethical question. Employer policies should not override ethical practice without that review (3.09(d)).

**D** **Incorrect** (trap: Misreads law or ethics). Quietly altering required entries is covert noncompliance that could put funding and services at risk and breaks commitments to the employer (3.09(a)). Disagreement belongs in consultation, not hidden action.

**Takeaway:** When agency policy conflicts with ethics and the supervisor cannot resolve it, seek formal consultation before deciding how to act.

**Sources:** NASW Code of Ethics (2021), Purpose; NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 3.09(d); NASW Code of Ethics (2021), 3.09(a)
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## 55. ETH-1084 (rev 2) · Best answer: B

A client at a community agency receives disability benefits that limit how much the client may earn. In session, the client mentions doing cash yard work most weekends. The client then asks the social worker to sign a letter to the benefits office stating that the client has no income. What is the **MOST** appropriate response?

**A.** Sign a letter worded as the client's own report of no income
**B.** Decline to sign while keeping the disclosure private
**C.** Report the unreported earnings to the benefits office

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Framing the statement as the client's report seems to keep the social worker honest. The worker knows about the cash earnings, though, so signing still lends the worker's name to a claim the worker believes is false, which is the deception 4.04 bars.

**B** **Correct.** Two duties pull apart here. Without consent, the Code allows disclosure only for compelling professional reasons, such as preventing serious, foreseeable and imminent harm (NASW Code 1.07(c)), and unreported earnings are not such a reason. But signing a statement the social worker knows is false would mean taking part in fraud (4.04). Declining the letter while keeping the disclosure private honors both duties, and the worker can discuss options such as benefits counseling. B beats A because wording the letter as the client's own report still puts the worker's signature behind a statement the worker knows is false.

**C** **Incorrect** (trap: Misreads law or ethics). Reporting would disclose confidential information without consent and without a compelling professional reason such as serious, imminent harm. Disagreeing with a client's conduct does not create grounds to breach confidentiality.

**Takeaway:** Keep client disclosures confidential unless serious imminent harm is at stake, but never sign or endorse a statement known to be false.

**Sources:** NASW Code of Ethics (2021), 1.07(c); NASW Code of Ethics (2021), 4.04
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 56. ETH-1085 (rev 3) · Best answer: A

A hospital social worker has one remaining charity-funded home health slot this month, and two patients being discharged qualify. The program's written criteria rank applicants by medical risk at home. A nurse suggests giving the slot to the patient who asked about it first. What is the **BEST** way for the social worker to decide?

**A.** Use the established risk ranking for both patients
**B.** Give the slot to the patient who asked first
**C.** Ask the program director to choose between the patients

**Why each option is right or wrong**

**A** **Correct.** When resources cannot meet every need, allocation should be open and fair, nondiscriminatory, and based on principles applied consistently (NASW Code 3.07(b)). The program already has written criteria based on medical risk, so applying them to both patients is the fair and defensible choice. A beats B because first-come order is consistent in itself but is not the program's stated standard, and it rewards knowing to ask rather than need.

**B** **Incorrect** (trap: Good but not best). First-come order can be a fair rule when a program adopts it in advance. Here the written criteria rank by medical risk, and switching standards case by case makes allocation inconsistent.

**C** **Incorrect** (trap: Hands off too soon). A director can settle a true tie or a question about the criteria. Here the written risk ranking already supplies the standard the social worker is expected to apply, so passing the decision up avoids a choice the policy makes possible and delays both discharges.

**Takeaway:** Allocate scarce services by open, consistently applied criteria rather than case-by-case preference.

**Sources:** NASW Code of Ethics (2021), 3.07(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 57. ETH-1086 (rev 1) · Best answer: B

Which situation is an ethical dilemma rather than a clear-cut ethical violation?

**A.** A supervisor asks a worker to backdate a missing progress note
**B.** A capable client refuses a shelter bed on a freezing night
**C.** An agency bills insurance for canceled group sessions
**D.** A worker invests in a current client's small business

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Backdating creates a false record. No competing professional duty justifies it, so it is a clear violation rather than a dilemma (NASW Code 4.04).

**B** **Correct.** An ethical dilemma exists when two or more professional duties conflict, so honoring one compromises another (Reamer, 2018). A capable client's refusal of shelter in dangerous cold sets self-determination against protection from harm; both are legitimate duties, and the right balance depends on further facts such as risk and capacity. B beats D because investing with a client may look like a judgment call, but the Code clearly bars exploiting professional relationships for business gain (NASW Code 1.06(b)).

**C** **Incorrect** (trap: Wrong model or concept). Billing for sessions that did not occur is fraud, which 4.04 prohibits. Nothing about it requires weighing competing duties.

**D** **Incorrect** (trap: Wrong model or concept). A business relationship with a current client creates a conflict of interest and a risk of exploitation that 1.06(b) addresses directly. It is a clear boundary problem, not a conflict between duties.

**Takeaway:** A dilemma pits two legitimate duties against each other; a clear violation has no competing duty to weigh.

**Sources:** Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical dilemmas: conflicting duties; NASW Code of Ethics (2021), 1.06(b); NASW Code of Ethics (2021), 4.04
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 58. ETH-1087 (rev 1) · Best answer: C

At an adult day program, a client with mild neurocognitive disorder wants to keep paying their own bills. After weighing options and consulting a supervisor, the social worker supports the plan, and the client agrees to a weekly check-in with a trusted friend. The reasoning is documented. What should the social worker do **NEXT**?

**A.** Request a formal capacity evaluation of the client
**B.** Treat the question as resolved and close it
**C.** Set a date to review how the plan is working

#### Why each option is right or wrong

**A** **Incorrect** (trap: Skips assessment). A capacity evaluation fits when there are signs a person cannot understand or weigh a decision. The diagnosis alone does not show that, and the choice was already weighed with consultation.

**B** **Incorrect** (trap: Good but not best). The decision and its documentation are complete, but neurocognitive disorders can progress. Closing the matter skips the monitoring step that would catch problems early.

**C** **Correct.** Ethical decision-making does not end with the decision. Reamer's framework closes with monitoring, evaluating and documenting the outcome (Reamer, 2018). A client with a progressive condition may change, so a set review date checks whether the plan still protects both autonomy and finances. C beats A because nothing in the stem suggests the client cannot understand or weigh this decision; an evaluation now would second-guess a choice already analyzed and supported.

**Takeaway:** After an ethical decision is made and documented, monitor and evaluate its outcome, especially when circumstances may change.

**Sources:** Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework: monitor and evaluate
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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### 59. ETH-1088 (rev 1) · Best answer: A

A hospice social worker visits a family at home. The parent who is dying has asked that their 9-year-old not be told about the prognosis yet. While drawing with the social worker, the child asks directly whether the parent is going to die. What is the **BEST** response?

**A.** Validate the worry and bring the question to the parent
**B.** Answer honestly that the parent is expected to die soon
**C.** Say the parent's doctors will help them feel better

#### Why each option is right or wrong

**A** **Correct.** Several duties meet here: honesty, the parent's authority over what a young child is told, and the child's well-being. Deceiving the child is never acceptable (NASW Code 4.04), but overriding the parent can harm the relationship the child will rely on. Acknowledging the worry and helping the parent plan an honest, age-appropriate talk reduces harm without deception (Dolgoff et al., 2012, least harm). A beats B because the parent is at home and can be helped to answer soon, so taking the decision away is not needed.

**B** **Incorrect** (trap: Overrides the client's choice). Children often sense the truth, and honesty matters. Giving the news against the parent's stated wish, however, undermines the parent at a fragile time when the parent can be supported to share it soon.

**C** **Incorrect** (trap: Misreads law or ethics). False reassurance comforts the child briefly but is deception (4.04). It can also damage the child's trust in the adults around them after the parent dies.

**Takeaway:** When a parent's wishes and a child's question collide, avoid both deception and overriding the parent; help the parent share the truth.

**Sources:** NASW Code of Ethics (2021), 4.04; Dolgoff, R., Harrington, D., & Loewenberg, F. M. (2012). Ethical Decisions for Social Work Practice (9th ed.). Brooks/Cole Cengage, Ethical Principles Screen: least harm
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### 60. ETH-1089 (rev 2) · Best answer: D

When values in the NASW Code of Ethics conflict, the Code asks social workers to use their informed judgment. According to the Code, how else should such a decision be evaluated?

**A.** By whether the client prefers the outcome
**B.** By the employing agency's written policies
**C.** By the Ethical Principles Screen's ranking
**D.** By how a peer review process would view it

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Client preference carries great weight under self-determination, but it is not the measure the Code names for judging whether a professional resolved an ethical conflict well.

**B** **Incorrect** (trap: Misreads law or ethics). Agency policy is one consideration, but the Code says employer policies should not interfere with ethical practice (3.09(d)). It is not the standard the Code names for judging decisions.

**C** **Incorrect** (trap: Wrong model or concept). The screen ranks principles for hard cases, but it comes from an ethics textbook, not from the NASW Code, which deliberately declines to rank its values.

**D** **Correct.** The Code's Purpose section explains that it does not rank its values, principles or standards, and that reasonable social workers can disagree about priorities. Decisions rest on the worker's informed judgment and should also consider how the issues would be judged in a peer review process applying the profession's standards (NASW Code, Purpose). D beats C because the Ethical Principles Screen is a useful textbook tool, not part of the Code, and the Code itself sets no ranking.

**Takeaway:** The Code does not rank its values; resolutions should be defensible to a peer review applying professional standards.

**Sources:** NASW Code of Ethics (2021), Purpose; NASW Code of Ethics (2021), 3.09(d)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 61. ETH-1091 (rev 1) · Best answer: C

A parent working toward reunification tells the child welfare social worker about drinking heavily one night last weekend while the children were at the foster home. The parent has met every other case plan goal and asks the worker to leave the drinking out of next week's court report. What is the **MOST** appropriate response?

**A.** Omit it, since the children were elsewhere
**B.** Include it in the report without telling the parent
**C.** Tell the parent the report will include it, with the progress

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). The children's absence lowers the immediate risk but does not make the drinking irrelevant to a case plan the court is monitoring. Leaving it out would mislead the court.

**B** **Incorrect** (trap: Good but not best). Accurate reporting is right, but clients should be told when the worker's obligations override their wishes (1.01). Surprising the parent in court harms trust and engagement.

**C** **Correct.** Loyalty to clients is real but limited: when legal obligations supersede it, clients should be told (NASW Code 1.01). A court report must be honest, and leaving out a relapse the court would consider material is deception (4.04). Telling the parent first, and presenting the lapse alongside the strong progress, keeps the report accurate and preserves trust. C beats B because including the information without warning blindsides the parent and damages the working relationship reunification depends on.

**Takeaway:** Report material facts honestly to the court, and tell the client beforehand when obligations override their wishes.

**Sources:** NASW Code of Ethics (2021), 1.01; NASW Code of Ethics (2021), 4.04
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 62. ETH-1092 (rev 3) · Best answer: B

A social worker staffs a tenant association that is developing a cooperative housing building. At a meeting, members vote to bar anyone with a criminal record from applying. Several members have relatives returning from prison who had hoped to apply. What is the **BEST** response by the social worker?

**A.** Help members write the vote into the building's rules
**B.** Raise how the rule excludes people and discuss alternatives
**C.** Tell members the ban is unjust and ask them to repeal it

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Respecting the group's decision-making is important, but helping put an exclusionary rule into effect without raising its impact sets aside the duty to promote justice and access.

**B** **Correct.** Group self-determination matters, but social justice ranks above it in the Ethical Principles Screen (Dolgoff et al., 2012), and social workers should work to expand choice and opportunity for all people, especially those who are disadvantaged (NASW Code 6.04(b)). A blanket ban shuts out people, including members' own relatives, regardless of offense or time since release. Naming that effect and exploring alternatives, such as individual review, informs the group's choice without taking it over. B beats C because naming the injustice is right, but pressing for repeal takes the decision from the group instead of informing it.

**C** **Incorrect** (trap: Too directive). The concern is valid, which makes a firm stand tempting. Pressing members to repeal the ban replaces the group's decision with the social worker's, and a lecture is less likely to change minds than laying out the rule's effects and workable alternatives.

**Takeaway:** When a client group's choice would exclude vulnerable people, raise its justice impact and alternatives while leaving the decision with the group.

**Sources:** NASW Code of Ethics (2021), 6.04(b); Dolgoff, R., Harrington, D., & Loewenberg, F. M. (2012). Ethical Decisions for Social Work Practice (9th ed.). Brooks/Cole Cengage, Ethical Principles Screen: social justice; self-determination
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 63. ETH-1169 (rev 3) · Best answer: B

A parole officer asks a reentry program social worker how a client is 'doing overall.' The client's release covers only attendance at mandated treatment, and the client also attends an optional peer group. After reviewing the release and the Code and talking with a supervisor, the social worker tells the officer that the client attends all mandated sessions and explains this choice to the client. What should the social worker do **NEXT**?

**A.** Ask a second colleague whether they agree
**B.** Record the options weighed, consultation and reasoning
**C.** Set a date to review how the decision works out

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). A second opinion adds little once a supervisor has been consulted and the decision has been made and carried out. More consultation fits when a dilemma remains unresolved, not after a reasoned decision is in place.

**B** **Correct.** Ethical decision-making models end with documenting the decision process and then monitoring its effects (Reamer, 2018). The social worker has identified the issue, reviewed the release and the Code, consulted a supervisor, decided and acted. The record should now show the options considered, the consultation and the reasons for sharing only mandated attendance, as the framework advises, and the Code asks that records be sufficient to support future services (NASW Code 3.04(b)). It beats C because reviewing the outcome comes after documentation; without a record there is nothing to evaluate against, and the reasoning is lost if questions arise later.

**C** **Incorrect** (trap: Right action, wrong time). Reviewing how the decision works out is the final step of the process, and it should happen. It comes after the decision and its reasoning are documented, so there is a record to evaluate against.

**Takeaway:** After resolving an ethical dilemma, document the options, consultation and reasoning, then monitor the outcome.

**Sources:** Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework: decide, document, monitor; NASW Code of Ethics (2021), 3.04(b)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.3 Recognizing and working through ethical dilemmas · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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### 64. ETH-1170 (rev 2) · Best answer: A

A social worker at a nonprofit counseling agency meets with a client who works nights and can attend only evening sessions, which the agency does not offer. A free clinic nearby runs evening groups for the client's concern. The agency director has told staff not to mention that clinic because funding depends on client numbers. Which action is **MOST** consistent with the social worker's ethical duties?

**A.** Tell the client about the clinic's evening services
**B.** Help the client ask the employer for a daytime shift
**C.** Ask the director to approve mentioning the evening clinic

**Why each option is right or wrong**

**A** **Correct.** Clients' interests are primary (NASW Code 1.01), and informed consent includes telling clients about reasonable alternatives (1.03(a)). An employer's policy should not interfere with ethical practice (3.09(d)). This client cannot attend any session the agency offers, so withholding a free clinic that fits the schedule would leave the client without care to protect agency numbers. It beats C because seeking permission delays information the client needs now, and access cannot hinge on approval; the social worker can raise the policy with the director afterward (3.09(b)).

**B** **Incorrect** (trap: Good but not best). Solving a scheduling barrier can help, but asking the client to rearrange their job to fit the agency's hours puts the burden on the client while an option that already fits goes unmentioned. The client should hear about the alternative first.

**C** **Incorrect** (trap: Right action, wrong time). Raising the policy with the director is worthwhile, and the social worker should do it. Making the client wait for approval, though, puts the agency's funding concern ahead of the client's access to timely care.

**Takeaway:** When an employer's instruction conflicts with a client's interests, inform the client of fitting alternatives and address the policy separately.

**Sources:** NASW Code of Ethics (2021), 1.01; NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 3.09(d); NASW Code of Ethics (2021), 3.09(b)
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## 65. ETH-1175 (rev 1) · Best answer: C

A new state law requires mental health providers to notify a parent before a minor's fourth counseling session. A social worker in a school-based counseling program believes the law will keep some teens from seeking help. The program's attorney has confirmed that the law applies, and several students are already in counseling. What is the **BEST** action for the social worker to take?

**A.** Keep the current practice until a court rules on the law
**B.** Stop taking new student clients until the law changes
**C.** Comply with the law and advocate for changing it
**D.** Ask an ethics committee whether to follow the law

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Ignoring a law that the program's attorney says applies exposes students and the program to harm, including possible loss of the service itself. Disagreeing with a law is a reason to work for its change, not to disregard it while services continue.

**B** **Incorrect** (trap: Hands off too soon). Closing the door to new students removes help from the very teens the social worker wants to protect. Withdrawing services does not resolve the conflict; it shifts the cost of the law onto students who need care.

**C** **Correct.** When law and ethics conflict, the NASW Code asks social workers to make a responsible effort to resolve the conflict consistently with its values (Purpose) and to advocate for changes in legislation (6.04(a)). The attorney has confirmed the law applies, so the social worker complies, tells current students about the new limit on

confidentiality before it affects them (1.07(e)), and works to change the law. It beats D because the Code reserves consultation for conflicts with no reasonable resolution; here the legal question is settled and a resolution exists.

**D** **Incorrect** (trap: Good but not best). The Code calls for consultation when a reasonable resolution does not appear possible. Here the legal question is already answered and a workable resolution exists, so another round of review delays compliance and leaves current students uninformed about the change.

**Takeaway:** When a law conflicts with ethical values, comply, tell affected clients about new limits, and advocate through legitimate channels to change it.

**Sources:** NASW Code of Ethics (2021), Purpose; NASW Code of Ethics (2021), 6.04(a); NASW Code of Ethics (2021), 1.07(e)
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## 66. ETH-1176 (rev 1) · Best answer: A

An ethics committee debates whether a hospital social worker may share a patient's history with a landlord to save the patient's housing. One member grants that this single disclosure might help. The member argues that keeping patient information private, case after case, yields the best results for patients overall, so the history should stay private. Which ethical theory is this member **MOST** likely using?

**A.** Rule utilitarianism
**B.** Act utilitarianism
**C.** Deontology

#### Why each option is right or wrong

**A** **Correct.** Utilitarian (teleological) theories judge actions by their consequences. Rule utilitarianism asks which general practice produces the best outcomes when followed consistently, even if breaking it once might help. The member concedes the single disclosure could help but defends privacy because the consistent practice yields the best results for patients overall, which is a rule-utilitarian argument. It beats C, deontology, because deontology treats a duty as binding regardless of outcomes; this member justifies privacy by its results (Reamer, 2018).

**B** **Incorrect** (trap: Wrong model or concept). Act utilitarianism weighs the consequences of this one act. On that reasoning, the member would favor disclosure if it helps this patient. The member instead sets the single case aside in favor of the general practice.

**C** **Incorrect** (trap: Wrong model or concept). Deontology treats certain duties, such as keeping confidences, as binding in themselves, whatever the outcome. This member's argument rests on the results of the practice, not on the duty's built-in rightness, so it is consequentialist.

**Takeaway:** Rule utilitarianism judges a practice by its overall consequences when followed consistently; deontology holds duties binding regardless of consequences.

**Sources:** Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, Ethical decision making: deontological and teleological (act and rule utilitarian) theories
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### 67. ETH-1177 (rev 2) · Best answer: C

A hospital patient with cancer and full decision-making capacity is visited by an adult child, who does most of the talking. When the social worker mentions the upcoming scan, the patient says, 'Just give the results to my child. I don't want to hear them.' The oncologist's usual practice is to give results directly to patients. What should the social worker do **FIRST**?

**A.** Encourage the patient to hear the results from the oncologist
**B.** Request an ethics consultation on keeping results from a patient
**C.** Meet privately with the patient to confirm the wish
**D.** Help the team record the wish and give results to the adult child

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). Hearing results directly is the usual practice, but a capable patient may decline information. Urging the patient to receive results they have refused puts the team's view of truthfulness ahead of the patient's autonomy.

**B** **Incorrect** (trap: Hands off too soon). Ethics consultation helps when duties remain in conflict after analysis. Here a short private conversation can settle whether the wish is the patient's own, so a consultation is premature.

**C** **Correct.** Respect for self-determination includes a capable patient's choice not to receive information and to involve family in decisions (NASW Code 1.02), but the social worker must first be sure the wish is the patient's own. The request came with the adult child present and doing most of the talking, so a private conversation about what the patient wants to know, and who should hear it, comes first. It beats D because recording the wish and routing results is the right next step once the private talk confirms the preference.

**D** **Incorrect** (trap: Right action, wrong time). Documenting the wish and sharing results with the adult child, which HIPAA permits when the patient agrees (45 CFR 164.510(b)), is right once the preference is confirmed as the patient's own. Acting on a request made in front of a relative who dominates the conversation skips that check.

**Takeaway:** When a capable patient waives information in front of family, confirm the wish privately; then the patient may direct results to family.

**Sources:** NASW Code of Ethics (2021), 1.02; HIPAA Privacy Rule, 45 CFR 164.510(b)
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### 68. ETH-1178 (rev 3) · Best answer: C

A hospice patient with full decision-making capacity wants to ride 300 miles by car to see a sibling one last time. The nurse has told the patient that the drive will be exhausting and could worsen symptoms. The team asks the social worker, who has not yet met the patient, to talk the patient out of it. What should the social worker do **FIRST**?

**A.** Suggest a video visit instead, as a safer way to see the sibling
**B.** Help the patient plan the trip with the team's precautions
**C.** Explore with the patient what the visit means to them

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). A video visit can be meaningful and may be offered as one option. Proposing it before hearing what the in-person visit means steers the patient toward the team's preference instead of the patient's own goals.

**B** **Incorrect** (trap: Right action, wrong time). Helping a capable patient carry out a risky choice as safely as possible, with rest stops and symptom medication, fits hospice values and is likely to follow. It comes after the social worker has heard what the visit means to the patient and how the patient weighs the risks.

**C** **Correct.** In hospice, care follows what matters most to the patient, and a capable patient's choice deserves to be understood before it is supported or questioned (NASW Code 1.02; Ethical Principles: Dignity and Worth of the Person). The social worker has not yet spoken with the patient and the team wants persuasion, so learning what the visit means comes first; it also equips the worker to help the team see the choice clearly. It beats B because planning the trip is likely the next step, but planning before hearing the patient skips the conversation that grounds the plan in the patient's goals.

**Takeaway:** When a capable hospice patient makes a risky choice, first learn what it means to the patient, then help carry it out as safely as possible.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), Ethical Principles: Dignity and Worth of the Person

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## 69. ETH-0102 (rev 1) · Best answer: B

At intake at a community mental health agency, a client says they do not read well and asks the social worker to 'just show me where to sign' the consent form. The client speaks English fluently. What is the **MOST** effective way for the social worker to obtain informed consent?

**A.** Read the form aloud, then have the client sign it

**B.** Explain each part plainly and check understanding

**C.** Point to the signature line, as the client requested

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Reading the form aloud helps a client who reads little, but it is a one-way step. Without checking what the client understood and inviting questions, the signature may still fall short of informed consent.

**B** **Correct.** Valid consent requires that the client understands the purpose, risks, costs, alternatives and the right to refuse. When literacy is limited, the Code calls for a detailed verbal explanation and steps to ensure comprehension (NASW Code 1.03(a), 1.03(b)). Explaining each element in plain language and asking the client to say it back confirms understanding. It beats A because reading the form aloud gives the client the words but does not show that the client grasped them or had a chance to ask questions.

**C** **Incorrect** (trap: Misreads law or ethics). A signature is not the same as informed consent. Honoring the request to skip the explanation leaves the client agreeing to services they may not understand, which the Code does not treat as valid consent.

**Takeaway:** Informed consent means understanding, not a signature; with limited literacy, explain in plain words and confirm comprehension.

**Sources:** NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.03(b)
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### 70. ETH-1020 (rev 1) · Best answer: C

A supervisor at a family service agency asks a social worker to video-record sessions with an adult client for use in supervision. The client has not yet heard about the request. Before the next session, what is the **MOST** appropriate way for the social worker to proceed?

**A.** Tell the client the agency requires recording for supervision
**B.** Start recording and tell the client they may ask to stop
**C.** Explain the recording's purpose and use and ask for consent

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). Presenting recording as an agency requirement pressures the client and suggests they have no choice. Recording for supervision depends on the client's informed agreement, and services should continue if the client declines.

**B** **Incorrect** (trap: Misuses the technique). Letting the client stop once recording has started treats silence as agreement. Consent must be informed and given before recording begins, after the client understands its purpose and use.

**C** **Correct.** Social workers obtain clients' informed consent before audio or video recording, or before observation by a third party (NASW Code 1.03(h)). Valid consent means explaining the purpose, who will see the recording, how it will be stored and destroyed, and that the client may decline without losing services (NASW Code 1.03(a)). C beats B because an opt-out after recording starts is not informed consent; the client must agree before any recording begins.

**Takeaway:** Get informed consent before recording a session, explaining purpose, viewers, storage and the right to decline.

**Sources:** NASW Code of Ethics (2021), 1.03(h); NASW Code of Ethics (2021), 1.03(a)
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### 71. ETH-1021 (rev 1) · Best answer: C

A resident of a skilled nursing facility has major neurocognitive disorder and has been found to lack capacity to make health care decisions. The resident's adult child, the legally appointed health care agent, has consented to the resident joining a weekly reminiscence group. What should the social worker do **NEXT**?

**A.** Enroll the resident now, since the agent has already consented
**B.** Have the resident sign the group consent form too
**C.** Explain the group simply and ask the resident's assent
**D.** Postpone the group until capacity improves

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). The agent's consent authorizes participation, so enrolling is the step that follows. Doing it before explaining the group to the resident skips the Code's duty to inform the client and seek their agreement.

**B** **Incorrect** (trap: Misuses the technique). A person found to lack decision-making capacity cannot give valid legal consent, so a signature adds nothing legally. The resident's assent is sought through a simple explanation, not a form.

**C** **Correct.** When a client lacks capacity, the social worker seeks permission from an appropriate third party and also informs the client in a way the client can understand, acting consistently with the client's wishes and interests (NASW Code 1.03(c); NASW Code 1.14). With the agent's consent obtained, the next step is to explain the group simply and invite the resident's agreement. C beats A because the agent's consent makes participation lawful, but enrolling without involving the resident skips the duty to inform and seek assent.

**D** **Incorrect** (trap: Good but not best). Major neurocognitive disorder is usually progressive, so waiting for capacity to return could deny the resident a helpful activity indefinitely. Third-party consent plus the resident's assent allows participation now.

**Takeaway:** When a client lacks capacity, get consent from the authorized third party and still explain services to the client and seek assent.

**Sources:** NASW Code of Ethics (2021), 1.03(c); NASW Code of Ethics (2021), 1.14
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## 72. ETH-1022 (rev 1) · Best answer: A

A social worker trained in prolonged exposure therapy has provided supportive therapy for 4 months to an adult client with posttraumatic stress disorder. The social worker now believes prolonged exposure would help and wants to begin it. What should the social worker do **FIRST**?

**A.** Discuss the approach, its risks and alternatives with the client
**B.** Have the client sign an updated treatment consent form
**C.** Begin with a short trial exposure to gauge the response

**Why each option is right or wrong**

**A** **Correct.** Informed consent is an ongoing process that is revisited whenever services change. Before starting a new treatment, the social worker explains its purpose, likely benefits and risks, such as a temporary rise in distress, reasonable alternatives and the client's right to refuse, and gives the client a chance to ask questions (NASW Code 1.03(a)). A beats B because a signature without discussion records agreement without making sure the client understands what exposure involves.

**B** **Incorrect** (trap: Misuses the technique). An updated signature may be part of the process, but signing alone does not ensure understanding. The discussion of purpose, risks and alternatives comes first; the form documents it.

**C** **Incorrect** (trap: Right action, wrong time). A trial exposure is still the new treatment. Starting it before the client understands and agrees to the approach bypasses informed consent, even if the session is short.

**Takeaway:** Revisit informed consent whenever treatment changes: explain the new approach, its risks and alternatives before starting.

**Sources:** NASW Code of Ethics (2021), 1.03(a)
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### 73. ETH-1023 (rev 1) · Best answer: B

A client ordered by a court to attend a driving-under-the-influence program asks the social worker at intake, 'What happens if I don't want to do this?' What is the **MOST** appropriate response?

**A.** Tell the client the court order removes the right to refuse
**B.** Explain the right to decline and what is reported
**C.** Suggest the client ask their attorney before going further

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). A court order creates consequences for refusing, but the client can still choose to decline and accept them. Saying the right to refuse is gone misstates the client's position and undermines informed consent.

**B** **Correct.** Mandated clients keep the right to make informed choices. The social worker explains the nature and extent of services and the extent of the client's right to refuse (NASW Code 1.03(d)), including what the program will report to the court and the likely consequences of declining. B beats C because legal advice belongs to the attorney, but explaining the program, its reporting and the client's choices is the social worker's own consent duty.

**C** **Incorrect** (trap: Hands off too soon). The attorney can advise on legal consequences, and the worker might suggest that as well. The social worker must still explain the services, reporting and right to refuse; sending the client away first leaves the consent duty unmet.

**Takeaway:** Mandated clients may refuse services; explain the services, what will be reported, and the consequences of declining.

**Sources:** NASW Code of Ethics (2021), 1.03(d)
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### 74. ETH-1024 (rev 2) · Best answer: C

A parent brings an 8-year-old to an outpatient clinic for anxiety treatment. The parent says the child's other parent lives across town after a recent divorce and 'doesn't need to know about this.' The parent is unsure what the custody order says about medical decisions. What should the social worker do **FIRST**?

**A.** Begin treatment with the consent of the parent who is present
**B.** Call the other parent to ask for permission to treat
**C.** Review the custody order to confirm who may consent
**D.** Require both parents' signatures before any sessions

**Why each option is right or wrong**

**A** **Incorrect** (trap: Skips assessment). Starting treatment assumes the present parent has authority to consent alone. That may be true, but it is unknown until the custody order is checked; treatment can begin once authority is confirmed.

**B** **Incorrect** (trap: Right action, wrong time). The other parent may need to consent, depending on the custody order. Calling before checking is premature and may disclose the child's treatment unnecessarily.

**C** **Correct.** Treating a child requires consent from a person with legal authority to make the child's health care decisions (NASW Code 1.03(c); 45 CFR 164.502(g)(3)). After a divorce, that authority depends on the custody order, which may give decision-making to one parent or require both. The social worker first reviews the order. C beats B because contacting the other parent may be needed, but only after the order shows whether that parent's consent is required.

**D** **Incorrect** (trap: Good but not best). Requiring both signatures may be more than the law requires if one parent holds sole decision-making authority, and it can delay care. The custody order shows what consent is actually needed.

**Takeaway:** Before treating a child of separated parents, check the custody order to confirm who has authority to consent.

**Sources:** NASW Code of Ethics (2021), 1.03(c); HIPAA Privacy Rule, 45 CFR 164.502(g)(3)
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### 75. ETH-1025 (rev 2) · Best answer: A

An adult client at a group private practice asks to use their employer health plan to pay for counseling. The plan requires a diagnosis on every claim and reviews progress after 12 sessions before approving more. The client is about to sign the consent form. What is **MOST** important for the social worker to explain before services begin?

**A.** What the plan will receive and how it may limit sessions
**B.** That session content will stay between the client and worker
**C.** That the client may drop the plan and self-pay later

**Why each option is right or wrong**

**A** **Correct.** Informed consent includes the limits that third-party payers place on services (NASW Code 1.03(a)). The decisive facts are that the plan will see a diagnosis on every claim and can stop approving sessions after review. Both affect the client's privacy and the length of care, so the client needs them before agreeing. A beats C because self-pay is a useful alternative, but the client can weigh it only after learning what using insurance involves. Any disclosure to the plan also needs the client's authorization (NASW Code 1.07(h)).

**B** **Incorrect** (trap: Misreads law or ethics). Promising that session content stays private is inaccurate here, because the plan will receive a diagnosis and may review progress. A false assurance undermines consent and damages trust once the client learns what the insurer sees.

**C** **Incorrect** (trap: Good but not best). Self-pay is a reasonable alternative worth mentioning, and it may become the client's choice. It helps only after the client understands what insurance involves; offered first, it skips the payer disclosure the client needs to decide.

**Takeaway:** Before services begin, tell clients what a third-party payer will receive and how it may limit their care.

**Sources:** NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.07(h)
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## 76. ETH-1026 (rev 1) · Best answer: B

A 16-year-old comes to a community health clinic without a parent and asks the social worker for help with daily drinking. The teen does not want a parent contacted and is ready to start today. What should the social worker do **FIRST**?

**A.** Call a parent to get consent before any counseling
**B.** Determine whether state law lets the teen consent alone
**C.** Begin counseling, since the teen is asking for help

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). In states that let minors consent to substance use treatment, calling a parent without the teen's agreement could breach confidentiality and drive the teen away. Parental consent is the right route only where state law requires it.

**B** **Correct.** Minors usually need a parent's consent, but many states let minors consent on their own to substance use or mental health care, and the rules vary. The first step is to learn which rule applies, because it decides who may consent and who controls information about the care (45 CFR 164.502(g)(3); NASW Code 1.03(a)). B beats C because starting today may be lawful, but only if the state allows minor consent. It beats A because calling a parent may override protections the teen has under that law.

**C** **Incorrect** (trap: Right action, wrong time). Starting today is the right move if state law allows the teen to consent alone. Beginning before checking risks treating a minor without valid consent in a state that requires a parent's permission.

**Takeaway:** When a minor seeks care alone, determine whether state law permits minor consent before deciding whom to involve.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.502(g)(3); NASW Code of Ethics (2021), 1.03(a)
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### 77. ETH-1027 (rev 3) · Best answer: D

A 28-year-old client with moderate intellectual developmental disorder lives in supported housing. The client's legal guardian, a sibling, has signed consent for the client to attend a sheltered day program five days a week. In planning meetings, which the guardian has not attended, the client says again and again that they want to keep their part-time grocery job instead. What should the social worker do **FIRST**?

**A.** Start the day program, since the guardian has consented
**B.** Treat the client's preference as overriding the guardian
**C.** Petition the court to review the guardianship
**D.** Bring the client's stated wishes to the guardian

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). The guardian does hold legal authority, so enrolling can look correct. Acting on that consent while the client's clear objection has not reached the decision-maker skips the worker's duty to see that the surrogate's choice reflects the client's wishes.

**B** **Incorrect** (trap: Misreads law or ethics). The client's voice matters, but a guardian holds legal authority for this decision. Treating the client's preference as automatically overriding it misreads the law; the worker's role is to make sure the guardian hears and weighs it.

**C** **Incorrect** (trap: Skips assessment). Court review is a serious step for a guardian who acts against the person's interests or refuses to consider their wishes. Nothing yet shows the guardian has heard the objection, so a petition comes far too early.

**D** **Correct.** When a client cannot give informed consent, the social worker seeks permission from an appropriate third party, explains matters at the client's level, and makes sure the third party acts in line with the client's wishes and interests (NASW Code 1.03(c)). The client's repeated, clear preference for the job is exactly what the guardian must weigh, so the worker first brings it forward (NASW Code 1.14). D beats A because the guardian's signature is legally valid, but enrolling before the guardian considers the client's objection skips that duty.

**Takeaway:** Surrogate consent should reflect the client's wishes and interests; bring the client's stated preferences to the decision-maker before acting.

**Sources:** NASW Code of Ethics (2021), 1.03(c); NASW Code of Ethics (2021), 1.14
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### 78. ETH-1028 (rev 1) · Best answer: C

Before a first session at an outpatient clinic, a social worker's colleague suggests looking up the new client's public social media accounts 'to get the full picture.' The referral describes mild anxiety about a job change. How should the social worker handle the suggestion?

**A.** Look at public posts, which anyone can already see
**B.** Search now, then tell the client what turned up
**C.** Seek the client's consent before doing any search

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Public availability does not cancel the consent rule. The Code treats an electronic search about a client as needing consent unless safety or another compelling reason applies, and mild job-related anxiety offers neither.

**B** **Incorrect** (trap: Misreads law or ethics). Telling the client afterward is honest, but it turns consent into notice. The client loses the chance to decide before the search happens, which is the point of the consent standard.

**C** **Correct.** Social workers obtain client consent before searching electronically for information about a client, unless the search protects someone from serious, foreseeable and imminent harm or another compelling reason exists (NASW Code 1.03(i)). A referral for mild anxiety about a job change gives no such reason. C beats A because public visibility does not remove the requirement; the Code asks workers to avoid gathering client information online without a compelling reason and, when appropriate, consent (NASW Code 1.07(q)).

**Takeaway:** Get a client's consent before searching online for information about them, unless imminent harm or another compelling reason exists.

**Sources:** NASW Code of Ethics (2021), 1.03(i); NASW Code of Ethics (2021), 1.07(q)
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### 79. ETH-1029 (rev 1) · Best answer: B

A couple receives services at a university training clinic. At intake, both partners consented in writing to a faculty team observing sessions through a one-way mirror. Midway through the fifth session, one partner says they no longer want the team watching. What should the social worker do **FIRST**?

**A.** Continue, since both partners consented in writing at intake
**B.** Stop the team's observation from this point on
**C.** Ask the team lead to explain why observation helps
**D.** Explore the partner's concerns before making any change

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Signed consent at intake does not bind a client for the rest of treatment. Consent is ongoing and can be withdrawn, and continuing after a clear withdrawal ignores it.

**B** **Correct.** Third-party observation requires informed consent (NASW Code 1.03(h)), and consent includes the right to withdraw it at any time (NASW Code 1.03(a)). The partner's statement withdraws consent now, so the observation stops first. B beats D because exploring the concern is useful, but doing it while the team keeps watching continues observation the client has just refused. Afterward, the worker can discuss what prompted the change and how the couple wants to proceed.

**C** **Incorrect** (trap: Overrides the client's choice). Having the team argue for observation pressures a client who has already said no. Information about benefits belongs in the original consent discussion, not in an effort to reverse a withdrawal.

**D** **Incorrect** (trap: Right action, wrong time). Understanding the partner's concerns is a good next step and may lead to a new agreement. Doing it before stopping the observation lets the team keep watching after consent was withdrawn.

**Takeaway:** Consent to observation or recording can be withdrawn at any time; stop when a client withdraws, then discuss the concern.

**Sources:** NASW Code of Ethics (2021), 1.03(h); NASW Code of Ethics (2021), 1.03(a)
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## 80. ETH-1030 (rev 2) · Best answer: A

A client comes to an outpatient substance use program's walk-in hours asking to start treatment today. The client smells of alcohol, slurs some words, says they had 'a few drinks' an hour ago, and walks steadily. The intake packet includes the consent to treatment. What is the **BEST** way for the social worker to proceed?

**A.** Offer brief support now and hold consent until sober
**B.** Complete the consent forms while motivation is high
**C.** Ask the client to return another day when sober

#### Why each option is right or wrong

**A** **Correct.** Valid consent requires that the client understand the services, risks and rights and choose freely (NASW Code 1.03(a)). Recent drinking with slurred speech can impair that understanding for now. The best response keeps the motivated client engaged with brief support and a set return time while postponing formal consent until the client is sober. A beats B because a signature given while impaired may not reflect informed consent. It beats C because sending the client away entirely risks losing a moment of readiness.

**B** **Incorrect** (trap: Misreads law or ethics). Motivation matters, but a signature given while intoxicated may not reflect real understanding of services, risks and rights. Consent obtained now could need redoing and weakens trust in the process.

**C** **Incorrect** (trap: Good but not best). Waiting for sobriety is right for the formal consent, but turning the client away entirely risks losing a motivated person. Brief support and a set return time keep engagement while protecting consent.

**Takeaway:** Postpone formal consent when intoxication impairs understanding, but keep a motivated client engaged until consent can be valid.

**Sources:** NASW Code of Ethics (2021), 1.03(a)
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## 81. ETH-1031 (rev 1) · Best answer: C

When a parent gives legal permission for a 10-year-old's treatment, what term describes the child's own agreement to take part?

**A.** Proxy consent
**B.** Implied consent
**C.** Assent
**D.** Substituted judgment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Proxy consent is permission given by someone authorized to decide for another person, such as the parent here. It names the parent's role, not the child's agreement.

**B** **Incorrect** (trap: Wrong model or concept). Implied consent is inferred from conduct or assumed in emergencies, such as treating an unconscious patient. It does not describe a child's stated agreement to planned services.

**C** **Correct.** Assent is a child's agreement to services, sought at a level the child can understand, while the parent provides legal permission (Katz & Webb, 2016). Seeking assent respects the child's growing autonomy and parallels the Code's duty to inform people who cannot give legal consent (NASW Code 1.03(c)). C beats A because proxy consent describes the parent's permission given on the child's behalf, not the child's own agreement.

**D** **Incorrect** (trap: Wrong model or concept). Substituted judgment is a standard a surrogate uses to decide as the person would have chosen. It guides decision-makers for adults who have lost capacity, not a child's agreement.

**Takeaway:** Parents give legal permission for a child's treatment; the social worker also seeks the child's assent at a developmentally suitable level.

**Sources:** Katz, A. L., Webb, S. A., & AAP Committee on Bioethics (2016). Informed consent in decision-making in pediatric practice. Pediatrics, 138(2), e20161485, Assent and parental permission; NASW Code of Ethics (2021), 1.03(c)
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## 82. ETH-1032 (rev 2) · Best answer: B

A community mental health agency has started offering text-message check-ins between sessions. An adult client in the depression program asks to begin receiving them this week. What should the social worker do **FIRST**?

**A.** Text the client the policy and start once the client replies OK
**B.** Review the texting policy and its risks, then get consent
**C.** Agree, and invite the client to text during any crisis

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Sharing the written policy is part of informed consent, which makes this tempting. A one-word reply by text, though, is not a discussion of privacy risks, response times and crisis limits, and it gives the client no real chance to ask questions before services start.

**B** **Correct.** Before technology-based services start, social workers discuss their technology policies and obtain informed consent (NASW Code 1.03(e), 1.03(f)). For texting, that means explaining privacy risks on shared phones, expected response times, what goes in the record, and that texts are not for emergencies. B beats A because a texted policy and a reply of 'OK' skip the discussion of risks and limits that makes consent informed.

**C** **Incorrect** (trap: Ignores risk). Texting is poorly suited to crises because messages can go unseen for hours. Inviting crisis texts creates a false safety net; clients need a crisis plan that uses phone lines or emergency services.

**Takeaway:** Before starting any technology-based service, explain the policies and risks and obtain informed consent.

**Sources:** NASW Code of Ethics (2021), 1.03(e); NASW Code of Ethics (2021), 1.03(f)
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## 83. ETH-1135 (rev 2) · Best answer: B

A patient on an inpatient psychiatric unit has schizophrenia and is preparing for discharge. The team suggests that the patient's parent sign consent for a supported housing program. The patient has no guardian. In a capacity evaluation, the psychiatrist documents that the patient can describe the program, weigh it against living alone and state a choice. What should the social worker do **NEXT**?

**A.** Ask the parent to sign the program consent as the team suggested
**B.** Review the program with the patient and obtain their own consent
**C.** Ask the patient and parent to sign the consent together

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). A parent who is not a guardian has no legal authority to consent for an adult patient. Third-party consent is for clients who lack capacity, and this patient's capacity has been documented.

**B** **Correct.** Adults are presumed able to make their own decisions, and a diagnosis alone does not establish incapacity (Appelbaum, 2007). The psychiatrist has documented that this patient understands the program, weighs options and states a choice, so the patient gives informed consent in clear language with a chance to ask questions (NASW Code 1.03(a)). Third-party consent is for clients who lack capacity (1.03(c)). B beats C because a parent's co-signature adds no legal authority and suggests the patient cannot decide alone; the parent can be involved in planning if the patient wishes.

**C** **Incorrect** (trap: Overrides the client's choice). Joint signing looks like a respectful way to include family, but the parent has no legal role in this adult's consent, and a

co-signature implies the patient's own consent is not enough. The parent can join discharge planning if the patient wants that.

**Takeaway:** A psychiatric diagnosis is not incapacity; when capacity is documented, the client gives their own informed consent.

**Sources:** NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.03(c); Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, presumption of competence; diagnosis alone not determinative
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## 84. ETH-1136 (rev 2) · Best answer: A

Shelter staff tell a new resident that their bed will be given to someone else unless they sign up for the agency's counseling program today. The resident accurately explains the program's purpose, risks and alternatives, then signs the consent form. Which element of valid consent is **MOST** likely missing?

**A.** Voluntariness
**B.** Disclosure
**C.** Comprehension

#### Why each option is right or wrong

**A** **Correct.** Valid consent rests on disclosure, comprehension, capacity and voluntariness, and consent given under coercion or undue influence is not valid (Reamer, 2018). The resident's accurate account of the program's purpose, risks and alternatives shows that information was given, understood and weighed, but the threat of losing a bed makes the choice unfree. The Code also expects clients to know they may refuse or withdraw consent (NASW Code 1.03(a)). A beats C because the resident's grasp of the program is intact; what is missing is a free choice.

**B** **Incorrect** (trap: Wrong model or concept). Disclosure means the needed information was given. The resident can describe the purpose, risks and alternatives, so the information was provided.

**C** **Incorrect** (trap: Wrong model or concept). Comprehension is the client's understanding of what was disclosed. The resident explains the program accurately, so understanding is present; the problem is the pressure behind the signature.

**Takeaway:** Valid consent must be voluntary; agreement given under threat of losing housing is not free, even when the client fully understands.

**Sources:** Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, informed consent: coercion and undue influence; NASW Code of Ethics (2021), 1.03(a)
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### 85. ETH-1137 (rev 2) · Best answer: A

A 17-year-old requests counseling at a community clinic for panic attacks. The client lives independently and gives intake a copy of a court order of emancipation. The intake coordinator says a parent must still sign the treatment consent because the client is under 18. What is the **MOST** appropriate way for the social worker to proceed?

**A.** Have the client sign the treatment consent themselves
**B.** Contact a parent to sign the consent before treatment begins
**C.** Have the client sign, then let a parent know care has begun

**Why each option is right or wrong**

**A** **Correct.** Emancipated minors are treated as adults for consent and may agree to or refuse their own care without a parent (Davis & Fang, 2023). HIPAA's parental-representative rule applies to unemancipated minors (45 CFR 164.502(g)(3)), so this client also controls their own health information. The social worker obtains informed consent from the client in clear language (NASW Code 1.03(a)). A beats B because the court order gives the client adult decision-making authority; requiring a parent's signature ignores it and could disclose the client's treatment.

**B** **Incorrect** (trap: Misreads law or ethics). Parental consent is the general rule for minors, which is why intake raised it. Emancipation removes that requirement, and contacting a parent without the client's agreement would also disclose the client's treatment.

**C** **Incorrect** (trap: Misreads law or ethics). The client's own signature is right, which makes this tempting. Telling a parent without the client's agreement, though, discloses treatment the client controls; an emancipated minor decides who is told (45 CFR 164.502(g)(3)).

**Takeaway:** An emancipated minor consents to their own treatment and controls their own health information, without a parent's signature.

**Sources:** Davis, M., & Fang, A. (2023). Emancipated Minor. StatPearls. NCBI Bookshelf, consent to medical care without parental permission; HIPAA Privacy Rule, 45 CFR 164.502(g)(3); NASW Code of Ethics (2021), 1.03(a)
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### 86. ETH-1138 (rev 1) · Best answer: C

A child welfare court orders a parenting capacity assessment, and a social worker is assigned to complete it. At the first meeting, the parent says, 'I'm so glad I finally have someone to talk to about everything.' What should the social worker do **FIRST**?

**A.** Begin the interview while the parent is open and engaged
**B.** Offer supportive counseling sessions alongside the assessment
**C.** Explain the evaluator role and who will receive the report

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Interviewing is the core of the assessment and follows soon. Starting while the parent believes the worker is a confidential helper means they share information without knowing it will go to the court.

**B** **Incorrect** (trap: Boundary slip or collusion). Combining evaluator and counselor roles creates a dual relationship with a risk of harm, since the worker would judge the same person they are helping (1.06(c)). A referral for separate counseling is the better route if the parent wants support.

**C** **Correct.** When services are involuntary, clients are told what the service involves and how far they can refuse it, and consent rests on a clear explanation of purpose and limits (NASW Code 1.03(a), 1.03(d)). The parent's comment shows they expect a confidential helping relationship, but a court-ordered assessment goes to the court and the child welfare agency. Clarifying the role, the report's recipients and the limits of confidentiality comes before any interviewing. C beats A because information gathered while the parent misunderstands the role is not given with informed consent.

**Takeaway:** In court-ordered evaluations, explain the role, report recipients and confidentiality limits before gathering any information.

**Sources:** NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.03(d); NASW Code of Ethics (2021), 1.06(c)

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## 87. ETH-1139 (rev 2) · Best answer: A

A new client has filed a lawsuit against a former employer, claiming emotional distress from workplace harassment. The client wants therapy focused on the harassment and its effects on sleep and mood. At intake, the client asks what the social worker will write down. What is the **BEST** response?

**A.** Explain that the lawsuit may lead to requests for the records
**B.** Recommend waiting to start therapy until the lawsuit is resolved
**C.** Agree to keep notes brief and leave out harassment details
**D.** Assure the client that therapy records are privileged in court

#### Why each option is right or wrong

**A** **Correct.** Informed consent includes the risks of services in clear language, and the limits of confidentiality are discussed early (NASW Code 1.03(a), 1.07(e)). When a client claims emotional distress in a lawsuit, the other side may seek therapy records, and privilege may not protect them. Knowing this, the client can decide what to discuss and can consult their attorney. A beats D because promising privilege overstates a protection that a client can lose by putting their mental state at issue.

**B** **Incorrect** (trap: Too directive). Delaying care would leave the client without help for sleep and mood problems now. Whether to start despite the legal risk is the client's decision once informed; the worker's role is to inform, not to steer.

**C** **Incorrect** (trap: Misreads law or ethics). Brief notes can protect privacy, but records must be accurate and reflect the services provided (3.04(a)). Leaving out the main focus of treatment to shape a lawsuit makes the record misleading.

**D** **Incorrect** (trap: Misreads law or ethics). Privilege belongs to the client, but a client who claims emotional distress in court may waive it for related records. Promising protection gives the client false security about a real risk.

**Takeaway:** Before treatment, tell clients in litigation that their records may be sought, so they can decide with that knowledge.

**Sources:** NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.07(e); NASW Code of Ethics (2021), 3.04(a)
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### 88. ETH-1140 (rev 2) · Best answer: B

Police bring an adult to the emergency department on an emergency hold after the person threatened to jump from a bridge. The person refuses to sign any forms and says, 'I'm not agreeing to anything.' The social worker is assigned to complete the psychosocial assessment. What should the social worker do **FIRST**?

**A.** Start the assessment interview and return to the forms later
**B.** Explain the purpose of the assessment and what may be shared
**C.** Wait to begin until the person agrees to sign the forms

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). The interview is the core of this assessment and comes next. Starting it before explaining its purpose and limits leaves the person unaware of how their answers will be used.

**B** **Correct.** Clients receiving services involuntarily are told what the service involves and how far they can refuse it (NASW Code 1.03(d)), and the limits of confidentiality are discussed early (1.07(e)). The emergency hold allows the assessment to go forward without a signature, but the person still deserves to know why the worker is there, what it will cover and who will see it. B beats A because starting the interview without this explanation treats the person as having no say in the process.

**C** **Incorrect** (trap: Ignores risk). The hold permits the assessment without signed consent, and the person was at risk of jumping. Waiting for a signature delays an urgent safety assessment.

**Takeaway:** Even when consent is not required, tell involuntary clients what the service involves, what they may refuse and who will see results.

**Sources:** NASW Code of Ethics (2021), 1.03(d); NASW Code of Ethics (2021), 1.07(e)
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### 89. ETH-1103 (rev 2) · Best answer: B

A clinic's waitlist for therapy intake has grown to about 6 weeks. Before a meeting with hospital discharge planners, the clinic director tells the outreach social worker to keep promising 'appointments within the week' so referrals and funding stay steady. Several patients referred last month waited weeks without care. What should the social worker do **FIRST**?

**A.** Describe the program to planners as the director instructed
**B.** Tell the director that planners need the actual wait time
**C.** Report the director's instruction to the clinic's board
**D.** Give planners the actual wait without telling the director

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Repeating a claim the social worker knows is false makes the worker part of the deception (4.04). An employer's instruction does not excuse misrepresenting services to the public (3.09(d)).

**B** **Correct.** Social workers make sure their representations to the public about services are accurate, and they do not take part in deception (NASW Code 4.06(c), 4.04; Integrity principle). Promising same-week care when the wait is 6 weeks misleads referral sources, and patients have already gone without timely care. Social workers make employers aware of their ethical obligations (3.09(c)) and do not let administrative orders interfere with ethical practice (3.09(d)), so a direct conversation with the director comes first. B beats D because talking with the director first gives the agency a chance to correct its message and plan for referrals; accurate information to planners follows either way.

**C** **Incorrect** (trap: Skips assessment). Going to the board may become necessary if the director insists on misleading referral sources. Doing it before any conversation skips the direct step the Code expects first.

**D** **Incorrect** (trap: Right action, wrong time). Planners must get accurate information, but bypassing the director leaves the instruction in place for other staff and later meetings. Raising it first lets the agency fix its message; accuracy at the meeting is required whatever the director says.

**Takeaway:** Descriptions of services must be accurate; when told to mislead, raise it with the decision-maker first and never repeat the false claim.

**Sources:** NASW Code of Ethics (2021), 4.06(c); NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 3.09(d); NASW Code of Ethics (2021), 3.09(c); NASW Code of Ethics (2021), Ethical Principles: Integrity
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## 90. ETH-1104 (rev 1) · Best answer: C

A hospital social worker helped a patient begin a disability benefits appeal and said the filing deadline was 30 days away. A week later, rereading the denial letter, the social worker sees the deadline is actually 3 days away. The patient was discharged home yesterday and has a working phone. What should the social worker do **FIRST**?

**A.** Tell the supervisor and wait for direction on the patient
**B.** Correct the deadline date in the patient's chart
**C.** Call today with the correct date and help submit it

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Supervisors should learn of errors that affect clients, but waiting for direction risks missing a 3-day deadline. Informing the supervisor fits right after the patient has the correct date and a plan to file.

**B** **Incorrect** (trap: Good but not best). The correction belongs in the record, but a chart note does nothing to help the patient file on time. Documentation follows the call to the patient.

**C** **Correct.** The Integrity principle calls for honesty and responsibility, which includes telling clients about one's own errors that affect them, and client well-being is the

primary responsibility (NASW Code 1.01; Integrity principle). The deadline is 3 days away, so the patient needs the correct date and help filing now. C beats A because waiting for supervisory direction could cost the appeal; the supervisor should be told promptly, but not before the patient knows. Concealing the error would also conflict with the bar on deception (4.04).

**Takeaway:** When a social worker's own error threatens a client's interests, tell the client promptly and help fix it before other steps.

**Sources:** NASW Code of Ethics (2021), Ethical Principles: Integrity; NASW Code of Ethics (2021), 1.01; NASW Code of Ethics (2021), 4.04

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## 91. ETH-1105 (rev 1) · Best answer: B

A social worker in a group practice completes a 2-day workshop in a trauma treatment model. The practice's marketing coordinator then updates the website to call the social worker a 'certified trauma specialist.' Certification in the model requires supervised cases that the social worker has not yet begun. What should the social worker do?

**A.** Keep the description while starting the certification cases

**B.** Have the profile describe the training actually received

**C.** Tell new clients at intake that certification is pending

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Planning to finish certification is good, and the Code allows accepting work while acquiring competence. Advertising the credential before earning it, though, misrepresents qualifications to the public (4.06(c)).

**B** **Correct.** Social workers make sure representations of their credentials and competence are accurate, claim only credentials they hold, and correct others' inaccurate statements about them (NASW Code 4.06(c)). A workshop is a start, but competence in a new technique also takes training, consultation and supervision (1.04(b)), which the NASW Competence principle reinforces. The coordinator made the error, yet correcting it is the social worker's duty. B beats A because intending to earn a credential does not make the claim accurate now, and the public reads the website today.

**C** **Incorrect** (trap: Good but not best). Telling clients at intake helps those who arrive, but the website keeps misleading everyone who reads it, including referral sources. The inaccurate public statement itself must be corrected.

**Takeaway:** Claim only credentials actually held, and correct others' inaccurate descriptions of one's qualifications.

**Sources:** NASW Code of Ethics (2021), 4.06(c); NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), Ethical Principles: Competence

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### 92. ETH-1106 (rev 2) · Best answer: D

A client with panic disorder asks a social worker to use a 'one-session trauma release' technique that online videos say ends panic in a single visit. The social worker, who is trained in cognitive-behavioral therapy for panic, finds no published research supporting the technique. The client has not had any prior therapy for panic and says quick relief is the goal. What is the **BEST** response?

**A.** Ask what draws the client to this technique
**B.** Try the technique once, as the client prefers
**C.** Refer the client to a practitioner who offers the technique
**D.** Explain the evidence and offer an established treatment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Understanding a request helps when its reasons are unclear. This client has explained the appeal (the videos' promise of one-visit relief) and named quick relief as the goal, so the useful next move is to share what the evidence shows and offer effective options.

**B** **Incorrect** (trap: Outside the social work role). Self-determination covers choices among legitimate options, but it does not oblige a social worker to deliver an unsupported technique without training. Competent practice rests on recognized knowledge and training (4.01(c), 1.04(b)).

**C** **Incorrect** (trap: Hands off too soon). Referral fits when another professional's expertise is needed (1.16(a)). Here it would steer the client toward an unsupported approach before offering the effective treatment the worker can provide.

**D** **Correct.** Social workers base practice on recognized knowledge, including empirically based knowledge (NASW Code 4.01(c)), and informed consent includes discussing alternatives (1.03(a)). Explaining that the technique lacks research support while offering an established panic treatment, which the worker is trained to provide, respects the client's goal of quick relief and keeps the choice informed. Self-determination is honored by informing the client, not by delivering an unsupported technique (1.02). D beats A because the client has already named the reason for the request; more exploration only delays the information needed to choose.

**Takeaway:** Clients choose among informed options; explain the evidence and offer effective treatment rather than deliver an unsupported technique on request.

**Sources:** NASW Code of Ethics (2021), 4.01(c); NASW Code of Ethics (2021), 1.03(a); NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), 1.16(a)
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### 93. ETH-1107 (rev 1) · Best answer: A

Residents of a public housing complex tell a community social worker that the city plans to close the neighborhood's only public health clinic. Several residents want to speak at next month's city council hearing but have never addressed public officials. What is the **MOST** appropriate role for the social worker?

**A.** Help residents prepare and deliver their own testimony
**B.** Testify at the hearing on the residents' behalf
**C.** Stay neutral because the closure is a political matter

**Why each option is right or wrong**

**A** **Correct.** Social workers help the public take informed part in shaping social policies and institutions (NASW Code 6.02) and take political action for access to needed services (6.04(a)). Residents have said they want to speak, so helping them prepare and present testimony builds their capacity and puts the people affected in front of decision-makers, in keeping with the NASW Social Justice principle. A beats B because speaking for residents who want to speak for themselves replaces their voice; the worker might add testimony too, but not instead of theirs.

**B** **Incorrect** (trap: Good but not best). Professional testimony can support a community's case, but these residents want to speak. Speaking in their place limits their participation and the weight that comes from people directly affected.

**C** **Incorrect** (trap: Misreads law or ethics). The Code calls for political action and public participation to protect access to services (6.04(a)). Staying neutral on a closure that cuts a neighborhood's health access would ignore that responsibility.

**Takeaway:** Support people affected by a policy to take part in shaping it themselves, rather than speaking for them.

**Sources:** NASW Code of Ethics (2021), 6.02; NASW Code of Ethics (2021), 6.04(a); NASW Code of Ethics (2021), Ethical Principles: Social Justice
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### 94. ETH-1108 (rev 2) · Best answer: C

A client in treatment for depression works at a warehouse. The client describes unpaid overtime, a paycheck $600 short, and a supervisor who threatens to cut hours if workers complain. The social worker has explored how the job affects the client's mood, and the client says recovering the wages would ease both the bills and the hopelessness. What should the social worker do **NEXT**?

**A.** Focus sessions on coping skills for workplace stress
**B.** Explore further what the lost wages mean to the client
**C.** Connect the client with a free workers' rights legal clinic
**D.** Call the employer to request the unpaid wages

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Coping skills can ease stress, but the client has named a concrete injustice whose resolution would lift the depression. Treating only the internal response misses the environmental cause the client identified.

**B** **Incorrect** (trap: Right action, wrong time). Exploration has already been done, and the client has stated a clear goal. Going back over meaning delays action on the problem the client asked for help with.

**C** **Correct.** The Social Justice principle and 6.04(a) direct social workers to challenge injustice, including barriers to fair employment, and 1.16(a) calls for referral when another professional's expertise is needed to serve a client fully (NASW Code). Unpaid wages and retaliation threats are legal problems a workers' rights clinic can address, and the client has said recovering the money would ease both finances and mood. C beats A because coping skills alone treat the distress while leaving the injustice driving it untouched; skills work can continue alongside the referral (NASW Code 1.16(a)).

**D** **Incorrect** (trap: Misreads law or ethics). Contacting the employer would reveal that the client is in treatment without consent (1.07(b)) and could trigger the retaliation the client fears. Any contact with the employer needs the client's informed direction.

**Takeaway:** When a client's distress stems from an injustice such as wage theft, link the client to resources that can remedy it, not only coping help.

**Sources:** NASW Code of Ethics (2021), Ethical Principles: Social Justice; NASW Code of Ethics (2021), 1.16(a); NASW Code of Ethics (2021), 6.04(a); NASW Code of Ethics (2021), 1.07(b)
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## 95. ETH-1109 (rev 1) · Best answer: B

A family shelter starts requiring residents to wear colored wristbands in the dining hall that show whether they receive public benefits, so staff can track meal funding. Residents tell a social worker they feel marked in front of other families. Which core value of the profession does this practice **MOST** directly conflict with?

**A.** Social justice
**B.** Dignity and worth of the person
**C.** Importance of human relationships

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Social justice addresses unequal access and discrimination, and the policy does single out low-income families. The stem describes no denied service, though; the central harm is public stigma to each resident's dignity.

**B** **Correct.** The Dignity and Worth of the Person principle asks social workers to treat each person in a caring and respectful fashion, mindful of individual differences (NASW Code, Ethical Principles). Marking families by benefit status in a shared dining hall exposes and stigmatizes them for an administrative purpose that could be met privately, and residents say they feel marked. B beats A because the harm here is to each resident's standing and respect in daily life; social justice concerns access, opportunity and discrimination, which overlaps but is less direct.

**C** **Incorrect** (trap: Wrong model or concept). This value treats relationships as a primary vehicle for change and engages people as partners in helping. The wristbands affect how residents are seen by others, but the core harm is to each person's dignity, not to a helping relationship.

**Takeaway:** Practices that publicly mark or stigmatize clients conflict most directly with the value of dignity and worth of the person.

**Sources:** NASW Code of Ethics (2021), Ethical Principles: Dignity and Worth of the Person; NASW Code of Ethics (2021), Ethical Principles: Social Justice; NASW Code of Ethics (2021), Ethical Principles: Importance of Human Relationships
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## 96. ETH-1110 (rev 2) · Best answer: C

A child protective services social worker arrives with a court order and a police officer to remove a 6-year-old after substantiated severe neglect. The parent is crying but calm and cooperative and asks for a few minutes to say goodbye and to pack the child's stuffed animal and pajamas. What is the **BEST** response?

**A.** Complete the removal quickly to limit the child's distress
**B.** Let the officer decide whether a goodbye is safe
**C.** Permit a brief supervised farewell and the comfort items

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). A quick exit makes sense when a parent is escalating or the child faces danger in the moment. Here the parent is calm and cooperative, so rushing takes away comforts that ease the transition.

**B** **Incorrect** (trap: Good but not best). Officers keep the scene safe, and their view matters if the situation turns tense. Here the parent is calm and cooperative, and shaping a removal that limits trauma is the social worker's responsibility (NCTSN, 2013); handing the decision to police gives that role away.

**C** **Correct.** The Dignity and Worth principle calls for treating parents and children with care and respect, even during protective action (NASW Code, Ethical Principles). With a court order in hand and a calm, cooperative parent, a brief supervised goodbye and familiar items cost little and help the child keep connection and comfort through the transition (NCTSN, 2013). C beats A because speed for its own sake does not reduce distress when no safety threat is present; an abrupt removal without a goodbye or comfort objects can add to the child's trauma.

**Takeaway:** Protective action does not suspend respect: when safe, allow goodbyes and comfort items, and give children honest explanations.

**Sources:** NASW Code of Ethics (2021), Ethical Principles: Dignity and Worth of the Person; National Child Traumatic Stress Network (2013). Child Welfare Trauma Training Toolkit (2nd ed.), Essential Element 1: Maximize physical and psychological safety
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### 97. ETH-1184 (rev 1) · Best answer: D

Which core value in the NASW Code asks social workers to promote clients' right to make their own choices?

**A.** Social justice
**B.** Integrity
**C.** Service
**D.** Dignity and worth

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Social justice centers on challenging poverty, discrimination and other injustice and on equal access and meaningful participation. Participation in decisions sounds related, but the Code places the self-determination language under dignity and worth.

**B** **Incorrect** (trap: Wrong model or concept). Integrity means acting honestly and trustworthily, in line with the profession's values, and since 2021 caring for oneself. It guides the worker's own conduct rather than the client's right to choose.

**C** **Incorrect** (trap: Wrong model or concept). Service means helping people in need and addressing social problems, putting service above self-interest. It does not contain the Code's language on client self-determination.

**D** **Correct.** The Code's Dignity and Worth of the Person principle states that social workers promote clients' socially responsible self-determination, alongside treating each person with care and respect and balancing clients' interests with society's. Self-determination therefore rests on respect for each person's inherent worth. It beats A, social justice, which focuses on challenging injustice and ensuring access and participation for vulnerable groups rather than on the individual client's right to choose (NASW Code, Ethical Principles).

**Takeaway:** In the NASW Code, promoting socially responsible self-determination is part of the Dignity and Worth of the Person principle.

**Sources:** NASW Code of Ethics (2021), Ethical Principles: Dignity and Worth of the Person
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### 98. ETH-1185 (rev 3) · Best answer: A

A client who has met with the social worker twice about job stress asks for a letter stating a need for an emotional support dog. The landlord will begin eviction over the unapproved dog in 10 days. Neither session has touched on the dog or the client's housing. What should the social worker do **FIRST**?

**A.** Assess whether a disability-related need for the animal exists
**B.** Refer the client to a provider who evaluates support animal requests
**C.** Write the requested letter so the eviction can be stopped

**Why each option is right or wrong**

**A** **Correct.** The NASW Code's Integrity principle requires honest, trustworthy conduct, and the Code bars association with deception (4.04). A disability-related letter must rest on the social worker's own assessment; federal housing guidance treats notes from health professionals with personal knowledge of the person as reliable documentation (HUD, 2020). The eviction deadline makes a prompt assessment important but does not replace it. It beats B because the social worker already knows the client and can assess within the 10 days, and personal knowledge is what makes such documentation reliable.

**B** **Incorrect** (trap: Hands off too soon). A specialist evaluator could assess the request, which makes this tempting. This social worker already has a relationship with the client and can assess promptly within the 10-day window, so referring out first adds delay and a new relationship without need.

**C** **Incorrect** (trap: Right action, wrong time). If the assessment supports a disability-related need, a letter is appropriate. Writing it now, without any assessment, would state something the social worker does not know, which conflicts with integrity and the Code's standard on dishonesty.

**Takeaway:** Document a client's need only after assessing it yourself; urgency speeds the assessment but never replaces it.

**Sources:** NASW Code of Ethics (2021), Ethical Principles: Integrity; NASW Code of Ethics (2021), 4.04; U.S. Department of Housing and Urban Development (HUD) (2020). FHEO Notice FHEO-2020-01: Assessing a Person's Request to Have an Animal as a Reasonable Accommodation Under the Fair Housing Act, Documentation of disability-related need from health care professionals

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## 99. ETH-1186 (rev 2) · Best answer: C

A hospital social worker notices that uninsured patients discharged to shelters rarely leave with a follow-up appointment, while insured patients almost always do. For 3 months, the social worker has arranged appointments case by case and tracked the gap, which has not narrowed. What should the social worker do **NEXT**?

**A.** Keep arranging follow-up appointments for each affected patient

**B.** File a complaint about the hospital with a federal civil rights office

**C.** Present the findings to leadership with proposed changes

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Arranging appointments helps each patient and should continue, but 3 months of data show the pattern persists. Staying at the individual level leaves the process that produces the gap unchanged.

**B** **Incorrect** (trap: Skips assessment). Outside complaints may be appropriate if internal efforts fail. Going there first skips the hospital's own channels, which have not yet seen the data and may correct the process quickly.

**C** **Correct.** The NASW Code's Social Justice principle asks social workers to challenge injustice and ensure access to needed services, and 3.09(b) asks them to work to improve their organizations' policies and procedures. The social worker has already

handled individual cases and documented a pattern that case-by-case work has not fixed, so the next step is to bring the data to the people who can change the process. It beats A because continuing case by case helps each patient but leaves the inequity in place for patients the social worker never sees.

**Takeaway:** When individual help cannot fix a documented pattern of unequal access, take the data to organizational decision-makers to change the system.

**Sources:** NASW Code of Ethics (2021), Ethical Principles: Social Justice; NASW Code of Ethics (2021), 3.09(b)
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## 100. ETH-1187 (rev 2) · Best answer: A

At a community mental health agency, a psychiatrist asks the social worker on a 9-year-old's treatment team to give the child a standardized intelligence test to help clarify a diagnosis. The social worker has never been trained to administer or interpret such tests. A psychologist on staff provides cognitive testing. What is the **MOST** appropriate response by the social worker?

**A.** Refer the child to the psychologist for testing
**B.** Give the test after studying its administration manual closely
**C.** Do the testing and have the psychologist score it
**D.** Describe the child's abilities from interview observations

**Why each option is right or wrong**

**A** **Correct.** The NASW Code's Competence principle and 1.04(a) limit practice to areas of education, training and supervised experience, and 1.16(a) calls for referral when another professional's specialized knowledge is needed. The social worker is untrained in standardized cognitive testing, and a qualified psychologist is on staff, so referral serves the child and the diagnostic question. It beats C because scoring help does not fix the problem: valid scores depend on standardized administration, which the social worker has not learned (AERA, APA, & NCME, 2014).

**B** **Incorrect** (trap: Outside the social work role). Reading a manual is not the supervised training that standardized testing requires. Errors in giving the test can produce misleading scores that shape the child's diagnosis and services.

**C** **Incorrect** (trap: Outside the social work role). Having an expert score the test still leaves administration to someone untrained, and administration errors can make scores invalid. Shared work fits only after the social worker has been trained and supervised in the instrument.

**D** **Incorrect** (trap: Good but not best). Interview observations add useful context, but they cannot substitute for a standardized measure when the psychiatrist needs one. Offering them in place of testing leaves the diagnostic question unanswered.

**Takeaway:** When a task requires training you lack and a qualified colleague is available, refer rather than attempt it.

**Sources:** NASW Code of Ethics (2021), Ethical Principles: Competence; NASW Code of Ethics (2021), 1.04(a); NASW Code of Ethics (2021), 1.16(a); AERA, APA, & NCME (2014). Standards for Educational and Psychological Testing. American Educational Research Association, Test Administration, Scoring, Reporting, and Interpretation: standardized administration

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## 101. ETH-1717 (rev 2) · Best answer: D

A social worker has treated an asylum seeker for PTSD for 4 months. In earlier sessions the client described being beaten in detention in the home country. The client's immigration attorney asks the social worker to write a forensic evaluation stating whether the client's symptoms are consistent with torture, for the asylum hearing. The social worker has never conducted forensic evaluations. What is the **BEST** response?

**A.** Write the asylum evaluation from the 4 months of session notes

**B.** Write the evaluation after consulting a forensic evaluator

**C.** Ask the client to describe the abuse again in full detail

**D.** Refer the client to an evaluator trained in asylum cases

#### Why each option is right or wrong

**A** **Incorrect** (trap: Outside the social work role). Treatment notes record history and symptoms, but a forensic opinion on consistency with torture calls for structured methods and training the worker lacks. An unsound evaluation could weaken the asylum claim and blur the treating role.

**B** **Incorrect** (trap: Boundary slip or collusion). Consultation can support work in a new area (NASW Code 1.04(b)), but one consultation does not supply forensic training. The worker would also be evaluating their own client, a treating-forensic role conflict that can weaken both the therapy and the evaluation's credibility (Greenberg & Shuman, 1997).

**C** **Incorrect** (trap: Ignores risk). The client has already described the abuse, and repeating it in detail can be retraumatizing. More detail would not give the worker the forensic skills the request requires.

**D** **Correct.** Social workers practice within the limits of their training (NASW Code 1.04(a)). Asylum evaluations are forensic work with specialized standards for documenting torture (Istanbul Protocol; OHCHR, 2022), and acting as both treating clinician and forensic evaluator creates role conflict (Greenberg & Shuman, 1997). Referring to a trained evaluator protects the client's claim and the treatment relationship; with consent, the worker can still share a factual treatment summary. It beats A because notes gathered for treatment do not meet forensic standards, and an untrained opinion could hurt the claim.

**Takeaway:** Forensic asylum evaluations need specialized training; treating clinicians without it should refer, and may share factual treatment summaries with consent.

**Sources:** NASW Code of Ethics (2021), 1.04(a); NASW Code of Ethics (2021), 1.04(b); OHCHR (2022). Istanbul Protocol: Manual on the Effective Investigation and Documentation of Torture and Other Cruel, Inhuman or Degrading Treatment or Punishment (Professional Training Series No. 8/Rev.2), whole manual; Greenberg, S. A., & Shuman, D. W. (1997). Irreconcilable conflict between therapeutic and forensic roles. Professional Psychology: Research and Practice, 28(1), 50-57, whole article
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## 102. ETH-0003 (rev 1) · Best answer: A

A social worker hears many clients describe abuse and violence. Over time, the social worker develops intrusive images, avoidance and hyperarousal. Which term describes this pattern?

**A.** Secondary traumatic stress
**B.** Occupational burnout
**C.** Countertransference reaction

#### Why each option is right or wrong

**A** **Correct.** Secondary traumatic stress is the set of PTSD-like symptoms, such as intrusive images, avoidance and hyperarousal, that come from indirect exposure to other people's traumatic experiences (Bride, 2007). The symptoms in the question match it directly. It differs from B, burnout, which builds from chronic workload and organizational stress and shows up as exhaustion, cynicism and reduced effectiveness rather than trauma symptoms. Recognizing it matters ethically: the 2021 Code's Purpose section treats self-care as essential to competent practice.

**B** **Incorrect** (trap: Wrong model or concept). Burnout grows out of chronic job demands and organizational stress. Its hallmarks are emotional exhaustion, depersonalization and a reduced sense of accomplishment, as described by Maslach, rather than intrusive trauma images.

**C** **Incorrect** (trap: Wrong model or concept). Countertransference is the social worker's emotional reaction to a particular client, often shaped by the worker's own history. It can occur alongside secondary traumatic stress, but it does not name this trauma-symptom pattern.

**Takeaway:** Trauma symptoms from hearing clients' trauma are secondary traumatic stress; exhaustion and cynicism from workload are burnout.

**Sources:** Bride, B. E. (2007). Prevalence of secondary traumatic stress among social workers. Social Work, 52(1), 63-70, whole article; Maslach, C., Schaufeli, W. B., & Leiter, M. P. (2001). Job burnout. Annual Review of Psychology, 52, 397-422, burnout dimensions; NASW Code of Ethics (2021), Purpose (self-care paragraph)
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### 103. ETH-0104 (rev 2) · Best answer: D

A child welfare social worker has carried a caseload far above agency limits for a year. The social worker feels drained before each workday, has grown cynical about the families served and doubts that the work makes a difference. At night, the social worker lies awake thinking about overdue paperwork. Which condition is **MOST** likely?

**A.** Secondary traumatic stress
**B.** Vicarious trauma
**C.** Countertransference
**D.** Occupational burnout

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Secondary traumatic stress is a trauma response to hearing about clients' traumatic experiences, with intrusive images, avoidance or hyperarousal. The sleeplessness here centers on overdue paperwork, not on reminders of clients' trauma, so the picture points to workload rather than trauma exposure.

**B** **Incorrect** (trap: Wrong model or concept). Vicarious trauma is a gradual shift in the worker's core beliefs about safety, trust and the world after long empathic engagement with trauma. The stem describes exhaustion and cynicism tied to caseload, not changed beliefs about danger or trust.

**C** **Incorrect** (trap: Wrong model or concept). Countertransference is a worker's emotional reaction to a particular client, shaped by the worker's own history. Here the exhaustion and cynicism are general, tied to workload across all families rather than to one client.

**D** **Correct.** Burnout is a work-driven syndrome marked by emotional exhaustion, cynicism toward the people served and a reduced sense of accomplishment (Maslach et al., 2001). An overloaded caseload, feeling drained, cynicism about families and doubting one's impact match all three features. It beats A because secondary traumatic stress involves trauma-like symptoms, such as intrusive images of clients' experiences, from exposure to their trauma (Bride, 2007); this worker's nighttime worry is about paperwork. Both call for supervision and self-care, but the response to burnout centers on workload and the work environment.

**Takeaway:** Exhaustion, cynicism and low sense of accomplishment from workload signal burnout; trauma symptoms from clients' stories signal secondary traumatic stress.

**Sources:** Maslach, C., Schaufeli, W. B., & Leiter, M. P. (2001). Job burnout. Annual Review of Psychology, 52, 397-422, burnout dimensions; Bride, B. E. (2007). Prevalence of secondary traumatic stress among social workers. Social Work, 52(1), 63-70, whole article; McCann, I. L., & Pearlman, L. A. (1990). Vicarious traumatization: A framework for understanding the psychological effects of working with victims. Journal of Traumatic Stress, 3(1), 131-149, whole article

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### 104. ETH-1117 (rev 2) · Best answer: C

A social worker has counseled adult survivors of sexual assault for 8 years. Over time, the worker has come to assume most strangers have harmful motives, has stopped letting their teenager ride public transit, and tells a colleague that the world now feels unsafe in a way it never did. The worker says these views developed gradually. Which concept **MOST** likely explains these changes?

**A.** Secondary traumatic stress
**B.** Burnout
**C.** Vicarious trauma
**D.** Countertransference

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Secondary traumatic stress involves symptoms resembling PTSD, such as intrusive images or avoidance, after hearing clients' trauma. The changes here are gradual shifts in core beliefs, which fit a different concept.

**B** **Incorrect** (trap: Wrong model or concept). Burnout involves exhaustion, cynicism and a reduced sense of accomplishment driven by workplace conditions (Maslach & Leiter, 2016). It does not center on altered beliefs about personal safety.

**C** **Correct.** Vicarious trauma describes cumulative changes in a helper's beliefs about safety, trust and control that develop through empathic engagement with clients' trauma over time (McCann & Pearlman, 1990; NCTSN, 2011). This worker's changes are in worldview: distrust of strangers, new limits on a child's freedom and a sense that the world is unsafe, and they built up gradually over years. C beats A because secondary traumatic stress centers on PTSD-like symptoms, such as intrusive images, avoidance and arousal; the stem describes gradual changes in beliefs instead.

**D** **Incorrect** (trap: Wrong model or concept). Countertransference is a reaction to a particular client shaped by the worker's own history. These changes are general shifts in worldview, not responses to one client.

**Takeaway:** Vicarious trauma means gradual change in a helper's core beliefs about safety and trust; secondary traumatic stress means PTSD-like symptoms.

**Sources:** McCann, I. L., & Pearlman, L. A. (1990). Vicarious traumatization: A framework for understanding the psychological effects of working with victims. Journal of Traumatic Stress, 3(1), 131-149, whole article; National Child Traumatic Stress Network, Secondary Traumatic Stress Committee (2011). Secondary Traumatic Stress: A Fact Sheet for Child-Serving Professionals, Sidebar: Secondary Traumatic Stress and Related Conditions; Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience: recent research and its implications for psychiatry. World Psychiatry, 15(2), 103-111, burnout dimensions

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### 105. ETH-1118 (rev 2) · Best answer: A

What term names the good feelings helpers get from doing trauma work well?

**A.** Compassion satisfaction
**B.** Vicarious resilience
**C.** Posttraumatic growth

**Why each option is right or wrong**

**A** **Correct.** Compassion satisfaction is the positive feeling that comes from performing competently as a trauma professional (NCTSN, 2011). It is the positive counterpart to burnout and secondary traumatic stress in a helper's professional quality of life. A beats B because vicarious resilience refers to a helper's own growth in coping that comes from witnessing clients' resilience (Hernández et al., 2007), not the reward of competent work and collegial support.

**B** **Incorrect** (trap: Wrong model or concept). Vicarious resilience is a positive transformation in helpers who learn from the ways clients overcome adversity. It describes learning from clients' resilience, not the reward of doing the work well.

**C** **Incorrect** (trap: Wrong model or concept). Posttraumatic growth describes positive change in a person after their own struggle with trauma (Tedeschi & Calhoun, 2004). It is not specific to the rewards of helping others.

**Takeaway:** Compassion satisfaction, the reward of competent helping, is the positive counterpart to burnout and secondary traumatic stress.

**Sources:** National Child Traumatic Stress Network, Secondary Traumatic Stress Committee (2011). Secondary Traumatic Stress: A Fact Sheet for Child-Serving Professionals, Sidebar: Secondary Traumatic Stress and Related Conditions; Hernández, P., Gangsei, D., & Engstrom, D. (2007). Vicarious resilience: A new concept in work with those who survive trauma. Family Process, 46(2), 229-241, whole article; Tedeschi, R. G., & Calhoun, L. G. (2004). Posttraumatic growth: Conceptual foundations and empirical evidence. Psychological Inquiry, 15(1), 1-18, whole article

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## 106. ETH-1119 (rev 2) · Best answer: B

A social worker conducts forensic interviews at a child advocacy center. In recent weeks, the worker has begun cutting interviews short when children start giving details, startles at small noises, and lies awake replaying children's statements. Two interviews this month had to be repeated. What should the social worker do **FIRST**?

**A.** Complete a self-screening measure to gauge the symptoms
**B.** Tell the supervisor how the reactions affect interviews
**C.** Request a transfer out of forensic interviewing
**D.** Begin a daily mindfulness practice to ease symptoms

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Screening tools can track symptoms over time, but the impact is already clear: interviews are being repeated. Measuring first delays the consultation the Code calls for once work is affected.

**B** **Correct.** When personal distress impairs professional performance, social workers seek consultation immediately and take remedial action, such as getting help or adjusting workload (NASW Code 4.05(b)). Cutting interviews short and needing repeat interviews show the secondary traumatic stress symptoms are already affecting children's cases. Supervision is a core response to secondary traumatic stress and can address caseload and next steps (NCTSN, 2011). B beats D because self-care helps but does not address the immediate effect on interviews; it belongs in the plan made with the supervisor.

**C** **Incorrect** (trap: Skips assessment). A role change may become part of the plan, but requesting a transfer before consulting skips options such as caseload adjustment, supervision and treatment. That decision is best made with the supervisor.

**D** **Incorrect** (trap: Right action, wrong time). Mindfulness and rest support recovery, but they work slowly and privately. With interviews already compromised, the supervisor needs to know now so cases can be protected.

**Takeaway:** Once secondary traumatic stress affects job performance, consult a supervisor promptly; self-care and screening belong inside that plan.

**Sources:** NASW Code of Ethics (2021), 4.05(b); National Child Traumatic Stress Network, Secondary Traumatic Stress Committee (2011). Secondary Traumatic Stress: A Fact Sheet for Child-Serving Professionals, Sections Four and Five: Strategies for Prevention; Strategies for Intervention

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## 107. ETH-1120 (rev 1) · Best answer: C

After staff cuts, social workers on a hospital's medical units have handled nearly double their usual discharges for a year. A social worker notices that several colleagues, like the worker, feel drained, have grown detached from patients, and doubt their work makes a difference. Management's response so far has been an optional lunchtime wellness webinar. What is the **BEST** action for the social worker?

**A.** Encourage colleagues to attend the wellness webinars

**B.** Build a personal routine of exercise, sleep and breaks

**C.** Propose workload relief to leaders using discharge data

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Wellness programs can help, but they treat a workload problem as an individual coping problem. Promoting them alone leaves the main driver of the team's exhaustion untouched.

**B** **Incorrect** (trap: Good but not best). Healthy routines support resilience and are worth keeping. When a whole team shows the same signs after a workload surge, though, individual coping cannot fix a cause that sits in the organization.

**C** **Correct.** Burnout, marked by exhaustion, detachment and a reduced sense of accomplishment, arises mainly from chronic workplace stressors such as excessive workload (Maslach & Leiter, 2016). Here the pattern is shared across the team and tied to a year of doubled discharges, so the cause is organizational. Social workers work to improve agency policies and services, and administrators should remove conditions that interfere with ethical practice (NASW Code 3.09(b), 3.07(d)). C beats B because personal self-care helps one person but leaves in place the workload driving the team's burnout.

**Takeaway:** Burnout driven by workload calls for organizational change through agency channels, not only individual self-care.

**Sources:** Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience: recent research and its implications for psychiatry. World Psychiatry, 15(2), 103-111, burnout dimensions and workplace causes; NASW Code of Ethics (2021), 3.09(b); NASW Code of Ethics (2021), 3.07(d)

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### 108. ETH-1191 (rev 1) · Best answer: B

Two weeks after a child on the unit's caseload died from abuse, a child welfare supervisor notices that a supervisee who worked the case startles easily, avoids that neighborhood, and snaps at coworkers. The supervisee has not mentioned any of this. What should the supervisor do **FIRST**?

**A.** Refer the supervisee to the employee assistance program for help
**B.** Meet privately to ask how the case is affecting the supervisee
**C.** Reassign the supervisee's cases in that neighborhood

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). An employee assistance referral may well follow, but sending the supervisee elsewhere before talking can feel like a brush-off. It also skips the supervisor's own role in recognizing and responding to the reaction.

**B** **Correct.** Startle, avoidance and irritability after a traumatic case event fit secondary traumatic stress, and reflective supervision is a primary way to recognize and address it (NCTSN, 2011). The NASW Code also encourages agencies to support workers' self-care (Purpose). The supervisor has seen signs but has not heard from the supervisee, so a private conversation about how the case is affecting them comes first and shows what support fits. It beats A because a referral before that conversation hands off the concern without understanding it.

**C** **Incorrect** (trap: Skips assessment). Changing assignments may help, but deciding for the supervisee before talking assumes what they need and can feel punitive. Workload changes work best when planned with the supervisee after the conversation.

**Takeaway:** When a supervisee shows trauma-related stress after a case event, start with a private supportive conversation before referring or reassigning.

**Sources:** National Child Traumatic Stress Network (2011). Secondary Traumatic Stress: A Fact Sheet for Child-Serving Professionals, Supervision and organizational strategies; NASW Code of Ethics (2021), Purpose
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### 109. ETH-1192 (rev 1) · Best answer: C

A hospital discharge planner whose caseload has doubled has started calling patients by bed number and diagnosis in team meetings. When a family calls in tears about a delayed discharge, the planner answers curtly and feels little concern. Which component of burnout is **MOST** likely present?

**A.** Emotional exhaustion
**B.** Reduced personal accomplishment
**C.** Depersonalization

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Emotional exhaustion means feeling drained and depleted by the work. A doubled caseload makes it likely, but the behaviors described, treating patients as cases and responding without concern, point to depersonalization.

**B** **Incorrect** (trap: Wrong model or concept). Reduced personal accomplishment is a sense of ineffectiveness and lack of achievement at work. The stem describes how the worker treats patients and families, not how the worker judges their own effectiveness.

**C** **Correct.** Maslach's model describes burnout through three components: emotional exhaustion, depersonalization (a callous, detached response to the people served) and reduced personal accomplishment. Calling patients by bed number and answering a distressed family coldly show the detached stance that defines depersonalization. It beats A because exhaustion, feeling drained and depleted, is often the first and most common component, but the signs in this stem concern how the worker treats people (Maslach et al., 2001).

**Takeaway:** Depersonalization, the interpersonal component of burnout, shows as a detached, callous response to the people served.

**Sources:** Maslach, C., Schaufeli, W. B., & Leiter, M. P. (2001). Job burnout. Annual Review of Psychology, 52, 397-422, Three dimensions of burnout

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## 110. ETH-1121 (rev 1) · Best answer: D

In 2021, the NASW Code of Ethics added a sentence on caring for one's own well-being. Which ethical principle now holds it?

**A.** Competence
**B.** Service
**C.** Dignity and worth of the person
**D.** Integrity

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Competence covers practicing within one's areas of expertise and building skill. Self-care supports competent work, but the 2021 sentence was added to the Integrity principle.

**B** **Incorrect** (trap: Wrong model or concept). Service concerns putting helping others above self-interest and offering some pro bono work. The self-care sentence does not appear under this principle.

**C** **Incorrect** (trap: Wrong model or concept). Dignity and worth concerns respect for each person and support for clients' self-determination. The 2021 self-care language was added elsewhere.

**D** **Correct.** The 2021 revision added self-care language in two places: a Purpose paragraph calling professional self-care "paramount for competent and ethical social work practice," and a sentence in the Integrity principle saying social workers should care for themselves professionally and personally (NASW Code, Ethical Principles; Purpose). Placing it under integrity frames self-care as part of trustworthy conduct. D beats A because competence concerns practicing within one's expertise and enhancing it; self-care supports competence, but the added principle sentence sits under integrity.

**Takeaway:** Since 2021, the Code treats self-care as an ethical matter, in the Purpose section and under the Integrity principle.

**Sources:** NASW Code of Ethics (2021), Ethical Principles: Integrity; NASW Code of Ethics (2021), Purpose (self-care paragraph)
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## 111. ETH-1122 (rev 1) · Best answer: B

A community mental health social worker learns in the morning that a long-term client died by suicide overnight. The supervisor offers to have colleagues cover the worker's afternoon, which includes two new crisis evaluations, and mentions a support meeting for staff. The worker feels numb and would rather stay busy. What is the **MOST** appropriate response for the worker?

**A.** Keep the afternoon's crisis evaluations to stay in routine
**B.** Accept coverage for this afternoon's crisis evaluations
**C.** Ask a colleague to sit in on the crisis evaluations

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Routine comforts some people after a loss, and lower-stakes tasks may be fine. Crisis evaluations, though, require full judgment, and numbness on the same morning signals that judgment may be affected.

**B** **Correct.** Self-care is part of competent, ethical practice (NASW Code, Purpose), and social workers must not let personal distress impair judgment in ways that could harm the people they serve (4.05(a)). The worker reports feeling numb only hours after the loss, and new crisis evaluations demand full attention to risk. Accepting coverage for those evaluations protects the new clients and gives the worker room to use staff support. B beats A because staying busy may feel better, but high-stakes risk assessments are the wrong place to test one's functioning on the day of a client's death.

**C** **Incorrect** (trap: Good but not best). A second clinician adds a check, but the worker would still lead high-stakes risk assessments hours after the loss while feeling numb. Full coverage protects the new clients better and frees the worker to use support.

**Takeaway:** After a client's death or other acute loss, accept support and coverage for high-stakes work rather than proving resilience.

**Sources:** NASW Code of Ethics (2021), Purpose (self-care paragraph); NASW Code of Ethics (2021), 4.05(a)
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## 112. ETH-1123 (rev 2) · Best answer: A

A social worker in solo private practice gave clients a personal cell number when the practice opened and now answers their texts most evenings and weekends. The worker sleeps poorly, feels irritable in sessions, and recently answered a client's late-night scheduling text sharply. What is the **MOST** effective way to address the problem?

**A.** Set after-hours limits and explain crisis options to clients
**B.** Begin personal counseling for the stress and poor sleep
**C.** Stop replying to evening texts without telling clients

**Why each option is right or wrong**

**A** **Correct.** Professional self-care is essential to competent, ethical practice (NASW Code, Purpose), and constant availability is draining the worker's sleep and patience with clients. Social workers discuss their technology policies with clients (1.03(e)), so a clear after-hours policy, with information on crisis lines and emergency services, protects both the worker's well-being and clients' safety. A beats B because counseling can ease the strain, but the cause, unlimited availability through a personal phone, would continue.

**B** **Incorrect** (trap: Good but not best). Personal counseling is a sound form of self-care, which the Code treats as essential, so this is tempting. It treats the effects while the arrangement causing them, constant after-hours texting, goes on unchanged.

**C** **Incorrect** (trap: Ignores risk). Going silent without notice could leave a client in crisis waiting for a reply that never comes. Any change in availability must be explained, with alternatives for urgent needs.

**Takeaway:** Sustainable self-care includes clear limits on availability, explained to clients along with crisis alternatives.

**Sources:** NASW Code of Ethics (2021), Purpose (self-care paragraph); NASW Code of Ethics (2021), 1.03(e)
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## 113. ETH-1193 (rev 2) · Best answer: A

After a flood, a county mental health agency asks staff to add evening shifts at evacuation shelters. A social worker whose own home flooded is staying with relatives, sleeping poorly, and handling insurance claims. The social worker wants to help and has not told the supervisor about the damage. What is the **BEST** action for the social worker?

**A.** Tell the supervisor about the losses and agree on limited shifts
**B.** Work the shifts and seek supervision if stress becomes a problem
**C.** Ask the supervisor to excuse the social worker from shelter work

**Why each option is right or wrong**

**A** **Correct.** The NASW Code calls professional self-care paramount for competent, ethical practice (Purpose) and asks social workers to care for themselves professionally and personally (Integrity). It also asks them to serve in public emergencies to the greatest extent possible (6.03), which allows for personal limits. Telling the supervisor about the damage lets the agency plan shifts the social worker can sustain. It beats B because waiting to seek supervision until stress shows lets known risks, poor sleep and personal losses, reach distressed residents first.

**B** **Incorrect** (trap: Ignores risk). Planning to seek supervision later sounds responsible, but it waits for problems to show while the social worker is already sleeping poorly and handling personal losses. Telling the supervisor now lets the agency set sustainable shifts before strain affects distressed residents.

**C** **Incorrect** (trap: Good but not best). Stepping away entirely may be needed if the social worker cannot function, but nothing in the stem suggests that. Asking to be excused from all shelter work gives up help the social worker wants to offer and could safely provide in part.

**Takeaway:** In emergencies, serve within sustainable limits: tell your supervisor about your own losses and plan a workload you can safely carry.

**Sources:** NASW Code of Ethics (2021), Purpose; NASW Code of Ethics (2021), Ethical Principles: Integrity; NASW Code of Ethics (2021), 6.03
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### 114. ETH-1194 (rev 1) · Best answer: B

A social worker newly assigned to a sexual assault program finds that clients' accounts stir memories of an assault the social worker experienced years ago. The social worker has discussed this in supervision, and the supervisor suggested personal counseling. The social worker worries that seeking counseling would look like incompetence. Which step is **MOST** consistent with ethical self-care?

**A.** Ask the supervisor to provide personal counseling in supervision
**B.** Begin personal counseling while continuing in supervision
**C.** Request a transfer away from the sexual assault program

#### Why each option is right or wrong

**A** **Incorrect** (trap: Boundary slip or collusion). Supervision addresses the social worker's practice, not personal treatment. Turning supervision into therapy blurs the supervisor's evaluative role and creates a dual relationship (Kadushin & Harkness, 2014).

**B** **Correct.** The NASW Code's Integrity principle states that social workers should take measures to care for themselves professionally and personally. The Code names seeking professional help as a remedy when personal difficulties interfere with practice (4.05(b)); using it earlier, as here, is sound prevention. Personal counseling addresses the worker's own history, while supervision keeps addressing the work with clients. It beats C because a transfer is a later option if reactions cannot be managed, and the social worker has not yet tried the support the supervisor recommended.

**C** **Incorrect** (trap: Hands off too soon). A transfer may become appropriate if reactions cannot be managed even with support. Requesting it first gives up work the social worker chose before trying the counseling the supervisor recommended.

**Takeaway:** Counseling for one's own trauma reactions is ethical self-care that runs alongside supervision, not inside it.

**Sources:** NASW Code of Ethics (2021), Ethical Principles: Integrity; NASW Code of Ethics (2021), 4.05(b); Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Supportive supervision: supervision versus therapy
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### 115. ETH-0105 (rev 2) · Best answer: C

A program director asks a social worker to start offering eye movement desensitization and reprocessing (EMDR) to clients next month. The social worker already treats several clients with trauma using cognitive behavioral methods but has had no EMDR training. A trained EMDR provider works at a clinic across town. What is the **MOST** appropriate response?

**A.** Begin EMDR using an online training video as a guide
**B.** Transfer current trauma clients to the trained EMDR provider
**C.** Explain the need for formal training and supervision first

**Why each option is right or wrong**

**A** **Incorrect** (trap: Outside the social work role). A video can introduce a method, but it does not replace formal training and supervised practice. Using EMDR on that basis puts clients at risk and practices outside the social worker's competence.

**B** **Incorrect** (trap: Hands off too soon). Transferring ongoing clients disrupts relationships with people the social worker is already competent to treat with cognitive behavioral methods, and it does not answer the director's request. Referral to the trained provider fits a specific client who needs EMDR before the social worker completes training.

**C** **Correct.** Social workers offer new techniques only after appropriate study, training, consultation and supervision from people competent in them (NASW Code 1.04(b)). Accepting the assignment is fine if the social worker commits to gaining that competence before using EMDR with clients (NASW Code 4.01(a)). Explaining this to the director keeps clients safe and gives the agency a workable plan. It beats B because transferring current trauma clients abandons people the social worker already serves competently; referral fits only a client who needs EMDR now.

**Takeaway:** Use a new technique only after formal training and supervision; until then, serve clients within current competence or refer when needed.

**Sources:** NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), 4.01(a)
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### 116. ETH-1111 (rev 1) · Best answer: A

On Monday morning, a licensed clinical social worker in solo private practice checks the state board's website and finds that the license expired 10 days ago because required continuing education hours were never reported. Eight clients are scheduled this week. What should the social worker do?

**A.** Pause client sessions until the license is reinstated
**B.** Finish the missing hours online and keep this week's sessions
**C.** See current clients while the license renewal is pending

**Why each option is right or wrong**

**A** **Correct.** Once a license expires, continuing to provide licensed services misrepresents a credential the social worker does not currently hold (NASW Code 4.06(c)) and is the kind of unauthorized practice the Code asks social workers to prevent (5.01(e)). The social worker pauses sessions until the board reinstates the license, while telling clients promptly and helping any who need care in the meantime find it (1.17(e)). A beats B because finishing the hours does not renew the license; only the board's action does.

**B** **Incorrect** (trap: Misreads law or ethics). Completing the hours is needed for reinstatement, but the license stays expired until the board acts. Seeing clients this week would still be practice without a current license.

**C** **Incorrect** (trap: Misreads law or ethics). Limiting practice to current clients feels like a compromise, but license status is the same for every client. Any licensed service this week would be unauthorized.

**Takeaway:** An expired license means no licensed practice until reinstatement; pause services and help clients with interim care.

**Sources:** NASW Code of Ethics (2021), 4.06(c); NASW Code of Ethics (2021), 5.01(e); NASW Code of Ethics (2021), 1.17(e)

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## 117. ETH-1112 (rev 1) · Best answer: B

The only social worker at a rural health clinic has a new client with gambling disorder who wants treatment. The nearest gambling specialist is 3 hours away, and the client cannot travel or use video. The social worker has general addiction training but has never treated gambling disorder. What should the social worker do **FIRST**?

**A.** Make a referral to the distant specialist

**B.** Arrange consultation with a gambling expert

**C.** Begin weekly gambling-focused sessions with the client

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). Referral is right when a qualified provider is reachable (1.16(a)). This client cannot travel or use video, so the referral would leave the client without treatment.

**B** **Correct.** Social workers practice within their competence and, before offering an intervention new to them, obtain appropriate study, training, consultation and supervision (NASW Code 1.04(a), 1.04(b)). This client cannot reach the only specialist, so a referral would leave no care at all. Consulting a gambling specialist by phone or email lets the social worker build a sound plan that draws on existing addiction skills (2.05(a), 2.05(b)). B beats C because starting gambling-specific treatment with no expert input risks ineffective care; sessions begin once consultation shapes the plan.

**C** **Incorrect** (trap: Right action, wrong time). Treating the client is the goal, but beginning a specialized treatment with no training or consultation risks harm. Sessions follow once a specialist's guidance shapes the plan.

**Takeaway:** When no qualified provider is reachable, build competence through expert consultation before and while serving the client.

**Sources:** NASW Code of Ethics (2021), 1.04(a); NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), 2.05(a); NASW Code of Ethics (2021), 2.05(b); NASW Code of Ethics (2021), 1.16(a)
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## 118. ETH-1113 (rev 2) · Best answer: A

For years, an employee assistance social worker has responded to workplace deaths at a large company with a required group meeting within 48 hours, in which each employee describes the event and their reactions. While updating training, the social worker reads a systematic review finding that single-session debriefing does not prevent PTSD and may increase symptoms for some people. What is the **BEST** response?

**A.** Switch to psychological first aid with follow-up
**B.** Keep the meetings but make attendance voluntary
**C.** Continue the meetings since employees rate them highly

#### Why each option is right or wrong

**A** **Correct.** Social workers keep up with emerging knowledge and base practice on recognized, empirically based knowledge (NASW Code 4.01(b), 4.01(c)). A Cochrane review found single-session individual debriefing did not prevent PTSD and concluded that compulsory debriefing should stop (Rose et al., 2002). Psychological first aid guidance says not to debrief by asking for details and offers practical support instead (Brymer et al., 2006), with follow-up for those who need more. A beats B because voluntary attendance still leaves in place the core element the evidence questions: early, detailed recounting of the event.

**B** **Incorrect** (trap: Good but not best). Voluntary attendance removes the compulsion the review warned against, but those who attend still recount the event in detail soon after it. The evidence questions the technique itself, not only its required use.

**C** **Incorrect** (trap: Ignores risk). Satisfaction ratings do not show that a practice prevents harm. When research suggests a practice is ineffective and may increase symptoms, popularity is not a reason to continue it.

**Takeaway:** When research shows a familiar practice is ineffective or harmful, update to evidence-supported approaches rather than relying on habit or satisfaction.

**Sources:** NASW Code of Ethics (2021), 4.01(b); NASW Code of Ethics (2021), 4.01(c); Rose, S., Bisson, J., Churchill, R., & Wessely, S. (2002). Psychological debriefing for preventing post traumatic stress disorder (PTSD). Cochrane Database of Systematic Reviews, (2), CD000560, Authors' conclusions; Brymer, M., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Introduction and Overview: Behaviors to Avoid
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### 119. ETH-1114 (rev 1) · Best answer: D

A pre-licensure associate social worker at an outpatient clinic learns that the clinical supervisor will be on medical leave for 6 weeks. The supervisor suggests the associate keep seeing clients and save questions until the leave ends. The associate replies that two clients have recent self-harm and waiting is not workable, but the supervisor repeats the suggestion. What should the associate do **NEXT**?

**A.** Raise the concern with the supervisor once more
**B.** Continue sessions and keep a list of questions
**C.** Pause client sessions until the supervisor returns
**D.** Ask the program director to arrange interim supervision

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Talking with the supervisor first was right, and it has been done. Repeating it after a clear answer delays action while clients with recent self-harm go without timely oversight.

**B** **Incorrect** (trap: Misreads law or ethics). Saving questions for 6 weeks leaves an associate practicing without the oversight the license requires and delays guidance on high-risk clients. Interim supervision is needed now.

**C** **Incorrect** (trap: Ignores risk). Stopping all sessions would leave clients, including two with recent self-harm, without care. The gap is in supervision, which can be filled without interrupting services (1.17(b)).

**D** **Correct.** Pre-licensure practitioners provide clinical services under the supervision their regulations require (NASW & ASWB, 2013, Standard 3), and administrators should ensure adequate resources for staff supervision (NASW Code 3.07(c)). Clients with recent self-harm need timely clinical guidance (2.05(a)). The associate has already raised the problem with the supervisor without result, so the next step is the program director, who can assign a qualified interim supervisor. D beats A because repeating the same request to the same person has already failed and delays help for clients at risk.

**Takeaway:** Pre-licensure practice requires ongoing supervision; if the supervisor cannot provide it, go up the agency's chain to arrange interim coverage.

**Sources:** NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 3: Legal and Regulatory Issues; NASW Code of Ethics (2021), 3.07(c); NASW Code of Ethics (2021), 2.05(a); NASW Code of Ethics (2021), 1.17(b)

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### 120. ETH-1115 (rev 2) · Best answer: C

A licensed clinical social worker kept the license current through continuing education during a 5-year break from practice. Before the break, the worker provided school-based counseling to children. A community clinic now offers the worker a full caseload of adults with serious mental illness, starting in 2 weeks. What is the **MOST** appropriate way for the worker to proceed?

**A.** Accept the full caseload after a weekend training on serious mental illness
**B.** Decline the position because the population is new
**C.** Accept with gradual intake, training and supervision

**Why each option is right or wrong**

**A** **Incorrect** (trap: Outside the social work role). Training is part of the answer, which makes this tempting. One weekend course before a full caseload starts skips the consultation, supervision and gradual intake that 1.04(b) expects for a new area, leaving adults with serious mental illness served by a worker still learning the basics.

**B** **Incorrect** (trap: Good but not best). Declining is unnecessary. The Code allows accepting work with the intention to acquire competence, and a structured plan can close the gap while meeting the clinic's need.

**C** **Correct.** Social workers accept employment only on the basis of existing competence or the intention to acquire it (NASW Code 4.01(a)), and they build competence in new areas through study, training, consultation and supervision (1.04(a), 1.04(b)). The license is current, but the worker's recent experience is with children, not adults with serious mental illness. A gradual caseload with targeted training and clinical supervision builds competence while serving clients safely. C beats B because the Code does not require existing expertise before accepting work, only a real plan to acquire it.

**Takeaway:** Accept work in a new area only with a concrete plan to gain competence through training, consultation and supervision.

**Sources:** NASW Code of Ethics (2021), 4.01(a); NASW Code of Ethics (2021), 1.04(a); NASW Code of Ethics (2021), 1.04(b)
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### 121. ETH-1116 (rev 2) · Best answer: B

What is the main purpose of the continuing education that state boards require for social work license renewal?

**A.** Raising the profession's standing among other disciplines
**B.** Protecting the public by keeping practitioners competent
**C.** Preparing licensees for specialty certification

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Professional standing may rise when practitioners stay current, but licensing boards are government agencies whose mandate is public protection, not the profession's reputation.

**B** **Correct.** Licensing exists to protect the public, and continuing education requirements serve that goal by helping licensees maintain competence as knowledge and practice

change. ASWB's continuing education approval program frames acceptable courses around supporting ongoing professional competence so social workers serve the public better (ASWB, 2023). The Code likewise expects social workers to pursue continuing education to stay proficient (NASW Code 4.01(b)). B beats A because any gain in professional standing is a side effect, not the regulatory purpose.

**C** **Incorrect** (trap: Wrong model or concept). Some courses count toward specialty credentials, but boards require continuing education of every licensee to maintain competence, whether or not they pursue certification.

**Takeaway:** Regulation, including continuing education for renewal, exists to protect the public by maintaining practitioner competence.

**Sources:** ASWB (2023). ACE Handbook (December 2023 revision), Course relevance criteria and course review checklist; NASW Code of Ethics (2021), 4.01(b)

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## 122. ETH-1188 (rev 1) · Best answer: B

At a training, a social worker at an opioid treatment program learns that the federal confidentiality rule for substance use disorder records was revised in 2024. The program's consent forms date from 2019, and the social worker will ask new clients to sign them this week. The program has a compliance officer who oversees confidentiality policies. What should the social worker do **FIRST**?

**A.** Continue using the forms until the program issues new ones

**B.** Ask the compliance officer whether the forms meet the current rule

**C.** Revise the consent form using a summary of the new rule

**D.** Pause new consents until the compliance officer approves new forms

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Waiting for the program to act could mean new clients sign forms that do not match the rule they are told protects them. Knowing about a possible gap creates a duty to check, not to wait.

**B** **Correct.** Social workers should strive to remain proficient (4.01(b)) and seek consultation from colleagues with relevant expertise (2.05(b)). The 2024 revision of 42 CFR Part 2 changed consent requirements (2.31), and the social worker does not know whether the 2019 forms still comply. The compliance officer oversees these policies, so checking there is the first step before more clients sign. It beats A because waiting passively leaves a known question unanswered while clients keep signing forms that may be outdated.

**C** **Incorrect** (trap: Outside the social work role). Rewriting a legal consent form from a summary risks errors and bypasses the person responsible for compliance. Helping with revisions is fine once the compliance officer confirms what must change.

**D** **Incorrect** (trap: Skips assessment). Stopping consents assumes the forms are invalid before anyone has checked, and it can delay care coordination for new clients. If the compliance officer finds a problem, a short pause might then be warranted.

**Takeaway:** When a rule governing your practice changes, verify current requirements with the responsible expert before relying on old procedures.

**Sources:** NASW Code of Ethics (2021), 4.01(b); NASW Code of Ethics (2021), 2.05(b); 42 CFR Part 2, 2.31
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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### 123. ETH-1189 (rev 2) · Best answer: C

After a weekend continuing education workshop, a social worker is eager to try a new brief technique for trauma. The presenter supported it with case stories and client testimonials. The social worker has several clients with trauma histories and a supervisor available for consultation. Which action is **BEST** before the social worker uses the technique with clients?

**A.** Try it with consenting clients while getting supervision
**B.** Ask the presenter for more detailed case examples
**C.** Search for published outcome studies of the method

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Study, training, consultation and supervision belong in adopting any new technique (1.04(b)), and clients' informed consent is needed too (1.03(a)). They address how to use a method, though, not whether it works; with only testimonials, the evidence question comes first.

**B** **Incorrect** (trap: Misuses the technique). More case examples are the same kind of evidence the workshop already offered. Anecdotes cannot show whether improvements came from the technique, so collecting more of them does not answer the central question.

**C** **Correct.** Social workers should base practice on recognized knowledge, including empirically based knowledge (4.01(c)). Case stories and testimonials cannot show whether a technique works or is safe, and clients with trauma histories can be harmed by an unproven method. Checking the research is the first question; training and supervision (1.04(b)) matter once the evidence supports using the technique. It beats A because supervision helps apply a sound method well but cannot make up for missing evidence that the method works.

**Takeaway:** Testimonials and case stories are not evidence of effectiveness; check peer-reviewed outcome research before adopting a technique with clients.

**Sources:** NASW Code of Ethics (2021), 4.01(c); NASW Code of Ethics (2021), 1.04(b); NASW Code of Ethics (2021), 1.03(a)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 124. ETH-1190 (rev 2) · Best answer: A

A licensed social worker opens a solo practice and no longer has a supervisor. Which habit is **MOST** appropriate for keeping clinical judgment sound over time?

**A.** Meeting regularly with a consultation group to review cases
**B.** Completing the continuing education hours required for renewal
**C.** Reviewing past supervision notes when a case feels difficult

**Why each option is right or wrong**

**A** **Correct.** Social workers should seek colleagues' advice whenever it serves clients (2.05(a)) and strive to remain proficient (4.01(b)). A solo practitioner without a supervisor loses routine outside feedback on actual cases, so regular case consultation is the habit that catches blind spots and drift in judgment as the work happens. It beats B because continuing education, which 4.01(b) names as one way to stay current, builds general knowledge but gives no feedback on the practitioner's own clinical decisions.

**B** **Incorrect** (trap: Good but not best). Continuing education is required for renewal and is expected by the Code (4.01(b)), so it belongs in any professional's routine. It updates general knowledge but does not examine the practitioner's own cases, so it cannot catch blind spots the way regular consultation can.

**C** **Incorrect** (trap: Good but not best). Looking back at old supervision notes can recall useful advice, but it offers no fresh outside view of current cases. Judgment drifts most when no one else reviews the work, which regular consultation with colleagues addresses directly.

**Takeaway:** Licensed practitioners without supervision should build in regular case consultation; continuing education updates knowledge but gives no feedback on actual cases.

**Sources:** NASW Code of Ethics (2021), 4.01(b); NASW Code of Ethics (2021), 2.05(a)
**Area** I Values and Ethics · **Sub-area** I.A Ethical Principles and Responsibilities · **Topic** I.A.8 Keeping knowledge current: training, licensing CE, supervision, consultation · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 3 of 3

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## I.B Ethical Service Delivery

### 125. ETH-0004 (rev 2) · Best answer: C

An adult client with schizophrenia has been stable for a year on an antipsychotic medication. The client tells the social worker that weight gain from the medication is unbearable and that they plan to stop taking it. The client is coherent and understands the risk of relapse. How should the social worker respond?

**A.** Tell the client that stopping the medication would be a mistake
**B.** Ask the client's family to encourage continued medication use
**C.** Explore the concerns and link the client with the prescriber
**D.** Suggest that the client lower the dose gradually before stopping

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Telling the client the decision is a mistake is advice-giving before the reasons are understood. It tends to provoke defensiveness and can make the client less likely to share future medication changes with the treatment team.

**B** **Incorrect** (trap: Overrides the client's choice). Recruiting family to push adherence goes around a competent adult's decision and discloses clinical information without consent. Family involvement is appropriate when the client agrees to it.

**C** **Correct.** A competent adult has the right to make treatment decisions, including stopping a medication (NASW Code 1.02). The social worker's role is to explore what drives the decision, here a real and common side effect, and to help the client bring it to the prescriber, who may have alternatives. This keeps the client informed and engaged without overriding the choice. It beats A because pressure tends to end the conversation, while exploration keeps the client talking with the team. Recovery-oriented care treats the person as the driver of treatment choices (SAMHSA, 2012).

**D** **Incorrect** (trap: Outside the social work role). Advising how to taper is medication management, which is outside the social work role and the worker's competence (NASW Code 1.04(a)). The prescriber guides any dose change, so the social worker connects the client with the prescriber instead.

**Takeaway:** A competent adult may refuse medication; the social worker explores the reasons and links the client to the prescriber.

**Sources:** NASW Code of Ethics (2021), 1.02; SAMHSA (2012) Working Definition of Recovery (PEP12-RECDEF), guiding principles: recovery is person-driven; NASW Code of Ethics (2021), 1.04(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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## 126. ETH-0107 (rev 2) · Best answer: A

An adult client with bipolar I disorder left a psychiatric unit 3 weeks ago. The client now declines the mood stabilizer the psychiatrist prescribed, preferring to manage symptoms with sleep routines and a peer support group. The client sleeps normally, is not manic and denies suicidal thoughts. What is the social worker's **BEST** response?

**A.** Respect the choice and build a plan for early warning signs
**B.** Ask the psychiatrist to seek a court order for the medication
**C.** Tell the client that relapse is likely without the medication

**Why each option is right or wrong**

**A** **Correct.** Competent adults may refuse treatment, and the Code permits limiting self-determination when a client's actions pose a serious, foreseeable and imminent risk (NASW Code 1.02). The client is stable, not manic and not suicidal, so no override applies. Building a plan for early warning signs, with the client's own steps and contacts, respects the choice while reducing relapse risk, consistent with person-driven recovery (SAMHSA, 2012). It beats C because accurate information about relapse belongs in the conversation, but stating it as a prediction lectures the client and does not help them act on their choice.

**B** **Incorrect** (trap: Overrides the client's choice). Court-ordered medication is reserved for people who lack capacity or pose serious danger under state law. A stable, competent client who declines a medication does not meet that bar, so this overrides a lawful choice.

**C** **Incorrect** (trap: Too directive). Sharing relapse risk is part of informed choice, but leading with a warning becomes persuasion. It tends to raise defensiveness and may make the client less open with the team about symptoms later.

**Takeaway:** A competent adult may decline medication; respect the choice, share information and plan for warning signs unless risk becomes imminent.

**Sources:** NASW Code of Ethics (2021), 1.02; SAMHSA (2012) Working Definition of Recovery (PEP12-RECDEF), guiding principles: recovery is person-driven
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 127. ETH-1520 (rev 1) · Best answer: C

During a home visit, an assertive community treatment social worker finds that a client with schizophrenia has refused all food and water for 3 days, believing it is poisoned. The client is dizzy, confused and unsteady when standing, and refuses to go to the hospital. What should the social worker do **FIRST**?

**A.** Explore the client's reasons for refusing the hospital
**B.** Respect the refusal and return tomorrow with sealed food
**C.** Seek emergency evaluation despite the client's refusal

#### Why each option is right or wrong

**A** **Incorrect** (trap: Ignores risk). Exploring the reasons for a refusal is the right first step when risk is uncertain and the client can weigh choices. Here the client is confused after 3 days without fluids, so the condition for overriding the refusal is already met.

**B** **Incorrect** (trap: Misreads law or ethics). Respecting a refusal is correct when a competent client faces no imminent danger. Dehydration with confusion is a serious and imminent risk, and waiting until tomorrow could lead to collapse or death.

**C** **Correct.** Self-determination may be limited when, in professional judgment, a client's actions pose a serious, foreseeable and imminent risk to self (NASW Code 1.02). Three days without fluids, with confusion and unsteadiness, signals a medical emergency driven by psychosis, and the confusion undercuts the client's capacity to decide (NASW Code 1.14). The social worker follows applicable law to obtain emergency evaluation now. It beats A because exploring the refusal fits when risk is unclear; here the risk is established and delay could be fatal.

**Takeaway:** Self-determination yields when a client's choice poses serious, foreseeable and imminent risk to self; then secure emergency evaluation.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.14
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 128. ETH-1525 (rev 1) · Best answer: C

In an integrated primary care clinic, a client who left home last month after a partner pushed them down the stairs says they have decided to move back in. The couple has no children. The client asks the social worker for help 'being smart about it.' What is the **BEST** response?

**A.** Urge a shelter stay until the partner gets help
**B.** Explore the client's mixed feelings about going back
**C.** Develop a safety plan with the client for living at home
**D.** Contact the partner to set conditions for the return

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Urging a shelter stay substitutes the social worker's judgment for the client's. Shelter is a resource to offer, but pressing it after the client has decided can damage trust and make the client less likely to return when danger rises.

**B** **Incorrect** (trap: Good but not best). Exploring mixed feelings fits a client who is still weighing whether to go back. This client has made a decision and asked for help carrying it out safely, so more exploration delays the safety work the client requested.

**C** **Correct.** A competent adult may choose to return to an abusive partner, and the social worker respects that choice while reducing risk (NASW Code 1.02). The client has decided and is asking for practical help, so the most useful step is a safety plan built on the client's own knowledge of the partner: warning signs, an exit route, a packed bag and trusted contacts (Davies, Lyon & Monti-Catania, 1998). It beats B because exploring ambivalence suits a client who is still deciding; this client has decided and asked for something specific.

**D** **Incorrect** (trap: Ignores risk). Contacting the partner could alert an abusive person to the client's plans and raise the danger. It also shares client information without consent. Any contact with the partner is the client's decision, made with a safety plan in place.

**Takeaway:** When a competent adult returns to an abusive partner, respect the choice and build a safety plan from the client's own knowledge of the risks.

**Sources:** NASW Code of Ethics (2021), 1.02; Davies, J., Lyon, E., & Monti-Catania, D. (1998). Safety Planning with Battered Women: Complex Lives/Difficult Choices. Sage, woman-defined advocacy and safety planning
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 129. ETH-1527 (rev 1) · Best answer: B

A client with bipolar I disorder has been stable for a year at a community mental health center. The client says that during past manic episodes, hospital staff gave medications that caused severe side effects and ignored the client's wishes. The client wants future care to reflect those preferences. What is the **BEST** way for the social worker to help?

**A.** Ask the psychiatrist to add the preferences to the chart
**B.** Help the client complete a psychiatric advance directive
**C.** Suggest the client tell a relative which medications to refuse
**D.** Reassure the client that hospitals follow stated wishes

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). A chart note helps the local team, but it may not reach a hospital during a future crisis and carries little legal weight there. It is a useful extra step, not the best way to protect the client's choices.

**B** **Correct.** A psychiatric advance directive is a legal document, written while the client is well, that records treatment preferences and can name a trusted agent for future crises (SAMHSA, 2019). It turns this client's wishes about medications into a document hospital staff can find and weigh, which supports self-determination even when capacity is reduced (NASW Code 1.02). It beats A because a chart note at one clinic may never reach an emergency hospital and carries little legal weight; the directive travels with the client and is built for this purpose.

**C** **Incorrect** (trap: Good but not best). Telling a relative helps, but informal word of mouth can be doubted or forgotten in a crisis, and the relative has no recognized role. A directive can formally name that person as an agent if the client wishes.

**D** **Incorrect** (trap: Misreads law or ethics). This reassurance is inaccurate. Wishes stated only in conversation can be hard to verify during a crisis, and treatment decisions vary with law and circumstances. Recording the preferences in a directive gives them more weight.

**Takeaway:** Help clients with recurring crises record treatment preferences in a psychiatric advance directive while they are well.

**Sources:** SAMHSA (2019). A Practical Guide to Psychiatric Advance Directives, What is a psychiatric advance directive; NASW Code of Ethics (2021), 1.02
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## 130. ETH-1529 (rev 1) · Best answer: A

An adult with an intellectual disability chooses trusted people who help them understand options and communicate choices, while the adult keeps the final say. Which arrangement does this describe?

**A.** Supported decision-making
**B.** Limited guardianship
**C.** Substituted judgment
**D.** Health care power of attorney

**Why each option is right or wrong**

**A** **Correct.** Supported decision-making lets a person with a disability choose supporters who help them understand information and express choices, while the person keeps legal authority to decide (United Nations Convention on the Rights of Persons with Disabilities, Article 12). It is a less restrictive alternative to guardianship and fits the duty to enhance a client's own ability to decide (NASW Code 1.03(c)). It beats B because even a limited guardianship moves some decisions to a court-appointed person; here the adult keeps the final say.

**B** **Incorrect** (trap: Wrong model or concept). A limited guardianship is court ordered and moves specific decisions to a guardian. It is narrower than full guardianship, but in this arrangement nobody else holds decision authority, so it is not guardianship of any kind.

**C** **Incorrect** (trap: Wrong model or concept). Substituted judgment is a standard a surrogate uses when the person cannot decide: the surrogate chooses what the person would have chosen. Here the adult makes the decision with help, so no surrogate is involved.

**D** **Incorrect** (trap: Wrong model or concept). A health care power of attorney names an agent to make medical decisions if the person later loses capacity. It covers health care and transfers authority when it applies; here the adult keeps authority over all choices.

**Takeaway:** Supported decision-making keeps authority with the person while chosen supporters help; it is a less restrictive alternative to guardianship.

**Sources:** United Nations Convention on the Rights of Persons with Disabilities (2006), Article 12: Equal recognition before the law; NASW Code of Ethics (2021), 1.03(c)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 131. ETH-1530 (rev 1) · Best answer: B

A 22-year-old client at a community counseling center has a job offer in another city. The client tells the social worker, 'In my family, big decisions are made together. I want my parents to decide this with me.' What is the **BEST** response?

**A.** Encourage the client to make this choice independently
**B.** Help the client plan how to include the parents
**C.** Explore whether the parents are pressuring the client

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). Encouraging an independent choice imposes one view of adulthood on a client who has stated a different, considered preference. Respecting self-determination means supporting how the client wants to decide, not only what the client decides.

**B** **Correct.** Respect for autonomy includes a person's choice to make decisions together with family (Beauchamp & Childress, 2019), and social workers help clients pursue goals in their own way (NASW Code 1.02). The client has said plainly that including the parents is the client's own wish, so the social worker helps carry it out, for example by planning the family conversation or a joint session. It beats C because probing for coercion fits when there are signs of pressure, such as fear or contradiction; here the client describes a shared family value.

**C** **Incorrect** (trap: Skips assessment). Checking for pressure is wise when a client seems fearful, hesitant or contradicts themselves. This client presents family involvement as a personal value, so probing for coercion without any sign of it treats the client's choice as suspect.

**Takeaway:** Self-determination includes choosing to decide with family; support the client's chosen decision process unless signs of coercion appear.

**Sources:** NASW Code of Ethics (2021), 1.02; Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). Oxford University Press, Ch. 4: respect for autonomy; choosing to delegate decisions
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.1 Protecting and promoting client self-determination, including right to refuse · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 132. ETH-1532 (rev 2) · Best answer: A

A client who entered an inpatient psychiatric unit voluntarily for depression with psychotic features asks to leave in 2 days, sooner than the team advises, to keep a new job. The psychosis has cleared, and the psychiatrist's assessment this morning found no suicidal thoughts or intent. The team asks the social worker to help. What is the **MOST** appropriate action?

**A.** Plan the client's discharge with follow-up care arranged
**B.** Repeat a full suicide risk assessment before planning
**C.** Ask the psychiatrist to consider a hold to finish treatment

**Why each option is right or wrong**

**A** **Correct.** A voluntary patient keeps the right to withdraw consent (NASW Code 1.03(a)), and the Code permits limiting self-determination when a client's actions pose a serious, foreseeable and imminent risk (1.02), which this morning's assessment did not find. The psychosis has cleared and this morning's assessment found no suicidal thoughts or intent, so the client's choice to protect the job stands. The social worker makes the earlier discharge as safe as possible: outpatient appointments, medication access and a crisis plan. It beats B because the psychiatrist just assessed risk; repeating it adds delay without new information, though new warning signs would call for reassessment.

**B** **Incorrect** (trap: Good but not best). Reassessment is essential when new warning signs appear or time has passed. Here a full assessment was done this morning with no concerning findings, so repeating it now delays planning without adding information.

**C** **Incorrect** (trap: Overrides the client's choice). A hold is meant for imminent danger, not for completing a recommended course of treatment. Using it to override a low-risk client's choice would violate self-determination and could damage the client's trust in future care.

**Takeaway:** A voluntary patient without imminent risk may leave earlier than advised; help make that discharge as safe as possible.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.03(a)
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### 133. ETH-1533 (rev 2) · Best answer: A

A child protective services investigation closed with no findings, and the agency offered the family voluntary in-home parenting support. At the first home visit, before the social worker can describe the program, the parent says sharply, 'We were cleared. We don't need anybody else checking up on us.' What should the social worker do **FIRST**?

**A.** Acknowledge the frustration behind the comment
**B.** Notify the investigator that the family refused services
**C.** Accept the refusal and explain how to return later
**D.** Explain that the service is voluntary and what it offers

**Why each option is right or wrong**

**A** **Correct.** A parent just cleared by an investigation may meet any follow-up visit with anger and fear of continued surveillance. Acknowledging that frustration first lowers defensiveness and makes room to explain that the service is voluntary, so whatever the parent decides is informed and freely chosen (NASW Code 1.02; Hepworth et al.). It beats D because explaining the program is the right next step, but launching into it over the parent's anger can sound like the agency is pressing on regardless.

**B** **Incorrect** (trap: Misreads law or ethics). Refusing voluntary services is not a new safety concern and is not reportable on its own. Telling the investigator implies a suspicion the closed investigation did not support; a report would be required only if new signs of abuse or neglect emerged.

**C** **Incorrect** (trap: Right action, wrong time). If the parent declines after learning that the service is voluntary, the social worker accepts the refusal and explains how to return (NASW Code 1.02, 1.03(a)). Treating an angry first remark as a final, informed refusal skips the chance to clarify what is being offered.

**D** **Incorrect** (trap: Right action, wrong time). Explaining that the service is voluntary and what it offers is the right next step, and it makes any refusal an informed one. Delivered over the parent's anger, before the feeling is acknowledged, it can sound like a sales pitch from the agency that just investigated the family.

**Takeaway:** When a reluctant client opens with anger, acknowledge the feeling first, then clarify choices so any decision is informed and free.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.03(a); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, engaging involuntary and reluctant clients
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## 134. ETH-1535 (rev 1) · Best answer: A

A 15-year-old in foster care asks to attend the upcoming meeting where the team will discuss changing the permanency goal from reunification to adoption. The foster parent suggests keeping the youth away because the discussion may be upsetting. What is the **MOST** appropriate response by the child welfare social worker?

**A.** Include the youth after preparing them for the meeting
**B.** Hold the meeting without the youth and report back
**C.** Ask the youth to send written views instead

**Why each option is right or wrong**

**A** **Correct.** Law limits minors' self-determination, but social workers still involve young people in decisions about their lives at a developmentally appropriate level (NASW Code 1.02). Federal law requires that case plans for youth 14 and older be developed in consultation with the youth (42 U.S.C. 675(1)(B)). A 15-year-old asking to take part in a decision about adoption should be included, with preparation and support during the meeting. It beats C because written input is a fallback when attendance is not possible; it keeps the youth out of the actual discussion.

**B** **Incorrect** (trap: Overrides the client's choice). Shielding the youth from distress may seem protective, but it shuts the youth out of a decision central to their future. Preparation and support address the risk of upset without removing the youth's voice.

**C** **Incorrect** (trap: Good but not best). Written input respects the youth's views more than exclusion does, and it suits a youth who prefers not to attend. This youth asked to be present, so substituting a letter limits participation without good reason.

**Takeaway:** Involve youth in decisions about their own care at a developmentally appropriate level; prepare and support them rather than excluding them.

**Sources:** NASW Code of Ethics (2021), 1.02; 42 U.S.C. 675 (Social Security Act, Title IV-E case plan requirements), 675(1)(B)

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### 135. ETH-1537 (rev 2) · Best answer: D

A hospital oncology social worker meets for the first time with a client newly diagnosed with early-stage colon cancer. The oncologist has twice explained that surgery offers a strong chance of cure, and the client can restate the risks accurately. The client wants to try herbal treatment from a trusted practitioner for 3 months first. The oncologist asks the social worker to help. What should the social worker do **FIRST**?

**A.** Request a capacity evaluation before honoring the choice
**B.** Respect the choice and keep follow-up care open
**C.** Ask the client's family to encourage the surgery
**D.** Ask about the client's reasons and concerns

#### Why each option is right or wrong

**A** **Incorrect** (trap: Skips assessment). Capacity should be assessed when confusion, psychosis or poor reasoning raises doubt. Ordering an evaluation only because the client disagrees with the physician treats an unwelcome choice as a symptom and can damage trust.

**B** **Incorrect** (trap: Right action, wrong time). Supporting a competent client's informed refusal while keeping follow-up open is right once the social worker understands the decision; competence is presumed, and patients may make choices others consider unreasonable (Appelbaum, 2007). Moving straight to it at a first meeting skips learning what lies behind the choice, which may include fears the client would want addressed.

**C** **Incorrect** (trap: Overrides the client's choice). Recruiting family to push surgery pressures a competent adult and may share health information without consent. Family can be involved if the client wants that, but not to override the decision.

**D** **Correct.** A competent adult may decline recommended treatment (NASW Code 1.02), and the social worker's first task is to understand the decision. The client already knows the medical facts, so the open questions are what draws the client to the herbal plan and what the client fears about surgery; the answers may reveal worries that can be addressed or values that should shape the plan (Miller & Rollnick, 2023). It beats B because respecting the choice may be where this ends, but doing so before hearing the client's reasons skips the conversation the decision deserves.

**Takeaway:** When a competent client declines recommended treatment, first explore the reasons and fears behind the choice, then support the informed decision.

**Sources:** NASW Code of Ethics (2021), 1.02; Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, presumption of competence; right to unreasonable choices; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, engaging: understanding the client's perspective before planning
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### 136. ETH-1539 (rev 3) · Best answer: B

A client with schizophrenia lives in a Housing First apartment program. The client wants to stop attending the program's weekly skills group and use that time at a peer-run drop-in center, explaining that peer staff there helped them find a part-time job and that the extra hours would help them keep it. A case manager warns that skipping the group could put the client's apartment at risk. Which response by the social worker is **MOST** helpful?

**A.** Encourage attending both until the client seems more stable
**B.** Support the change and revise the plan together
**C.** Explain that the apartment depends on attending the group
**D.** Explore what the client hopes to gain at the drop-in center

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Asking the client to do both until they seem more stable keeps a service the client wants to drop and sets a vague condition the client cannot meet. Nothing in the situation suggests a risk that would justify overriding the choice.

**B** **Correct.** In Housing First, housing is not contingent on treatment participation, and tenants choose which services to use (Tsemberis, 2010). Self-determination also covers refusing one service and choosing another (NASW Code 1.02). The client has named a specific alternative that has already supported recovery and a concrete goal for the time (keeping the new job), so the social worker backs the change and revises the plan. It beats D because the client has already said what the center offers and what they hope to gain. It beats C because tying the apartment to group attendance contradicts the program model; the case manager's concern needs correcting.

**C** **Incorrect** (trap: Misreads law or ethics). This repeats the case manager's misunderstanding. In a Housing First program, tenancy rests on ordinary lease obligations, not group attendance. Presenting attendance as a housing condition pressures the client with an inaccurate threat.

**D** **Incorrect** (trap: Good but not best). Exploring a client's hopes is useful when the reasons are unclear. Here the client has already said what the center offers and what the extra hours are for, keeping the new job, so more exploration repeats ground already covered and delays acting on a clear, reasonable choice.

**Takeaway:** Clients may refuse one service and choose another; in Housing First, housing does not depend on treatment participation.

**Sources:** Tsemberis, S. (2010). Housing First: The Pathways Model to End Homelessness for People with Mental Illness and Addiction. Hazelden, separating housing from treatment; consumer choice; NASW Code of Ethics (2021), 1.02

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## 137. ETH-1540 (rev 2) · Best answer: A

Which term describes overriding a competent client's choice, or withholding information from the client, on the grounds that doing so is for the client's own good?

**A.** Paternalism
**B.** Beneficence
**C.** Nonmaleficence

#### Why each option is right or wrong

**A** **Correct.** Paternalism is interfering with a person's choices or information, against their wishes, for their own benefit (Reamer, 2018). It is the central tension with self-determination, which the Code allows a social worker to limit when a client's actions pose a serious, foreseeable and imminent risk (NASW Code 1.02). It beats B because beneficence is the general duty to act for the client's benefit; paternalism is one way of pursuing benefit, by overriding the client's choice (Beauchamp & Childress, 2019).

**B** **Incorrect** (trap: Wrong model or concept). Beneficence is the general duty to promote the client's welfare. It motivates paternalism but does not describe overriding a competent person's choice; most beneficent acts respect the client's decisions.

**C** **Incorrect** (trap: Wrong model or concept). Nonmaleficence is the duty to avoid causing harm. It concerns what the social worker does to the client, not overriding the client's own decisions for the client's benefit.

**Takeaway:** Paternalism means overriding a competent client's choice, or withholding information, for the client's own good; it is the classic threat to self-determination.

**Sources:** Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, client self-determination and paternalism; NASW Code of Ethics (2021), 1.02; Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). Oxford University Press, Ch. 6: paternalism

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## 138. ETH-1542 (rev 3) · Best answer: C

A client with bipolar I disorder arrives at an outpatient session talking rapidly and says they slept 3 hours in two nights. The client plans to withdraw their entire retirement savings tomorrow to fund a business idea from a late-night video. On a call from the session, the prescriber judges that a visit first thing tomorrow morning is soon enough. What should the social worker do **NEXT**?

**A.** Accept the withdrawal as the client's own financial choice
**B.** Send the client for a same-day crisis evaluation
**C.** Ask the client to delay the withdrawal until after the appointment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Respecting financial choices is right when a client can weigh them. Here sleep loss, rapid speech and an impulsive plan to empty retirement savings raise real doubt about judgment, so simply accepting the plan risks lasting harm.

**B** **Incorrect** (trap: Skips assessment). A same-day crisis evaluation fits when mood symptoms bring danger to self or others, such as threats, psychosis or reckless acts with physical risk. Here the prescriber has already judged that the morning visit is soon enough for a financial risk, so this escalation overrides that judgment and the client. New signs of danger before then would call for it.

**C** **Correct.** Self-determination assumes the ability to weigh a decision, and symptomatic bipolar disorder can impair decision making (Appelbaum, 2007). The client shows signs of an emerging episode and plans an irreversible step that could happen before the prescriber visit. Asking the client to wait until after the visit keeps the decision in the client's hands while preventing harm that cannot be undone, which honors self-determination without ignoring risk (NASW Code 1.02). It beats B because the prescriber has already set the timing of the evaluation; the remaining gap is the withdrawal itself.

**Takeaway:** When symptoms may impair judgment, ask the client to postpone irreversible decisions rather than overriding or ignoring the choice.

**Sources:** NASW Code of Ethics (2021), 1.02; Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, bipolar disorder and impaired decision making

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## 139. ETH-1545 (rev 2) · Best answer: B

A nursing home resident in their 80s recently chaired the residents' council and manages their own finances. The resident has begun a romantic relationship with another resident. The resident's adult children call the social worker, upset, ask what staff know about the relationship, and demand that staff keep the two apart. What should the social worker do **FIRST**?

**A.** Assure the children that staff will limit the residents' time alone

**B.** Ask the resident how they want staff to respond to the children

**C.** Explain to the children that the relationship is the resident's choice

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Limiting a capable resident's time with a partner because relatives object overrides the resident's self-determination and privacy rights. Family concern does not give relatives authority over a competent adult's relationships.

**B** **Correct.** Nursing home residents have rights to self-determination and personal privacy, including in their visits and relationships (42 CFR 483.10(f), (h)). The resident shows clear capacity, and the relationship is the resident's private matter, so the first step is to learn the resident's wishes: what may be shared with the children and how to

handle their concerns (NASW Code 1.02). It beats C because the children may need that explanation, but discussing the resident's private life with family before checking with the resident bypasses the resident's control over their own information (HIPAA, 45 CFR 164.510(b)).

**C** **Incorrect** (trap: Right action, wrong time). Explaining the resident's rights may help the children accept the relationship, and it could follow. Doing it before asking the resident means discussing the resident's private life, which the children are asking about, without the resident's agreement, and it takes the matter out of the resident's hands.

**Takeaway:** A capable older adult controls their own relationships and information; ask the resident before responding to family objections.

**Sources:** 42 CFR 483.10 (Resident rights), (f) self-determination; (h) privacy; HIPAA Privacy Rule, 45 CFR 164.510(b); NASW Code of Ethics (2021), 1.02
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### 140. ETH-1651 (rev 2) · Best answer: A

A client with schizophrenia has had a representative payee for Social Security benefits since a hospital stay 5 years ago. For the past 18 months, the client has worked part-time, paid rent from wages on time and kept a written budget. The client tells the social worker, 'I want my check sent to me again.' What is the **MOST** appropriate response?

**A.** Help the client gather evidence and request direct payment
**B.** Ask the payee to give the client a larger weekly allowance
**C.** Advise the client to keep the payee until the next yearly review
**D.** Arrange a formal assessment of the client's money skills first

#### Why each option is right or wrong

**A** **Correct.** Self-determination includes managing one's own money when able (NASW Code 1.02). The Social Security Administration says a beneficiary with a payee can receive benefits directly by showing the ability to manage them, for example with a doctor's statement or other evidence (SSA payee FAQs). Eighteen months of on-time rent and a written budget are that kind of evidence. It beats B because a larger allowance keeps the payee in control and treats a rights request as a spending problem. The client should also know that Social Security may review disability status when a condition improves.

**B** **Incorrect** (trap: Good but not best). A bigger allowance may ease day-to-day frustration and is a reasonable interim step if the client wants it. It leaves the payee in charge, though, and sidesteps what the client asked for: control of their own benefits.

**C** **Incorrect** (trap: Misreads law or ethics). No rule makes a beneficiary wait for a review. Social Security's payee guidance sets no waiting period: the beneficiary can ask to manage their own benefits with evidence such as a doctor's statement, and a payee decision can be appealed.

**D** **Incorrect** (trap: Good but not best). A formal skills assessment would make sense with no track record to go on. Here 18 months of budgeting and on-time rent already show the ability, so another assessment adds a hurdle the client does not need to clear.

**Takeaway:** When a client with a payee shows they can manage money, help them pursue direct payment rather than keep the restriction in place.

**Sources:** NASW Code of Ethics (2021), 1.02; Social Security Administration. FAQs for Beneficiaries Who Have a Representative Payee (ssa.gov/payee), accessed 2026., managing your own benefits; appeals
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### 141. ETH-1654 (rev 1) · Best answer: A

A 20-year-old comes to a first appointment at a community clinic because the parents insisted. The social worker explains that services are voluntary and the client may decline them. The client folds their arms and says, 'They think I'm depressed. I'm just tired of them running my life.' What should the social worker do **NEXT**?

**A.** Ask what the client would want to change, if anything
**B.** End the meeting, since the client is declining services
**C.** Give a brief depression screen to check the parents' concern

**Why each option is right or wrong**

**A** **Correct.** A voluntary adult decides whether to use services, and the social worker helps clients identify and clarify their own goals (NASW Code 1.02). The complaint about being controlled hints at a goal: more say over their own life. Inviting the client to name what they might want, if anything, respects the choice and keeps the door open (Miller & Rollnick, 2023). It beats B because the client has voiced frustration, not a decision to leave; ending now closes off engagement the client may want once their own agenda is heard.

**B** **Incorrect** (trap: Good but not best). Ending is the right response if, after hearing what is offered, the client chooses to stop. The client has not said that yet. They have described feeling controlled, which is an opening rather than a refusal.

**C** **Incorrect** (trap: Right action, wrong time). A depression screen may become useful later, with the client's agreement. Starting with the parents' concern repeats the very dynamic the client objects to and is likely to end engagement before it begins.

**Takeaway:** With a reluctant voluntary client, invite the client's own goals before screening, persuading or closing.

**Sources:** NASW Code of Ethics (2021), 1.02; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, engaging: the client's own goals
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### 142. ETH-1658 (rev 3) · Best answer: D

A 79-year-old hospital patient recovering from a hip fracture wants to go home alone instead of to a rehabilitation facility. A psychiatric consultant found the patient able to make this decision, and the patient knows another fall could happen. Worried, the team asks the social worker to hold off on discharge planning until the patient agrees to rehab. What should the social worker do **NEXT**?

**A.** Request a second capacity evaluation from another consultant
**B.** Ask the patient's adult child to urge the move to rehab
**C.** Have the patient sign a discharge-against-advice form first
**D.** Plan a home discharge with services the patient accepts

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). A repeat evaluation fits when capacity seems to fluctuate or the first assessment was incomplete. Here a consultant has assessed this specific decision; seeking another opinion because the team disagrees treats disagreement as incapacity.

**B** **Incorrect** (trap: Overrides the client's choice). Family can help when the patient invites them in. Asking a relative to push for rehab enlists others to change a capable adult's decision, which undermines the patient's choice and trust in the team.

**C** **Incorrect** (trap: Misreads law or ethics). Against-advice forms are meant for patients who leave before the team considers any discharge appropriate. This patient is ready for discharge and, with capacity, has chosen among settings; the label treats a lawful choice as defiance, can strain trust and follow-up, and does nothing to make the home plan safer.

**D** **Correct.** A capable adult may choose a riskier option; the Code permits limiting self-determination when a client's actions pose a serious, foreseeable and imminent risk (NASW Code 1.02), which the possibility of a future fall does not. Capacity is decision-specific; it has been assessed for this choice, and the patient understands the fall risk (Appelbaum, 2007). The social worker's task now is to make the chosen plan as safe as possible, with home health, therapy, a medical alert device and home changes. It beats A because a second opinion sought only because the team dislikes the answer turns assessment into a tool for overriding the patient.

**Takeaway:** Once a patient is found capable of a decision, help make the chosen plan as safe as possible instead of looking for ways around it.

**Sources:** NASW Code of Ethics (2021), 1.02; Appelbaum, P. S. (2007). Assessment of patients' competence to consent to treatment. New England Journal of Medicine, 357(18), 1834-1840, decision-specific capacity
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### 143. ETH-1662 (rev 1) · Best answer: C

A client with bipolar I disorder has been stable on medication for 3 years and works full-time. The client tells the community mental health social worker that they and their partner plan to try for a baby next year. At a team meeting, a colleague suggests that the worker urge the client to reconsider. What is the **MOST** appropriate response?

**A.** Encourage the client to wait for 5 years of stability
**B.** Share the team's concern that pregnancy may trigger relapse
**C.** Help the client plan pregnancy care with the prescriber

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). An arbitrary stability target puts the team's comfort in charge of a personal decision. Three stable years and full-time work show the client is well placed to plan; what matters now is good preparation.

**B** **Incorrect** (trap: Too directive). Relapse risk after birth is real and belongs in the conversation, but as information for the client's planning, not as a case for changing the decision. Framed as a reason to reconsider, it pressures the client and may discourage openness about the pregnancy.

**C** **Correct.** Decisions about having children belong to the client, and the social worker helps clients pursue their own goals (NASW Code 1.02). Planning ahead is what guidelines favor: medication decisions made with a prescriber before and during pregnancy, because some mood stabilizers carry fetal risks and stopping medication abruptly raises relapse risk (ACOG, 2023). Helping the client prepare for that conversation, and for postpartum support, respects the choice and addresses the real risks. It beats B because presenting the team's worry as a reason to reconsider pressures the client instead of equipping them.

**Takeaway:** Support a client's reproductive choices; help them plan with the prescriber instead of steering them away because of a diagnosis.

**Sources:** NASW Code of Ethics (2021), 1.02; American College of Obstetricians and Gynecologists (2023). Clinical Practice Guideline No. 5: Treatment and Management of Mental Health Conditions During Pregnancy and Postpartum, psychiatric medication decisions in pregnancy
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### 144. ETH-1664 (rev 1) · Best answer: A

A tenant in supportive housing keeps stacks of newspapers and boxes that block the apartment's only exit path. After an inspection, the fire marshal gives the tenant 14 days to clear the path or the lease will be ended. The tenant tells the social worker, 'It's my home. I decide what stays.' What should the social worker do **FIRST**?

**A.** Explain the deadline and what is at stake if it passes
**B.** Arrange a crew to clear the path while the tenant is out
**C.** Ask the property manager to respect the tenant's choice
**D.** Refer the tenant to a treatment group for hoarding

**Why each option is right or wrong**

**A** **Correct.** Self-determination may be limited when a choice poses a serious, foreseeable and imminent risk to the client or others (NASW Code 1.02), and a blocked exit endangers the tenant, neighbors and firefighters. When legal obligations override a client's wishes, the client should be told (NASW Code 1.01). Explaining plainly what the order requires and what is at stake sets up a joint plan the tenant can shape, such as what to move and in what order. It beats D because hoarding treatment may help over time, but the 14-day order comes first.

**B** **Incorrect** (trap: Overrides the client's choice). Clearing the path without the tenant removes the hazard but overrides the tenant in their own home, and forced cleanouts often damage trust and lead to quick reaccumulation. The tenant should lead the work within the safety limit.

**C** **Incorrect** (trap: Ignores risk). A tenant's control over their home matters, but a blocked exit threatens neighbors and emergency responders too. Asking the manager to drop the requirement ignores a real fire risk and a legal order.

**D** **Incorrect** (trap: Right action, wrong time). Hoarding treatment can help the tenant keep the path clear over the long run and may be part of the plan. Starting there leaves the 14-day deadline and the immediate fire hazard unaddressed.

**Takeaway:** When safety law limits a client's choice, tell the client plainly, then let them shape how the requirement is met.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.01
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## 145. ETH-1667 (rev 2) · Best answer: A

A firefighter with PTSD after a fatal blaze has had 4 sessions of prolonged exposure therapy. The client says, 'I can't keep retelling that night. Can we just talk instead?' The client still has nightmares and wants help with them. What should the social worker do **FIRST**?

**A.** Ask what has made these sessions feel so hard
**B.** Describe other effective treatments and let the client choose
**C.** Shift to the supportive talk sessions the client asked for

**Why each option is right or wrong**

**A** **Correct.** Before changing a treatment at a client's request, the social worker learns what is driving the request. Retelling can feel overwhelming midway through exposure, and the client's answer shows whether an adjustment or a different trauma-focused therapy would fit; understanding the client's own view comes before planning (Miller & Rollnick, 2023). The client may still decline exposure (NASW Code 1.02). It beats B because describing alternatives is the right next step, but offering them before hearing the client's reasons skips what makes the choice informed.

**B** **Incorrect** (trap: Right action, wrong time). Clients may refuse a treatment and should hear reasonable alternatives (NASW Code 1.03(a)), and guidelines recommend several trauma-focused therapies, such as cognitive processing therapy and EMDR, that do not rely on repeated detailed retelling (VA/DoD, 2023). This is the right next step once the

social worker knows what makes retelling so hard, so the options match the client's concern.

**C** **Incorrect** (trap: Good but not best). Moving to supportive talk honors the request, and it may be what the client chooses. Doing it at once, without learning what made retelling unbearable or describing trauma-focused options, leaves the nightmares without an effective treatment.

**Takeaway:** When a client asks to stop a treatment midway, first learn what drives the request, then present effective options so the choice is informed.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.03(a); VA/DoD (2023). Clinical Practice Guideline for the Management of Posttraumatic Stress Disorder and Acute Stress Disorder. U.S. Department of Veterans Affairs and Department of Defense, recommended individual trauma-focused psychotherapies (CPT, EMDR, PE); Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, engaging: understanding the client's perspective before planning
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### 146. ETH-1670 (rev 3) · Best answer: B

A group home for adults with serious mental illness supports a resident's wish to ride the city bus alone to a new job. A staff member asks what happens if the resident gets lost. The team leader says an occasional wrong turn is part of ordinary adult life. The team arranges travel training and lets the resident ride alone. Which principle is the team **MOST** likely applying?

**A.** Least restrictive alternative
**B.** Dignity of risk
**C.** Harm reduction
**D.** Beneficence

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). The least restrictive alternative favors the least confining setting or intervention that meets a need, such as community care over hospitalization. It fits partly, but the team leader's stated reason is that occasional mishaps are part of adult life, not the choice of a setting.

**B** **Correct.** Dignity of risk holds that people with disabilities have the right to take the ordinary risks of daily life, and that shielding them from all risk undermines dignity and growth (Perske, 1972). The team weighs the resident's goal against staff worry and supports the attempt with preparation, which promotes self-determination (NASW Code 1.02). It beats A because the least restrictive alternative concerns choosing the least confining setting or intervention, while this team's stated reason is that occasional mishaps belong to an ordinary adult life.

**C** **Incorrect** (trap: Wrong model or concept). Harm reduction lowers the harms of a behavior a person will continue, such as drug use, without requiring them to stop. Riding a bus to work is not a harmful behavior to be reduced.

**D** **Incorrect** (trap: Wrong model or concept). Beneficence is the duty to act for another's good. It can justify protective limits as easily as freedom, so it does not capture the specific idea that facing everyday dangers is part of a full life.

**Takeaway:** Dignity of risk: people with disabilities have the right to face ordinary risks; prepare them rather than prohibit.

**Sources:** Perske, R. (1972). The dignity of risk and the mentally retarded. Mental Retardation, 10(1), whole article; NASW Code of Ethics (2021), 1.02
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### 147. ETH-0008 (rev 1) · Best answer: A

A social worker provides weekly video sessions to an adult client with depression. At the start of a session, the client mentions that they are calling from a vacation rental in another state where the social worker is not licensed. What should the social worker do **FIRST**?

**A.** Determine whether practice is permitted in that state
**B.** Continue the session because the client is already established
**C.** Identify emergency resources in the client's current state
**D.** End the session and terminate services with the client

**Why each option is right or wrong**

**A** **Correct.** Telehealth is generally considered to take place where the client is located. The social worker must follow the laws of that jurisdiction as well as their own (Technology Standards 2.02; NASW Code 1.04(e)). Before going on, the social worker determines whether practice there is allowed, for example through a temporary permit or compact privilege, and then decides how to proceed. It beats C because emergency planning for the client's location matters only if the session can lawfully continue, so it follows once that is settled.

**B** **Incorrect** (trap: Misreads law or ethics). Being an established client does not change where the service takes place. Continuing without checking risks practicing without authorization in the client's current jurisdiction.

**C** **Incorrect** (trap: Right action, wrong time). Knowing emergency resources near a remote client is required (Technology Standards 2.13). It comes after the social worker confirms that serving the client in that state is permitted.

**D** **Incorrect** (trap: Hands off too soon). Ending services is premature, because the social worker does not yet know that state's rules. Even if this session must stop, the social worker would plan for continuity rather than terminate an ongoing client.

**Takeaway:** Telehealth happens where the client is: confirm you may practice in the client's location before continuing.

**Sources:** NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.02; NASW Code of Ethics (2021), 1.04(e)
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### 148. ETH-0113 (rev 2) · Best answer: A

A social worker sees an adult client with generalized anxiety disorder by video each week. The client will spend the next month at a relative's home in another state. The social worker confirms that this state's rules permit the sessions to continue. What should the social worker do **NEXT**?

**A.** Identify emergency services near where the client will stay
**B.** Continue weekly sessions as usual under the current safety plan
**C.** Ask the client to pause sessions until returning home

**Why each option is right or wrong**

**A** **Correct.** Telehealth practice takes place where the client is located, and social workers should know the emergency resources in that location (Tech Standard 2.13). Legal authority is settled, so the next step is updating the emergency plan with services near the relative's home, along with the client's address there. It beats B because the current plan lists resources in the home state that cannot help the client during the stay. Social workers follow the law where the client is located (NASW Code 1.04(e); Tech Standard 2.02).

**B** **Incorrect** (trap: Ignores risk). The existing safety plan points to emergency resources in the home state. If a crisis occurs during the stay, those contacts cannot respond, so continuing unchanged leaves a gap in safety planning.

**C** **Incorrect** (trap: Hands off too soon). Pausing treatment for a month interrupts care the client needs when the social worker has already confirmed the sessions are allowed. A pause would fit only if the other state did not permit practice.

**Takeaway:** For telehealth with a client in a new location, confirm legal authority, then update the emergency plan for that location.

**Sources:** NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.13; NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.02; NASW Code of Ethics (2021), 1.04(e)
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### 149. ETH-1058 (rev 2) · Best answer: A

A client in an intensive outpatient program for alcohol use sends a friend request to the social worker's personal social media account. The program gave each client written social media rules at intake. What is the **MOST** appropriate response?

**A.** Leave it unaccepted and review the policy at the next session
**B.** Send the client a private message explaining the policy
**C.** Delete the request without mentioning it to the client

**Why each option is right or wrong**

**A** **Correct.** Social workers should not accept friend requests from clients on personal social media (NASW Code 1.06(h)), and a social media policy shared at intake is reviewed with clients as needed (Tech Standards 2.10). Raising it at the next session treats the request as clinical material and keeps the relationship clear. A beats C because silently deleting the request avoids the topic; the client may feel rejected without knowing why, and the boundary is never explained.

**B** **Incorrect** (trap: Boundary slip or collusion). Explaining the policy promptly is the right idea, which makes this tempting. Doing it by message from the personal account opens the very channel the policy closes and starts personal electronic contact (NASW Code 1.06(e)); the conversation belongs in session.

**C** **Incorrect** (trap: Good but not best). Not accepting is right, but deleting the request in silence misses the chance to explain the boundary. The client may read it as rejection, which can harm engagement.

**Takeaway:** Decline client friend requests on personal accounts and discuss the social media policy openly in session.

**Sources:** NASW Code of Ethics (2021), 1.06(h); NASW Code of Ethics (2021), 1.06(e); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.10
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### 150. ETH-1059 (rev 1) · Best answer: C

A former client posts an online review of a social worker's private practice. The review claims the social worker 'never returned calls' and mentions the client's divorce. Phone logs show the calls were returned the same day. What is the **BEST** way for the social worker to handle the review?

**A.** Reply with the call dates without naming the client
**B.** Call the former client and ask to have the review removed
**C.** Refrain from any reply that confirms a client relationship

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Leaving out the name does not protect confidentiality: any factual reply confirms the person was a client and discloses service details. Protecting the worker's reputation does not justify the disclosure (1.07(r)).

**B** **Incorrect** (trap: Boundary slip or collusion). Contacting a former client to request removal pressures that person and serves the worker's interests rather than the client's. It risks a further boundary problem and can escalate the conflict.

**C** **Correct.** Social workers must not post identifying or confidential client information online (NASW Code 1.07(r); Tech Standards 2.09). Even a reply that omits the name confirms the reviewer was a client and reveals details of services, and the client's own public post does not waive the worker's duty. C beats A because correcting the facts with call dates is the natural impulse, yet it discloses the relationship. At most, a general statement of practice policies that refers to no one may be posted.

**Takeaway:** Never confirm a client relationship when answering online reviews; a client's public post does not waive confidentiality.

**Sources:** NASW Code of Ethics (2021), 1.07(r); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.09
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### 151. ETH-1060 (rev 1) · Best answer: B

At 10 p.m., a social worker scrolling a public community page sees a post made 15 minutes earlier by a current adult client. It shows a pile of pill bottles with the words 'Done fighting. Goodbye.' The social worker calls the client's phone and reaches voicemail. What should the social worker do **NEXT**?

**A.** Text the client to ask about current safety
**B.** Request an emergency welfare check at the client's home
**C.** Message the client on the platform asking for a call back
**D.** Discuss at the next session how the post was found

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). Asking about safety is right when risk is unclear. Here a goodbye message with pills and an unanswered call already establish imminent danger, and waiting on a text reply loses critical time.

**B** **Correct.** Consent is normally required before gathering client information online, but an exception exists to protect a client from serious, imminent harm (NASW Code 1.03(i)), and disclosure to prevent such harm is permitted (1.07(c)). A goodbye message with pills, posted minutes ago, plus an unanswered call, shows imminent risk. B beats A because a text, like the call, may go unanswered while time passes; once direct contact has failed, emergency responders need to check on the client. The worker can be open with the client about the post afterward.

**C** **Incorrect** (trap: Misuses the technique). Reaching out through the social media platform mixes personal channels with clinical contact and, like the call, may go unanswered. Imminent risk needs emergency responders, not another message.

**D** **Incorrect** (trap: Right action, wrong time). Being open with the client about how the post was found is appropriate, but only after the client is safe. Waiting until the next session ignores imminent risk.

**Takeaway:** Online information found without consent may be acted on to prevent imminent harm; when direct contact fails, send emergency help.

**Sources:** NASW Code of Ethics (2021), 1.03(i); NASW Code of Ethics (2021), 1.07(c)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 152. ETH-1061 (rev 1) · Best answer: D

A client receiving video sessions for depression from home says, in a low voice, that their partner works from home, listens at the door and checks the client's phone. The client wants to keep meeting and works in an office two blocks from the clinic. What is the **BEST** way for the social worker to proceed?

**A.** Continue video sessions using headphones and a code word
**B.** Assess the client's comfort with the clinic's video platform
**C.** Switch to secure text messaging between sessions
**D.** Offer in-person sessions at the clinic during the workday

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). Headphones keep the worker's voice private, but the client must still speak in a home where the partner listens. A code word helps end a call quickly; it does not make disclosure safe.

**B** **Incorrect** (trap: Good but not best). Assessing a client's relationship with technology is useful at the start of remote services. Here the problem is already clear: privacy and possible control at home, not the client's comfort with the platform.

**C** **Incorrect** (trap: Misuses the technique). Text messages arrive on the same phone the partner checks, so this would expose the client's disclosures and could increase danger.

**D** **Correct.** Social workers assess whether remote services suit each client and help arrange alternatives when they do not (NASW Code 1.03(g)); electronic services carry confidentiality risks that must be weighed (Tech Standards 2.04). A partner who listens at the door and checks the phone means privacy at home cannot be assured, and the controlling pattern raises safety concerns. The client's office near the clinic makes in-person sessions practical. D beats A because headphones protect only what the client hears; the partner can still overhear what the client says.

**Takeaway:** When home lacks privacy or safety, remote sessions may be unsuitable; offer a workable in-person or safer alternative.

**Sources:** NASW Code of Ethics (2021), 1.03(g); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.04
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 153. ETH-1062 (rev 1) · Best answer: A

A hospital social worker's work phone breaks, and a replacement will take a week. Several discharged patients need follow-up calls about appointments. A coworker suggests using a personal cell phone in the meantime. What is the **BEST** way to handle patient contact this week?

**A.** Use the office landline and secure messaging system
**B.** Text patients from the personal phone without names
**C.** Use the personal phone and delete messages each day

**Why each option is right or wrong**

**A** **Correct.** Using personal devices for client contact can compromise boundaries and confidentiality, so workers rely on employer-provided systems where available (Tech Standards 2.11) and protect electronic communication with safeguards such as encryption (NASW Code 1.07(m)). The hospital already has an office line and secure messaging, so a personal phone is unnecessary. A beats B because leaving names out of texts does not stop replies, numbers and message histories from sitting on a personal device outside the hospital's safeguards.

**B** **Incorrect** (trap: Misuses the technique). Omitting names reduces exposure, but texts still sit on an unsecured personal device, patients gain the worker's personal number, and replies arrive outside work systems. A personal phone is a last resort, not the first choice.

**C** **Incorrect** (trap: Boundary slip or collusion). Giving patients a personal number blurs the professional boundary, and daily deletion does not undo that. It also erases communications that may need to be documented.

**Takeaway:** Use employer-provided, secure channels for client contact; personal devices blur boundaries and put confidential information at risk.

**Sources:** NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.11; NASW Code of Ethics (2021), 1.07(m)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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## 154. ETH-1063 (rev 1) · Best answer: C

During a video session, a client tearfully discusses the anniversary of a sibling's death and says, 'Sometimes I wonder what the point is.' Before the social worker can ask more, the video freezes and the call drops. Reconnecting fails. At intake, they agreed the social worker would phone the client if video failed. What should the social worker do **FIRST**?

**A.** Ask local police to check on the client at home
**B.** Send a secure message offering to finish next week
**C.** Call the client's number on file

#### Why each option is right or wrong

**A** **Incorrect** (trap: Skips assessment). A welfare check fits imminent risk or a failed attempt to reach a client after a high-risk statement. The worker has not yet tried the agreed call or assessed the remark, so this escalates too soon.

**B** **Incorrect** (trap: Ignores risk). Waiting a week leaves a possible suicidal statement unassessed and ignores the backup plan. Rescheduling fits a routine outage, not one that interrupts a risk disclosure.

**C** **Correct.** Social workers plan for technology failures and discuss backup procedures with clients early (Tech Standards 2.12), and they make reasonable efforts to maintain services when communication is disrupted (NASW Code 1.15). The agreed backup is a phone call, which also lets the worker finish assessing a possible suicidal statement. C beats A because the remark is ambiguous and no intent, plan or means is known; emergency responders become appropriate if the client cannot be reached or the assessment shows imminent risk.

**Takeaway:** When video fails, use the agreed backup channel first, especially if a risk disclosure was interrupted; escalate if contact fails.

**Sources:** NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.12; NASW Code of Ethics (2021), 1.15
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 155. ETH-1064 (rev 2) · Best answer: B

A family caregiver support program moves to remote-only services. An 80-year-old caregiver in a rural area has a basic cell phone and no home internet, and wants to keep meeting with the social worker. Which response is **MOST** helpful?

**A.** Lend a tablet and teach the caregiver to use video
**B.** Hold the sessions as regular voice calls
**C.** Refer the caregiver to an in-person group 60 miles away

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Loaning devices can close access gaps, but this caregiver has no home internet, so a tablet would also need a data plan or hotspot plus training before any session could happen. Services would stall while those barriers are solved.

**B** **Correct.** Social workers recognize differences in clients' access to technology and work to keep those differences from becoming barriers to service (NASW Code 1.05(e)); they also assess each client's ability to use a given technology (1.03(g)). The caregiver already has a cell phone and wants to continue, so phone sessions keep services going right away. B beats A because a tablet would need a separate data connection and new skills before sessions could resume, which adds strain for an already burdened caregiver, while the phone works now.

**C** **Incorrect** (trap: Hands off too soon). A distant in-person group hands off a client who can be served now. The travel burden for a rural caregiver makes this likely to end services rather than continue them.

**Takeaway:** Match remote services to the client's real access; use the technology the client already has rather than requiring new tools.

**Sources:** NASW Code of Ethics (2021), 1.05(e); NASW Code of Ethics (2021), 1.03(g)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 156. ETH-1066 (rev 1) · Best answer: C

A client with low vision asks the agency to email a copy of their record to a personal email account. The agency usually uses an encrypted portal, but the client finds it hard to use with screen-reading software. What is the **BEST** response?

**A.** Require the client to use the encrypted portal for access
**B.** Mail a printed copy to the client's home address instead
**C.** Warn of email risks, then send it if the client accepts
**D.** Email a brief summary rather than the complete record

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Encryption is the right default (1.07(m)), but once warned, a client may choose regular email for their own records. Insisting on a portal the client's screen reader cannot use blocks access.

**B** **Incorrect** (trap: Good but not best). A printed copy is hard for a client with low vision to read and ignores the format requested. Paper is reasonable only if the client prefers it.

**C** **Correct.** Clients have a right to reasonable access to their records, and agencies inform clients how technology is used to provide that access (NASW Code 1.08(a), 1.08(b)). Federal guidance says a client may receive records by unencrypted email on request after being warned that others could read them in transit (HHS FAQ 2060). The portal is a barrier for this client, so honoring the request after a clear warning respects both access and informed choice. C beats A because a security preference cannot deny a client's chosen, lawful form of access.

**D** **Incorrect** (trap: Good but not best). Clients are entitled to their records, not a summary they did not request (1.08(a)). A summary may be offered in addition, but it cannot replace the copy asked for.

**Takeaway:** After warning about risks, honor a client's request to receive their own records by regular email; security defaults cannot bar chosen access.

**Sources:** NASW Code of Ethics (2021), 1.08(a); NASW Code of Ethics (2021), 1.08(b); NASW Code of Ethics (2021), 1.07(m); HHS Office for Civil Rights, HIPAA FAQ, FAQ 2060: transmitting PHI to an individual by an unsecure method on request
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 157. ETH-1067 (rev 1) · Best answer: A

A social worker leads a weekly video support group for adults with chronic pain. Members agreed to join from private spaces. Midway through, one member speaks up from a crowded coffee shop, with nearby voices clearly audible, just as another member begins describing a recent hospitalization. What should the social worker do **FIRST**?

**A.** Pause and ask the member to move somewhere quiet or wear headphones
**B.** Remind everyone of the group's confidentiality agreement and continue
**C.** Mute the member's microphone and continue the discussion

#### Why each option is right or wrong

**A** **Correct.** In group work, social workers seek agreement on each member's duty to protect what others share and explain that confidentiality cannot be guaranteed (NASW Code 1.07(f)). In a remote group, that duty includes who else can hear. Strangers can hear another member's disclosure through this member's speaker, so the session should pause before more is shared. A beats B because a general reminder lets the disclosure continue while bystanders can still hear it.

**B** **Incorrect** (trap: Good but not best). A reminder is useful, but continuing lets the next member's disclosure be overheard by strangers right now. The immediate exposure has to be stopped first.

**C** **Incorrect** (trap: Misuses the technique). Muting stops the member from speaking but not from hearing; people nearby can still hear other members through the member's speaker. The privacy risk remains.

**Takeaway:** In remote groups, stop and fix an immediate privacy exposure before members continue sharing.

**Sources:** NASW Code of Ethics (2021), 1.07(f)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 158. ETH-1068 (rev 1) · Best answer: B

A client logs on for a scheduled video session during a lunch break. The social worker sees that the client is driving, with the phone mounted on the dashboard. The client says it is fine to talk because the drive is 20 minutes. What should the social worker do **FIRST**?

**A.** Switch to audio so the client can watch the road
**B.** Ask the client to park safely or reschedule the session
**C.** Keep the session to a short check-in during the drive

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Audio-only can make a session workable when video is the barrier, but here the risk is driving while engaged in therapy. Emotional conversation distracts a driver whether or not the camera is on.

**B** **Correct.** Social workers assess whether remote services are suitable at the time of service (NASW Code 1.03(g)) and act to promote the client's well-being (1.01). A session while driving divides attention, endangers the client and others, and leaves no privacy for difficult topics. Pausing until the client is parked, or rescheduling, protects safety without ending care. B beats A because audio alone still asks the client to hold an emotional conversation while driving, which remains a dangerous distraction.

**C** **Incorrect** (trap: Ignores risk). Shortening the session reduces but does not remove the danger of talking through emotional material while driving, and it signals that the risk is acceptable.

**Takeaway:** Check each remote session's suitability; do not hold sessions while a client is driving or otherwise unsafe.

**Sources:** NASW Code of Ethics (2021), 1.03(g); NASW Code of Ethics (2021), 1.01
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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### 159. ETH-1157 (rev 1) · Best answer: D

An outpatient substance use program sends automatic text reminders before appointments. A new client says they often forget appointments and would like reminders. However, everyone in the household shares one phone, and relatives read the messages on it. Which plan for reminders is **MOST** appropriate?

**A.** Send the standard reminder, since the client asked for one
**B.** Skip reminders to keep the treatment private
**C.** Mail written reminders to the client's home instead
**D.** Agree with the client on a discreet reminder method

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). The client wants reminders, but a standard message from a substance use program would show relatives where the client is being treated. Honoring the request without adjusting the method ignores the privacy risk the client described.

**B** **Incorrect** (trap: Good but not best). Skipping reminders removes the privacy risk but also the help the client asked for with forgetting appointments. Missed sessions could undermine treatment when a discreet method could serve both needs.

**C** **Incorrect** (trap: Misuses the technique). Household mail is another shared channel that relatives may open. Switching to letters without asking the client trades one exposure for another and is slower than the reminder the client needs.

**D** **Correct.** Providers must accommodate reasonable requests to receive communications by alternative means (HIPAA, 45 CFR 164.522(b)), and social workers should protect the confidentiality of electronic communications (NASW Code 1.07(m)). This client needs reminders to keep appointments yet faces a real privacy risk on a shared phone. Working out a method together, such as a call at a set time or a generic text that names no program, meets both needs and leaves the client in control. It beats B because dropping reminders protects privacy at the cost of the support the client asked for.

**Takeaway:** Tailor reminders and other electronic contact to each client's privacy situation, agreeing on a method that meets needs without revealing treatment.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.522(b); NASW Code of Ethics (2021), 1.07(m)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 1 of 3

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### 160. ETH-1158 (rev 1) · Best answer: C

A client living with bipolar I disorder tells the social worker about a public video they posted describing their first year back at work after a long hospitalization. The client says they would like the social worker to watch it and talk about it at the next session. What is the **MOST** appropriate response?

**A.** Explain that the Code bars viewing a client's online videos
**B.** Follow the client's channel to show support
**C.** View the video as invited and discuss it next session

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). The Code does not ban looking at client content; it requires consent or a compelling reason before an electronic search. A refusal here misreads the rule and may leave the client feeling that a hard-won milestone is unwelcome.

**B** **Incorrect** (trap: Boundary slip or collusion). Following the client's channel creates an ongoing online connection, and the Code advises against personal relationships with clients on social networking sites (1.06(h)). Support can be shown in session instead.

**C** **Correct.** The Code asks for client consent before an electronic search about a client (NASW Code 1.03(i)). Here the client has invited the social worker to view one specific public video and wants to discuss it, so consent is clear and the purpose is clinical. Watching it and processing it together can affirm the client's recovery. It beats A because refusing a clear, consented, treatment-related request is rigid and could feel like rejection. Viewing should stay limited to what the client shared, not a broader look at their accounts (Tech Standard 3.09).

**Takeaway:** With a client's clear invitation and a clinical purpose, viewing content they shared is ethical; keep it limited and process it in session.

**Sources:** NASW Code of Ethics (2021), 1.03(i); NASW Code of Ethics (2021), 1.06(h); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 3.09
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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## 161. ETH-1159 (rev 1) · Best answer: A

While scrolling a community events page on a personal account, a social worker sees a friend's recent party photo that shows a client holding a drink. The client is in treatment for alcohol use disorder and said at the last session that they have been sober for 4 months. What is the **BEST** action for the social worker?

**A.** Tell the client what was seen and discuss it
**B.** Record a possible relapse in the client's chart
**C.** Keep it private to protect the client's trust

#### Why each option is right or wrong

**A** **Correct.** When a social worker comes across client information online by accident, the technology standards advise telling the client and discussing what the worker now knows and what it may mean for the relationship (Tech Standard 3.09). Information found online must also be verified before it is used, and a photo of a drink may not show what it seems. Raising it openly lets the client explain and keeps the relationship honest. It beats C because silence leaves the social worker holding unspoken information that can shape the work, which undermines trust more than a transparent conversation.

**B** **Incorrect** (trap: Skips assessment). Charting a relapse from one unverified photo records speculation as fact. Online information must be checked with the client or the original source before it guides care or enters the record (3.04(a)).

**C** **Incorrect** (trap: Good but not best). Saying nothing can feel like protecting the client, but the social worker now knows something that may affect treatment. The standards favor telling the client how it was found and discussing it, rather than keeping a secret within the relationship.

**Takeaway:** Client information found online by accident should be disclosed to the client and discussed, and verified before it shapes care or the record.

**Sources:** NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 3.09; NASW Code of Ethics (2021), 3.04(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 162. ETH-1160 (rev 1) · Best answer: B

When should a social worker who uses social media professionally review their social media policy with clients?

**A.** When a client first sends a connection request
**B.** At the first interview, updating it as needed
**C.** Once a working alliance has formed

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Explaining the policy only after a client sends a request makes it reactive, and the client may feel rejected. The standards call for setting expectations at the start, so a request can be declined by pointing to a known policy.

**B** **Correct.** Social workers who use social media professionally should have a written policy, share it with clients, review it at the first interview and update it as circumstances change (Tech Standard 2.10). The Code likewise asks workers to discuss their technology policies with clients (NASW Code 1.03(e)). Early review sets expectations about connection requests, messaging, online searches and reviews before a problem arises. It beats C because waiting for an alliance to form leaves common boundary questions unaddressed when they are most likely to come up.

**C** **Incorrect** (trap: Right action, wrong time). Waiting for a working alliance can seem gentler, but clients often reach out online early. The policy belongs with other informed-consent topics at the first interview, then gets updated as needed.

**Takeaway:** Share a written social media policy at the first interview, alongside informed consent, and update it as circumstances change.

**Sources:** NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.10; NASW Code of Ethics (2021), 1.03(e)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 163. ETH-1161 (rev 1) · Best answer: A

A social worker at a group practice learns that the new video platform automatically records every telehealth session and stores the files for quality review. The consent forms do not mention recording, and clients have not been told. Only the practice's owners can change the platform's settings. The office manager says the vendor contract requires it. What should the social worker do **FIRST**?

**A.** Raise the recording problem with the practice's leadership

**B.** Tell each of their clients about the recordings at the next session

**C.** Continue sessions as usual since the contract requires recording

**D.** Report the practice to the state licensing board

**Why each option is right or wrong**

**A** **Correct.** Clients must give informed consent before sessions are recorded (NASW Code 1.03(h)), and an employer's policies or contracts should not interfere with ethical practice, which workers should improve through the organization (3.09(b), 3.09(d)). Because only the owners can stop the recording and every client is affected, the first step is to bring it to practice leadership so recording stops or consent is obtained practice-wide. It beats B because telling one's own clients matters but leaves the recordings running and other clinicians' clients uninformed. Disclosure to clients should follow promptly as part of the fix.

**B** **Incorrect** (trap: Right action, wrong time). Clients deserve to know, and informing them is part of the remedy. Done alone and first, it leaves the recordings running for

everyone and does not reach other clinicians' clients. Disclosure should follow once the practice acts.

**C** **Incorrect** (trap: Misreads law or ethics). A vendor contract does not replace each client's informed consent to recording. Employer policies and contracts should not interfere with ethical practice (3.09(d)), so continuing as usual lets an unconsented practice go on.

**D** **Incorrect** (trap: Skips assessment). A board complaint skips the internal channel that can fix the problem today. Formal outside reporting fits if the practice refuses to act after the concern is raised, not before anyone in the practice has been asked.

**Takeaway:** When an agency practice violates client consent, raise it first with whoever can fix it, then make sure clients are informed.

**Sources:** NASW Code of Ethics (2021), 1.03(h); NASW Code of Ethics (2021), 3.09(b); NASW Code of Ethics (2021), 3.09(d)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.10 Ethics of electronic and technology-mediated practice · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 164. ETH-0114 (rev 1) · Best answer: A

Which approach to recruiting current clients for an agency research study is **MOST** appropriate?

**A.** Assure clients that declining has no effect on services
**B.** Offer participants priority placement on the agency's waitlist
**C.** Ask each client's own social worker to invite them

#### Why each option is right or wrong

**A** **Correct.** Research consent must be voluntary, free of coercion or undue inducement, and clients must know they can decline or withdraw without penalty (NASW Code 5.02(e), 5.02(i)). Current clients may fear that saying no will affect their care, so stating plainly that services do not depend on participation protects voluntariness. Federal human subjects rules likewise require minimizing coercion and undue influence (45 CFR 46.116). It beats C because a client's own social worker holds influence over the client, which makes refusing harder.

**B** **Incorrect** (trap: Misreads law or ethics). Linking participation to faster access to services is an undue inducement. Clients who need help may feel they have to join to get it, which undermines voluntary consent.

**C** **Incorrect** (trap: Boundary slip or collusion). A request from the client's own social worker carries the weight of the helping relationship, so clients may agree to please the worker. Having neutral research staff recruit reduces this pressure and the dual-role conflict.

**Takeaway:** Recruit clients for research so refusal is easy: no link to services and no pressure from the treating social worker.

**Sources:** NASW Code of Ethics (2021), 5.02(e); NASW Code of Ethics (2021), 5.02(i); 45 CFR Part 46 (Common Rule), 45 CFR 46.116
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** MOST APPROPRIATE · **Cognitive** recall · **Difficulty** 1 of 3

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### 165. ETH-1075 (rev 2) · Best answer: A

An outpatient clinic ran a quality-improvement project tracking depression scores for clients in a new group program. The results look strong, and the program director asks the social worker to prepare the client data for a journal article. The clinic has a research partnership with a nearby university. What should the social worker do **FIRST**?

**A.** Submit the project for IRB review before using the data
**B.** Remove client identifiers from the data set
**C.** Ask former group members for permission to publish
**D.** Draft the article, since the project used routine care data

**Why each option is right or wrong**

**A** **Correct.** Planning to publish does not by itself turn quality improvement into research, but analyzing client data for a journal article may be a systematic investigation designed to contribute to generalizable knowledge (45 CFR 46.102(l)). Social workers engaged in research follow participant-protection guidelines and consult institutional review boards (NASW Code 5.02(d)). The IRB, not the social worker, decides whether the work is research, whether it is exempt, what consent is needed and how data must be handled. A beats B because removing identifiers may well be part of the approved plan, but the social worker does not decide alone that the data need no review.

**B** **Incorrect** (trap: Good but not best). De-identifying data is often required and may support an exemption. Doing it first and treating the review question as settled skips the step that decides how the data may be used; it belongs within IRB review.

**C** **Incorrect** (trap: Right action, wrong time). Seeking permission sounds respectful, but the IRB determines whether consent is needed and what it must say. Contacting former clients without an approved procedure can itself raise privacy concerns.

**D** **Incorrect** (trap: Misreads law or ethics). Data gathered in routine care can still become research when they are used to produce generalizable knowledge. Publishing without review skips the protections that 5.02(d) expects.

**Takeaway:** Before using practice data for publication, let the IRB decide whether it is research and what review applies; don't decide alone.

**Sources:** NASW Code of Ethics (2021), 5.02(d); 45 CFR Part 46 (Common Rule), 45 CFR 46.102(l)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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### 166. ETH-1076 (rev 1) · Best answer: C

A university study will interview youth in residential treatment about their placement experiences. A 15-year-old's guardian has signed permission. The social worker helping with recruitment explains the study to the youth, who listens, asks one question, and then says they do not want to take part. What is the **MOST** appropriate response?

**A.** Enroll the youth, since the guardian gave written permission
**B.** Review the study's benefits again and encourage joining
**C.** Accept the decision and note the refusal in the study log

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). A guardian's permission is necessary but not sufficient. For a youth able to understand the study, the youth's own agreement is also sought, so permission alone cannot override a clear refusal.

**B** **Incorrect** (trap: Overrides the client's choice). Answering questions is appropriate, but re-selling the study after a clear refusal pressures a youth in residential care, where staff shape daily life. Consent must be free of coercion (5.02(e)).

**C** **Correct.** When participants cannot give legal consent, researchers obtain permission from a proxy and also seek the person's own assent to the extent they are able (NASW Code 5.02(g)). Federal rules direct IRBs to provide for soliciting the assent of children capable of giving it (45 CFR 46.408(a)), and an interview about placement offers no direct benefit that could justify setting a refusal aside. Declining must carry no penalty (5.02(e)). C beats B because repeating the pitch to a youth who depends on program staff turns encouragement into pressure.

**Takeaway:** In research, a capable minor's refusal stands even when a parent or guardian has given permission.

**Sources:** NASW Code of Ethics (2021), 5.02(g); NASW Code of Ethics (2021), 5.02(e); 45 CFR Part 46 (Common Rule), 45 CFR 46.408(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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## 167. ETH-1077 (rev 1) · Best answer: B

A social worker conducts interviews for an IRB-approved study of job-training outcomes. A question about past jobs leads one participant to describe a workplace assault and become very distressed. The social worker stops the interview, stays with the participant until calm, and, with consent, links them to a counseling service. What should the social worker do **NEXT**?

**A.** Call the participant's emergency contact about the distress
**B.** Report the event to the IRB promptly
**C.** Describe the event at the IRB's annual continuing review

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). The participant is a calm adult who has accepted a referral. Calling an emergency contact would reveal study participation and private details without consent, breaking the confidentiality promised under 5.02(m).

**B** **Correct.** Participant protection comes first, and that step is done: the interview stopped and support was arranged (NASW Code 5.02(j), 5.02(k)). An unexpected, study-related harm that suggests more risk than the protocol anticipated must be reported promptly, so the IRB can decide whether the questions, consent form or procedures need to change (45 CFR 46.108(a)(4)(i)). B beats C because waiting months for continuing review leaves other participants exposed to the same question in the meantime.

**C** **Incorrect** (trap: Right action, wrong time). Continuing review is the place for routine updates, not for an unanticipated problem involving risk. IRBs must have procedures for prompt reporting, so this timing is wrong.

**Takeaway:** After protecting a research participant, report unanticipated study-related harm to the IRB promptly, not at the next scheduled review.

**Sources:** NASW Code of Ethics (2021), 5.02(j); NASW Code of Ethics (2021), 5.02(k); NASW Code of Ethics (2021), 5.02(m); 45 CFR Part 46 (Common Rule), 45 CFR 46.108(a)(4)(i)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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## 168. ETH-1078 (rev 2) · Best answer: A

A researcher designs an online survey that collects no names, contact details, IP addresses, or linking codes. Which protection does this design provide?

**A.** Anonymity
**B.** Confidentiality
**C.** De-identification

#### Why each option is right or wrong

**A** **Correct.** Anonymity means no one, including the researcher, can link responses to an individual, because identifying information is never collected. That matches a survey with no names, contact details, IP addresses or codes. NASW Code 5.02(m) asks researchers to protect participants' anonymity or confidentiality. A beats C because de-identification strips identifiers from data that were collected with them, whereas here identifiers never existed. Data without identifiers are not identifiable private information (45 CFR 46.102(e)(5)).

**B** **Incorrect** (trap: Wrong model or concept). Confidentiality means the researcher can identify participants but promises not to reveal who said what. This survey cannot identify anyone, so the stronger protection, anonymity, applies.

**C** **Incorrect** (trap: Wrong model or concept). De-identification removes identifiers after data are collected, so the original data could once be linked to people. Here identifiers are never gathered at all.

**Takeaway:** Anonymity means no one can link data to a person; confidentiality means the researcher can but protects the link.

**Sources:** NASW Code of Ethics (2021), 5.02(m); 45 CFR Part 46 (Common Rule), 45 CFR 46.102(e)(5)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 169. ETH-1079 (rev 2) · Best answer: C

A social worker at a family shelter is helping a university team design a study of residents' health needs. To boost enrollment, a team member proposes giving each participant priority on the shelter's waitlist for permanent housing. What is the **BEST** way for the social worker to respond to the proposal?

**A.** Offer participants added case management hours instead
**B.** Recommend offering no compensation for taking part
**C.** Suggest a modest cash payment separate from services

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Extra case management sounds modest and helpful, so this is tempting. It still ties a needed shelter service to joining the study, so residents who depend on the shelter may feel unable to refuse; 5.02(e) calls for consent free of undue inducement or penalty.

**B** **Incorrect** (trap: Good but not best). Paying participants for their time is not unethical in itself. Dropping all compensation solves the inducement problem but goes further than needed and may undervalue residents' time.

**C** **Correct.** Research consent must be voluntary and free of coercion or undue influence (45 CFR 46.116(a)(2); NASW Code 5.02(e)). Tying a scarce, life-changing service like housing priority to enrollment would pressure residents to join and make shelter services depend on research. Modest payment for time is common and acceptable when it is not large enough to override judgment. C beats B because removing all compensation goes further than needed; fair payment for time respects participants without coercing them.

**Takeaway:** Research incentives must never involve access to services or be large enough to override a person's judgment about joining.

**Sources:** 45 CFR Part 46 (Common Rule), 45 CFR 46.116(a)(2); NASW Code of Ethics (2021), 5.02(e)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 170. ETH-1166 (rev 1) · Best answer: C

Under the NASW Code of Ethics, what must social workers tell research participants about withdrawing from a study?

**A.** They may withdraw before data collection begins
**B.** They may withdraw if the review board approves
**C.** They may withdraw at any time without penalty

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Withdrawal is not limited to the start of a study. Participants keep the right throughout, including after data collection begins, because consent continues for as long as they take part.

**B** **Incorrect** (trap: Misreads law or ethics). Review boards approve a study's design, not each participant's choice to leave. Requiring approval to withdraw would pressure people to stay, which conflicts with voluntary participation (5.02(e)).

**C** **Correct.** The Code directs those doing evaluation or research to inform participants of their right to withdraw at any time without penalty (NASW Code 5.02(i)). Consent is ongoing, not a one-time signature, so a participant can stop midway, after data collection has started, without losing services. It beats A because limiting withdrawal to the period before data collection would turn consent into a binding contract. Consent must also be free of coercion and of penalty for declining (5.02(e)).

**Takeaway:** Research consent is ongoing: participants may stop at any time, without penalty, and must be told so.

**Sources:** NASW Code of Ethics (2021), 5.02(i); NASW Code of Ethics (2021), 5.02(e)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 171. ETH-1167 (rev 2) · Best answer: A

A social worker leads the evaluation of a parenting program that runs at four sites. Outcomes improved at three sites but not at the fourth, which had heavy staff turnover during the year. Worried about renewal, the agency director asks the social worker to leave the fourth site out of the report to the funder. What is the **BEST** response?

**A.** Report every site and explain the fourth site's context
**B.** Drop the fourth site from the report as asked
**C.** Combine the four sites into one overall average

#### Why each option is right or wrong

**A** **Correct.** Evaluators must report findings accurately and must not fabricate or falsify results (NASW Code 5.02(o)); leaving out an unfavorable site misrepresents the program's results. Accurate reporting can still be fair to the agency: describing the staff turnover at the fourth site gives the funder a plausible explanation and a lesson for improvement. Social workers monitor and evaluate programs (5.02(a)), and that work can guide decisions only if the funder sees the full picture. It beats C because pooling the sites technically includes the data but hides the site-level difference the funder most needs to see.

**B** **Incorrect** (trap: Misreads law or ethics). Dropping the site because its results are disappointing is selective reporting, which falsifies the overall findings. Funding pressure does not change the duty to report results accurately.

**C** **Incorrect** (trap: Misuses the technique). Pooling all sites into a single average includes the data but hides the site-level difference that matters most. Reporting choices made to conceal an unfavorable result still misrepresent the findings.

**Takeaway:** Funding pressure never justifies dropping or masking unfavorable evaluation results; report them with honest context.

**Sources:** NASW Code of Ethics (2021), 5.02(o); NASW Code of Ethics (2021), 5.02(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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### 172. ETH-1168 (rev 1) · Best answer: B

A social worker is conducting an IRB-approved research interview with an older adult about caring for a spouse who died last year. Partway through, the participant begins crying while describing the final weeks of the spouse's illness. What should the social worker do **FIRST**?

**A.** Continue with the protocol so the data stay consistent
**B.** Pause and acknowledge how painful this is to recall
**C.** End the interview and give a list of grief resources

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). Sticking to the protocol puts data ahead of the participant's well-being. The Code requires protecting participants from unwarranted distress, and data consistency does not outweigh responding to visible grief.

**B** **Correct.** Researchers must protect participants from unwarranted distress and make sure they can reach supportive services (NASW Code 5.02(k), 5.02(j)). The first step when a participant becomes upset is to pause and respond to the person, acknowledging the grief rather than pressing on or shutting down. Then the social worker can offer choices: a break, skipping questions, stopping or continuing, which respects the right to withdraw (5.02(i)). It beats C because ending the interview without checking what the participant wants removes their choice and can feel like dismissing their grief.

**C** **Incorrect** (trap: Right action, wrong time). Offering grief resources is appropriate, and ending is an option if the participant chooses it. Doing it first, without pausing or asking, decides for the participant and can feel abrupt at a vulnerable moment.

**Takeaway:** When a research participant becomes distressed, pause and respond to the person first, then offer the choice to continue, skip or stop.

**Sources:** NASW Code of Ethics (2021), 5.02(k); NASW Code of Ethics (2021), 5.02(j); NASW Code of Ethics (2021), 5.02(i)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.11 Research ethics: IRBs, human subjects protections, consent · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 173. ETH-0005 (rev 1) · Best answer: C

A social worker is the only mental health provider in a small rural town. A new client's child plays on the same youth soccer team as the social worker's child, so they will run into each other at weekly games. The nearest other provider is 2 hours away. What is the **MOST** appropriate action?

**A.** Refer the client to the provider who is 2 hours away

**B.** Continue treatment without raising the overlap with the client

**C.** Agree with the client on how to handle outside contact

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). Referral fits when an overlapping relationship risks exploitation or impaired judgment. Here the contact is incidental, and a 2-hour trip could put care out of reach. Referring would shift the social worker's boundary task onto the client.

**B** **Incorrect** (trap: Misreads law or ethics). Saying nothing leaves the client unprepared for public contact and risks an accidental breach, such as a friendly greeting that reveals the relationship. Managing boundaries is the social worker's job, and it starts with a conversation.

**C** **Correct.** Dual relationships that cannot reasonably be avoided are not automatically unethical. The social worker is responsible for setting clear, appropriate boundaries that protect the client (NASW Code 1.06(c)). Talking with the client ahead of time about greetings, privacy and conversation at games, then documenting the plan and consulting as needed, manages the risk. It beats A because sending the client 2 hours

away creates a real barrier to care when the overlap is incidental and manageable. Rural practice ethics accepts that some overlap cannot be avoided (Reamer, 2020).

**Takeaway:** When a dual relationship can't be avoided, the social worker sets clear boundaries with the client, documents the plan and consults.

**Sources:** NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, unavoidable dual relationships in small communities **Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 174. ETH-0106 (rev 1) · Best answer: B

In a remote town with one mental health provider, a social worker treats a client who owns the only hair salon. At intake, the two agreed on how to handle contact at the salon. During the social worker's next haircut, the client refuses payment, saying, 'You've helped me so much.' What is the social worker's **BEST** response?

**A.** Accept the free haircut as a sign of the client's gratitude

**B.** Pay the usual price and explore the gesture at the next session

**C.** Use a salon in another town for the rest of treatment

#### Why each option is right or wrong

**A** **Incorrect** (trap: Boundary slip or collusion). Accepting a free service adds a financial benefit to an existing overlap and blurs the boundary the two agreed on. Small tokens of gratitude can sometimes be accepted, but a service with real value given at the client's expense is different.

**B** **Correct.** In small communities, overlapping roles can be unavoidable, and the social worker is responsible for keeping them from becoming exploitative (NASW Code 1.06(c)). Paying as any customer would keeps the salon contact ordinary and avoids taking a financial benefit from a client. Exploring the gesture in session respects the client's gratitude and uses it clinically. It beats C because the overlap was already managed at intake; the problem is the free service, not the salon visits, and avoiding the only salon disrupts community life without addressing what the gesture means (Reamer, 2020).

**C** **Incorrect** (trap: Good but not best). Avoiding the salon could be reasonable if the contact became harmful, but here the overlap was planned and working. Changing salons does not address the gift and treats an unavoidable small-town overlap as something that has to be eliminated.

**Takeaway:** When overlapping roles are unavoidable, keep outside contact ordinary, decline financial benefits from clients, and explore gestures clinically.

**Sources:** NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, unavoidable dual relationships in small communities **Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 175. ETH-1500 (rev 1) · Best answer: B

During a home visit, an 84-year-old client who lives alone on a fixed income hands the social worker a holiday card with $150 in cash inside. The client says the social worker has been 'like family' this year and would be hurt by a refusal. What is the social worker's **BEST** response?

**A.** Accept the cash and give it to the agency's client fund
**B.** Decline the cash warmly and talk about what the gift means
**C.** Accept the cash to spare the client's feelings
**D.** Return the cash later by mail with a policy note

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Passing the money to an agency fund feels like a compromise, but the client still loses $150 they may need, and the meaning of the gesture goes unexplored. Some agencies accept gifts made to the organization itself; this gift was personal.

**B** **Correct.** The Code has no gift standard, so gifts are weighed as conflict-of-interest and boundary questions: value, timing, motive and the client's means (NASW Code 1.06(a)). Cash worth $150 from a client on a fixed income is a real financial benefit, so the social worker declines it. Declining in person, with warmth, and talking about what the gesture means honors the client's feelings and protects the relationship. It beats A because handing the money to the agency still takes it from the client and leaves its meaning unaddressed (Reamer, 2020).

**C** **Incorrect** (trap: Boundary slip or collusion). Accepting a meaningful sum to avoid hurt feelings puts the worker's comfort ahead of the client's interests and blurs the professional role. A small token, such as the card itself, could be kept; the cash is different.

**D** **Incorrect** (trap: Misuses the technique). Returning the money is the right decision carried out the wrong way. A mailed return with a policy note can feel like rejection and skips the conversation in which the client's gratitude and isolation can be acknowledged.

**Takeaway:** Weigh gifts by value, motive and the client's means; decline money or valuable gifts warmly and discuss their meaning.

**Sources:** NASW Code of Ethics (2021), 1.06(a); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, gifts from clients
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 176. ETH-1501 (rev 1) · Best answer: A

At the last of 10 sessions at a community clinic, a client gives the social worker a small jar of homemade jam. The client explains that in their family, sharing food is how people show gratitude. What is the **MOST** appropriate response?

**A.** Accept the jam and express appreciation
**B.** Decline the jam, explaining that gifts blur roles
**C.** Suggest the client share the jam with clinic staff

**Why each option is right or wrong**

**A** **Correct.** Small, low-value gifts with cultural meaning can generally be accepted. The Code has no gift ban; gifts are weighed under the conflict-of-interest standard by value, timing and meaning (NASW Code 1.06(a)). A jar of jam at the final session is a modest, culturally rooted thank-you with no risk of exploitation, and accepting it graciously honors the client and the ending. It beats C because politely redirecting the gift still sidesteps it and can read as rejection at a sensitive moment (Reamer, 2020).

**B** **Incorrect** (trap: Misreads law or ethics). Refusing treats every gift as a boundary violation, which the Code does not require. Turning down a small, culturally meaningful token at termination can shame the client and undo the goodwill of a good ending. Declining fits money or valuable gifts.

**C** **Incorrect** (trap: Good but not best). Redirecting the gift to staff avoids accepting it personally, but the client offered it to the social worker as a personal thank-you. The polite deflection can feel like rejection and misses the cultural meaning of the gesture.

**Takeaway:** Small, culturally meaningful gifts can usually be accepted graciously; refusing every gift can harm the relationship.

**Sources:** NASW Code of Ethics (2021), 1.06(a); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, gifts from clients
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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## 177. ETH-1502 (rev 1) · Best answer: D

A social worker who is going through a divorce reviews notes from 4 recent sessions with a client whose marriage is ending. In each session, the social worker spent much of the time describing their own separation. The client recently said, 'It helps to know you get it.' What should the social worker do **FIRST**?

**A.** Keep sharing, since the client finds it helpful
**B.** Transfer the client to a colleague without a similar history
**C.** Tell the client that future sessions will focus on them
**D.** Seek supervision about the pattern of self-disclosure

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). The client's comment shows the disclosures feel validating, but the sessions now center on the worker. Disclosure that meets the worker's needs blurs the role and crowds out the client's own grief, even when the client says it helps.

**B** **Incorrect** (trap: Hands off too soon). Transfer is a large step for a problem the worker can likely correct with supervision. It disrupts the client's care and hands off a boundary issue the worker is responsible for fixing. Transfer fits if supervision shows the worker cannot regain objectivity.

**C** **Incorrect** (trap: Right action, wrong time). Bringing the focus back to the client is a needed change and will come soon. Done first, without supervision, it treats the symptom but leaves the worker's own unresolved reaction, which produced the pattern, unexamined.

**D** **Correct.** Self-disclosure is appropriate only when it serves the client, stays brief and returns the focus to the client (Hepworth et al.). Long, repeated accounts of the worker's own divorce suggest the worker's needs are driving the sessions, a sign that a personal difficulty is affecting practice. The Code directs social workers to seek consultation when personal problems interfere with their work (NASW Code 4.05(b)). It beats C because refocusing is the right change, but without supervision the worker may not understand or resist the pull to disclose.

**Takeaway:** When self-disclosure starts serving the worker's needs, seek supervision first, then refocus sessions on the client.

**Sources:** NASW Code of Ethics (2021), 4.05(b); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Ch. 5: authenticity and self-disclosure

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 178. ETH-1503 (rev 1) · Best answer: C

In an intensive outpatient program for alcohol use disorder, a client says nobody really stays sober and asks the social worker, 'Have you ever been through this?' The social worker has been in recovery for 12 years, and program policy allows staff to share their recovery status. What is the **BEST** response?

**A.** Describe the social worker's own path to sobriety in detail

**B.** Explain that the social worker's personal life stays private

**C.** Confirm it briefly, then turn back to the client's doubts

#### Why each option is right or wrong

**A** **Incorrect** (trap: Boundary slip or collusion). Some disclosure can help here, but a detailed story shifts the session to the worker and can burden the client or invite comparison. Disclosure should be short enough that the focus quickly returns to the client's own recovery.

**B** **Incorrect** (trap: Good but not best). Keeping personal information private is a sound default and is often correct. In this program, though, brief recovery disclosure is allowed, and a flat refusal misses a chance to answer the client's hopelessness directly.

**C** **Correct.** Self-disclosure is appropriate when it is brief, relevant and offered for the client's benefit, with the focus returned to the client (Hepworth et al.). The clinical issue is the client's hopelessness about recovery, the program permits recovery disclosure, and a short confirmation can show that sobriety is possible. Turning back to the client's doubts keeps the session theirs. It beats B because privacy is a reasonable default, but here a brief, permitted disclosure speaks directly to the hopelessness the client raised (Reamer, 2020).

**Takeaway:** Self-disclose only briefly, for the client's benefit, and return the focus to the client right away.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Ch. 5: authenticity and self-disclosure; Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, self-disclosure

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 179. ETH-1504 (rev 1) · Best answer: B

At a neighborhood street fair, a social worker runs into a former client whose services ended 3 years ago. After a friendly conversation, the former client asks the social worker out on a date. How should the social worker respond?

**A.** Accept, since over 2 years have passed
**B.** Decline, citing the lasting professional duty
**C.** Offer to meet as friends instead of dating

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). The 2-year idea comes from some state laws and other professions, not from the NASW Code, which sets no waiting period. The national standard for social workers is to avoid romantic or sexual relationships with former clients.

**B** **Correct.** The NASW Code directs social workers to avoid sexual activity or contact with former clients and sets no time limit; a worker who claims an exception bears the full burden of showing no exploitation (NASW Code 1.09(c)). The power and personal knowledge gained in the helping relationship persist after services end, so a clear, kind decline is correct. It beats C because a friendship is also a dual relationship with a former client, carrying the same risk of exploitation (NASW Code 1.06(c)).

**C** **Incorrect** (trap: Boundary slip or collusion). Offering friendship sounds like a gentle middle ground, but it creates a new dual relationship with a former client, which the Code cautions against when there is a risk of exploitation or harm.

**Takeaway:** The NASW Code sets no time limit after which a romantic relationship with a former client becomes acceptable.

**Sources:** NASW Code of Ethics (2021), 1.09(c); NASW Code of Ethics (2021), 1.06(c)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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### 180. ETH-1505 (rev 2) · Best answer: A

A social worker is providing family therapy for a 19-year-old client. The client's 27-year-old sibling attended 3 of the sessions. Between sessions, the sibling messages the social worker to ask for a dinner date, adding, 'I'm technically outside the treatment.' What is the **MOST** ethical response?

**A.** Decline the invitation and keep the professional role
**B.** Accept, since the sibling is outside the client role
**C.** Accept after the family's treatment has ended
**D.** Decline and transfer the family to a colleague

**Why each option is right or wrong**

**A** **Correct.** Social workers avoid romantic or sexual involvement with clients' relatives or others close to them when there is a risk of exploitation or harm, and the worker, not the other person, sets the boundary (NASW Code 1.09(b)). The sibling took part in sessions, so the worker holds information and influence that could harm the client and family. A clear decline that keeps the professional role protects the treatment. It beats D because the invitation can be handled without disrupting the family's care; transfer is needed only if the boundary cannot be held.

**B** **Incorrect** (trap: Misreads law or ethics). Standing outside the identified-client role does not remove the risk. The sibling took part in therapy, and a relationship would affect the client and the family's treatment. The Code extends this caution to clients' relatives and close others.

**C** **Incorrect** (trap: Boundary slip or collusion). Waiting until treatment ends does not remove the risk, because the worker would still hold what was learned in family sessions. The family might also need to return for services, which the relationship would make difficult (Reamer, 2020).

**D** **Incorrect** (trap: Hands off too soon). Declining is right, but transfer is an overreaction here. The family's work would be disrupted over a boundary the worker can simply hold. Transfer would fit if the worker's objectivity were compromised or the sibling kept pressing.

**Takeaway:** Avoid romantic involvement with clients' relatives or close others; the social worker, not the other person, holds the boundary.

**Sources:** NASW Code of Ethics (2021), 1.09(b); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, intimate relationships with clients' relatives
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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## 181. ETH-1506 (rev 1) · Best answer: C

An employee assistance program social worker reviews the next day's schedule and sees that a newly assigned client is someone the social worker had a brief sexual relationship with 10 years ago. The two have had no contact since. The program has 4 other staff social workers. What should the social worker do?

**A.** Begin services, since the relationship ended long ago
**B.** Meet once to learn whether the history concerns the client
**C.** Ask that the case be reassigned to a colleague
**D.** Disclose the history and let the client decide

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Time does not change the rule. The Code bars clinical services to a former sexual partner with no time limit, because the prior intimacy can distort judgment and the client's trust no matter how long ago it ended.

**B** **Incorrect** (trap: Boundary slip or collusion). Even a single session would be clinical service to a former sexual partner. Checking how the client feels about the history misses the point when the past relationship itself is the barrier to serving this client.

**C** **Correct.** Social workers do not provide clinical services to people with whom they have had a prior sexual relationship, because the history makes clear boundaries hard to keep and risks harm (NASW Code 1.09(d)). With 4 colleagues available, reassigning the case before any contact protects the client and spares them an awkward first meeting. It beats D because the client's agreement does not cure the conflict; the Code's bar applies regardless of time passed or the client's preference (Reamer, 2020).

**D** **Incorrect** (trap: Misreads law or ethics). Openness is usually a good instinct, but here it shifts the decision onto the client and may pressure them. The Code's bar does not depend on the client's consent, and the disclosure itself could be uncomfortable for the client.

**Takeaway:** Do not provide clinical services to a former sexual partner; arrange for a colleague to serve the client before services begin.

**Sources:** NASW Code of Ethics (2021), 1.09(d); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, intimate relationships: former partners as clients
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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## 182. ETH-1507 (rev 1) · Best answer: C

A hospital social worker is helping a 79-year-old patient choose a skilled nursing facility for rehabilitation after a stroke. The social worker's partner is the admissions director at one of the 5 facilities with open beds. The patient asks which facility the social worker would pick. What is the **BEST** response?

**A.** Recommend the partner's facility, which has strong ratings
**B.** Leave the partner's facility off the list of options
**C.** Disclose the connection and review each option neutrally

#### Why each option is right or wrong

**A** **Incorrect** (trap: Boundary slip or collusion). Even with strong ratings, a recommendation shaped by the partner's job is the kind of conflict the Code warns about. The patient cannot weigh the advice without knowing about the connection.

**B** **Incorrect** (trap: Good but not best). Removing the facility avoids the appearance of favoritism, but it hides the conflict and takes away an option that might suit the patient best. Disclosure lets the patient decide with full information.

**C** **Correct.** Social workers inform clients of real or potential conflicts of interest and resolve them in the client's interest (NASW Code 1.06(a)). The partner's job could color any advice, so the social worker names the connection and then reviews all 5 facilities against the patient's own priorities, such as location, therapy services and ratings. This keeps the patient's choice informed and free. It beats B because quietly dropping a facility hides the conflict and narrows the options; that facility might be the best fit (Reamer, 2020).

**Takeaway:** Disclose real or potential conflicts of interest and keep the client's choice informed and free.

**Sources:** NASW Code of Ethics (2021), 1.06(a); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, conflicts of interest
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 183. ETH-1508 (rev 2) · Best answer: A

A social worker in private practice often refers clients with eating disorders to residential care. A new residential program's marketing director offers the social worker $500 for each referred client who enrolls. The social worker would provide no services to those clients after the referral. What should the social worker do **FIRST**?

**A.** Decline the payment and base choices on clinical fit
**B.** Ask the program to donate the fees to a client scholarship fund
**C.** Ask for the program's outcome data before referring

#### Why each option is right or wrong

**A** **Correct.** Social workers do not give or receive payment for a referral when they provide no professional service to the referred client (NASW Code 1.16(c)). A per-enrollment fee creates a financial incentive that could bend referral decisions away from clients' needs, a conflict of interest (NASW Code 1.06(a)). Declining the offer now, and referring only when the program fits a client clinically, settles the issue. It beats B because sending the fees to a scholarship fund removes personal gain but keeps a per-enrollment payment that can still bend referral decisions.

**B** **Incorrect** (trap: Misreads law or ethics). Redirecting the money avoids personal gain, which makes this tempting. The payment would still be tied to each client who enrolls, so the pull toward this program remains, and the Code's bar on referral payments when no service is provided (1.16(c)) is not avoided by changing where the money goes.

**C** **Incorrect** (trap: Right action, wrong time). Checking a program's outcomes is good practice before referring anyone, and the social worker may do it later. It does not address the fee, which is improper whatever the data show, so the offer should be declined first.

**Takeaway:** Refuse payment for referrals when no service is provided; base every referral on clinical fit.

**Sources:** NASW Code of Ethics (2021), 1.16(c); NASW Code of Ethics (2021), 1.06(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 184. ETH-1509 (rev 1) · Best answer: B

A social worker at a community agency is planning a kitchen remodel. The social worker's partner collected bids, and the lowest bid comes from a contractor who is a current client of the social worker. The client does not yet know the social worker is the homeowner. What is the **MOST** appropriate action?

**A.** Hire the client and pay the full bid amount
**B.** Accept one of the other bids instead
**C.** Ask the client in session how the job would feel

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Paying the full price avoids taking a discount, but it still makes the client the worker's paid contractor. Any disagreement about the work could spill into treatment, and the client may feel unable to complain or say no.

**B** **Correct.** Social workers avoid dual relationships with clients when there is a risk of exploitation or harm (NASW Code 1.06(c)). Hiring a current client creates a business tie in which disputes over cost, quality or schedule could damage treatment, and the client may feel unable to object. Unlike an unavoidable small-town overlap, this one is easy to avoid by choosing another bidder. It beats C because raising the job in session would put the client on the spot and turn treatment time toward the worker's own interests (NASW Code 1.06(b)).

**C** **Incorrect** (trap: Good but not best). Asking the client's view sounds respectful, but the condition is already clear: this is an avoidable dual relationship. Raising it would pressure the client and use session time for the worker's own business needs.

**Takeaway:** When a dual relationship with a client is easy to avoid, avoid it rather than try to manage it.

**Sources:** NASW Code of Ethics (2021), 1.06(c); NASW Code of Ethics (2021), 1.06(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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### 185. ETH-1510 (rev 1) · Best answer: D

At a community mental health center, a client tells the social worker that their prescription copay went up and they are $40 short this month. The client asks to borrow the money until payday, promising to repay it in 2 weeks. The center has an emergency assistance fund. What is the **BEST** response?

**A.** Lend the money with a written repayment date
**B.** Give the client $40 as a one-time gift
**C.** Suggest the client ask relatives for emergency help
**D.** Help the client apply to the emergency fund

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). A written repayment date makes the loan look businesslike, but it turns the worker into the client's creditor. If repayment fails, the debt can strain or end the helping relationship.

**B** **Incorrect** (trap: Boundary slip or collusion). A gift seems kind, but money from the worker blurs the role and can create dependence or a sense of obligation. It also sets a precedent the worker cannot sustain with this client or others.

**C** **Incorrect** (trap: Good but not best). Asking family can be part of problem solving, but it may be impossible and leaves the medication need unmet. The center's own fund is a more reliable resource the worker can help the client use today.

**D** **Correct.** Personal financial ties with clients create a conflict of interest and a dual relationship that can leave the client feeling indebted to the worker (NASW Code 1.06(a)). The client's need is real, and the center's emergency fund meets it through the agency rather than the worker's own money. Helping the client apply keeps the relationship professional and solves the problem now. It beats C because relatives may be unable or unwilling to help, while the fund is a resource the social worker can help the client reach today (Reamer, 2020).

**Takeaway:** Meet clients' financial needs through agency and community resources, not through personal loans or gifts from the worker.

**Sources:** NASW Code of Ethics (2021), 1.06(a); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, altruistic gestures
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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### 186. ETH-1511 (rev 3) · Best answer: A

A 14-year-old in residential treatment, who has a history of sexual abuse, greets the social worker with long, full-body hugs before each session. The client says the hugs make them feel safe. Other staff have started to comment on the greetings. What is the social worker's **BEST** response?

**A.** Offer a side hug or handshake and discuss the change
**B.** Keep the hugs but make each one brief
**C.** Step back from hugs and avoid raising the topic

#### Why each option is right or wrong

**A** **Correct.** Social workers avoid physical contact that could cause psychological harm and set clear, culturally sensitive boundaries when some contact is appropriate (NASW Code 1.10). For a teen with a sexual-abuse history, prolonged full-body hugs can confuse boundaries, yet a sudden rejection could feel like punishment. Offering a safer greeting and talking it through preserves the client's sense of safety while modeling healthy limits (SAMHSA, 2014). It beats C because pulling away without explanation leaves the client guessing and may feel like rejection by a trusted adult.

**B** **Incorrect** (trap: Boundary slip or collusion). Shortening the hugs moves toward a limit, which makes this tempting. Full-body hugs with a teen who has been sexually abused still blur what safe touch looks like, and an unexplained change teaches nothing; the social worker, not the client, is responsible for physical boundaries.

**C** **Incorrect** (trap: Misuses the technique). Ending the hugs is the right direction, but doing it silently is the wrong method. The client may feel rejected or blamed, and the chance to teach and practice a safe greeting is lost.

**Takeaway:** The social worker sets physical-contact boundaries, offering safer alternatives and discussing them, especially with trauma survivors.

**Sources:** NASW Code of Ethics (2021), 1.10; SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: safety
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 187. ETH-1512 (rev 1) · Best answer: B

A social worker in a state prison provides counseling to an incarcerated client. Near the end of a session, the client hands the social worker a sealed letter and asks the worker to mail it to the client's adult child from outside the facility. The client says the letter holds family news. What is the **MOST** appropriate response?

**A.** Mail the letter as a one-time favor to the client
**B.** Explain how to send it through the facility's mail
**C.** Read the letter first, then mail it if it is harmless
**D.** Report the request to facility security as misconduct

#### Why each option is right or wrong

**A** **Incorrect** (trap: Boundary slip or collusion). A small favor can feel like kindness, but carrying mail outside official channels gives the client a special arrangement, may break facility rules and can be used to press for more favors later.

**B** **Correct.** The power imbalance in a prison makes favors outside official channels a boundary risk: they create a special relationship, can become leverage and may break facility rules (NASW Code 1.06(c)). Explaining kindly how to send the letter through the facility's mail meets the real need, contact with family, while keeping the professional role clear. It beats D because the request may be innocent; declining and redirecting handles it without punishing the client, and the worker can follow facility reporting rules if pressure continues (Reamer, 2020).

**C** **Incorrect** (trap: Misreads law or ethics). Screening the letter does not fix the problem. The worker would still be bypassing the facility's mail process, and reading private mail adds a privacy intrusion that falls outside the counseling role.

**D** **Incorrect** (trap: Skips assessment). Reporting may be needed if the client keeps pressing or the request suggests contraband, but treating a first, possibly innocent request as misconduct jumps ahead of the facts and can damage trust.

**Takeaway:** In settings with large power imbalances, decline personal favors outside official channels and redirect clients to the proper process.

**Sources:** NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, altruistic gestures
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 188. ETH-1513 (rev 1) · Best answer: C

A social worker in private practice is asked by a close friend to provide therapy for the friend's 15-year-old, who has been anxious and refusing school. The friend says they trust no one else with their child. Several qualified child and adolescent providers practice nearby. What is the **BEST** response?

**A.** Agree, and keep the friendship separate from sessions
**B.** Agree once the teen and friend sign informed consent
**C.** Decline and give names of other local providers
**D.** Assess the teen once, then refer to another provider

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Keeping the friendship out of sessions is hard to do in practice. The worker would hold private information about the teen while socializing with the parent, and either relationship could strain the other.

**B** **Incorrect** (trap: Misreads law or ethics). Informed consent is required for any treatment, but it does not remove a conflict. Signed forms leave the overlapping roles, and the risks to confidentiality and objectivity, exactly where they were.

**C** **Correct.** Social workers avoid dual relationships when there is a risk of exploitation or harm and the relationship can reasonably be avoided (NASW Code 1.06(c)). Treating a close friend's child would mix friendship with a clinical role, threaten the teen's confidentiality and make honest feedback to the parent difficult. Because qualified providers are nearby, declining and offering referrals serves the teen best. It beats D because even one assessment session starts the dual relationship and makes the teen tell their story twice (Reamer, 2020).

**D** **Incorrect** (trap: Good but not best). A single assessment can feel like a helpful bridge, but it already places the worker in a clinical role with a friend's child. It also means the teen repeats their story when a direct referral is available.

**Takeaway:** When a dual relationship can reasonably be avoided, decline and refer, even when a friend or relative asks.

**Sources:** NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, dual relationships: avoidable overlapping roles
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 189. ETH-1514 (rev 3) · Best answer: B

A social worker provided couples therapy to two partners for 8 months. The partners have since separated and are disputing custody of their children. One former partner asks the social worker to write a letter to the court describing that partner as the better parent. What is the **MOST** appropriate response?

**A.** Write a letter describing that partner's parenting strengths
**B.** Explain why the past role rules out taking a side
**C.** Write the letter once the other partner also consents

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Writing for one partner abandons the neutral role the worker held with both and offers a custody opinion the worker never formed through an evaluation. It could also disclose couples material without the other partner's consent.

**B** **Correct.** When serving couples or families, social workers clarify their obligations to each person and anticipate conflicts, such as being asked to testify in a custody dispute (NASW Code 1.06(d)). As the couple's therapist, the worker owed duties to both partners and never evaluated either as a parent, so advocating for one would betray the other and exceed what the worker knows. Explaining this role limit answers the request honestly. It beats C because consent addresses confidentiality but not the role conflict or the missing custody evaluation (Reamer, 2020).

**C** **Incorrect** (trap: Good but not best). Getting both partners' consent solves the confidentiality problem and may seem fair. It leaves the role problem in place: the worker would still be taking one side and giving a parenting opinion without having done a custody evaluation.

**Takeaway:** In couples or family work, clarify duties to each person; a former couples therapist does not advocate for one partner in custody disputes.

**Sources:** NASW Code of Ethics (2021), 1.06(d); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, conflicts of interest: forensic and custody roles
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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## 190. ETH-1515 (rev 1) · Best answer: C

At intake, a social worker learns that a new client lives 3 houses away on the same block. The two have only exchanged casual greetings. The social worker discusses the overlap with the client, who wants to continue, and the supervisor agrees the risk is low. What should the social worker do **NEXT**?

**A.** Transfer the client to a colleague to be safe
**B.** Wait to record the overlap until a problem arises
**C.** Document the discussion and the boundary plan

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). Transfer would fit if the overlap threatened objectivity or the client wanted a change. Here the client prefers to continue and the supervisor judged the risk low, so transfer would override the client's choice without need.

**B** **Incorrect** (trap: Misreads law or ethics). Waiting leaves no record of how the worker assessed and managed the overlap. If a problem later arises, the missing documentation makes it hard to show that the boundary was handled with care.

**C** **Correct.** When a social worker continues with a client despite an overlapping relationship, the steps taken to manage it belong in the record: the discussion, the client's choice, the consultation and the plan for contact outside sessions (Reamer, 2020). Timely, sufficient documentation supports continuity of care and shows the

worker owns the boundary (NASW Code 3.04(b)). It beats A because the client chose to continue, the supervisor judged the risk low, and transfer would override that choice with no clear benefit.

**Takeaway:** After deciding to continue despite an overlapping relationship, document the discussion, consultation and boundary plan.

**Sources:** NASW Code of Ethics (2021), 3.04(b); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, risk management: documenting boundary decisions
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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### 191. ETH-1516 (rev 3) · Best answer: B

On the day of discharge from an inpatient psychiatric unit, a patient thanks the social worker and asks for the worker's personal cell phone number, saying they fear no one will answer at night 'if things get bad again.' The patient starts outpatient care next week. What is the social worker's **BEST** response?

**A.** Share the number for after-hours emergencies
**B.** Provide clinic and 24-hour crisis contacts
**C.** Record the request in the discharge note
**D.** Explain the policy against sharing personal numbers

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Sharing the number feels caring, but it creates a personal channel the worker cannot staff around the clock. In a real crisis the patient might wait for a reply instead of reaching help that is always available.

**B** **Correct.** Giving a client a personal phone number blurs the professional boundary and invites contact the worker cannot reliably answer (NASW Code 1.06(c); Technology Standard 2.11). The patient's stated fear is that no one will answer at night, so the best response gives dependable professional options that answer at any hour: the outpatient clinic's line and 24-hour crisis contacts such as the 988 Suicide and Crisis Lifeline. Offering them also declines the personal number without a lecture. It beats D because explaining the policy answers the request but leaves the patient's fear of a night-time crisis unmet on the day of discharge.

**C** **Incorrect** (trap: Good but not best). Documenting the request and the response is good practice, but a note is not a response to the patient. It leaves the patient without a crisis plan at the moment of discharge.

**D** **Incorrect** (trap: Good but not best). Holding the boundary is correct, and a brief explanation can accompany the crisis contacts. Explaining the policy alone, though, answers the request while ignoring the need the patient named, help at night if things get bad, at the moment the patient is leaving the unit.

**Takeaway:** Meet a client's need for crisis support through professional, always-available channels rather than a worker's personal phone.

**Sources:** NASW Code of Ethics (2021), 1.06(c); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.11
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 192. ETH-1517 (rev 1) · Best answer: A

Which approach to client testimonials is consistent with the NASW Code of Ethics?

**A.** Avoid soliciting testimonials from current clients
**B.** Request testimonials from clients who are doing well
**C.** Ask for reviews during a client's final session

**Why each option is right or wrong**

**A** **Correct.** The Code directs social workers to avoid soliciting testimonial endorsements, including permission to reuse past statements, from current clients or others vulnerable to undue influence (NASW Code 4.07(b)). The power and dependence in a helping relationship mean a client may feel unable to refuse. The Technology Standards apply the same caution to online testimonials (Technology Standard 2.14). It beats C because a client at a final session is still a current client, and the ending can heighten the wish to please the worker.

**B** **Incorrect** (trap: Misreads law or ethics). Choosing clients who are doing well does not reduce the pressure they may feel to agree. Current clients are vulnerable to undue influence regardless of progress, so the Code advises against asking them.

**C** **Incorrect** (trap: Misreads law or ethics). Timing the request at the last session seems to avoid affecting treatment, but the person is still a current client. Gratitude and the wish to end well can make refusal especially hard at that moment.

**Takeaway:** Do not solicit testimonials from current clients or others vulnerable to undue influence.

**Sources:** NASW Code of Ethics (2021), 4.07(b); NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.14
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 193. ETH-1518 (rev 2) · Best answer: D

Under the NASW Code of Ethics, which condition must be present before a social worker accepts goods or services from a client instead of a fee?

**A.** The goods match the fee's market value
**B.** The client's insurance coverage has lapsed
**C.** A supervisor approves the plan in writing
**D.** The client proposes it with informed consent

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Equal value can seem fair, but market value is not the Code's test. A barter of matched value can still be coercive or harmful, and the Code's conditions focus on initiative, consent, local acceptance and necessity.

**B** **Incorrect** (trap: Misreads law or ethics). Lost coverage often prompts barter, but financial hardship is not one of the Code's conditions. The worker should first consider other options, such as a sliding fee scale or referral to lower-cost care.

**C** **Incorrect** (trap: Misreads law or ethics). Supervisory approval may be wise, and some agencies require it, but it is not a Code condition. The Code places the burden on the social worker and requires the client's own initiative and informed consent.

**D** **Correct.** The Code says social workers should avoid accepting goods or services as payment. Barter is acceptable only in very limited circumstances: it is accepted among local professionals, essential to providing services, negotiated without coercion, and entered into at the client's initiative with informed consent (NASW Code 1.13(b)). The social worker bears the full burden of showing it is not harmful. It beats C because supervisory approval may be good agency practice but is not a Code condition and cannot substitute for the client's initiative and consent.

**Takeaway:** Barter only in very limited cases: client-initiated with informed consent, locally accepted, essential and uncoerced; the worker bears the burden.

**Sources:** NASW Code of Ethics (2021), 1.13(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 194. ETH-1519 (rev 1) · Best answer: A

After discussing the request with the hospice team, a social worker accepts a family's invitation to a patient's memorial service. The social worker sits near the back, offers brief condolences to the family and leaves before the reception. Which concept does the social worker's attendance **MOST** likely illustrate?

**A.** Boundary crossing
**B.** Boundary violation
**C.** Dual relationship
**D.** Countertransference

#### Why each option is right or wrong

**A** **Correct.** A boundary crossing is a departure from a strict professional role that is not harmful and may even help the client; a boundary violation is a departure that exploits or harms (Gutheil & Gabbard, 1993). Attending a memorial after consulting the team, briefly and in a professional manner, is a recognized crossing that can support the family's grief. It beats B because nothing here exploits the family or serves the worker's own needs; the planning and limits are what keep it a crossing (Reamer, 2020).

**B** **Incorrect** (trap: Wrong model or concept). A violation harms or exploits the client, for example by serving the worker's own needs. A brief, planned, team-approved appearance at a memorial does neither, so this label does not fit the facts given.

**C** **Incorrect** (trap: Wrong model or concept). A dual relationship means taking on a second role, such as friend, business partner or employer. Attending a service in a professional capacity, without forming a new relationship, stays within a single role.

**D** **Incorrect** (trap: Wrong model or concept). Countertransference is the worker's emotional reaction to a client. The stem describes a planned professional action, and nothing suggests the worker's own feelings drove the decision.

**Takeaway:** Boundary crossings are harmless or helpful departures from strict roles; violations exploit or harm the client.

**Sources:** Gutheil, T. G., & Gabbard, G. O. (1993). The concept of boundaries in clinical practice: Theoretical and risk-management dimensions. American Journal of Psychiatry, 150(2), 188-196, boundary crossings versus boundary violations; Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, boundary crossings and violations
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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## 195. ETH-1650 (rev 2) · Best answer: C

A social worker in long-term recovery attends a weekly community 12-step meeting, which is the worker's main recovery support. One evening, a current outpatient client walks in, sees the social worker and sits down. The social worker is unsure whether to keep attending. The client's next session is in 4 days. What should the social worker do **FIRST**?

**A.** Discuss the encounter with the client at the next session
**B.** Find a different meeting to attend from now on
**C.** Talk through the overlap with a supervisor or consultant

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Raising the encounter with the client is necessary, and it should happen soon. Doing it before the worker has sorted out how personal recovery needs shape the plan risks steering the client toward whatever is easiest for the worker.

**B** **Incorrect** (trap: Good but not best). Leaving the meeting gives up the worker's main recovery support and settles the question alone, before talking with anyone. Changing meetings may end up being part of the plan, but only after consultation and a conversation with the client.

**C** **Correct.** When a boundary question involves the social worker's own life, self-interest can cloud judgment, so consultation comes before acting. The meeting supports the worker's recovery, and the client has an equal claim to it, so both people's needs are at stake. A consultant helps the worker plan how to raise the encounter, protect the client's privacy and weigh options such as attending different meetings (NASW Code 2.05(a), 1.06(c); Reamer, 2020). It beats A because the worker is still unsure whether to keep attending, and the 4 days before the session leave time to sort out personal interests first; the talk with the client comes next.

**Takeaway:** When an overlapping relationship touches the social worker's own personal life, seek consultation before deciding how to handle it with the client.

**Sources:** NASW Code of Ethics (2021), 2.05(a); NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, unanticipated encounters with clients in personal settings
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 196. ETH-1652 (rev 2) · Best answer: B

On a pediatric intensive care unit, a patient's parent has just learned that the child died. The hospital social worker has supported the family through the 2-week stay. Sobbing, the parent turns and reaches out to hug the social worker. What is the **BEST** response?

**A.** Offer the parent a hand on the shoulder instead
**B.** Accept the brief hug and stay with the parent
**C.** Step back gently and offer a chair and tissues

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). A hand on the shoulder is a respectful form of contact and would fit if the parent seemed unsure about touch. Redirecting a hug the parent has already begun, at the moment of the death, signals rejection without reducing any real risk.

**B** **Correct.** The Code does not ban touch. It warns against contact that could cause psychological harm and makes the social worker responsible for clear, culturally sensitive boundaries when contact is appropriate (NASW Code 1.10). A brief hug, started by a grieving parent at the moment of a child's death on an open unit, is a boundary crossing that meets the parent's need, not the worker's (Reamer, 2020). It beats A because swapping in a different touch at that moment reads as rejection while adding no protection that the brief hug lacks.

**C** **Incorrect** (trap: Misreads law or ethics). Stepping back treats all physical contact as forbidden, which the Code does not require, and it can feel like abandonment at the worst moment. Declining touch is right when contact could harm the client or confuse the relationship.

**Takeaway:** Brief, client-initiated, comforting touch that serves the client's needs can be appropriate; the social worker remains responsible for the boundary.

**Sources:** NASW Code of Ethics (2021), 1.10; Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, boundary crossings and physical contact
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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### 197. ETH-1655 (rev 2) · Best answer: C

A social worker in a housing program volunteers for a ballot campaign to fund affordable housing, a measure that would help many clients. The campaign asks volunteers to share flyers urging a yes vote. The social worker plans to hand them out on home visits and tell clients that support is optional. Which action is **MOST** consistent with the NASW Code of Ethics?

**A.** Hand out the flyers, stressing that support is optional
**B.** Keep the election out of home-visit conversations
**C.** Offer nonpartisan voting information in place of the flyers
**D.** Give campaign flyers to clients who ask about the measure

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Saying support is optional sounds respectful, but the power gap remains: clients who rely on the worker for housing help may agree just to keep the relationship smooth. Stating that a request is voluntary does not remove the pressure of a partisan request from one's case manager.

**B** **Incorrect** (trap: Good but not best). Steering clear of the campaign avoids exploitation, but silence misses a chance to support clients' civic participation, which the Code encourages. Neutral voting information serves that aim without promoting the worker's side.

**C** **Correct.** Social workers should not use a professional relationship to advance their own political interests (NASW Code 1.06(b)), yet they should help the public take an informed part in shaping policy (6.02). Nonpartisan information on registering, voting and where to learn about ballot measures serves clients without pressing the worker's position. It beats A because clients who depend on the worker for housing help may not feel free to refuse, so calling support optional does not remove the pressure. The worker remains free to campaign on personal time, away from clients.

**D** **Incorrect** (trap: Boundary slip or collusion). Waiting for clients to ask is less pushy, but handing over campaign material still enlists the professional relationship for the worker's cause. If a client asks, the worker can point to neutral sources that describe the measure.

**Takeaway:** Support clients' civic participation with nonpartisan information; do not use the professional relationship to advance the worker's political cause.

**Sources:** NASW Code of Ethics (2021), 1.06(b); NASW Code of Ethics (2021), 6.02
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 198. ETH-1657 (rev 1) · Best answer: B

A hospital social worker serves on the ethics committee. A case comes to the committee in which an ICU patient's adult child, the health care agent, disagrees with the medical team. The social worker recognizes the agent as a former client from 3 years ago. The committee meets in an hour. What is the **BEST** action?

**A.** Tell the chair the agent is a former client and step aside
**B.** Step aside, saying a conflict exists without naming it
**C.** Take part, since services ended 3 years ago
**D.** Ask the agent for permission before taking part

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Disclosing and stepping aside is the usual way to handle a conflict, which makes this tempting. Naming the agent as a former client, though, reveals that the person received services, which is confidential. The chair needs to know that a conflict exists, not its nature.

**B** **Correct.** Two duties apply. A conflict of interest that could affect impartial judgment calls for stepping back (NASW Code 1.06(a)), and the fact that someone was a client is itself confidential (1.07(c)). Recusing while saying only that a conflict exists meets both duties: the committee's review stays fair, and the former client's privacy holds. It beats

A because naming the past treatment relationship, even to a colleague, discloses confidential information that the committee does not need in order to manage the conflict.

**C** **Incorrect** (trap: Boundary slip or collusion). The Code sets no expiration date on conflicts involving former clients. Knowing the agent's history from treatment could color the worker's view of the agent's motives, and the agent might later learn that the former provider weighed in.

**D** **Incorrect** (trap: Good but not best). Seeking permission involves the former client in the worker's committee role and still leaves the worker's impartiality in question. One party's consent cannot cure a conflict that affects the whole committee's deliberation.

**Takeaway:** Resolve a conflict of interest by stepping back, and disclose only that a conflict exists when its nature is confidential.

**Sources:** NASW Code of Ethics (2021), 1.06(a); NASW Code of Ethics (2021), 1.07(c)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 199. ETH-1659 (rev 1) · Best answer: A

A client in outpatient treatment arrives upset. The client found the social worker's personal social media account and saw a public post supporting a cause the client strongly opposes. 'I don't know if I can trust you now,' the client says. What should the social worker do **FIRST**?

**A.** Invite the client to describe how the post affected the relationship
**B.** Make the account private and remove the post
**C.** Explain that personal views stay separate from the client's care

#### Why each option is right or wrong

**A** **Correct.** A client's report of shaken trust signals an alliance rupture, and repair starts by inviting the client to say more and taking the concern seriously without defending (Safran & Muran, 2000). The Code warns that personal information posted online can cause boundary confusion or harm to clients (NASW Code 1.06(f)). Hearing the client's experience comes before deciding what to change. It beats B because tightening privacy settings is a sensible later step for the worker, but doing it first leaves the client's question about the relationship unanswered in the room.

**B** **Incorrect** (trap: Right action, wrong time). Reviewing privacy settings is worth doing, and the worker should do it soon. As a first response, it attends to the worker's exposure while the client sits with an open question about the relationship.

**C** **Incorrect** (trap: Good but not best). Reassurance that personal views won't affect care may be true, but offered first it defends the worker and can feel dismissive. It is more useful after the client has been heard, as part of repairing the relationship.

**Takeaway:** When a client finds the worker's personal online content and trust is shaken, explore the rupture before defending or changing settings.

**Sources:** Safran, J. D., & Muran, J. C. (2000). Negotiating the Therapeutic Alliance: A Relational Treatment Guide. Guilford Press, rupture resolution; NASW Code of Ethics (2021), 1.06(f)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 200. ETH-1661 (rev 1) · Best answer: B

A community mental health agency hires a former client as a peer support specialist. The new hire finished treatment with a staff social worker 2 years ago and did well. The program director asks that social worker to be the new hire's direct supervisor, citing their good rapport. What is the **MOST** appropriate response from the social worker?

**A.** Accept, setting clear rules that keep supervision work-focused
**B.** Ask that another staff member supervise the new hire
**C.** Object to the agency hiring its former clients as staff

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Clear rules are the right tool when an overlapping relationship cannot be avoided, as in a small town. Here another supervisor is available, and evaluative power over a former client carries real risk, so avoiding the overlap is better.

**B** **Correct.** A dual relationship with a former client should be avoided when it carries a risk of exploitation or harm, and that applies to relationships that follow one another (NASW Code 1.06(c)). As supervisor, the worker would judge the job performance of someone who once shared private struggles; the employee may find it hard to disagree, and the worker's view may be colored by the treatment history (Reamer, 2020). It beats A because boundary rules manage an overlap that cannot be avoided, and this one is easy to avoid.

**C** **Incorrect** (trap: Good but not best). Hiring people with lived experience is a recognized strength of recovery-oriented services. The ethical problem lies in the supervisory pairing, not in employing a former client, so opposing the hire overcorrects.

**Takeaway:** Avoid taking a role with power over a former client when someone else can readily fill it.

**Sources:** NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, dual relationships with former clients
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 201. ETH-1663 (rev 1) · Best answer: A

In a group practice, a social worker learns at the second session that a new client manages the department where the social worker's adult child works. The client is deciding on layoffs in that department. After the social worker explains the conflict, the client says, 'It doesn't bother me. I'd rather stay with you.' What should the social worker do **NEXT**?

**A.** Arrange for a colleague to take over the client's care
**B.** Continue, with a written agreement on outside contact
**C.** Ask how the client feels about the overlap

**Why each option is right or wrong**

**A** **Correct.** When a conflict of interest could impair impartial judgment, the social worker resolves it in the client's favor, which can mean ending the relationship with a proper referral (NASW Code 1.06(a)) and an orderly transfer (1.16(b)). The client's layoff choices could affect the worker's own child, so the worker cannot respond neutrally to the client's work stress, whatever the client prefers. A colleague in the same practice makes transfer easy. It beats B because written boundaries handle incidental contact, not a personal stake in the client's decisions.

**B** **Incorrect** (trap: Good but not best). A boundary agreement fits an unavoidable overlap, such as living on the same block. It cannot remove the worker's personal stake in the client's layoff decisions, which affects judgment inside sessions, not just contact outside them.

**C** **Incorrect** (trap: Right action, wrong time). Checking the client's comfort is part of disclosing a conflict, and the client has already answered. Asking again delays a decision that rests on the worker's impartiality, which the client's comfort cannot restore.

**Takeaway:** A conflict that compromises the worker's impartial judgment calls for transfer, even when the client prefers to stay.

**Sources:** NASW Code of Ethics (2021), 1.06(a); NASW Code of Ethics (2021), 1.16(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 202. ETH-1665 (rev 1) · Best answer: C

A social worker begins voluntary in-home parenting support with a family. At each visit, the parent sets out tea and a small plate of homemade pastries, explaining that in their culture a guest is always served. The family has offered nothing else. What is the **MOST** appropriate response?

**A.** Decline politely, explaining that workers cannot accept gifts
**B.** Accept the tea and bring a small treat to the next visit
**C.** Accept the refreshments graciously as hospitality

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). A blanket refusal misreads hospitality as a gift that creates obligation. Declining makes sense for items of real value or offers tied to a request, but here it would likely offend the parent and strain the work.

**B** **Incorrect** (trap: Boundary slip or collusion). Returning the favor turns a guest courtesy into an exchange of gifts and moves the relationship toward the social. The worker can accept hospitality without reciprocating; warm thanks is enough.

**C** **Correct.** The Code has no specific gift standard; gifts and hospitality are weighed under its boundary standards by value, motive, timing and cultural meaning (NASW Code 1.06(c)). Simple refreshments offered as customary hospitality are a minor boundary crossing that honors the family's culture, and refusing can shame the host and harm engagement (Reamer, 2020). The offer is what any guest receives, and nothing else is given. It beats A because declining treats a cultural courtesy as a gift that creates obligation, when nothing suggests any expectation.

**Takeaway:** Customary hospitality of small value, offered as a cultural courtesy, can be accepted; weigh value, motive and meaning.

**Sources:** NASW Code of Ethics (2021), 1.06(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, gifts, favors and home visits
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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## 203. ETH-1668 (rev 1) · Best answer: C

A nursing home resident with no living family has worked with the facility social worker for 3 years. The resident, who has full decision-making capacity, asks the social worker to serve as health care agent, saying, 'You know what I want better than anyone.' What is the **BEST** response?

**A.** Agree, since no family is available to serve
**B.** Suggest that a court appoint a guardian for the resident
**C.** Decline, and help the resident consider other possible agents
**D.** Ask the administrator whether staff may serve as agents

#### Why each option is right or wrong

**A** **Incorrect** (trap: Boundary slip or collusion). The request is sincere and the resident trusts the worker, which makes agreeing tempting. As agent, though, the worker could face decisions that conflict with the facility's interests or their own job, so the dual role puts the resident at risk.

**B** **Incorrect** (trap: Misreads law or ethics). Guardianship is for people who cannot make their own decisions. This resident has full capacity and can name an agent, so seeking a guardian would remove rights the resident is able to exercise.

**C** **Correct.** Serving as agent would add a second, powerful role with a current client, the kind of dual relationship the Code says to avoid when it risks exploitation or harm (NASW Code 1.06(c)). An agent may face decisions where the worker's job, the facility's interests and the resident's wishes collide (1.06(a)). Declining while helping the resident find another agent, such as a trusted friend, faith community member or professional fiduciary, respects the purpose of the request. It beats A because a lack of family is a reason to search widely, not to merge the roles.

**D** **Incorrect** (trap: Good but not best). Facility rules matter, but checking them first frames the issue as one of permission. Even if staff may serve, the conflict between the agent role and the worker's job remains, so the worker should decline either way.

**Takeaway:** Decline to take on a client's personal legal roles, such as health care agent, and help find someone outside the professional relationship.

**Sources:** NASW Code of Ethics (2021), 1.06(c); NASW Code of Ethics (2021), 1.06(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 204. ETH-1671 (rev 2) · Best answer: A

Under the NASW Code of Ethics, when should a social worker avoid a dual relationship with a client or former client?

**A.** When it carries a risk of exploitation or harm to the client
**B.** When it begins less than 2 years after services have ended
**C.** When it involves social contact outside the office

**Why each option is right or wrong**

**A** **Correct.** The Code says to avoid dual or multiple relationships with clients or former clients when there is a risk of exploitation or potential harm, and when overlap cannot be avoided, the social worker sets clear, culturally sensitive boundaries (NASW Code 1.06(c)). The test is risk, not timing or setting. It beats B because the Code sets no waiting period after which a dual relationship becomes acceptable; time-based rules come from some state laws and other professions' codes, mostly about sexual relationships.

**B** **Incorrect** (trap: Misreads law or ethics). Waiting periods appear in some state rules and other professions' codes, usually for sexual relationships. The NASW Code uses a risk test for dual relationships and sets no time after which they become acceptable.

**C** **Incorrect** (trap: Misreads law or ethics). Incidental social contact, such as meeting at a store or a child's game, is not prohibited in itself. In small communities overlap is often unavoidable, and the Code asks the social worker to manage it with clear boundaries.

**Takeaway:** The Code's test for dual relationships is risk of exploitation or harm; unavoidable overlaps are managed with clear boundaries.

**Sources:** NASW Code of Ethics (2021), 1.06(c)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.2 Boundaries: power, conflicts of interest, self-disclosure, dual relationships · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 205. ETH-0006 (rev 1) · Best answer: A

A 9-year-old client in a school-based program tells the social worker that a parent hit them with a belt last night. The client shows the social worker red marks on the back of their legs. What should the social worker do **FIRST**?

**A.** Make a report to child protective services
**B.** Call the parent to ask about the marks
**C.** Interview the child in detail about other incidents

**Why each option is right or wrong**

**A** **Correct.** A child's disclosure of being hit with an object, with visible marks, gives reasonable suspicion of physical abuse. That triggers the legal duty to report promptly to child protective services. Reporters need suspicion, not proof. It beats C because investigating belongs to child protective services and law enforcement, and detailed questioning can distress the child and taint a later forensic interview. Every state must have mandatory reporting provisions (CAPTA), and reporting is a recognized limit of confidentiality (NASW Code 1.07(c)). CA aside: California requires a phone report immediately or as soon as practicable and a written report within 36 hours (Penal Code 11166).

**B** **Incorrect** (trap: Ignores risk). Contacting the alleged abuser first can put the child at greater risk and lead to pressure on the child to take back the disclosure. Getting the parent's account is part of the protective services investigation, not the reporter's task.

**C** **Incorrect** (trap: Outside the social work role). A detailed interview goes beyond what reporting requires and can contaminate a later forensic interview. The social worker gathers only enough information to make the report, then lets trained investigators take over.

**Takeaway:** Reasonable suspicion of child abuse triggers a prompt report. The social worker reports; investigators investigate.

**Sources:** Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i); NASW Code of Ethics (2021), 1.07(c)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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## 206. ETH-0108 (rev 1) · Best answer: B

During a session, an adult client who is angry about a recent layoff says, 'My manager is going to get what's coming.' The client has no known history of violence and quickly changes the subject. What should the social worker do **FIRST**?

**A.** Notify the manager and local police of a possible threat

**B.** Ask the client directly what the statement meant

**C.** Document the statement and revisit it next session

**D.** Consult a supervisor about the duty to warn

**Why each option is right or wrong**

**A** **Incorrect** (trap: Skips assessment). Warning the manager and police is a protective step if assessment shows a serious threat. Taking it on an ambiguous remark breaches confidentiality without a basis and may damage the relationship and the client's livelihood.

**B** **Correct.** Confidentiality yields to protective action when disclosure is needed to prevent serious, foreseeable and imminent harm (NASW Code 1.07(c)). This remark is vague, so the social worker first assesses it: what the client meant, and any intent, plan, means or past violence. That assessment decides whether a protective disclosure, which HIPAA permits for a serious and imminent threat, is needed (45 CFR 164.512(j)). It beats D because consultation is valuable once the facts are known, and only the

client can supply them now, in session. If the client voices intent or a plan, protective steps follow.

**C** **Incorrect** (trap: Ignores risk). Waiting until the next session leaves a possible threat unexplored. Documentation matters, but the meaning of the remark needs to be assessed now, while the client is present.

**D** **Incorrect** (trap: Right action, wrong time). Consulting about a duty to warn is wise once the social worker knows whether a real threat exists. Without asking the client first, the supervisor has too little information to guide the decision.

**Takeaway:** An ambiguous threat calls for immediate assessment of intent, plan and means before deciding on any protective disclosure.

**Sources:** NASW Code of Ethics (2021), 1.07(c); HIPAA Privacy Rule, 45 CFR 164.512(j)(1)(i)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 207. ETH-1033 (rev 2) · Best answer: A

On an inpatient psychiatric unit, a patient scheduled for discharge this afternoon tells the social worker they plan to go to their former partner's home tonight and shoot the partner with the handgun kept in their closet. The patient says the decision is made. What should the social worker do **FIRST**?

**A.** Have the team hold the discharge for reevaluation
**B.** Warn the former partner and proceed with the discharge
**C.** Ask more about how firmly the patient intends to act
**D.** Notify the outpatient provider to follow up this week

#### Why each option is right or wrong

**A** **Correct.** A serious threat against an identifiable person, with a plan, a weapon and a time, calls for protective action under the duty recognized in Tarasoff (1976). The patient is still hospitalized, so the most protective step is to have the team hold the discharge and reevaluate risk. A beats B because warning the former partner is a recognized protective step, but releasing a patient who has stated a plan and has a gun at home leaves the danger in place. A warning can follow as the law allows (45 CFR 164.512(j)(1)(i)).

**B** **Incorrect** (trap: Ignores risk). Warning an identifiable victim is one way to meet the duty to protect and may still be needed. Here the patient is about to leave with a stated plan and access to a gun, so holding the discharge addresses the danger more directly.

**C** **Incorrect** (trap: Right action, wrong time). Clarifying intent is the right first move when a statement is vague. This patient has named the target, the weapon, the time and a firm decision, so more questioning delays protective action as discharge nears.

**D** **Incorrect** (trap: Ignores risk). Outpatient follow-up suits ongoing care after a safe discharge. A stated plan to shoot someone tonight is an imminent risk that cannot wait for a visit later in the week.

**Takeaway:** A serious threat with an identifiable victim, plan and means requires protective action now; while the client is hospitalized, use that protection first.

**Sources:** Tarasoff v. Regents of the University of California, 17 Cal. 3d 425 (1976); HIPAA Privacy Rule, 45 CFR 164.512(j)(1)(i)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 208. ETH-1034 (rev 1) · Best answer: D

A client who gave birth 6 weeks ago tells the social worker at an outpatient perinatal mental health clinic about sudden, unwanted images of the baby slipping under the water during baths. The client cries, calls the images 'horrifying,' and now asks a partner to give all baths. The social worker finds the client's thinking clear and organized, and the client wants help. What is the **BEST** response?

**A.** Notify the child abuse hotline about the disclosure

**B.** Send the client to the emergency department for evaluation

**C.** Tell the client's partner to oversee every feeding and bath

**D.** Continue treatment and record the risk assessment

#### Why each option is right or wrong

**A** **Incorrect** (trap: Skips assessment). Child abuse reports require reasonable suspicion of abuse or neglect. Unwanted, feared thoughts with protective avoidance are symptoms, not maltreatment. A report would be warranted if the parent described intent, harmful acts or psychotic thinking about the baby.

**B** **Incorrect** (trap: Skips assessment). Emergency evaluation is right when there is confusion, psychotic features or intent to harm, as in postpartum psychosis. This client's thinking is clear and the images are unwanted, so emergency care overreacts and may frighten the client away from treatment.

**C** **Incorrect** (trap: Overrides the client's choice). Directing the partner to supervise all care treats the client as dangerous and discloses without consent. The partner can join in support if the client chooses, but restricting the parent's care of the baby is not indicated.

**D** **Correct.** Unwanted, distressing harm images that a new parent avoids acting on are typical of perinatal obsessions, which are not linked to a higher risk of harming the infant when psychosis is absent (Hudak & Wisner, 2012). With clear thinking, distress and protective avoidance, there is no reportable maltreatment or imminent danger, so the worker continues treatment and documents the assessment. D beats A because a report needs reasonable suspicion of maltreatment, and reporting feared thoughts would breach confidentiality and discourage disclosure (NASW Code 1.07(c)).

**Takeaway:** Distinguish unwanted, distressing harm obsessions from intent; obsessions without psychosis call for treatment and documentation, not a report.

**Sources:** Hudak, R., & Wisner, K. L. (2012). Diagnosis and treatment of postpartum obsessions and compulsions that involve infant harm. American Journal of Psychiatry, 169(4), 360-363, whole article; NASW Code of Ethics (2021), 1.07(c)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 209. ETH-1035 (rev 1) · Best answer: C

A social worker at a community youth center learns from a 7-year-old that a parent's partner burned the child's arm with a lighter, and sees a fresh round burn. The social worker informs the center director, who says staff will talk with the parent at pickup tomorrow and asks the social worker to hold off. What should the social worker do **NEXT**?

**A.** Wait until the director has spoken with the parent
**B.** Ask the child more questions to firm up the details
**C.** Call the child abuse hotline today

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Agency staff cannot delay or block an individual's reporting duty. Waiting for the director to talk with the parent postpones protection and may alert the person who harmed the child.

**B** **Incorrect** (trap: Skips assessment). Gathering more detail can feel careful, but the disclosure and the burn already meet the reporting standard. Extra questioning risks shaping the child's account; investigative interviews belong to child protective services and law enforcement.

**C** **Correct.** The duty to report known or suspected child abuse rests with each mandated reporter and is not met by telling a director (CAPTA, 42 U.S.C. 5106a(b)(2)(B)(i)). A disclosure plus a fresh burn clearly meets the threshold, and legal duties like this can outweigh other loyalties (NASW Code 1.01). C beats A because waiting lets a supervisor delay a mandated report, and contacting the household first could put the child at greater risk.

**Takeaway:** Each mandated reporter must report reasonable suspicion personally; a supervisor's plan or request never replaces or delays the report.

**Sources:** Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i); NASW Code of Ethics (2021), 1.01
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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### 210. ETH-1036 (rev 1) · Best answer: B

In outpatient treatment for depression, an adult client says they left their 5-year-old alone overnight twice last month while working night shifts. The social worker determines that a report is mandated by law. The client has no history of violence, and the child is staying with a grandparent this week. What is the **BEST** way to proceed?

**A.** File the report without telling the client, to avoid conflict
**B.** Tell the client about the report, then make it
**C.** Give the client a week to arrange child care first
**D.** Make the report, then transfer the client to a colleague

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Reporting without notice is appropriate when warning the client could put the child, the worker or the investigation at risk. None of those risks appears here, so silence needlessly damages trust and departs from the Code's preference for advance notice.

**B** **Correct.** When feasible, social workers tell clients before disclosing confidential information, including disclosures the law requires, and explain the likely consequences (NASW Code 1.07(d)); clients should know when legal duties override other loyalties (NASW Code 1.01). Telling this client is safe: there is no history of violence and the child is with a grandparent. B beats A because a silent report is reserved for cases where notice could endanger the child or the worker, and advance notice keeps the alliance intact.

**C** **Incorrect** (trap: Misreads law or ethics). The reporting duty arises once the threshold is met and is not suspended while a parent fixes the problem. Delaying a week substitutes the worker's judgment for the law's requirement.

**D** **Incorrect** (trap: Hands off too soon). A report does not end the helping relationship. Transferring the client right after reporting can feel like punishment, and continued services are often how the worker helps the family through the process.

**Takeaway:** When it is safe, tell the client before making a mandated report, and keep working with the client afterward.

**Sources:** NASW Code of Ethics (2021), 1.07(d); NASW Code of Ethics (2021), 1.01
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 211. ETH-1037 (rev 2) · Best answer: A

At a community health center, a 34-year-old client says their partner shoved them into a wall during an argument last week. The client, who lives alone and works full time, asks the social worker not to involve the police. What should the social worker do?

**A.** Work out a plan with the client for staying safe
**B.** Report the assault to adult protective services
**C.** Urge the client to seek a protective order this week

#### Why each option is right or wrong

**A** **Correct.** Violence between competent adults is generally not a mandated report to protective services, and competent clients decide how to respond to it (NASW Code 1.02). HIPAA allows a report about an adult victim only when law requires it, the client agrees, or narrow exceptions apply (45 CFR 164.512(c)). The right response is a safety plan built with the client. A beats B because adult protective services generally serve older or dependent adults, not a working adult who lives alone.

**B** **Incorrect** (trap: Misreads law or ethics). Adult protective services generally serve older adults and adults with disabilities who depend on others. This client lives alone and works, so a report would breach confidentiality without a legal basis. A report could apply to an older or dependent victim.

**C** **Incorrect** (trap: Too directive). A protective order is one option to describe, and some clients choose it. Urging it before planning with the client puts the worker's solution ahead of the client's own judgment about what will keep them safe (NASW Code 1.02), and filing can raise danger if the partner retaliates.

**Takeaway:** Partner violence between capable adults generally falls outside mandated reporting; follow the client's wishes and plan for safety.

**Sources:** NASW Code of Ethics (2021), 1.02; HIPAA Privacy Rule, 45 CFR 164.512(c)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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### 212. ETH-1038 (rev 1) · Best answer: C

At a residential program for adolescents, a social worker arriving for an evening group sees a colleague clocking in for the overnight shift. The colleague smells strongly of alcohol, slurs words, and will be the only staff member awake with 12 residents. What should the social worker do **FIRST**?

**A.** Speak with the colleague one-on-one about the alcohol smell
**B.** Stay late to cover and raise it with the colleague tomorrow
**C.** Notify the on-call supervisor right away

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Speaking with the colleague first is the Code's usual starting point and would be right if no clients were at immediate risk. An apparently intoxicated sole overnight staff member creates danger tonight, so the supervisor must know now.

**B** **Incorrect** (trap: Ignores risk). Covering the shift helps tonight, but it hides the problem from the people responsible for staffing and safety. The impairment and the agency's response to it still need to reach a supervisor today.

**C** **Correct.** The Code's first step for an impaired colleague is to talk with them, but only when feasible (NASW Code 2.08(a)). This colleague appears intoxicated and is about to be the only awake staff member for 12 adolescents, so client safety requires going straight to the agency's channel (NASW Code 2.08(b)). C beats A because a private talk suits impairment that is not putting clients at immediate risk; tonight's shift cannot wait for the colleague to respond.

**Takeaway:** When a colleague's impairment puts clients at immediate risk, report through agency channels at once rather than starting with a private talk.

**Sources:** NASW Code of Ethics (2021), 2.08(a); NASW Code of Ethics (2021), 2.08(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 213. ETH-1039 (rev 2) · Best answer: D

What threshold of belief triggers a mandated reporter's legal duty to report possible child abuse or neglect?

**A.** Probable cause
**B.** Clear and convincing evidence
**C.** Preponderance of the evidence
**D.** Reasonable suspicion

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Probable cause is the standard police and courts use for arrests and search warrants. Requiring it before reporting would push social workers into investigating, which reporting laws leave to child protective services.

**B** **Incorrect** (trap: Wrong model or concept). Clear and convincing evidence is a high civil standard used in decisions such as terminating parental rights. A reporter needs far less, and waiting for that level of certainty would delay protection.

**C** **Incorrect** (trap: Wrong model or concept). Preponderance of the evidence, meaning more likely than not, is the usual civil trial standard and is used in many states to substantiate a finding after investigation. A report requires only reasonable suspicion beforehand.

**D** **Correct.** Federal law requires states to mandate reports of known and suspected abuse (CAPTA, 42 U.S.C. 5106a(b)(2)(B)(i)), and state laws set the trigger as reasonable suspicion, or reason to believe, not proof. Deciding whether abuse occurred is the job of child protective services and the courts. D beats A because probable cause is a law enforcement standard for arrests and searches, a higher bar than the suspicion that triggers a report.

**Takeaway:** Mandated reporters report on reasonable suspicion; proof and investigation belong to child protective services and the courts.

**Sources:** Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 214. ETH-1040 (rev 1) · Best answer: B

In an outpatient session, a client says they will stab a former business partner tonight, gives the partner's home address, and shows a folding knife. The social worker urges voluntary hospitalization, but the client refuses and walks out. What should the social worker do **NEXT**?

**A.** Call the client tomorrow to reassess the threat
**B.** Notify law enforcement and warn the intended victim
**C.** Raise the case at the next team consultation

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). Reassessment fits when a threat is vague or the client remains engaged. This client named a victim, a time and a weapon, then left after refusing help, so waiting a day leaves the partner unprotected tonight.

**B** **Correct.** A serious threat of violence against an identifiable person triggers a duty to take reasonable protective steps (Tarasoff, 1976). The client named the victim, set a time, showed a weapon and refused hospitalization, so the protective options left are notifying police and warning the target, which HIPAA permits for a serious and imminent threat (45 CFR 164.512(j)(1)(i)). B beats A because tonight's threat cannot wait for tomorrow's call. The worker then documents and follows the applicable state law.

**C** **Incorrect** (trap: Ignores risk). Consultation is valuable, but a scheduled team meeting comes too late for a threat set for tonight. Same-day consultation is fine only if it does not delay protective action.

**Takeaway:** When a client with a specific threat, victim and weapon leaves care, notify police and the intended victim without delay.

**Sources:** Tarasoff v. Regents of the University of California, 17 Cal. 3d 425 (1976); HIPAA Privacy Rule, 45 CFR 164.512(j)(1)(i)

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## 215. ETH-1041 (rev 1) · Best answer: C

At an opioid treatment program, a client says their partner burned their 6-year-old with a cigarette this morning, and the client saw the mark. The social worker knows federal rules strictly protect substance use treatment records and is unsure whether a report is allowed. What is the **BEST** action?

**A.** Ask the client to sign a release before reporting

**B.** Get guidance from the program's attorney before acting

**C.** Make the abuse report with just the facts it requires

**D.** Make the report and send the client's treatment file

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). A release is the usual route for disclosing substance use treatment information, which makes this tempting. Abuse reports under state law are an express exception, and a mandated report cannot depend on the client's agreement.

**B** **Incorrect** (trap: Ignores risk). Legal advice helps with unclear questions, and the worker can consult later about any records request. The child abuse exception is clear, and waiting on an attorney delays a report that protects a child burned this morning.

**C** **Correct.** The federal confidentiality rules for substance use programs do not apply to reports of suspected child abuse made under state law (42 CFR 2.12(c)(6)). A burn the client witnessed gives reasonable suspicion, so the worker reports. The same rule keeps the original treatment records protected, so the report includes only what the law requires. C beats A because requiring a release would let a refusal block a mandated report, and asking for one delays protection.

**D** **Incorrect** (trap: Misreads law or ethics). The exception covers the report, not the treatment record. Sending the file discloses protected information beyond what reporting needs; disclosure should be limited to the least information required (NASW Code 1.07(c)).

**Takeaway:** Part 2 permits the initial child abuse report under state law, but treatment records stay protected; report only what is required.

**Sources:** 42 CFR Part 2, 42 CFR 2.12(c)(6); NASW Code of Ethics (2021), 1.07(c)

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### 216. ETH-1042 (rev 1) · Best answer: A

An adult client in outpatient therapy discloses being sexually abused by a family friend from ages 8 to 11. The client recently learned that this person now coaches a youth soccer team. The client does not want to be part of any report. The social worker is unsure whether the law requires a report in this situation. What is the **BEST** way to proceed?

**A.** Seek consultation about the applicable reporting law
**B.** Report the family friend to child protective services now
**C.** Encourage the client to make the report personally
**D.** Explain that abuse of an adult in childhood cannot be reported

**Why each option is right or wrong**

**A** **Correct.** Whether an adult's disclosure of childhood abuse must be reported, especially when the alleged abuser has current access to children, varies by state. When the worker is unsure, the next step is consultation with a supervisor or legal counsel on the applicable law (NASW Code 2.05(a)), because the answer decides whether the worker must report regardless of the client's wishes. A beats C because the client has already declined involvement, and urging a self-report does not settle the worker's own possible duty.

**B** **Incorrect** (trap: Skips assessment). A report may prove to be required, and many states would allow one. Filing before learning the law risks disclosing an adult client's history without a legal basis and against the client's stated wishes (NASW Code 1.07(c)).

**C** **Incorrect** (trap: Good but not best). Supporting the client's own choice to report can be empowering and might be offered later. The client has already declined, and the worker's possible legal duty does not depend on what the client decides.

**D** **Incorrect** (trap: Misreads law or ethics). Some states do require or allow reports when an adult discloses past abuse and the abuser still has access to children. Telling the client nothing can be done misstates the law and may leave children unprotected.

**Takeaway:** When a reporting duty is unclear, as with adult disclosures of past abuse, consult on the applicable law before acting or dismissing it.

**Sources:** NASW Code of Ethics (2021), 2.05(a); NASW Code of Ethics (2021), 1.07(c)
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### 217. ETH-1043 (rev 2) · Best answer: C

A hospice social worker noticed a colleague nodding off in team meetings and slurring words on a call. Two weeks ago, the social worker raised this privately, and the colleague denied any problem. This week the colleague missed two patient visits and seemed sedated at a family meeting. What should the social worker do **NEXT**?

**A.** Meet with the colleague a second time to urge help
**B.** Keep a log of incidents for a few more weeks
**C.** Take the concerns to the colleague's supervisor

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). A second conversation can feel kinder, and talking privately was the right first step. The colleague has already denied a problem and the signs are getting worse, so the Code's next step is the agency channel.

**B** **Incorrect** (trap: Ignores risk). Recording specific incidents is useful, but weeks more of watching while patients miss visits and the colleague appears sedated with families leaves the harm unaddressed.

**C** **Correct.** With direct knowledge of a colleague's impairment, the social worker first talks with the colleague when feasible (NASW Code 2.08(a)), which has already happened. When the colleague does not take adequate steps, the next move is action through appropriate channels such as the employer (NASW Code 2.08(b)). Missed patient visits and apparent sedation show the problem continues. C beats A because repeating a talk the colleague has already rebuffed delays protection for patients and families.

**Takeaway:** If an impaired colleague does not act after a direct conversation, take the concern to the employer or agency channel.

**Sources:** NASW Code of Ethics (2021), 2.08(a); NASW Code of Ethics (2021), 2.08(b)
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## 218. ETH-1044 (rev 2) · Best answer: B

In a caregiver support program, a client caring for a parent with major neurocognitive disorder admits shoving the parent twice this week when the parent resisted bathing. The parent has bruises on both upper arms. The client is tearful and asks for help managing stress. What should the social worker do **FIRST**?

**A.** Arrange respite care to ease the caregiver's strain
**B.** Contact adult protective services about the incidents
**C.** Explore the client's guilt about the shoving
**D.** Teach the client calmer ways to handle bathing

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Respite addresses the stress behind the shoving and belongs in the plan. As a first step it treats repeated physical harm to a dependent adult as a caregiver-support problem and leaves the report undone.

**B** **Correct.** Physical abuse of a dependent adult, with injuries, is reportable to adult protective services, which most state laws require. Legal duties can outweigh loyalty to the client, who should be told about the report (NASW Code 1.01), and the disclosure is limited to what the report needs (NASW Code 1.07(c)). B beats A because respite is an important part of the response, but it does not replace the report or the protective assessment adult protective services provides.

**C** **Incorrect** (trap: Right action, wrong time). Exploring the client's guilt can support the caregiver and the relationship later. With repeated physical harm and bruises already disclosed, the protective report comes before emotional processing.

**D** **Incorrect** (trap: Good but not best). Skills for handling care resistance can reduce future incidents and belong in the plan. Teaching them first assumes the risk is managed and delays the report the injuries call for.

**Takeaway:** Disclosed physical abuse of a dependent adult calls for a protective report first; caregiver support follows and does not replace it.

**Sources:** NASW Code of Ethics (2021), 1.07(c); NASW Code of Ethics (2021), 1.01
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 219. ETH-1045 (rev 1) · Best answer: A

A staff member at an after-school program tells the program's social worker that an 8-year-old has lingered at pickup each Friday this month and once asked to stay late. The staff member describes the child as otherwise cheerful and engaged. What should the social worker do **FIRST**?

**A.** Talk with the child using open, non-leading questions
**B.** Report the staff member's concern to child protective services
**C.** Ask the parent at pickup whether anything is wrong at home

**Why each option is right or wrong**

**A** **Correct.** Reporting laws cover known or suspected abuse (CAPTA, 42 U.S.C. 5106a(b)(2)(B)(i)), and lingering at pickup has many explanations, from enjoying the program to a change at home that is not harmful. A brief, open, non-leading conversation within the worker's role clarifies whether suspicion exists without shaping any later disclosure. A beats B because reporting a vague behavior acts before there is cause; if the conversation raises suspicion, the worker reports right away.

**B** **Incorrect** (trap: Skips assessment). A report is required once there is reasonable suspicion, and the worker should not hesitate if one develops. Lingering at pickup alone does not reach that point, so a report now discloses information without a compelling reason (NASW Code 1.07(c)).

**C** **Incorrect** (trap: Ignores risk). Raising concerns with a parent before understanding them could alert a caregiver who may be harming the child and put the child at risk. Parent contact may fit later if the concern proves harmless.

**Takeaway:** When a concern falls short of reasonable suspicion, clarify it briefly and without leading questions; report as soon as suspicion forms.

**Sources:** Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i); NASW Code of Ethics (2021), 1.07(c)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 220. ETH-1046 (rev 1) · Best answer: B

A social worker makes an honest report of suspected child abuse that is later not confirmed. What legal protection generally applies?

**A.** Psychotherapist-patient privilege
**B.** Statutory immunity
**C.** Sovereign immunity

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Psychotherapist-patient privilege protects confidential communications from compelled disclosure in court. It belongs to the client and limits disclosure; it does not protect a reporter from being sued over a report.

**B** **Correct.** CAPTA requires states, as a condition of funding, to provide immunity from civil and criminal liability for people who make good-faith reports of suspected child abuse or neglect (42 U.S.C. 5106a(b)(2)(B)(vii)). State reporting laws therefore protect an honest reporter even when the report is not substantiated. B beats C because sovereign immunity protects governments, not individual reporters, and does not turn on good faith.

**C** **Incorrect** (trap: Wrong model or concept). Sovereign immunity shields governments, and sometimes their agencies, from certain lawsuits. It does not depend on good faith, and it is not the protection reporting laws give individual reporters, including those in private practice.

**Takeaway:** Reporting laws give reporters immunity for good-faith reports, even when the report is not substantiated.

**Sources:** Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(vii)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 221. ETH-1047 (rev 1) · Best answer: C

An adult client seen in outpatient care after a painful divorce describes recurring thoughts of hurting the former spouse that 'come out of nowhere' at night and frighten the client. The client has given their handgun to a sibling for safekeeping and is eager to keep working on the anger. What is the **BEST** response?

**A.** Warn the former spouse about the client's thoughts
**B.** Arrange a hospital evaluation of dangerousness today
**C.** Intensify treatment with a written safety plan

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). A warning is a protective action reserved for a serious threat of violence against an identifiable person. Disclosing unwanted thoughts the client fears and is working on breaches confidentiality and may end the treatment that is lowering the risk.

**B** **Incorrect** (trap: Skips assessment). Hospital evaluation fits when risk is imminent, as with intent, a plan or a refusal to stay safe. Here the client has moved the gun away and seeks help, so hospitalization would be more restrictive than the risk calls for.

**C** **Correct.** The duty to protect arises from a serious threat of violence against an identifiable person (Tarasoff, 1976). This client's thoughts are unwanted and frightening, and the client has already moved the gun away, so the risk is best managed within treatment: more frequent sessions and a written plan that keeps the gun with the sibling. C beats A because warning the former spouse breaches confidentiality without a serious threat (NASW Code 1.07(c)) and could undo the treatment that is reducing risk.

**Takeaway:** Unwanted violent thoughts without intent call for intensified treatment and means safety; warnings are for serious threats against identifiable people.

**Sources:** Tarasoff v. Regents of the University of California, 17 Cal. 3d 425 (1976); NASW Code of Ethics (2021), 1.07(c)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 222. ETH-1048 (rev 2) · Best answer: A

At a group home for adults with intellectual developmental disorder, a resident tells the social worker that a night staff member slapped them, and the social worker sees a red mark on the resident's cheek. The social worker informs the house manager, who says the resident 'makes things up' and plans an internal review next week. What should the social worker do **NEXT**?

**A.** Report the incident to the designated outside agency
**B.** Wait for the results of the internal review
**C.** Ask the staff member for an account of the night

#### Why each option is right or wrong

**A** **Correct.** Suspected abuse of a dependent adult by facility staff goes to the outside agency that state law designates, and a manager's doubts about the resident do not lower the threshold. The resident's account plus a visible mark give reasonable suspicion. HIPAA permits disclosures about abuse victims that law requires, limited to what the law asks (45 CFR 164.512(c)(1)(i)). A beats B because an internal review does not replace an outside investigation, and next week is too long for a resident still exposed to the staff member.

**B** **Incorrect** (trap: Misreads law or ethics). Internal reviews help agencies correct practice, but they do not replace the outside investigation that abuse laws call for. Waiting a week leaves the resident exposed and lets the facility judge itself.

**C** **Incorrect** (trap: Outside the social work role). Questioning the accused staff member is investigative work that belongs to the designated agency. It could alert the staff member, shape the account, and put pressure on the resident before investigators arrive.

**Takeaway:** Doubts about an alleged victim's credibility do not replace an outside report; suspected staff abuse goes to the designated investigating agency.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.512(c)(1)(i)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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### 223. ETH-1141 (rev 1) · Best answer: D

A hospital social worker plans discharge for an 82-year-old patient with moderate major neurocognitive disorder. The patient's grandchild holds power of attorney. The billing office says the patient's nursing home bills are 5 months unpaid, and bank statements show $40,000 moved to the grandchild's account. When asked, the patient cannot say where the money went. What should the social worker do **FIRST**?

**A.** Meet with the grandchild to request an accounting of the funds
**B.** Ask the patient more about their finances and wishes
**C.** Refer the family to an elder law attorney for advice
**D.** Report suspected financial exploitation to adult protective services

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). An accounting may be part of the eventual response, but asking the suspected exploiter for it first can alert them, lead to more transfers or put pressure on the patient. Investigation belongs to adult protective services.

**B** **Incorrect** (trap: Right action, wrong time). Exploring the patient's wishes is usually a good step, and it has been tried: the patient cannot explain where the money went. More questions now delay the report without adding needed facts.

**C** **Incorrect** (trap: Good but not best). Legal advice may help later, for example to change the power of attorney. A referral does not address the suspected exploitation now or bring in the agency responsible for investigating it.

**D** **Correct.** Suspected exploitation of a vulnerable adult calls for a report to adult protective services, which most states require of professionals; investigating is APS's role, not the worker's. Federal law defines exploitation to include improper use of an elder's resources for another's gain (Elder Justice Act, 42 U.S.C. 1397j(8)). This patient cannot protect their own interests, so the worker acts to protect them (NASW Code 1.14). D beats A because confronting the grandchild first could prompt more transfers or pressure on the patient before APS can act.

**Takeaway:** Reasonable suspicion of elder financial exploitation goes to adult protective services; do not confront the suspected exploiter first.

**Sources:** NASW Code of Ethics (2021), 1.14; Elder Justice Act, 42 U.S.C. 1397j(8)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 224. ETH-1142 (rev 1) · Best answer: C

During a video session, a 15-year-old client says they want to die. When asked directly, the teen says they plan to take a parent's sleeping pills tonight and knows where they are kept. The teen begs the social worker not to tell anyone. A parent is at home in another room. What should the social worker do **NEXT**?

**A.** Complete a safety plan with the teen before involving anyone else
**B.** Have emergency responders sent before telling the parent
**C.** Bring the parent into the session to address safety now

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). A safety plan matters, but it cannot hold an immediate plan with known means. The teen's request for secrecy does not outweigh imminent risk, and planning alone leaves the pills within reach tonight.

**B** **Incorrect** (trap: Good but not best). Emergency responders are the right call if no adult in the home can act or the teen leaves the session. Here a parent is in the home and can secure the pills and arrange evaluation faster and with less disruption.

**C** **Correct.** Confidentiality gives way when disclosure is needed to prevent serious, foreseeable and imminent harm to the client (NASW Code 1.07(c)), and HIPAA permits disclosure to a person able to lessen the threat, including family (45 CFR 164.512(j)(1)(i)). The teen has a plan for tonight and knows where the pills are, and a parent who can remove them and get the teen evaluated is steps away. The worker tells the teen first, as feasible (1.07(d)). C beats B because the parent can act at once while the teen stays connected; responders are right if the parent cannot be reached or the teen ends the call.

**Takeaway:** Imminent self-harm risk overrides a minor's request for secrecy; involve the nearest adult who can remove the means and get care.

**Sources:** NASW Code of Ethics (2021), 1.07(c); NASW Code of Ethics (2021), 1.07(d); HIPAA Privacy Rule, 45 CFR 164.512(j)(1)(i)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 225. ETH-1143 (rev 2) · Best answer: A

At a community after-school program, a 10-year-old shows the social worker a bruise on their side and says a parent's partner punched them last night. The social worker calls the child abuse hotline, and the intake worker accepts the report. What should the social worker do **NEXT**?

**A.** Document when, to whom and what was reported

**B.** Ask the child more questions to add detail for investigators

**C.** Call the parent to explain that a report has been made

**D.** Wait to document until child protective services shares findings

**Why each option is right or wrong**

**A** **Correct.** Records should be accurate, timely and sufficient for continuity of services (NASW Code 3.04(a), 3.04(b)). After a report, the worker documents when it was made, to whom, and what was reported, including the child's own words. Investigating and contacting the family belong to child protective services once a report is accepted (DePanfilis, 2018). A beats C because calling the parent could alert the partner, who is the alleged abuser, and put the child at risk before investigators act.

**B** **Incorrect** (trap: Skips assessment). The child's statement already supports the report, which has been made. More questioning risks leading the child and can complicate the investigation; detailed interviews belong to trained investigators.

**C** **Incorrect** (trap: Ignores risk). Telling a family about a report can be appropriate in some cases, but here the alleged abuser is the parent's partner. Contact with the family is for investigators to plan, so the child is not exposed to retaliation or pressure.

**D** **Incorrect** (trap: Right action, wrong time). Documentation should be timely, made while the details are fresh. Waiting for the agency's findings, which may take weeks or never come back to the reporter, leaves the record incomplete.

**Takeaway:** After making an abuse report, document when, to whom and what was reported; leave investigation and family contact to CPS.

**Sources:** NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 3.04(b); DePanfilis, D. (2018). Child Protective Services: A Guide for Caseworkers. U.S. Department of Health and Human Services, Children's Bureau, Ch. 6: Initial Assessment or Investigation

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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## 226. ETH-1144 (rev 1) · Best answer: B

In a private conversation, a colleague at a family services agency tells a social worker about completing treatment for alcohol use disorder 8 months ago and attending weekly recovery meetings. The colleague's supervisor rates their work highly, and their clients are making progress. A coworker who overheard urges a report to the licensing board. What is the **MOST** appropriate action?

**A.** Report the colleague's history to the state licensing board

**B.** Respect the colleague's privacy and make no report

**C.** Tell the supervisor so the agency can monitor the colleague

#### Why each option is right or wrong

**A** **Incorrect** (trap: Skips assessment). A board report fits impairment that harms practice and that the colleague has not addressed, or a report the law requires. Recovery with strong performance meets neither condition, and reporting would break the colleague's confidence.

**B** **Correct.** The duty to act on a colleague's impairment applies when an impairment interferes with practice effectiveness (NASW Code 2.08(a)); formal channels follow when the colleague has not addressed it (2.08(b)). This colleague completed treatment, stays engaged in recovery and practices well, and a history of treatment is not impairment. Social workers also respect confidential information colleagues share (2.02). B beats C because telling the supervisor treats recovery as a risk and breaks the colleague's confidence when clients are doing well.

**C** **Incorrect** (trap: Misreads law or ethics). Monitoring would make sense if there were signs of impairment at work. Alerting the supervisor here shares a private disclosure and treats recovery itself as the problem.

**Takeaway:** Recovery is not impairment; the duty to act on a colleague begins when an impairment interferes with practice.

**Sources:** NASW Code of Ethics (2021), 2.08(a); NASW Code of Ethics (2021), 2.08(b); NASW Code of Ethics (2021), 2.02

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### 227. ETH-1145 (rev 2) · Best answer: C

A social worker in a family support program reported suspected neglect after finding a client's 4-year-old alone at home, and explained the report to the client beforehand. At the next home visit, the client says, 'I trusted you, and you called CPS on me. Why should I keep seeing you?' What is the **BEST** response?

**A.** Offer to transfer the client to another worker in the program
**B.** Explain that the law required the report and move to today's plan
**C.** Invite the client to talk about feeling betrayed by the report
**D.** Promise to talk with the client before making any future report

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). A transfer may become right if the client still wants a different worker after talking it through. Offering it first treats the rupture as unfixable and can feel like rejection.

**B** **Incorrect** (trap: Good but not best). Explaining the law is accurate, and the client was already told before the report. Repeating it and moving on skips the sense of betrayal, which is what threatens the relationship.

**C** **Correct.** A mandated report does not end the helping relationship; the worker keeps serving a client who still needs help and works to avoid abandonment (NASW Code 1.17(b)). The client's question is an opening, not a decision to leave, and talking through the sense of betrayal is how the alliance is repaired. The report was explained beforehand (1.07(d)), so repeating the legal reason does not meet the client's hurt. C beats B because restating the law answers why the report happened but not the client's feeling of betrayal, and moving on may end engagement.

**D** **Incorrect** (trap: Misreads law or ethics). A worker cannot promise advance discussion of every future report, because some situations require reporting without delay or warning. The promise could not be kept and would mislead the client.

**Takeaway:** After a mandated report, keep serving the client and work through the breach of trust rather than defending the report or handing off.

**Sources:** NASW Code of Ethics (2021), 1.17(b); NASW Code of Ethics (2021), 1.07(d)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 228. ETH-1150 (rev 1) · Best answer: B

Two social workers share an office suite but run separate solo practices. One has noticed the other arriving smelling of alcohol several times, and a client in the waiting room says the colleague fell asleep during a session. Last month, the social worker raised the concern privately and urged the colleague to get help. The colleague denied any problem, and the incidents have continued. What should the social worker do **NEXT**?

**A.** Give the colleague a final month to seek treatment
**B.** File a complaint with the colleague's licensing board
**C.** Suggest that the colleague's clients seek care elsewhere
**D.** Get legal advice about liability before acting

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). A deadline extends the collegial approach that 2.08(a) calls for, but that approach has already been tried and rebuffed. Another month of waiting leaves the colleague's clients exposed to impaired sessions, one of which a client has already described.

**B** **Correct.** When a colleague's impairment affects practice and the colleague has not taken adequate steps after being approached, the Code directs action through formal channels such as employers, agencies, NASW or licensing boards (NASW Code 2.08(b)). Here the private talk already happened and failed, and a client reports that the colleague fell asleep in session. Because both are solo practitioners, there is no employer or supervisor to notify, so the licensing board is the fitting channel. It beats A because giving more time to a colleague who has denied the problem leaves clients exposed while the impairment continues.

**C** **Incorrect** (trap: Boundary slip or collusion). Contacting the colleague's clients bypasses the formal channels the Code names, spreads unverified concerns and may look like soliciting them. Protecting those clients runs through the bodies that can investigate and act on impairment.

**D** **Incorrect** (trap: Good but not best). Legal advice can reassure a worker who fears retaliation, and it may be sought alongside the complaint. Making it a precondition puts the worker's own protection ahead of clients who are being seen by an impaired practitioner now.

**Takeaway:** When direct outreach to an impaired peer fails, use a formal body; for a solo practitioner with no employer, that is the licensing board.

**Sources:** NASW Code of Ethics (2021), 2.08(a); NASW Code of Ethics (2021), 2.08(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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## 229. ETH-1151 (rev 2) · Best answer: C

In the fifth session, a client being treated for depression says that 6 years ago they took about $3,000 from a former employer's cash register and were never caught. The client has had no contact with that employer since. Looking anxious, the client asks whether the social worker now has to call the police. What should the social worker do **FIRST**?

**A.** Explore the guilt the client has carried about the theft
**B.** Ask a supervisor whether the theft is reportable
**C.** Explain that confidentiality covers this past theft

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Exploring the guilt is valuable and belongs right after the question is answered. While the client is waiting to learn whether the police will be called, turning to feelings first sidesteps a direct question about confidentiality and can feel evasive.

**B** **Incorrect** (trap: Good but not best). Consultation helps when a reporting duty is genuinely unclear. Here the answer is settled: an old property crime with no ongoing danger is not a mandated report, so stepping away to check adds worry without changing the answer.

**C** **Correct.** Confidentiality may be set aside only for compelling professional reasons, such as preventing serious, foreseeable and imminent harm (NASW Code 1.07(c)); the Code also recognizes disclosures that law requires (1.07(d)). Mandated reporting laws cover situations such as child or elder abuse, not a completed property crime from years ago with no ongoing danger. The client has asked directly about a limit of confidentiality, and the Code asks workers to explain those limits as needed (1.07(e)). It beats A because exploring guilt while the client still fears a police call leaves the question unanswered.

**Takeaway:** Past crimes with no ongoing danger are not mandated reports; when a client asks about confidentiality limits, answer clearly before exploring.

**Sources:** NASW Code of Ethics (2021), 1.07(c); NASW Code of Ethics (2021), 1.07(d); NASW Code of Ethics (2021), 1.07(e)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 230. ETH-1152 (rev 2) · Best answer: A

A parent in a contested custody case tells the social worker that their 6-year-old came back from the other parent's home with a burn on one hand. The parent says the child described the other parent holding the hand on a hot stove, and shows a dated photo of the burn. The two parents have accused each other of lying in court filings. The parent asks for a letter for the custody hearing. What should the social worker do **FIRST**?

**A.** Notify protective services about the burn
**B.** Write the letter describing what the parent reported
**C.** Ask to meet with the child to hear the account directly
**D.** Coach the parent on filing the hotline call themselves

#### Why each option is right or wrong

**A** **Correct.** Federal law requires states to mandate reports of known or suspected child abuse (CAPTA, 42 U.S.C. 5106a(b)(2)(B)(i)); reporters need reasonable suspicion, not proof, and judging credibility is the job of child protective services. A specific injury, the child's own account of how it happened and a dated photo create reasonable suspicion, even amid custody conflict. Possible motives are details to pass on, not reasons to wait. It beats C because meeting the child to test the story makes the social worker an investigator, risks shaping the child's account and delays protection. When safe, tell the parent about the report (NASW Code 1.07(d)).

**B** **Incorrect** (trap: Boundary slip or collusion). A letter for the hearing serves the custody case, not the child's safety, and it does not meet the reporting duty. Writing it could also pull the social worker into an advocacy role in the dispute. The report comes first.

**C** **Incorrect** (trap: Skips assessment). Hearing from the child seems careful given the custody conflict, but the duty rests on reasonable suspicion, which the injury, the child's account and the photo already supply. Questioning the child is investigative work that belongs to protective services and can taint later interviews.

**D** **Incorrect** (trap: Misreads law or ethics). The parent may also call, and supporting that is fine. But the reporting duty belongs to the social worker personally, and relying on the parent leaves the child's protection to someone in the middle of a court fight.

**Takeaway:** A litigant's possible motive does not raise the bar for reporting; pass a child's specific account to protective services to weigh.

**Sources:** Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106a(b)(2)(B)(i); NASW Code of Ethics (2021), 1.07(d)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.3 Mandated reporting: abuse, harm threats, impaired colleagues, duty to warn · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 231. ETH-0109 (rev 1) · Best answer: C

How should a social worker fix a mistake discovered in a completed progress note?

**A.** Replace the note with a corrected version
**B.** Mention the error in the next progress note entry
**C.** Add a dated, signed addendum that corrects it

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Replacing a note erases the original, which looks like alteration of the record and destroys the audit trail. Even well-meant changes made this way can be treated as falsification in an audit or legal review.

**B** **Incorrect** (trap: Good but not best). Noting the error elsewhere is honest, but the original entry stays unchanged and unlinked. A reader of the earlier note would still see the wrong information, so the record as a whole remains inaccurate.

**C** **Correct.** Records must be accurate and reflect the services actually provided (NASW Code 3.04(a)). Errors are corrected by a dated, signed addendum or late entry that leaves the original entry readable, so anyone reviewing the record can see what changed and when. HIPAA's amendment rule follows the same logic, appending or linking a correction rather than erasing the original (45 CFR 164.526(c)(1)). It beats B because a later note may never be read alongside the original, which would still appear uncorrected.

**Takeaway:** Correct records with a dated, signed addendum that leaves the original readable; never delete or overwrite entries.

**Sources:** NASW Code of Ethics (2021), 3.04(a); HIPAA Privacy Rule, 45 CFR 164.526(c)(1)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 232. ETH-1049 (rev 2) · Best answer: B

During an internal records audit at a community agency, a social worker discovers that notes for two counseling sessions held last month were never written. The social worker has detailed appointment logs and remembers both sessions clearly. What is the **BEST** way to handle the missing notes?

**A.** Write the notes now and date them on the session days
**B.** Write them now, dated today and labeled as late entries
**C.** Leave the gap rather than change the record afterward

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Dating the notes as if written on the session days misrepresents when they were made. Backdating is falsification even when the content is true, and it can destroy the record's credibility in an audit or court.

**B** **Correct.** Records must be accurate and timely enough to support continuity of care (NASW Code 3.04(a), 3.04(b)). When documentation was missed, the accepted fix is a late entry: written now, dated with the actual date of writing, and clearly labeled as late, so the record is complete without misrepresenting when it was made. B beats C because leaving the gap protects nothing; two sessions of care go undocumented, which undermines continuity and the record's accuracy.

**C** **Incorrect** (trap: Good but not best). Avoiding changes is sound for completed notes, which are corrected only by dated addenda. Missing notes are different: the services happened, and leaving them undocumented makes the record inaccurate and incomplete.

**Takeaway:** Document missed services as clearly labeled late entries dated when written; never backdate, and never leave delivered services unrecorded.

**Sources:** NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 3.04(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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### 233. ETH-1050 (rev 2) · Best answer: A

At intake in an outpatient clinic, a client discloses a suicide attempt 8 years ago. The client then asks the social worker to keep it out of the record and falls silent, looking at the intake form. What should the social worker do **FIRST**?

**A.** Ask what worries the client about having it recorded
**B.** Leave it out of the record and keep a private memo
**C.** Record the attempt and explain who can access the record
**D.** Describe the attempt vaguely as past difficulties

#### Why each option is right or wrong

**A** **Correct.** A request to keep something out of a record usually rests on a specific fear, such as who might see it, and the right reassurance depends on what that fear is. The client has not said why and has gone quiet, so asking comes first; the social worker can then record what care requires and explain how the record is protected (NASW Code 3.04(a), 1.07(e)). It beats C because recording the attempt and explaining access is the right next step, but doing it before learning the concern may answer the wrong worry.

**B** **Incorrect** (trap: Misreads law or ethics). A private memo that holds material clinical facts outside the record creates a shadow record. The official record becomes inaccurate, and the memo may still be reachable in legal proceedings. Risk history belongs in the clinical record (3.04(a)).

**C** **Incorrect** (trap: Right action, wrong time). Records must be accurate and include information directly relevant to service delivery (NASW Code 3.04(a), 3.04(c)), so a past suicide attempt, a major risk factor, belongs in the record, and explaining who can see it often eases the fear (1.07(e)). This is the right next step once the social worker knows what the client is worried about.

**D** **Incorrect** (trap: Misuses the technique). Leaving out nonessential sensitive detail is right under 3.04(c), but a past suicide attempt is essential risk information. A vague phrase misleads future providers and makes the record inaccurate.

**Takeaway:** Before answering a request to keep information out of the record, learn the client's concern; then record what care requires and explain access.

**Sources:** NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 3.04(c); NASW Code of Ethics (2021), 1.07(e)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 234. ETH-1052 (rev 1) · Best answer: A

A supervisor reviews a supervisee's unsigned draft progress note for a hospital patient awaiting discharge. The note calls the patient 'manipulative and drug-seeking.' It also records that the patient asked for pain medication 4 times in an hour and refused a walk with physical therapy. What should the supervisor do **FIRST**?

**A.** Have the supervisee revise the draft to describe observed behavior
**B.** Co-sign the draft, then discuss wording in the next supervision
**C.** Have the supervisee add an addendum explaining the terms
**D.** Revise the note personally so it is accurate before co-signing

#### Why each option is right or wrong

**A** **Correct.** Records must be accurate, and social workers use accurate, respectful language rather than derogatory labels when writing about clients (NASW Code 1.12, 3.04(a)). Because this note is an unsigned draft, it can still be revised directly; addenda are for completed entries. The note already contains the observable facts, so the fix is to replace the labels with those behaviors. A beats B because co-signing first would make the labels part of the permanent record, where they could then be corrected only by an addendum.

**B** **Incorrect** (trap: Right action, wrong time). Discussing wording is right, but signing first makes the labels part of the permanent record. The revision belongs before the note is finalized, with the teaching conversation alongside it.

**C** **Incorrect** (trap: Misuses the technique). An addendum is the correction method for a signed, completed note. This draft is unsigned, so an addendum would leave the derogatory labels in place when they can simply be revised.

**D** **Incorrect** (trap: Good but not best). Rewriting it personally fixes this note but takes over the supervisee's authorship of their own documentation and skips the learning that supervision exists to provide. The supervisee should make the revision.

**Takeaway:** Replace labels with observed behavior in records; revise drafts directly, and correct finalized notes only by addendum.

**Sources:** NASW Code of Ethics (2021), 1.12; NASW Code of Ethics (2021), 3.04(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 235. ETH-1053 (rev 1) · Best answer: C

A social worker is leaving a community agency in 3 weeks to open a private practice. Several clients will move to colleagues at the agency, and two have said they may follow the social worker later. What is the **BEST** way for the social worker to handle the client records before leaving?

**A.** Take a list of client contact details for future follow-up
**B.** Leave the records at the agency without adding notes
**C.** Leave the records with the agency and write transfer summaries

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Removing client contact information takes agency records without authorization and invites soliciting clients. Clients who want to follow can contact the social worker themselves or authorize the agency to release their records.

**B** **Incorrect** (trap: Good but not best). Leaving the records in place meets the custody duty, but without transfer summaries the colleagues inherit cases without a clear picture of goals and progress, which undermines continuity (3.04(b)).

**C** **Correct.** Agency records stay with the agency, stored securely with access limited to authorized people (NASW Code 1.07(l)). Documentation should also be sufficient to support continuity of future services (3.04(b)), and transfer summaries support an orderly transfer of responsibility (1.16(b)). C beats B because simply leaving the records meets the custody duty but not the continuity duty. Clients who later choose to follow can ask the agency, in writing, to send their records to the new provider.

**Takeaway:** When leaving an agency, records stay with the agency; add transfer summaries so care continues smoothly.

**Sources:** NASW Code of Ethics (2021), 1.07(l); NASW Code of Ethics (2021), 3.04(b); NASW Code of Ethics (2021), 1.16(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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### 236. ETH-1054 (rev 1) · Best answer: B

Under the HIPAA Privacy Rule, which records must a covered entity keep for 6 years?

**A.** Clinical progress notes for each client
**B.** Written policies and signed authorizations
**C.** Psychotherapy notes kept apart from the record
**D.** Records of clients who have died

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Retention of clinical progress notes is set by state law, agency policy and contracts (3.04(d)), and the periods vary. The HIPAA 6-year rule covers compliance documentation, not the clinical record.

**B** **Correct.** The Privacy Rule requires covered entities to keep required compliance documentation, such as policies and procedures, signed authorizations and other required written communications, for 6 years from creation or last effective date (45 CFR 164.530(j)). It does not set a retention period for the clinical record itself; state

law, agency policy and contracts do (NASW Code 3.04(d)). B beats A because confusing these two rules is a common error: the 6-year figure applies to HIPAA paperwork, not to progress notes.

**C** **Incorrect** (trap: Wrong model or concept). Psychotherapy notes get extra protection from disclosure under HIPAA, but the Privacy Rule sets no 6-year retention period for them. Their retention follows state law and agency policy.

**D** **Incorrect** (trap: Wrong model or concept). HIPAA continues to protect a deceased client's information, but it sets no special retention period for those records. State law, agency policy and contracts govern how long they are kept.

**Takeaway:** HIPAA's 6-year rule covers compliance documents like policies and authorizations; clinical record retention follows state law, policy and contracts.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.530(j); NASW Code of Ethics (2021), 3.04(d)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 237. ETH-1055 (rev 2) · Best answer: A

After reviewing a copy of their record, a client in an outpatient substance use program asks in writing that a note be deleted. The note states that the client arrived for group smelling of alcohol and slurring words, which the social worker observed directly. The client says the note is unfair. What is the **BEST** response?

**A.** Explain the client's right to add a written statement of disagreement
**B.** Amend the note, since clients have a right to request amendment
**C.** Explain that signed notes are final and cannot be amended

#### Why each option is right or wrong

**A** **Correct.** Under HIPAA, clients may request amendment of their records, and a provider may deny the request when the record is accurate and complete. The written denial must tell the client of the right to submit a statement of disagreement, which then accompanies the record (45 CFR 164.526(d)). This note records what the social worker directly observed, so it is accurate and stays (NASW Code 3.04(a)). A beats C because C tells the client that no amendment process exists, denying a right the client holds.

**B** **Incorrect** (trap: Misreads law or ethics). The right is to request amendment, not to have every request granted; a provider may deny it when the record is accurate and complete (45 CFR 164.526(a)(2)(iv)). The worker observed these facts directly, so changing the note would make the record less accurate (3.04(a)); the client's remedy is a statement of disagreement.

**C** **Incorrect** (trap: Misreads law or ethics). Accurate notes need not be changed, but saying signed notes can never be amended is wrong. Clients may request amendment, inaccurate entries must be corrected, and here the client may add a statement of disagreement.

**Takeaway:** Do not change accurate records at a client's request; deny in writing and offer the right to file a statement of disagreement.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.526(d); HIPAA Privacy Rule, 45 CFR 164.526(a)(2)(iv); NASW Code of Ethics (2021), 3.04(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 238. ETH-1056 (rev 1) · Best answer: B

A social worker in a home-based family program carries a large caseload. The agency's electronic record lets staff copy the previous visit's note into a new note. A colleague suggests copying notes forward to keep up with documentation. How should the social worker use this feature?

**A.** Copy each note forward and update the visit date
**B.** Edit each copied note to reflect that visit
**C.** Write one summary note every two weeks instead

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Changing only the date makes each note claim that the earlier visit's events happened again. Identical cloned notes misrepresent services and draw audit scrutiny, even when the visits occurred.

**B** **Correct.** Records must accurately reflect the services provided and be timely enough to support continuity (NASW Code 3.04(a), 3.04(b)). Copy-forward can serve as a starting draft, but each note must then be revised to describe what happened at that visit: who attended, what was done and how the family responded. B beats A because a note copied with only a new date repeats old content as if it recurred, which misrepresents services and can support inaccurate billing (3.05).

**C** **Incorrect** (trap: Misreads law or ethics). A summary every two weeks fails the timeliness standard (3.04(b)). Each visit needs its own note, written promptly, so that others can rely on the record.

**Takeaway:** Electronic shortcuts are acceptable only if every note is edited to reflect the service actually delivered.

**Sources:** NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 3.04(b); NASW Code of Ethics (2021), 3.05
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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### 239. ETH-1057 (rev 1) · Best answer: D

Under the HIPAA Privacy Rule, which content can be kept in psychotherapy notes, apart from the main record?

**A.** Session start and stop times
**B.** The treatment plan and progress summary
**C.** Current diagnosis and symptoms
**D.** The worker's analysis of session conversation

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Session start and stop times are expressly excluded from psychotherapy notes. They belong in the clinical record, where they support billing and continuity.

**B** **Incorrect** (trap: Wrong model or concept). Summaries of the treatment plan and progress are excluded from the definition. They must appear in the clinical record so that care can continue if another provider takes over.

**C** **Incorrect** (trap: Wrong model or concept). A summary of diagnosis and symptoms is excluded from psychotherapy notes. Diagnosis is core clinical record content used for treatment and billing.

**D** **Correct.** HIPAA defines psychotherapy notes as a mental health professional's notes documenting or analyzing the contents of counseling conversations, kept separate from the rest of the record (45 CFR 164.501). The definition excludes session times, medication monitoring, test results and summaries of diagnosis, symptoms, treatment plan and progress. D beats B because the plan and progress belong in the clinical record that other providers rely on. Psychotherapy notes add protection but never replace that record (NASW Code 3.04(b)).

**Takeaway:** Psychotherapy notes hold the worker's analysis of session content; times, diagnosis, plan and progress belong in the clinical record.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.501 (definition: psychotherapy notes); NASW Code of Ethics (2021), 3.04(b)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 240. ETH-1153 (rev 2) · Best answer: D

When writing progress notes for members of a therapy group, how should a social worker handle information about the other group members?

**A.** Refer to other members by their initials
**B.** Keep one shared note in a group file
**C.** List every attendee in each member's note
**D.** Leave other members' identities out of each note

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Initials feel like de-identification, but in a small group with a known schedule they can easily point to specific people. Anyone who later reads the record, including the member, could identify peers.

**B** **Incorrect** (trap: Misreads law or ethics). A single shared note mixes every member's information, so releasing one person's record would expose everyone else's. Each member needs an individual record that supports their own care and billing.

**C** **Incorrect** (trap: Misreads law or ethics). An attendance list in every chart reveals who else is in treatment. That is not relevant to the individual member's services, and the group's confidentiality agreement (1.07(f)) cannot protect names placed in another member's record.

**D** **Correct.** Each member's record should hold only information relevant to that member's services, and privacy should be protected as far as possible (NASW Code 3.04(c)). Other members' names, initials or disclosures do not belong in a member's chart, because that member, an insurer or a court may later see it. The usual approach is a separate note per member describing group themes and that member's participation. It beats A because initials in a small, known group can still identify people, so they are not real protection.

**Takeaway:** Each group member's record stands alone: document the member's own participation and leave other members' identities out.

**Sources:** NASW Code of Ethics (2021), 3.04(c); NASW Code of Ethics (2021), 1.07(f)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 241. ETH-1154 (rev 1) · Best answer: A

At the final session of a planned termination, a client who completed treatment for an eating disorder asks the social worker to shred their whole record that day. The client wants a 'clean slate' and worries that a future employer might see it. The agency must keep records for a set number of years. What is the **BEST** response?

**A.** Explain the retention rule and how the record is protected
**B.** Remove the most sensitive notes from the record
**C.** Agree to shred it once the client signs a request

#### Why each option is right or wrong

**A** **Correct.** Records must be kept after services end for as long as laws, agency policy and contracts require (NASW Code 3.04(d)), and this agency has such a requirement, so a client's wish cannot shorten it. The helpful response explains the rule and addresses the real worry: the record is stored securely, is not released to an employer without the client's authorization except as law requires, and will be destroyed securely when the period ends (1.07(s)). It beats C because a signed request, though it honors the client's wish, does not change a legal or policy retention duty.

**B** **Incorrect** (trap: Misreads law or ethics). Pulling sensitive notes would leave an incomplete record that no longer matches the services provided (3.04(a)) and breaks the retention duty for the removed material, whatever the client's later preference.

**C** **Incorrect** (trap: Misreads law or ethics). A written request shows respect for the client's choice, but consent cannot override a required retention period. Shredding early would violate agency policy and possibly law, and could leave the client without records they may need later.

**Takeaway:** Clients cannot waive record retention rules; explain the required period, how the record is protected and when it will be destroyed.

**Sources:** NASW Code of Ethics (2021), 3.04(d); NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 1.07(s)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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### 242. ETH-1155 (rev 1) · Best answer: C

A program director at a family counseling agency tells staff to keep notes for clients in custody disputes 'as thin as possible,' recording only the date and 'attended,' so the notes cannot hurt clients if subpoenaed. A social worker sees several of these clients, including one who recently self-harmed. Which documentation approach is **MOST** appropriate?

**A.** Follow the director's instruction for these clients
**B.** Keep thin notes but document risk events in full
**C.** Write sufficient notes limited to service-relevant facts
**D.** Keep detailed notes in a separate personal file

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Thin notes may seem protective, but a record that shows only attendance cannot support continuity, risk management or billing. Agency directives should not override ethical practice (3.09(d)); the better course is sufficient notes plus raising the concern with the director.

**B** **Incorrect** (trap: Good but not best). Recording risk events fully is right, but leaving every other session as 'attended' still fails the sufficiency standard. Progress, interventions and plans also need documenting so another provider could continue care.

**C** **Correct.** Records must be accurate and sufficient enough to support current care and continuity (NASW Code 3.04(a), 3.04(b)), while including only information relevant to services (3.04(c)). That balance answers both concerns: the client who self-harmed needs assessment and safety planning documented, yet private details unrelated to treatment can stay out. It beats B because sufficiency applies to every session, not only crises. Subpoenaed records are protected by asserting privilege, not by documenting less than care requires, and a director's order cannot override this (3.09(d)).

**D** **Incorrect** (trap: Misuses the technique). Separately kept process notes may hold a clinician's impressions, but they cannot replace the clinical record. Moving required content, such as risk assessments, into a private file leaves the official chart incomplete and inaccurate (3.04(a)).

**Takeaway:** Litigation risk does not justify thin records; document what care requires and leave out what is irrelevant to services.

**Sources:** NASW Code of Ethics (2021), 3.04(a); NASW Code of Ethics (2021), 3.04(b); NASW Code of Ethics (2021), 3.04(c); NASW Code of Ethics (2021), 3.09(d)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

### 243. ETH-1156 (rev 2) · Best answer: B

Members of a hospital patient's family disagree about whether to continue the patient's dialysis. The social worker consults the hospital's ethics consultant, who suggests a family meeting focused on the patient's previously stated wishes. The social worker schedules the meeting for later in the week. What is **MOST** important to document about the consultation now?

**A.** A brief entry noting that a consultation occurred
**B.** When it occurred, the advice given and the action taken
**C.** A summary in a separate file kept out of the chart
**D.** The family meeting's outcome, once the meeting has been held

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). A brief entry that a consultation happened seems discreet, but it leaves out the advice and the action taken, so the team cannot see how the plan was reached. Documentation should be sufficient for care and continuity while staying relevant to services (3.04(b), 3.04(c)).

**B** **Correct.** Records should be sufficient and timely enough to support care and continuity (NASW Code 3.04(b)). A consultation about an ethically difficult decision is a key event to record: who was consulted, when, what was advised and what the worker did in response. This lets the team see how the plan was reached and shows that the decision followed recognized steps, including consultation (Reamer, 2018). It beats D because waiting to record only the meeting's outcome leaves out why the meeting was arranged and the advice behind it.

**C** **Incorrect** (trap: Misuses the technique). Keeping the consultation in a side file leaves the chart without information the care team needs to understand the plan. Consultation that shapes care belongs in the record, written with the same attention to relevance as any other entry.

**D** **Incorrect** (trap: Right action, wrong time). The meeting's outcome will need documenting too, but that comes later. Waiting until then delays the entry and drops the reasoning and advice that led to the meeting, which other team members may need before it is held.

**Takeaway:** Document consultations promptly: who was consulted, when, what was advised and what action followed.

**Sources:** NASW Code of Ethics (2021), 3.04(b); NASW Code of Ethics (2021), 3.04(c); Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, ethical decision-making framework: consult and document

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.4 Legal and ethical record keeping and documentation · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

### 244. ETH-0110 (rev 1) · Best answer: D

After an assessment, a social worker diagnoses a client with adjustment disorder. The client's insurance plan pays for only a few sessions for that diagnosis, so the client asks the social worker to record major depressive disorder instead. The client does not meet criteria for major depressive disorder. What is the **BEST** response?

**A.** Record the requested diagnosis to keep the client in care
**B.** Refer the client to a provider who accepts self-pay clients
**C.** Explain that billing a false diagnosis is insurance fraud
**D.** Keep the accurate diagnosis and discuss payment options

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Recording a diagnosis the client does not meet misrepresents the client's condition to the insurer. Good intentions do not change that this is fraud, and the false diagnosis also follows the client in future records.

**B** **Incorrect** (trap: Hands off too soon). Referral hands the client off over a payment problem the social worker can help solve. Referral may fit if no affordable option exists here, but it should follow a discussion of fees and alternatives.

**C** **Incorrect** (trap: Too directive). The statement is true, but leading with fraud warnings lectures a client who made an understandable request. It leaves the real problem, affording continued care, unaddressed and can damage the relationship.

**D** **Correct.** Billing and diagnosis must accurately reflect the services and the client's condition; recording a diagnosis the client does not meet is fraud, even when meant to help (NASW Code 3.05, 4.04). Keeping the accurate diagnosis while exploring options, such as a sliding fee, an appeal or lower-cost services, protects honesty and addresses the client's real concern, paying for care (NASW Code 1.13(a)). It beats C because naming the fraud is accurate but stops at a warning; D declines the request and still helps the client keep getting services.

**Takeaway:** Diagnose and bill only what the assessment supports; address cost problems through fees, appeals or resources, never by changing the diagnosis.

**Sources:** NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 1.13(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 245. ETH-1069 (rev 1) · Best answer: D

An associate social worker at a community clinic notices that claims for the associate's therapy sessions list the licensed supervisor as the provider who delivered them. The health plan's contract requires claims to name the person who actually provided the service. The associate raised this with the supervisor, who replied, 'Everyone does it.' What should the associate do **NEXT**?

**A.** Report the billing practice to the health plan's fraud unit
**B.** Ask the supervisor to co-sign each note to support the claims
**C.** Keep providing sessions and document the objection in a memo
**D.** Take the concern to the organization's compliance officer

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Reporting to the payer may become necessary if the agency does not correct the practice. Going outside first skips the internal compliance office, which can stop the billing and correct past claims faster.

**B** **Incorrect** (trap: Misuses the technique). Co-signing shows supervisory review, but it does not make the supervisor the person who delivered the sessions. The claims would still misstate the provider (3.05).

**C** **Incorrect** (trap: Misreads law or ethics). A private memo records disagreement but leaves the inaccurate billing in place. Continuing in silence amounts to condoning fraud (4.04).

**D** **Correct.** Billing must accurately show the nature of services and who provided them (NASW Code 3.05), and social workers must not take part in or condone fraud (4.04). They also take reasonable steps to make employer practices consistent with the Code (3.09(d)). The associate already raised it with the supervisor without result, so the next step is the agency's formal compliance channel, which can correct current and past claims. D beats A because an outside report is premature while an internal channel that can fix the problem is untried; it becomes appropriate if the agency fails to act.

**Takeaway:** When billing misstates who provided services and a supervisor dismisses it, take it to the agency's compliance channel before going outside.

**Sources:** NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 3.09(d)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 246. ETH-1070 (rev 1) · Best answer: C

A client in a private practice signed a fee agreement at intake stating that sessions missed without 24 hours' notice cost $40. The client misses a session without calling. The office manager suggests billing the client's health plan for the missed hour instead. How should the social worker handle the missed session?

**A.** Bill the health plan for the missed session

**B.** Bill the plan at a shorter-session rate instead

**C.** Charge the client the agreed missed-session fee

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Health plans pay for services delivered, not appointments missed. Submitting a claim for a session that did not occur misrepresents the service and is insurance fraud.

**B** **Incorrect** (trap: Misreads law or ethics). Lowering the amount does not make the claim true; no service of any length took place. Any claim to the plan for this hour misrepresents services (3.05).

**C** **Correct.** Billing must accurately reflect the nature and extent of services actually provided (NASW Code 3.05), and billing a payer for a session that did not happen is fraud (4.04). Costs, including missed-session charges, are explained when services begin (1.03(a)), and this client agreed in writing to a $40 fee. C beats B because billing

a shorter session still claims a service that never occurred; a smaller false claim is still false.

**Takeaway:** Bill payers only for services delivered; missed sessions may be charged to the client only under a fee agreed in advance.

**Sources:** NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 1.03(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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### 247. ETH-1071 (rev 2) · Best answer: A

A clinic social worker spends 30 minutes on the phone helping a client complete a housing application. The client's health plan covers psychotherapy but not case management. A colleague notes that the call could be billed as psychotherapy because the client seemed calmer afterward. What is the **BEST** way for the social worker to handle the call?

**A.** Record it as case management with no therapy claim
**B.** Bill it as therapy rather than case management
**C.** Skip documenting the call because the plan won't pay

#### Why each option is right or wrong

**A** **Correct.** Billing must accurately reflect the nature and extent of the services provided (NASW Code 3.05). Helping with a housing application is case management, even if the client felt calmer, so labeling it psychotherapy to obtain payment would misrepresent the service (4.04). The call still belongs in the record, because documentation supports continuity of care (3.04(b)). A beats C because the payment question does not change the duty to document a service that took place.

**B** **Incorrect** (trap: Misreads law or ethics). A calming effect does not turn paperwork help into psychotherapy. Coding a service as something it was not, to fit coverage, is billing fraud (3.05, 4.04).

**C** **Incorrect** (trap: Misreads law or ethics). Whether a payer covers a service has no bearing on whether it is documented. The call is part of the client's care and needs a timely note (3.04(b)).

**Takeaway:** Bill services as what they actually were; coverage gaps never justify relabeling a service, and unbilled services are still documented.

**Sources:** NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 3.04(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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### 248. ETH-1072 (rev 2) · Best answer: B

A client at a community mental health center learns that extra weekly sessions there have a 6-week waitlist. The client asks to see the social worker at the social worker's evening private practice and offers to pay a reduced cash fee. What is the **MOST** appropriate response?

**A.** Agree to see the client privately at the reduced fee
**B.** Decline and pursue added services through the center
**C.** See the client outside the center in the evenings for free

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Even at a reduced fee, privately charging a client for services available through the agency conflicts with 1.13(c), which bars seeking such fees, and sets the worker's income against the client's interests (1.06(a)).

**B** **Correct.** Social workers should not seek a private fee for services clients are entitled to through their employer or agency (NASW Code 1.13(c)), and taking the client's offer would have the same effect. The client is entitled to services at the center, so the right path is to pursue options there, such as an interim group, a cancellation list or an urgent-need review. B beats A because a reduced fee does not cure the problem: it still moves an agency client into the worker's paid practice, creating a conflict of interest (1.06(a)).

**C** **Incorrect** (trap: Boundary slip or collusion). Free private sessions remove the fee but create a dual relationship outside the agency's structure and oversight. Added help should come through the center.

**Takeaway:** Do not charge agency clients privately for services they can get through the agency; seek more help within the agency.

**Sources:** NASW Code of Ethics (2021), 1.13(c); NASW Code of Ethics (2021), 1.06(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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### 249. ETH-1073 (rev 2) · Best answer: C

A group practice bills a client's health plan for weekly sessions. The client begins addressing gambling losses and asks that those sessions be kept from the plan, paying the full fee for each one at the visit. The social worker has confirmed that the client understands the cost. How should the social worker handle billing for those sessions?

**A.** Bill the plan but leave gambling off the claim
**B.** Explain that practice policy bills every session to the plan
**C.** Mark those sessions so nothing about them goes to the plan

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Billing the plan for sessions the client already paid for would collect twice and still disclose the services. Leaving gambling off the claim does not honor the restriction and makes the claim inaccurate (3.05).

**B** **Incorrect** (trap: Misreads law or ethics). Practice policy cannot override the client's federal right to restrict health plan disclosures for services paid in full. Telling the client otherwise misstates the client's rights.

**C** **Correct.** Under HIPAA, a provider must agree to a client's request not to disclose information to a health plan for payment or operations when it concerns services the client has paid for in full (45 CFR 164.522(a)(1)(vi)). This client pays in full for these sessions, so the practice must keep them off claims. Disclosure to payers also requires the client's authorization (NASW Code 1.07(h)). C beats B because an internal billing policy cannot override a restriction that federal law requires the practice to honor.

**Takeaway:** A client who pays in full out of pocket may keep those services from the health plan; providers must honor that restriction.

**Sources:** HIPAA Privacy Rule, 45 CFR 164.522(a)(1)(vi); NASW Code of Ethics (2021), 1.07(h); NASW Code of Ethics (2021), 3.05
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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### 250. ETH-1074 (rev 1) · Best answer: A

In Medicare billing, what does 'incident-to' billing refer to?

**A.** Billing staff services under a supervising practitioner's number
**B.** Billing an emergency service code after a critical incident
**C.** Billing services under the staff member's own provider number
**D.** Billing a liability insurer for care after an accident

#### Why each option is right or wrong

**A** **Correct.** Medicare's incident-to rules let services furnished by auxiliary personnel be billed under the supervising physician's or practitioner's provider number, but only when specific conditions, including required supervision, are met, and only the supervising practitioner may bill (42 CFR 410.26(b)). The ethics point is that claims must accurately show who provided the service (NASW Code 3.05); billing this way when the conditions are not met is fraud. A beats C because incident-to is defined by billing under the supervisor's number, not the staff member's own.

**B** **Incorrect** (trap: Wrong model or concept). The word 'incident' here refers to services that are incidental to a practitioner's care, not to critical incidents or crisis events.

**C** **Incorrect** (trap: Wrong model or concept). Billing under the staff member's own number is ordinary direct billing by an enrolled provider. Incident-to is the opposite arrangement: the service is billed under the supervising practitioner.

**D** **Incorrect** (trap: Wrong model or concept). Seeking payment from a liability insurer after an accident is a different process. Incident-to concerns who furnished a service versus whose number it is billed under.

**Takeaway:** Incident-to billing places auxiliary staff services under a supervisor's number only when payer conditions are met; otherwise it misstates the provider.

**Sources:** Medicare Part B regulations, 42 CFR 410.26(b); NASW Code of Ethics (2021), 3.05
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 251. ETH-1162 (rev 1) · Best answer: C

A social worker in private practice sees a 74-year-old client with Medicare for depression. The client's only income is a small pension. After a large dental bill, the client says they may have to stop sessions because of the copayment and asks whether it could be waived for a while. What is the **MOST** appropriate response?

**A.** Explain that Medicare rules bar waiving copays
**B.** Waive copays for every client living on a fixed income
**C.** Waive it after documenting the client's hardship

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Saying copays can never be waived overstates the rule. Federal guidance permits an occasional waiver based on an individual's good-faith hardship determination, and refusing outright may push this client out of needed treatment.

**B** **Incorrect** (trap: Misreads law or ethics). A standing waiver for a whole group of clients is a routine waiver, which the guidance treats as misstating the actual charge to Medicare. Hardship waivers must rest on each client's own circumstances.

**C** **Correct.** Federal fraud guidance treats routine copay waivers as improper but allows a provider to waive a copayment for an individual patient after a good-faith determination of financial hardship, applied occasionally rather than routinely (HHS OIG, 1994). This client has a specific change in finances and risks dropping needed care. A case-by-case waiver, recorded with its basis, keeps treatment going and honors the Code's call to consider ability to pay (NASW Code 1.13(a)). It beats B because a blanket policy for a class of clients is exactly the routine waiver the guidance warns against.

**Takeaway:** Copays may be waived for an individual client's documented financial hardship, but not routinely or for whole groups of clients.

**Sources:** HHS Office of Inspector General (1994). Special Fraud Alert: Routine Waiver of Copayments or Deductibles Under Medicare Part B, Federal Register 59(242), December 19, 1994; NASW Code of Ethics (2021), 1.13(a)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 252. ETH-1163 (rev 2) · Best answer: A

A social worker in private practice plans to raise session fees by 20%. One client, who pays out of pocket and lives on a fixed income, is midway through a planned course of trauma treatment. How should the social worker **BEST** handle the change with this client?

**A.** Give early notice and discuss a reduced fee if needed
**B.** Announce the new fee to clients with 30 days' notice
**C.** Refer the client to a lower-cost clinic before the increase

**Why each option is right or wrong**

**A** **Correct.** Fees should be fair, reasonable and in line with the services, with attention to the client's ability to pay (NASW Code 1.13(a)), and clients should understand costs as part of informed consent (1.03(a)). Giving notice well ahead and discussing options, such as keeping the current rate or a sliding fee, protects a client on a fixed income who is midway through trauma treatment. It beats B because notice alone treats fairness as identical fees for everyone, while the Code ties fees to the client's ability to pay.

**B** **Incorrect** (trap: Good but not best). Advance notice and the same rule for every client sound fair, which makes this tempting. Applying the full increase without discussing options ignores this client's ability to pay, which the Code asks social workers to consider, and risks ending trauma work midway.

**C** **Incorrect** (trap: Hands off too soon). Referral can be right if the client cannot afford care even with adjustments. As a first move, it ends a trauma course midway, when notice and a reduced fee could keep the work intact.

**Takeaway:** Before raising fees, give advance notice and weigh each client's ability to pay, offering adjustments before referral.

**Sources:** NASW Code of Ethics (2021), 1.13(a); NASW Code of Ethics (2021), 1.03(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 253. ETH-1164 (rev 2) · Best answer: B

A client arrives 25 minutes late for a scheduled 50-minute psychotherapy session at a group practice, and the session lasts 25 minutes. The office manager tells the social worker to bill the client's insurer for the full 50 minutes, since the client chose to come late. How should the social worker bill the session?

**A.** Bill the full scheduled session, as directed
**B.** Bill for the time actually provided
**C.** Submit no claim, since the session was incomplete

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). An office manager's direction does not make an inaccurate claim acceptable. Billing for 25 minutes that were not provided misrepresents the extent of service to the insurer, even though the lateness was the client's choice.

**B** **Correct.** Billing must accurately reflect the nature and extent of services provided and who provided them (NASW Code 3.05). The client received 25 minutes of psychotherapy, so the claim should reflect that shorter service, however the lateness came about. Billing the scheduled 50 minutes would claim service that never occurred, which is fraud even when the office directs it (4.04). Any late policy agreed with the client must follow payer rules and never justifies overbilling the insurer. It beats C because the 25 minutes of care were real and can be billed accurately.

**C** **Incorrect** (trap: Good but not best). Skipping the claim is not required: 25 minutes of therapy were delivered and can be billed accurately. Accuracy cuts both ways, and the record and claim should match the service that occurred.

**Takeaway:** Bills must match the service actually delivered; lateness may be handled by an agreed client policy, never by overbilling insurers.

**Sources:** NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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### 254. ETH-1165 (rev 2) · Best answer: B

A social worker at a community clinic treats an adult with PTSD whose Medicaid managed care plan reviews progress every 8 sessions. Symptom scores have improved modestly but remain clinically significant. On a review call, the plan's reviewer remarks that more sessions are approved more easily when scores show a client 'getting worse.' What is the **BEST** response?

**A.** Ask the client to give worst-day scores on the next measure
**B.** Submit the actual scores with a rationale for more care
**C.** Accept the likely denial and begin planning termination

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misuses the technique). Coaching the client to report only their worst days skews the measure, so the scores no longer reflect the client's condition. It is a quieter way of misrepresenting progress to the payer.

**B** **Correct.** Billing and reports to payers must accurately reflect services and the client's condition, and social workers may not take part in deception or fraud (NASW Code 3.05, 4.04). Modest gains with ongoing clinically significant symptoms are themselves a valid argument for continued care when explained. Submitting the real scores with a clear rationale, and appealing if needed, is honest advocacy. It beats C because conceding a denial before presenting the case for medical necessity abandons a legitimate argument that the client still needs treatment.

**C** **Incorrect** (trap: Good but not best). Planning for termination may become necessary if coverage ends, but starting it now gives up before the clinical case for more sessions has been made. Modest progress with ongoing symptoms supports continued care.

**Takeaway:** Advocate with payers using accurate data and clinical rationale; modest progress with ongoing symptoms can justify care without exaggeration.

**Sources:** NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.5 Ethical billing: managed care, insurance, incident-to billing · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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### 255. ETH-1526 (rev 1) · Best answer: A

During a home visit, a hospice patient with advanced lung disease tells the social worker, 'I've been saving my pain pills. I'm going to take them all tonight so my family can stop taking care of me.' The patient shows the social worker a full bottle hidden in a drawer. What should the social worker do **FIRST**?

**A.** Stay with the patient and get the care team's help now
**B.** Explore the patient's feeling of being a burden to the family
**C.** Explain how legal aid in dying differs from a private overdose

**Why each option is right or wrong**

**A** **Correct.** Self-determination and confidentiality yield when a client's plan poses a serious, foreseeable and imminent risk of harm (NASW Code 1.02, 1.07(c)). This patient has a plan for tonight, the means in hand and a reason rooted in feeling like a burden, which is different from a lawful end-of-life choice. Staying with the patient and bringing in the hospice team lets the team secure the medication and assess the patient now. It beats B because the sense of burden must be explored, but only after the immediate danger is contained.

**B** **Incorrect** (trap: Right action, wrong time). Feeling like a burden is a key driver to explore, and the team should address it soon. With a plan for tonight and pills within reach, though, talking first leaves the means available. It belongs after the safety steps.

**C** **Incorrect** (trap: Ignores risk). Information about lawful options may matter later if the patient raises them in a calmer moment. Right now the patient has a hidden overdose plan for tonight, so a discussion of legal processes ignores the immediate danger.

**Takeaway:** A terminally ill client with a plan, means and a timeline needs immediate protective action; terminal illness does not lower the response to suicide risk.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.07(c)
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## 256. ETH-1531 (rev 2) · Best answer: D

A hospital patient with advanced major neurocognitive disorder can no longer communicate. The patient's living will declines a feeding tube. The patient's health care agent, an adult child, insists on placing one, saying, 'I can't let my parent starve.' The team met with the agent yesterday to review the directive, but the agent's position has not changed, and the agent wants the tube placed this week. What should the social worker do **NEXT**?

**A.** Follow the agent's instructions as the legal decision-maker
**B.** Schedule another meeting to review the directive with the agent
**C.** Explore the agent's grief about the parent's decline
**D.** Request an ethics consultation on the conflict

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). The agent does hold decision authority, which is why this option tempts. But that authority is to decide as the patient would have chosen. When the agent's choice contradicts a clear written directive, simply following it sets aside the patient's own wishes.

**B** **Incorrect** (trap: Right action, wrong time). Another meeting was the right step before, and continued communication matters. The team has already reviewed the directive with the agent, who did not change position, so repeating it delays a fair resolution while a decision is pending.

**C** **Incorrect** (trap: Good but not best). The agent's grief is real and deserves ongoing support, which may ease the conflict over time. It cannot settle a decision being pressed this week that contradicts the patient's documented wishes; that now needs a formal, impartial review. Grief support can continue alongside the consultation.

**D** **Correct.** Social workers seek to ensure that a third party making decisions acts consistently with the client's wishes (NASW Code 1.03(c)). The living will clearly declines a feeding tube, a team meeting has failed, and placement is being pressed. An ethics consultation gives a structured, impartial review of the patient's wishes, the agent's role and the options. It beats C because grief support, though needed, cannot settle a pending decision that contradicts the directive. It beats A because an agent's authority is to carry out the patient's known wishes, not replace them (Beauchamp & Childress, 2019).

**Takeaway:** When a surrogate overrides a patient's clear advance directive and team discussion fails, seek an ethics consultation.

**Sources:** NASW Code of Ethics (2021), 1.03(c); Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). Oxford University Press, Ch. 5: standards of surrogate decision making
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### 257. ETH-1534 (rev 1) · Best answer: B

A surrogate makes a choice for a patient who can no longer decide. The surrogate picks what the patient would have wanted, based on the patient's values and past words. Which standard is this?

**A.** Best interests standard
**B.** Substituted judgment standard
**C.** Reasonable person standard

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). The best interests standard asks what would most benefit the patient, weighing benefits and burdens. It is used when the patient's wishes and values are unknown, such as for a person who never had decision-making capacity.

**B** **Correct.** Under substituted judgment, the surrogate decides as the patient would have decided, drawing on the patient's known values, statements and preferences (Beauchamp & Childress, 2019). It extends the patient's autonomy after capacity is lost, and it is what social workers help surrogates focus on when they support decisions consistent with the client's wishes (NASW Code 1.03(c)). It beats A because the best interests standard applies when the patient's wishes are unknown, so the surrogate weighs benefits and burdens instead.

**C** **Incorrect** (trap: Wrong model or concept). The reasonable person standard concerns how much a clinician must disclose for informed consent: what a reasonable patient would want to know. It does not guide a surrogate's choice for an incapacitated patient.

**Takeaway:** Substituted judgment: decide as the patient would have. Best interests: weigh benefits and burdens when the patient's wishes are unknown.

**Sources:** Beauchamp, T. L., & Childress, J. F. (2019). Principles of Biomedical Ethics (8th ed.). Oxford University Press, Ch. 5: standards of surrogate decision making; NASW Code of Ethics (2021), 1.03(c)
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### 258. ETH-1538 (rev 2) · Best answer: C

Before elective surgery, a nurse tells an adult patient that the hospital requires an advance directive on file. The upset patient tells the hospital social worker they do not want to make one and asks whether the surgery can go ahead without it. What should the social worker do **FIRST**?

**A.** Help the patient complete a brief directive
**B.** Clarify the hospital's duties with the nurse
**C.** Inform the patient that a directive is voluntary

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Helping the patient fill out a form treats the nurse's mistaken claim as true and pressures the patient into a document they declined. An advance directive must reflect the patient's voluntary choice.

**B** **Incorrect** (trap: Right action, wrong time). Correcting the nurse's understanding is an important follow-up so other patients are not misinformed. Doing it first leaves an upset patient believing surgery depends on a document they do not want.

**C** **Correct.** Federal rules require hospitals to give adult inpatients written information about advance directives at admission, but hospitals may not condition care on whether a patient has one (42 CFR 489.102). The patient has been misinformed, is upset and has asked directly, so the first step is to answer the patient and affirm the right to decline (NASW Code 1.02). It beats B because the nurse does need accurate information, but that conversation can follow; the patient's informed choice and distress come first.

**Takeaway:** Hospitals must offer advance directive information but cannot require a directive; patients may decline without any effect on care.

**Sources:** 42 CFR 489.102 (Patient Self-Determination Act requirements), (a)(1), (a)(3); NASW Code of Ethics (2021), 1.02
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### 259. ETH-1541 (rev 2) · Best answer: A

An 82-year-old patient on dialysis for 6 years tells the dialysis social worker they want to stop treatment and enter hospice. Over 3 weeks the patient has given the same reasons, and the nephrologist and a psychiatrist have confirmed decision-making capacity. The patient hopes the family will come to accept the decision. The patient's adult children ask the social worker to talk the patient out of it. What is the **BEST** response?

**A.** Offer the patient a family meeting to explain the choice
**B.** Ask the patient to continue until the family agrees
**C.** Request another psychiatric evaluation first
**D.** Remind the children the patient may legally decide

**Why each option is right or wrong**

**A** **Correct.** A competent adult may refuse life-sustaining treatment, including dialysis (Cruzan, 1990; NASW Code 1.02). Capacity is confirmed and the decision has held for weeks, so the social worker's task is support, not persuasion. The patient wants the family's acceptance, and a family meeting lets the patient explain the reasons and helps the children begin to grieve. It beats D because the legal point is accurate, but stating it alone shuts the children out; social workers elevate the patient's wishes while helping the family (NASW Standards for Palliative and End of Life Care, Standard 1).

**B** **Incorrect** (trap: Overrides the client's choice). Delaying until the family agrees lets the children's wishes override a competent patient's decision about their own treatment. The children's distress deserves support, but it does not change who decides.

**C** **Incorrect** (trap: Good but not best). A capacity and depression evaluation is essential before honoring a request to stop life-sustaining care. Here the nephrologist and a psychiatrist have already confirmed capacity, and the decision is stable, so repeating it delays the patient's choice.

**D** **Incorrect** (trap: Good but not best). The statement is legally accurate, which makes it tempting. Delivered alone, it dismisses grieving children, may strain the patient's remaining time with family, and misses the social worker's role in helping families through end-of-life decisions.

**Takeaway:** A capable patient may stop life-sustaining treatment; the social worker supports the choice and helps the family understand rather than persuading.

**Sources:** Cruzan v. Director, Missouri Department of Health, 497 U.S. 261 (1990); NASW Code of Ethics (2021), 1.02; NASW Standards for Palliative and End of Life Care, Standard 1: Ethics and Values
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## 260. ETH-1544 (rev 1) · Best answer: D

In a state where medical aid in dying is legal and the hospital permits it, a patient with terminal pancreatic cancer asks the palliative care social worker how to request it. The patient is calm and describes long-held views about dying on their own terms. What is the **MOST** appropriate response?

**A.** Refer the question to the hospital chaplain
**B.** Request an urgent suicide risk evaluation
**C.** Encourage the patient to focus on comfort care instead
**D.** Discuss the request and involve the physician

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). Spiritual care may help, and the patient can be offered it. Handing off the question entirely, though, avoids the social worker's own role in end-of-life decisions and leaves the patient without help with a legal process.

**B** **Incorrect** (trap: Skips assessment). Treating a lawful, consistent request from a terminally ill patient as a suicide emergency misreads the situation and can feel stigmatizing. Capacity and depression review is built into the legal process; an urgent evaluation fits signs of crisis, such as a hidden plan.

**C** **Incorrect** (trap: Overrides the client's choice). Steering the patient toward comfort care imposes the social worker's preference on a decision that belongs to the patient. Comfort care should be offered alongside the request, not as a substitute for discussing it.

**D** **Correct.** Where medical aid in dying is legal, social workers discuss it in line with the governing law, support the patient's informed choice and work with the team (NASW Standards for Palliative and End of Life Care, Standard 1; NASW Code 1.02). The patient is terminally ill, calm and consistent with long-held values, and the process runs through the physician. It beats B because urgent suicide evaluation fits signs of a mental health crisis, not a lawful, consistent request; eligibility review, including capacity, is part of the legal process.

**Takeaway:** Where aid in dying is legal, support the patient's informed choice and work through the physician-led legal process rather than steering or deflecting.

**Sources:** NASW Standards for Palliative and End of Life Care, Standard 1: Ethics and Values; NASW Code of Ethics (2021), 1.02

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### 261. ETH-1653 (rev 1) · Best answer: D

An ICU patient who cannot communicate after a stroke has no advance directive and no named agent. The patient's legal spouse, separated for 3 years, wants aggressive treatment. Two adult children want comfort care. The physician asks the social worker to help resolve the conflict. What should the social worker do **FIRST**?

**A.** Hold a family meeting to seek agreement on goals of care
**B.** Follow the adult children, who have been at the bedside
**C.** Ask the physician to decide based on the patient's interests
**D.** Find out whom the state's surrogate law authorizes to decide

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). A family meeting to clarify the patient's values and goals is usually the most helpful step. Holding it before anyone knows who has legal authority can produce an agreement the authorized surrogate rejects, or sideline the person entitled to decide.

**B** **Incorrect** (trap: Misreads law or ethics). Time at the bedside does not create legal authority. Many state laws rank a spouse ahead of adult children, so following the children could leave the legally authorized surrogate out of the decision.

**C** **Incorrect** (trap: Misreads law or ethics). Physicians decide alone in narrow situations, such as emergencies or when no surrogate can be found. Here relatives who may hold authority are present, so handing the choice to the physician bypasses them.

**D** **Correct.** Before mediating, the social worker needs to know who holds legal authority. Most states have default surrogate laws that rank relatives, usually placing a spouse first, and some set rules for disagreements (Wynn, 2014). Separation may or may not change a spouse's standing, so state law and hospital policy must be checked. Knowing the decision-maker protects the rights of a patient who cannot decide (NASW Code

1.14). It beats A because a family meeting is the right next step, and it works when everyone knows whose decision it is.

**Takeaway:** When an incapacitated patient left no directive, identify the legally authorized surrogate before mediating family disagreement.

**Sources:** Wynn, S. (2014). Decisions by surrogates: An overview of surrogate consent laws in the United States. Bifocal, 36(1). American Bar Association Commission on Law and Aging, default surrogate priority and disagreement rules; NASW Code of Ethics (2021), 1.14

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## 262. ETH-1660 (rev 1) · Best answer: C

Under U.S. law, how is a capable patient's request to have a ventilator removed generally classified?

**A.** Medical aid in dying

**B.** Voluntary active euthanasia

**C.** Refusal of medical treatment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Medical aid in dying means a capable, terminally ill adult takes a prescribed medication to end life, and it is lawful only in some jurisdictions. Removing a ventilator at the patient's request is a different act, recognized as a treatment refusal.

**B** **Incorrect** (trap: Wrong model or concept). Active euthanasia means a clinician directly causes death, for example by injection, and it is illegal throughout the United States. Withdrawing a treatment the patient refuses lets the underlying illness take its course.

**C** **Correct.** U.S. courts treat a capable patient's refusal of life-sustaining treatment, including a ventilator, as a protected choice (Cruzan, 1990). The Supreme Court distinguished refusing treatment from being helped to end one's life, because when treatment is refused the underlying disease causes death (Vacco v. Quill, 1997). It beats A because medical aid in dying involves a prescribed medication the patient takes to end life and is lawful only in some states, whereas the right to refuse treatment is recognized nationally.

**Takeaway:** Courts classify removing life support at a capable person's request as treatment refusal, a category separate from aid in dying and euthanasia.

**Sources:** Cruzan v. Director, Missouri Department of Health, 497 U.S. 261 (1990); Vacco v. Quill, 521 U.S. 793 (1997)

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### 263. ETH-1666 (rev 1) · Best answer: D

In a state where medical aid in dying is lawful, a hospice patient with terminal cancer asks the social worker for help understanding the request process and talking with family. The social worker objects to aid in dying on religious grounds and, after supervision, concludes they cannot discuss it without their views showing. What is the **BEST** action?

**A.** Tell the patient about the objection so the patient understands
**B.** Suggest that the patient take the questions to the physician
**C.** Continue with the patient while keeping personal views private
**D.** Arrange promptly for a colleague to help the patient

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Sharing the objection makes the patient manage the worker's beliefs and may feel like judgment at a vulnerable time. A brief, honest word about the handoff is fine; the reasons belong in supervision, not with the patient.

**B** **Incorrect** (trap: Good but not best). The physician handles the medical steps of a request, but the patient asked for help understanding the process and talking with family, which is social work's role. Redirecting without a handoff leaves that need unmet.

**C** **Incorrect** (trap: Outside the social work role). Setting views aside is the goal when a worker can manage it. This worker has concluded, after supervision, that their views would show, so continuing risks subtly steering the patient's choice.

**D** **Correct.** The patient has a right to informed, supported decisions about lawful end-of-life options (NASW Code 1.02). When a worker cannot serve a client effectively, the worker refers and arranges an orderly transfer (NASW Code 1.16(a), (b)); conflicts between personal and professional values are resolved in the client's favor (Reamer, 2018). A prompt handoff to a colleague keeps the patient from being delayed or judged. It beats C because the worker has already concluded, after supervision, that their views would show, and pressing on risks steering a dying patient.

**Takeaway:** When personal values block competent help with a lawful choice, arrange a prompt, orderly handoff without burdening the client.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.16(a), (b); Reamer, F. G. (2018). Social Work Values and Ethics (5th ed.). Columbia University Press, conflicts between personal and professional values

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### 264. ETH-0007 (rev 2) · Best answer: B

A social worker supervises a newly licensed social worker whose recent divorce is affecting their work. The supervisee has been late to sessions and tearful with clients. In supervision, the supervisee asks the supervisor to provide personal therapy for a few weeks, saying 'You already know my situation.' What is the supervisor's **BEST** response?

**A.** Agree to provide short-term therapy during supervision
**B.** Decline to provide therapy and address the impact on clients
**C.** Report the supervisee's impairment to the state licensing board

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Becoming the supervisee's therapist creates a dual relationship. It undermines the supervisor's ability to evaluate the supervisee fairly and to protect clients. The supportive function of supervision does not extend to treatment.

**B** **Correct.** Supervisors should avoid providing psychotherapy to supervisees, because the dual role compromises fair evaluation and blurs accountability for client care (Supervision Standards, Standard 4; NASW Code 3.01(c)). The supervisor's job is to name the effect on clients, plan protective steps such as adjusting the caseload, and encourage the supervisee to seek outside help, which the Code expects of any social worker whose personal problems affect practice (NASW Code 4.05(b)). It beats C because the problem is recent and the supervisee is still reachable through supervision, so supportive and corrective steps come before a formal report.

**C** **Incorrect** (trap: Misreads law or ethics). Formal reporting fits impairment that continues after the colleague has been approached and has not taken adequate steps (NASW Code 2.08). The supervisor has a direct, earlier route: addressing the problem in supervision.

**Takeaway:** Supervision supports but never becomes therapy; address the impact on clients and refer the supervisee for personal help.

**Sources:** NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 4 (Ethical Issues): Boundaries; NASW Code of Ethics (2021), 3.01(c); NASW Code of Ethics (2021), 4.05(b)
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## 265. ETH-0111 (rev 1) · Best answer: B

A social worker supervises a staff member who is working toward clinical licensure. The supervisee invites the supervisor to invest in a weekend wellness business the supervisee is starting. The supervisor will submit an evaluation of the supervisee's hours to the licensing board next month. What should the supervisor do **FIRST**?

**A.** Invest a small amount after submitting the evaluation
**B.** Decline and explain the conflict with the supervisory role
**C.** Record the invitation in the supervision notes

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Waiting until after the evaluation does not remove the conflict. The supervisor's power and influence over the supervisee's career continue, and a business tie would compromise future supervision and references.

**B** **Correct.** Supervisors must avoid dual relationships with supervisees that risk exploitation or harm (NASW Code 3.01(c)). A shared business investment would tie the supervisor's money to the supervisee's success while the supervisor holds evaluative power, including a licensing evaluation next month. Declining clearly and explaining why models the boundary the supervisee will need with clients (NASW & ASWB, 2013). It beats C because documenting the request is good practice, but it comes after the supervisor has responded; recording it first leaves the invitation open.

**C** **Incorrect** (trap: Right action, wrong time). Documenting the invitation and the response belongs in the supervision record. Doing it before responding delays the clear answer the supervisee needs and leaves the boundary question unsettled.

**Takeaway:** Supervisors hold evaluative power, so they decline business or social ties with supervisees and explain the boundary directly.

**Sources:** NASW Code of Ethics (2021), 3.01(c); NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 4 (Ethical Issues): Boundaries
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## 266. ETH-1551 (rev 1) · Best answer: D

During weekly chart review, a clinical supervisor at a community agency reads a supervisee's note from 4 days ago. The client said, 'Sometimes I think everyone would be better off without me.' The note shows no follow-up questions, and the client's next appointment is in 10 days. What should the supervisor do **FIRST**?

**A.** Ask the supervisee why the remark went unexplored
**B.** Arrange suicide risk assessment training for the supervisee
**C.** Note the lapse in the supervisee's performance record
**D.** Make sure the client is contacted today to assess risk

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Talking with the supervisee about the missed follow-up is an important step and part of the teaching role. Doing it first delays reaching a client whose possible suicidal statement has gone unassessed for 4 days.

**B** **Incorrect** (trap: Right action, wrong time). Training addresses the skill gap and protects future clients, so it belongs in the plan. It does nothing for this client, who remains unassessed with no contact scheduled for 10 days.

**C** **Incorrect** (trap: Right action, wrong time). Supervision records should capture the issue and the response, but recording it first puts paperwork ahead of the client. The supervision standards also treat errors as chances to learn, not grounds for punishment (Supervision Standards, Evaluation and Outcomes).

**D** **Correct.** Supervision exists to protect clients, and supervisors are responsible for ensuring that supervisees provide competent services (Supervision Standards, Overview). The client's remark may signal suicidal thinking, it was never assessed, and 10 days remain before the next contact. Arranging same-day outreach so risk is assessed comes before the supervisee's learning needs, because a social worker's primary duty is the client's well-being (NASW Code 1.01). It beats A because understanding the lapse matters and should follow within the day, but that talk can wait a few hours; an unassessed risk cannot.

**Takeaway:** When a supervisor finds an unaddressed client risk, protecting the client comes first; teaching and documenting the lapse follow.

**Sources:** NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Overview of Supervision; NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Evaluation and Outcomes; NASW Code of Ethics (2021), 1.01
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## 267. ETH-1553 (rev 1) · Best answer: C

A supervisee in a group practice misses clear signs that a client's condition is worsening, and the client is harmed. The supervisor had met with the supervisee weekly and reviewed cases as agreed. The client's lawsuit names both the supervisee and the supervisor. Which statement about the supervisor's legal exposure is accurate?

**A.** Regular weekly review shields the supervisor from liability
**B.** Liability requires proof that guidance to the supervisee was faulty
**C.** The supervisor may share liability for the supervisee's acts

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Diligent, documented supervision lowers risk and helps a supervisor's defense, but it does not remove vicarious liability, which rests on the supervisory relationship rather than on the supervisor's own fault.

**B** **Incorrect** (trap: Wrong model or concept). This describes direct liability, which applies when a supervisor's own recommendations or assignments cause harm. A supervisor can also face vicarious liability for a supervisee's errors without any personal fault.

**C** **Correct.** Under vicarious liability (respondeat superior), a supervisor can be held responsible for a supervisee's negligent acts or omissions within the scope of supervision, even when the supervisor did nothing wrong (Supervision Standards, Standard 3: Liability). Here the supervisor met weekly as agreed, yet the claim can still reach the supervisor through the supervisee's error. It beats B because B describes only direct liability, which requires the supervisor's own faulty guidance; vicarious liability does not.

**Takeaway:** Supervisors can be vicariously liable for supervisees' errors even without personal fault; direct liability arises from the supervisor's own faulty guidance.

**Sources:** NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 3 (Legal and Regulatory Issues): Liability
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### 268. ETH-1556 (rev 1) · Best answer: B

A supervisee at a youth services agency invites their clinical supervisor to the public commencement where the supervisee will receive an MSW. The invitation also includes a family dinner at the supervisee's home afterward. Supervision will continue for another year. What is the **MOST** appropriate response by the supervisor?

**A.** Politely decline both the public ceremony and the family dinner
**B.** Attend the public ceremony and decline the family dinner
**C.** Attend the public ceremony and the family dinner

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Declining everything looks safe, but it treats a brief, public, professional event as a violation. It can feel rejecting at a milestone and misses the difference between low-risk crossings and harmful dual relationships.

**B** **Correct.** Supervisors set clear, culturally sensitive boundaries and avoid dual relationships with supervisees that risk exploitation or harm (NASW Code 3.01(b), 3.01(c)). Briefly attending a public commencement is a low-risk boundary crossing that recognizes a professional milestone. A private family dinner moves toward a social relationship that could blur a year of evaluation ahead. It beats A because the Code bars risky dual relationships, not all contact outside the office, and a crossing with a clear purpose need not become a violation (Reamer, 2020).

**C** **Incorrect** (trap: Boundary slip or collusion). Joining a private family dinner while supervision continues for a year mixes social and evaluative roles. The supervisor's power over evaluations makes a social relationship risky, however sincere the invitation.

**Takeaway:** Low-risk, public boundary crossings with supervisees can be appropriate; avoid social ties that blur an ongoing evaluative role.

**Sources:** NASW Code of Ethics (2021), 3.01(b); NASW Code of Ethics (2021), 3.01(c); Reamer (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, Boundary crossings versus boundary violations
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### 269. ETH-1558 (rev 2) · Best answer: B

A hospital opens an outpatient eating disorders program and asks an experienced clinical supervisor to supervise its 4 newly hired social workers. The supervisor has strong skills with mood and anxiety disorders but no training in eating disorders. Several program clients have serious medical complications. What is the **BEST** response by the supervisor?

**A.** Accept and learn the specialty alongside the new staff
**B.** Ask program leaders to secure an eating disorders specialist to supervise
**C.** Accept and meet monthly with an outside eating disorders expert
**D.** Decline and let the program find someone else to supervise

**Why each option is right or wrong**

**A** **Incorrect** (trap: Outside the social work role). Learning alongside new staff means supervising high-risk work without the knowledge to catch errors. A new specialty is built through one's own training and supervision, not while serving as the team's supervisor.

**B** **Correct.** Supervisors work only within their knowledge and competence (NASW Code 3.01(a)). When a specialty is unfamiliar, the supervision standards direct supervisors to obtain assistance or refer supervisees to an appropriate source (Supervision Standards, Standard 2: Competency). With new staff and medically fragile clients, the team needs a supervisor already trained in eating disorders, and administrators are responsible for adequate supervision resources (NASW Code 3.07(c)). It beats C because monthly outside consultation leaves week-to-week supervision of high-risk cases with someone who lacks the needed skill.

**C** **Incorrect** (trap: Good but not best). The supervision standards do endorse getting expert help when a specialty is unfamiliar, and it would help here. But monthly contact cannot make a supervisor with no training competent to guide new staff week to week on medically complicated cases.

**D** **Incorrect** (trap: Good but not best). Declining is honest about the limits of competence, but simply stepping back leaves new staff and fragile clients without a plan. The stronger step names the need and helps secure qualified supervision.

**Takeaway:** Supervise only within competence; when a specialty is unfamiliar, help secure qualified supervision instead of learning on the job.

**Sources:** NASW Code of Ethics (2021), 3.01(a); NASW Code of Ethics (2021), 3.07(c); NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 2 (Conduct of Supervision): Competency
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 270. ETH-1562 (rev 2) · Best answer: A

A clinical supervisor in a group practice is asked to sign a former supervisee's licensure verification form. The form lists 24 months of supervision, but no supervision took place during 3 of those months while the supervisor was on medical leave. The supervisee needs the full total to apply this year. What is the **BEST** action by the supervisor?

**A.** Sign for the months in which supervision occurred
**B.** Sign for the 24 months of supervision and attach a note about the leave
**C.** Decline to sign any part of the verification form

**Why each option is right or wrong**

**A** **Correct.** Social workers do not take part in dishonesty, fraud or deception (NASW Code 4.04), and those who evaluate others do so fairly and on stated criteria (3.03). Licensing boards rely on supervisors' verification to protect the public. Certifying the 21 months that occurred is accurate and lets the supervisee complete the remaining hours. It beats C because refusing to verify supervision that did happen is unfair to the supervisee and protects no one.

**B** **Incorrect** (trap: Misreads law or ethics). A note seems transparent, but the signed form still certifies 24 months, 3 of which never happened. A verification to a licensing

board must be accurate on its face; a side note does not cure a false certification and still exposes the supervisor to discipline.

**C** **Incorrect** (trap: Good but not best). Refusing to sign anything penalizes the supervisee for the supervisor's leave and withholds accurate verification of 21 real months. Declining fits only if the supervisor cannot confirm any of the supervision.

**Takeaway:** Verify supervision accurately: certify the supervision that occurred, no more, and do not withhold verification of what did occur.

**Sources:** NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 3.03
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 271. ETH-1564 (rev 3) · Best answer: C

A field instructor at a middle school will complete an MSW intern's final evaluation in 3 weeks. For several months, the intern's case notes have been late and incomplete, but the field instructor has never raised the issue. The evaluation form includes documentation as a rated skill. What should the field instructor do **FIRST**?

**A.** Rate the intern's documentation as unsatisfactory on the evaluation
**B.** Email the intern a list of the late and incomplete notes
**C.** Meet with the intern now to review documentation against the criteria

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). An accurate rating matters and may still be low after the discussion. Issuing it with no prior feedback denies the intern the chance to understand and improve, which makes the evaluation unfair.

**B** **Incorrect** (trap: Misuses the technique). Flagging the missing notes is a start, and it is quick. A task list by email, though, does not review documentation against the evaluation criteria or give the intern a chance to discuss the skill and improve before it is rated.

**C** **Correct.** Evaluations should be fair, based on clearly stated criteria, and supported by planned, continuous feedback (NASW Code 3.02(b), 3.03; Supervision Standards, Evaluation and Outcomes). The field instructor has let a known problem go unmentioned for months. Meeting now to review the notes against the stated criteria gives the intern specific feedback and time to improve before the rating. It beats A because a low rating may still be warranted, but giving it first, with no prior feedback, turns the evaluation into a surprise rather than a fair judgment.

**Takeaway:** Fair evaluation means feedback throughout against stated criteria; raise concerns before the final rating, not for the first time in it.

**Sources:** NASW Code of Ethics (2021), 3.02(b); NASW Code of Ethics (2021), 3.03; NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Evaluation and Outcomes
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 272. ETH-1566 (rev 2) · Best answer: C

The clinical director of a psychiatric hospital learns that a unit supervisor has begun a romantic relationship with a social worker the supervisor directly supervises. Both say the relationship is consensual and does not affect their work. What should the director do **FIRST**?

**A.** Ask both to keep the relationship separate from work
**B.** Refer the supervisor for a human resources conduct review
**C.** Move the supervisee to a different supervisor right away
**D.** Wait unless a complaint or performance problem arises

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Keeping the relationship out of daily work leaves the supervisor evaluating a romantic partner. The conflict comes from the supervisory authority itself, which this request leaves in place.

**B** **Incorrect** (trap: Right action, wrong time). A conduct review under agency policy is a likely later step. Starting with it leaves the supervisee under a romantic partner's evaluation while the process runs, so the conflict continues.

**C** **Correct.** Supervisors should not have sexual relationships with people over whom they hold professional authority (NASW Code 2.06(a)), and a social worker in a relationship with a colleague has a duty to transfer professional responsibilities to avoid a conflict of interest (2.06(b)). The supervision standards say romantic involvement with a supervisee should be strictly avoided because it creates marked role conflict (Supervision Standards, Standard 4: Boundaries). Consent does not remove the power imbalance. The first step is to end the supervisory line by reassigning supervision. It beats B because a conduct review may follow, but it takes time, and the supervisee should not stay under the partner's evaluation meanwhile.

**D** **Incorrect** (trap: Misreads law or ethics). Waiting for a problem misreads the rule: the relationship itself creates the conflict, whatever the pair report about their work. Acting only after harm leaves the supervisee, and fairness to other staff, at risk (NASW Code 3.01(c)).

**Takeaway:** A romantic relationship between supervisor and supervisee requires ending the supervisory line at once; consent does not cure the power conflict.

**Sources:** NASW Code of Ethics (2021), 2.06(a); NASW Code of Ethics (2021), 2.06(b); NASW Code of Ethics (2021), 3.01(c); NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 4 (Ethical Issues): Boundaries
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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### 273. ETH-1568 (rev 1) · Best answer: B

According to the NASW Code of Ethics, what should field instructors ensure when social work students provide services to clients?

**A.** Students' work is reviewed before each session begins
**B.** Clients are routinely told that a student is serving them
**C.** Clients sign a separate consent form for student services

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Field instructors review students' work regularly, but the Code sets no rule requiring review before every session. The standard here concerns the client's right to know who is serving them.

**B** **Correct.** Educators and field instructors take reasonable steps to ensure that clients are routinely informed when services are provided by students (NASW Code 3.02(c)). Clients have a right to know who is serving them and in what role, which supports informed consent (1.03(a)). It beats C because the Code requires that clients be informed, not that a separate signed consent be obtained; an agency may add a form, but informing clients is the ethical standard.

**C** **Incorrect** (trap: Misreads law or ethics). Some agencies use written acknowledgment forms, but the Code's standard is that clients are routinely informed. A separate consent form is a local practice, not the ethical requirement.

**Takeaway:** Clients should be routinely told when a social work student is providing their services.

**Sources:** NASW Code of Ethics (2021), 3.02(c); NASW Code of Ethics (2021), 1.03(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 274. ETH-1570 (rev 1) · Best answer: D

A program manager at an outpatient clinic tells a clinical supervisor to drop suicide screening at intake to shorten wait times. The supervisor raised the safety risk with the manager in writing and proposed a shorter screening tool, but the manager repeated the directive. What should the supervisor do **NEXT**?

**A.** Have clinical staff keep screening without telling the manager
**B.** Follow the directive while keeping a record of the objection
**C.** Propose a trial of the shorter tool to the manager again
**D.** Take the concern to the clinical director or compliance office

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Quietly continuing screening protects clients for now but defies a directive in secret, damages trust and leaves the unsafe policy in place. Open escalation through proper channels addresses the policy itself.

**B** **Incorrect** (trap: Ignores risk). Recording the objection documents the concern but still removes a safety step for every new client. Employer directives do not override the duty to avoid practices that put clients at risk.

**C** **Incorrect** (trap: Good but not best). Offering the alternative again is reasonable advocacy, but the manager has already refused it in writing. Each week of delay means more new clients go unscreened while the supervisor waits.

**D** **Correct.** Social workers do not let employer policies interfere with ethical practice and work to bring agency practices in line with the Code (NASW Code 3.09(d)). The supervisor has already raised the risk directly with the manager and offered an alternative, and the manager refused. The next step is formal channels above the manager (2.10(d)), since administrators must keep a work environment that supports

Code compliance (3.07(d)). It beats C because repeating a refused proposal leaves screening suspended while new clients keep arriving.

**Takeaway:** When a manager refuses to reverse a directive that endangers clients, take the concern through formal channels above that manager.

**Sources:** NASW Code of Ethics (2021), 3.09(d); NASW Code of Ethics (2021), 2.10(d); NASW Code of Ethics (2021), 3.07(d)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 275. ETH-1677 (rev 1) · Best answer: A

Two weeks before completing a supervisee's final evaluation for licensure, a clinical supervisor at a family agency receives a gift from the supervisee: concert tickets worth about $200. The card thanks the supervisor for 2 years of guidance. Agency policy says nothing about gifts between staff. What should the supervisor do?

**A.** Return the tickets and explain the conflict with the evaluation
**B.** Hold the tickets until the evaluation has been submitted
**C.** Accept the tickets and thank the supervisee for the gesture

#### Why each option is right or wrong

**A** **Correct.** Supervisors avoid dual relationships that risk exploitation or harm and should evaluate supervisees fairly (NASW Code 3.01(c); 3.01(d)). A $200 gift arriving just before a licensure evaluation creates an obligation, or the appearance of one, at the moment the supervisor's judgment matters most. Returning it kindly, with a plain explanation, protects the supervisee as well as the evaluation. It beats B because the supervisor would still write the evaluation knowing a valuable gift is waiting, so delaying acceptance does not remove its influence.

**B** **Incorrect** (trap: Good but not best). Waiting until the evaluation is done looks like a fix, but the supervisor writes it knowing a valuable gift is waiting. The influence, and the appearance of it, stays in place. A small token after supervision ends would raise far fewer concerns.

**C** **Incorrect** (trap: Boundary slip or collusion). The gratitude is genuine, but accepting a $200 gift right before a licensure evaluation mixes a personal exchange into an evaluative relationship. The supervisee or colleagues could fairly question the evaluation's objectivity.

**Takeaway:** Supervisors decline valuable gifts from supervisees during evaluation periods, because the gift compromises, or appears to compromise, fair evaluation.

**Sources:** NASW Code of Ethics (2021), 3.01(c); NASW Code of Ethics (2021), 3.01(d)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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### 276. ETH-1681 (rev 2) · Best answer: C

A former supervisee asks a clinical supervisor for a reference letter for a hospital job. During supervision, the supervisee was warm with patients but often missed documentation deadlines, and two written improvement plans addressed it. The supervisee asks the supervisor to 'focus on the positives.' Which response is **MOST** consistent with ethical standards?

**A.** Write a letter on strengths alone, leaving out the deadline issues
**B.** Write a balanced letter and send it straight to the hospital
**C.** Explain that an honest letter would include the deadline issues

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Leaving out a problem that took two improvement plans creates a misleading picture through omission. The hospital relies on the letter to judge fitness for a role in which documentation affects patient care.

**B** **Incorrect** (trap: Right action, wrong time). A balanced letter is the right content, so this is tempting. Sending it without first telling the supervisee, who asked for a positives-only letter, takes away the supervisee's informed choice to use it or ask someone else, which fairness in evaluation calls for.

**C** **Correct.** Social workers should not participate in or be associated with dishonesty, and evaluations of others should be fair and based on clearly stated criteria (NASW Code 4.04; 3.03). A letter that omits a documented, repeated problem misleads the hospital about a skill that affects patient care. Telling the supervisee up front what an honest letter would say lets the supervisee decide whether to use it. It beats B because a balanced letter is the right product, but sending it unannounced, after the supervisee asked for something else, removes the supervisee's choice about whether to use it.

**Takeaway:** References for supervisees should be honest; tell the requester what an accurate letter will include rather than omitting documented concerns.

**Sources:** NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 3.03
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.7 Ethics of supervising and managing · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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### 277. ETH-1686 (rev 2) · Best answer: D

A hospital field instructor has given an MSW intern weekly feedback and a written learning plan for missed notes and late arrivals. At midterm, the intern still falls short on several competencies. The intern then discloses a learning disability, says the school has never been told, and asks the instructor not to give a failing rating. What should the field instructor do **NEXT**?

**A.** Give a passing midterm rating while the intern seeks help
**B.** Arrange accommodations with the intern without involving the school
**C.** End the field placement, since the learning plan has failed
**D.** Involve the school's field liaison in reviewing the plan

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). A passing rating that does not reflect performance misleads the school and the intern and could put patients at risk. The disclosure does not change what the intern has shown so far; it may change supports going forward.

**B** **Incorrect** (trap: Outside the social work role). Adjusting tasks informally is well meant, but formal accommodations run through the school, which decides what is reasonable within program standards. Private arrangements can be unfair to the intern and leave the evaluation on uncertain footing.

**C** **Incorrect** (trap: Hands off too soon). Ending the placement at midterm, right after a disability disclosure and before the school is involved, is premature and could look discriminatory. It may be considered later if the intern still falls short with proper supports.

**D** **Correct.** Field instructors evaluate students fairly and against clearly stated criteria (NASW Code 3.02(b); 3.03). The school runs the field program and its disability accommodation process, so when a student may not meet competencies, the field liaison shares the next decision. The liaison can review the learning plan and help the intern seek formal accommodations; the instructor shares the performance concerns and leaves disclosure of the disability to the intern. It beats B because accommodations arranged privately on site bypass the school's process and may not fit program standards.

**Takeaway:** When a student is falling short, involve the school's field liaison; accommodations go through the school, and evaluations stay honest and criteria-based.

**Sources:** NASW Code of Ethics (2021), 3.02(b); NASW Code of Ethics (2021), 3.03
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## 278. ETH-1692 (rev 1) · Best answer: B

A clinical supervisor at the only behavioral health agency in a rural county learns that a new supervisee will coach the supervisor's 10-year-old child's soccer team this season. Contact would be limited to weekend practices and games. The agency has no other supervisor with the supervisee's specialty. What is the supervisor's **MOST** appropriate action?

**A.** Arrange video supervision for the supervisee from another county
**B.** Discuss the overlap with the supervisee and agree on clear boundaries
**C.** Continue as usual, since the contact happens outside work

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). An outside supervisor would remove the overlap, but the overlap carries little risk: it is public, limited and unrelated to work. Moving supervision out of the agency fits when a dual relationship is significant, such as a close friendship or a family tie.

**B** **Correct.** Supervisors avoid dual relationships with supervisees that carry a risk of exploitation or harm, and they set clear, appropriate, culturally sensitive boundaries (NASW Code 3.01(b); 3.01(c)). In a small community some overlap cannot be avoided

(Reamer, 2020). Coaching a team means public, limited contact with nothing that bears on evaluation, so naming it and agreeing on how to handle it, such as keeping work talk out of games, manages the risk. It beats A because remote supervision by an outsider trades a low, manageable risk for supervision from someone who does not know the agency or its community.

**C** **Incorrect** (trap: Good but not best). The contact is low risk, but ignoring it leaves both people guessing about how to act at games and whether work topics will come up. Naming the overlap early prevents awkward moments and protects the evaluation.

**Takeaway:** Unavoidable, low-risk overlaps with supervisees, common in small communities, are managed by naming them and setting boundaries, not by ending supervision.

**Sources:** NASW Code of Ethics (2021), 3.01(b); NASW Code of Ethics (2021), 3.01(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, dual relationships in small and rural communities

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## 279. ETH-1552 (rev 3) · Best answer: A

A child welfare social worker meets with a parent whose 2 children entered foster care 4 months ago. The parent is starting substance use treatment and believes there is no deadline for reunification. What is the **MOST** appropriate way to respond to the parent's belief?

**A.** Explain the permanency time frames and how they shape the plan

**B.** Refer the parent to their attorney to explain the time frames

**C.** Tell the parent that rights end automatically after 15 months

#### Why each option is right or wrong

**A** **Correct.** Clients have a right to plain-language information about the limits and conditions that affect their services (NASW Code 1.03(a)). Under the Adoption and Safe Families Act, a permanency hearing is held within 12 months, and the state generally must seek termination of parental rights once a child has been in care 15 of the last 22 months, with exceptions (42 U.S.C. 675(5)). Explaining this now lets the parent use the remaining time well. It beats B because legal strategy belongs with the parent's attorney, but plain information about the case's time frames is part of the social worker's role, and handing it off leaves a costly misbelief in place now.

**B** **Incorrect** (trap: Hands off too soon). Parents in dependency cases usually have counsel, and questions of legal strategy belong with the attorney. Basic information about permanency time frames is part of the caseworker's job, though, and sending the parent elsewhere leaves the belief uncorrected while the federal clock runs.

**C** **Incorrect** (trap: Misreads law or ethics). This overstates the rule. The agency generally must file or join a petition at 15 of 22 months, but exceptions exist, such as care by a relative or a documented compelling reason, and a court decides. Accurate information, not threats, supports engagement.

**Takeaway:** Explain policy-driven time limits, such as federal permanency deadlines, early and accurately so clients can make informed choices.

**Sources:** NASW Code of Ethics (2021), 1.03(a); Adoption and Safe Families Act of 1997, 42 U.S.C. 675(5)(C), (E)
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### 280. ETH-1555 (rev 1) · Best answer: C

A client in an intensive outpatient program for alcohol use disorder lost Medicaid coverage 3 weeks ago. The state mailed the renewal form to an old address, and the client never received it. The client is doing well and wants to continue. Several other clients have lost coverage the same way. What should the social worker do **FIRST**?

**A.** Propose a fix for renewal mailings to state officials
**B.** Ask the client why the address was left outdated
**C.** Help the client submit the renewal paperwork this week
**D.** Discharge the client with referrals to free treatment programs

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). The pattern across several clients justifies systems advocacy, which can follow soon (NASW Code 6.04(a)). Starting there leaves this client without coverage while a slow policy process unfolds.

**B** **Incorrect** (trap: Good but not best). The stem already explains how coverage was lost. Asking why the address was not updated adds blame and delays help; correcting contact details is simply part of filing the renewal.

**C** **Correct.** When a policy or procedure cuts off care, the first response secures the client's own access, since the social worker's primary duty is the client's well-being (NASW Code 1.01). Medicaid rules require states to reconsider a person dropped for not returning a renewal form, without a new application, if the form comes in within 90 days (42 CFR 435.916). Filing now can restore coverage and keep treatment going. It beats A because the pattern across clients warrants advocacy, but a proposal to officials will not restore this client's coverage in time.

**D** **Incorrect** (trap: Hands off too soon). Ending treatment for a client who is doing well and wants to continue, when coverage can likely be restored, risks a needless break in care. Referral to free programs fits only if coverage cannot be restored or other payment arranged.

**Takeaway:** When a procedural policy cuts off a client's coverage, restore the client's access first; systems advocacy for the wider pattern follows.

**Sources:** NASW Code of Ethics (2021), 1.01; NASW Code of Ethics (2021), 6.04(a); Medicaid eligibility renewal regulations, 42 CFR 435.916
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### 281. ETH-1561 (rev 1) · Best answer: D

A city has begun clearing homeless encampments every month under a new ordinance. During each sweep, clients of a street outreach team lose medications, IDs and benefit papers. The team has replaced these items after every sweep, and the losses keep recurring. What is the **BEST** action for the team at this point?

**A.** Keep replacing lost items for clients after each sweep
**B.** Ask police to tell the team before each sweep happens
**C.** Encourage clients to move their camps outside city limits
**D.** Present documented harms to city leaders with proposed changes

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Replacing medications and documents is essential direct help, and the team has done it. Repeating it every month treats the effects while the ordinance keeps producing the same losses.

**B** **Incorrect** (trap: Good but not best). Advance notice would reduce losses and could be one of the changes the team proposes. As an informal request alone, it depends on goodwill and leaves the ordinance's harms unrecorded and unaddressed.

**C** **Incorrect** (trap: Overrides the client's choice). Steering clients to move shifts the burden onto them and may cut them off from services and support. Where clients live is their choice; the problem the team sees lies in the policy.

**D** **Correct.** When a policy keeps harming clients, social workers pair direct help with advocacy for policy change that expands choice for vulnerable people (NASW Code 6.04(a), 6.04(b)). The team has met immediate needs, and the harm recurs with each sweep. Bringing documented effects to city leaders, with proposals such as advance notice and safe storage, addresses the cause. It beats B because advance notice would help, but a private arrangement leaves the ordinance unchanged and can be withdrawn at any time; the proposal to leaders can include notice.

**Takeaway:** Once immediate needs are met and a policy keeps causing the same harm, document its effects and press decision-makers for change.

**Sources:** NASW Code of Ethics (2021), 6.04(a); NASW Code of Ethics (2021), 6.04(b)
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### 282. ETH-1565 (rev 2) · Best answer: A

A client with major depressive disorder has employer-sponsored health insurance through a large company. The plan denied payment for therapy after 12 visits this year, citing a visit cap. The social worker reviewed the plan documents with the client and found no visit cap on outpatient medical care. What should the social worker do **NEXT**?

**A.** Help the client appeal the decision under federal parity rules
**B.** Switch the client to a reduced self-pay fee starting right away
**C.** Tell the client the plan broke the law and should be sued

**Why each option is right or wrong**

**A** **Correct.** Federal parity law bars covered group health plans from placing visit limits on mental health benefits that are stricter than those applied to most medical and surgical benefits (29 U.S.C. 1185a). A therapy-only cap in a large employer's plan raises exactly this concern. Helping the client file an appeal that cites the law uses the plan's own process and keeps treatment going, serving the client's interests first (NASW Code 1.01). It beats B because a reduced fee may be needed if the appeal fails, but starting there gives up coverage the client may be owed.

**B** **Incorrect** (trap: Good but not best). A reduced fee can keep treatment going if coverage is truly unavailable. Moving to self-pay first gives up benefits the client may be entitled to and shifts a cost the plan may owe onto the client.

**C** **Incorrect** (trap: Outside the social work role). Declaring that the plan broke the law and recommending a lawsuit is a legal judgment outside the social work role. The appeal process comes first, and seeking legal counsel is the client's choice if it fails.

**Takeaway:** When insurance limits mental health care more strictly than medical care, help the client use appeal rights under federal parity law.

**Sources:** Mental Health Parity and Addiction Equity Act of 2008, 29 U.S.C. 1185a(a)(3)(A); NASW Code of Ethics (2021), 1.01

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## 283. ETH-1569 (rev 1) · Best answer: B

A parent tells a school social worker that their 8-year-old is struggling with reading and asks for a special education evaluation. The school's practice is to complete 12 weeks of classroom reading intervention before considering a referral, and 4 weeks remain. What is the **MOST** helpful response by the social worker?

**A.** Tell the parent an evaluation can be requested once intervention ends

**B.** Help the parent submit a written request for an evaluation now

**C.** Suggest a private evaluation to avoid waiting for the school

**D.** Ask the teacher to increase the intensity of reading support

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Intervention data are useful, but federal guidance says an intervention process cannot delay or deny an evaluation a parent requests. Telling the parent to wait misstates the parent's rights (OSEP Memo 11-07).

**B** **Correct.** Under the Individuals with Disabilities Education Act, a parent may request an initial evaluation (34 CFR 300.301(b)), and the U.S. Department of Education has stated that an intervention process cannot be used to delay or deny one (OSEP Memo 11-07, 2011). Putting the request in writing starts the school's required response: it must seek consent to evaluate or give written notice explaining a refusal (34 CFR 300.503). It beats D because stronger reading support helps in any case but does not act on the parent's request.

**C** **Incorrect** (trap: Hands off too soon). A private evaluation is the family's option, but it costs money and does not replace the school's duty to respond. The parent need not go outside the school to be heard.

**D** **Incorrect** (trap: Good but not best). More intensive reading support may help and can continue during an evaluation. It does not respond to the parent's request, which the school must act on.

**Takeaway:** School intervention routines cannot delay a parent's request for a special education evaluation; help families use their federal rights.

**Sources:** Individuals with Disabilities Education Act regulations, 34 CFR 300.301(b); Individuals with Disabilities Education Act regulations, 34 CFR 300.503; U.S. Department of Education, OSEP Memo 11-07 (2011), RTI process cannot delay or deny an evaluation

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.8 How local, state, federal policy affects service delivery · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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### 284. ETH-1679 (rev 1) · Best answer: B

A 45-year-old client with no dependents receives a notice that food benefits will stop after 3 months because the client is not working or in a work program 80 hours a month. The client has severe major depressive disorder, sleeps most of the day and has not held a job in a year. The client's psychiatrist confirms the client cannot work now. What should the social worker do **FIRST**?

**A.** Help the client search for part-time jobs that meet the hour rule
**B.** Help the client request a medical exemption from the time limit
**C.** Write to state legislators about the harm of the work rule
**D.** Refer the client to food pantries to replace the lost benefits

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Job searching runs against the psychiatrist's judgment that the client cannot work now, and a failed job could deepen the depression. Work-focused planning fits later, if recovery makes employment realistic.

**B** **Correct.** When a policy threatens a client's benefits, the first move is to use the protections the policy itself contains, then advocate for change (NASW Code 1.01; 6.04(a)). Federal SNAP rules exempt people certified as physically or mentally unfit for employment from the time limit for adults without dependents, and a physician's statement can supply that certification (7 CFR 273.24(c)(2)). The psychiatrist's confirmation makes this request strong. It beats D because a pantry only partly replaces benefits the client likely qualifies to keep.

**C** **Incorrect** (trap: Right action, wrong time). Advocating for policy change is part of the social work role and may follow, especially if other clients face the same rule. It will not keep this client's benefits from ending in 3 months.

**D** **Incorrect** (trap: Good but not best). Food pantries can bridge a gap and are worth mentioning. They do not replace a benefit the client likely qualifies to keep, and relying on them first gives up the exemption the rules provide.

**Takeaway:** When a policy threatens a client's benefits, first use the policy's own exemptions and appeals; then advocate for change.

**Sources:** NASW Code of Ethics (2021), 1.01; NASW Code of Ethics (2021), 6.04(a); SNAP time limit regulations, 7 CFR 273.24(c)(2)

**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.8 How local, state, federal policy affects service delivery · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 285. ETH-1690 (rev 2) · Best answer: A

Under the McKinney-Vento Act, what may a student do when the family becomes homeless and moves to a shelter in another district?

**A.** Remain in the school of origin, with transportation provided
**B.** Enroll in the new district once school records are transferred
**C.** Stay at the original school until the semester ends, then transfer

**Why each option is right or wrong**

**A** **Correct.** The McKinney-Vento Act presumes that keeping a homeless student in the school of origin, the school last attended when housed or last enrolled, is in the student's best interest unless the parent asks otherwise, and transportation must be provided at the parent's request (42 U.S.C. 11432(g)(3)(B); (g)(1)(J)(iii)). The family may instead choose the local school, which must enroll the student immediately. It beats C because the right lasts for the duration of homelessness, not just to the end of a term.

**B** **Incorrect** (trap: Misreads law or ethics). The family may choose the new district's school, but the law requires immediate enrollment even without the records normally required. Waiting for records would wrongly keep the student out of school.

**C** **Incorrect** (trap: Misreads law or ethics). The right to stay is not limited to the current semester, and the student need not transfer when it ends. It lasts for the duration of homelessness and through the end of the school year in which the student becomes permanently housed.

**Takeaway:** Homeless students may stay in their school of origin with transportation, or enroll immediately in the local school; the family's preference gets priority.

**Sources:** McKinney-Vento Homeless Assistance Act, 42 U.S.C. 11432(g)(3); McKinney-Vento Homeless Assistance Act, 42 U.S.C. 11432(g)(1)(J)(iii)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.8 How local, state, federal policy affects service delivery · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 286. ETH-0112 (rev 1) · Best answer: C

A social worker accepts a job at another agency and will leave in 6 weeks. The supervisor has approved a plan for telling clients this week. One client has worked on trauma symptoms with the social worker for 8 months. What should the social worker do **NEXT** with this client?

**A.** Refer the client to a trauma specialist outside the agency
**B.** Wait until the last 2 sessions to tell the client of the move
**C.** Tell the client about the move and discuss options for care

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). An outside referral may suit the client, but making it before the client hears about the departure takes away the client's say. A discussion of options, including staying at the agency with a new worker, comes first.

**B** **Incorrect** (trap: Right action, wrong time). Delaying the news shortens the time to work through the ending, which matters after a long trauma-focused relationship. Late notice can feel like abandonment and leaves little room for a careful transfer.

**C** **Correct.** Clients should get prompt notice of an anticipated interruption, and a social worker leaving an agency should explain the options for continuing services, with the benefits and risks of each (NASW Code 1.17(e), 1.17(f)). With 6 weeks left and an 8-month trauma relationship, early notice gives time to process the ending and choose the next step together. It beats A because a referral may be one of the options, but deciding it before the client knows about the move removes the client's choice. Any transfer then proceeds with consent (NASW Code 1.16(b)).

**Takeaway:** When leaving an agency, tell clients early and lay out their options for continuing care so they can choose.

**Sources:** NASW Code of Ethics (2021), 1.17(e); NASW Code of Ethics (2021), 1.17(f); NASW Code of Ethics (2021), 1.16(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 287. ETH-1528 (rev 1) · Best answer: C

A social worker in private practice has seen a client for depression for 4 months. The depression has resolved, but the client now spends hours on checking rituals. The social worker is not trained in exposure and response prevention, and two consultations confirmed the client needs it. The client wants to keep meeting with the social worker. What should the social worker do **NEXT**?

**A.** Attend a weekend workshop and then start the treatment
**B.** Continue supportive counseling, as the client prefers
**C.** Help the client transfer to a provider of that treatment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Outside the social work role). A single workshop does not create competence in a new treatment. Social workers use a new method only after appropriate study, training, consultation and supervision (NASW Code 1.04(b)). Treating the client as a practice case risks harm when trained clinicians are available.

**B** **Incorrect** (trap: Good but not best). Respecting the client's preference matters, and the attachment deserves discussion. But continuing sessions that do not treat the main problem leaves the client without effective care. Supportive work fits only if the client declines the referral after a full discussion.

**C** **Correct.** Social workers refer when another professional's specialized expertise is needed to serve the client fully, and they make the transfer orderly (NASW Code 1.16(a), (b)). The depression has resolved, the remaining need is exposure and response prevention, and two consultations confirmed it is beyond the social worker's training. A planned transfer includes discussing the reasons, sharing records with consent and holding a closing session. It beats B because honoring the wish to stay would leave the main problem untreated; the client still chooses among qualified providers.

**Takeaway:** When a client needs a treatment beyond the worker's training, refer and arrange an orderly transfer rather than continuing ineffective care.

**Sources:** NASW Code of Ethics (2021), 1.16(a)-(b); NASW Code of Ethics (2021), 1.04(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 288. ETH-1536 (rev 2) · Best answer: C

At a community mental health agency, a client became enraged when a housing letter was delayed and told the social worker, 'I know where you park. You'll regret this.' The next day the client left a similar voicemail. When the supervisor called the client to discuss the threats, the client repeated them. The agency has reported the threats under its safety policy. What is the **BEST** way to handle the clinical relationship?

**A.** Continue sessions in a room close to security staff
**B.** Address the threats with the client at the next session
**C.** End services and arrange care with another provider
**D.** Close the case by letter and decline further contact

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Security precautions are reasonable while a transition is arranged. Continuing the relationship as before, though, keeps the worker exposed to a client who has repeated the threats even after the supervisor raised them, and the relationship itself remains damaged.

**B** **Incorrect** (trap: Ignores risk). Talking through anger can repair minor ruptures, and a safe closing contact may acknowledge what happened. Here the supervisor already tried to discuss the threats and the client repeated them, so another attempt in session is unlikely to help and puts the worker at risk.

**C** **Correct.** Social workers avoid abandoning clients, withdraw services abruptly only in unusual circumstances while minimizing harm, and help arrange continued services (NASW Code 1.17(b)). Specific threats against the worker, repeated even after the supervisor addressed them, compromise both the worker's safety and the working relationship. Ending this relationship while arranging care with another provider protects the worker and keeps the client served. It beats A and B because added security or another talk leaves the client with a worker the client keeps threatening, after a direct attempt to address the threats has failed.

**D** **Incorrect** (trap: Misreads law or ethics). Ending contact by letter alone, with no referral, leaves a client in need without services, which is abandonment. Even when termination is justified, the social worker helps arrange continued care.

**Takeaway:** Threats against the worker can justify ending services, but termination still includes arranging care elsewhere to avoid abandonment.

**Sources:** NASW Code of Ethics (2021), 1.17(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 289. ETH-1543 (rev 2) · Best answer: B

A client in a group practice has 2 sessions left before insurance benefits run out for the year. The client still has frequent panic attacks and cannot afford to pay out of pocket. An appeal to the insurer was denied. What should the social worker do **FIRST**?

**A.** End services after the last covered session, with referrals
**B.** Discuss continuation options, such as a reduced fee
**C.** Keep meeting weekly and let the unpaid balance build

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Referrals may be the right outcome, and they belong in any ending plan. Deciding to end before discussing alternatives with the client, though, skips the client's preferences and options such as a reduced fee.

**B** **Correct.** Social workers who anticipate an interruption notify clients promptly and seek transfer, referral or continuation based on the client's needs and preferences (NASW Code 1.17(e)). With 2 covered sessions left and ongoing panic, the first step is a joint conversation about options: a reduced fee, less frequent sessions, a lower-cost program or a planned transfer. It beats A because referrals may well be part of the plan, but choosing termination before reviewing options with the client skips the client's preferences and risks a needless break in care (NASW Code 1.17(b)).

**C** **Incorrect** (trap: Good but not best). Letting a balance build without a clear agreement creates debt the client cannot pay and may later force an abrupt ending for nonpayment. Fees should be discussed openly and consider the client's ability to pay (NASW Code 1.13(a)).

**Takeaway:** When coverage ends mid-treatment, first discuss continuation options with the client, such as reduced fees or transfer, before ending services.

**Sources:** NASW Code of Ethics (2021), 1.17(b), (e); NASW Code of Ethics (2021), 1.13(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 290. ETH-1550 (rev 1) · Best answer: B

A client receiving depression treatment at a community mental health clinic has missed 3 sessions in a row. The social worker has called twice and left voicemails, but the client has not replied. The treatment goals are not yet met. What should the social worker do **NEXT**?

**A.** Close the case and record the client's lack of contact
**B.** Send a letter offering a closing session and referrals
**C.** Keep the case open and hold the client's weekly time

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Closing the file after two voicemails, with no final outreach and no referrals, leaves a client with unmet goals without a path back to care. Closing becomes appropriate after a final offer of closure or referral has gone unanswered.

**B** **Correct.** Social workers avoid abandoning clients who still need services and help arrange continued care when services end (NASW Code 1.17(b)). After two unanswered calls, a letter that offers a closing session and lists referrals gives this

client, whose goals are unmet, a clear chance to return or move to other care. It also shows a reasonable final effort before closure (NASW Code 1.17(e)). It beats C because holding a slot indefinitely leaves the client's status unresolved and offers no route to other help if the client does not come back.

**C** **Incorrect** (trap: Good but not best). Holding the time slot is well-meant, but it leaves the client's status unclear, delays services for others and offers no closure or referral. It fits only when the client has said they plan to return soon.

**Takeaway:** When a client stops attending, make a final outreach that offers closure and referrals, and document it before closing the case.

**Sources:** NASW Code of Ethics (2021), 1.17(b); NASW Code of Ethics (2021), 1.17(e)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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### 291. ETH-1554 (rev 1) · Best answer: A

In the fourth session at an outpatient clinic, a client voices strong contempt for people who receive public assistance. The social worker grew up in a family that relied on public assistance, finds the remarks upsetting and intends to refer the client elsewhere. The client is making progress on work-related anxiety. What should the social worker do **FIRST**?

**A.** Seek supervision about the reaction before changing the plan
**B.** Refer the client to a colleague with a different background
**C.** Tell the client how the remarks affect the social worker
**D.** Challenge the client's views on public assistance in session

#### Why each option is right or wrong

**A** **Correct.** Social workers do not end or transfer services simply because a client's views clash with their own. Ending care that is working risks abandonment (NASW Code 1.17(b)), and referral is meant for when special knowledge is needed or the worker is not being effective (1.16(a)). Here the worker's personal history is driving the plan to refer. Seeking supervision, as the Code encourages when it serves the client (2.05(a)), helps the worker manage the reaction and judge whether effective service can continue. It beats B because referral may become right later, but making it first ends care that is helping.

**B** **Incorrect** (trap: Hands off too soon). Referral may be appropriate if, after supervision, the worker cannot serve the client effectively. Making it first, because of the worker's reaction to the client's views, ends treatment that is helping the client's anxiety.

**C** **Incorrect** (trap: Boundary slip or collusion). Describing the worker's personal reaction shifts the session to the worker's needs and burdens the client. Any self-disclosure should serve the client's goals, which this does not.

**D** **Incorrect** (trap: Too directive). Arguing with the client's political views is not part of the anxiety treatment the client sought and risks a rupture. The worker's values conflict is the worker's to manage, not the client's to fix.

**Takeaway:** A values conflict alone does not justify ending or transferring services; take the reaction to supervision before changing the plan.

**Sources:** NASW Code of Ethics (2021), 1.17(b); NASW Code of Ethics (2021), 1.16(a); NASW Code of Ethics (2021), 2.05(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 292. ETH-1557 (rev 1) · Best answer: A

A private practice client owes for 6 sessions despite a written fee agreement and two talks about the balance. The social worker had planned to end services for nonpayment at the end of this month. Today the client reports thoughts of suicide most days since losing a job last week. How should the social worker handle the planned termination?

**A.** Postpone ending services while the risk is addressed
**B.** End services as planned and provide crisis line numbers
**C.** Refer the client to a sliding-scale clinic's waitlist

**Why each option is right or wrong**

**A** **Correct.** In fee-for-service settings, the Code allows ending services for nonpayment only when the financial terms were clear, the consequences were discussed, and the client poses no imminent danger to self or others (NASW Code 1.17(c)). This client's new, frequent suicidal thoughts mean the worker cannot conclude that no danger exists, so termination waits while risk is assessed and treated. It beats C because a waitlist leaves a client at heightened risk with no current provider; a transfer can follow once the client is stable and a new provider accepts the case (1.17(b)).

**B** **Incorrect** (trap: Ignores risk). The fee terms were clear, but ending care for a client who just disclosed frequent suicidal thoughts ignores the Code's danger condition. Crisis numbers do not replace an ongoing treatment relationship at a time of heightened risk.

**C** **Incorrect** (trap: Hands off too soon). A low-fee referral is a sensible step once the client is stable. Placing a client with new suicidal thoughts on a waitlist leaves them without care when risk is highest.

**Takeaway:** Ending services for nonpayment requires clear terms, discussed consequences and no imminent danger; new suicide risk postpones termination.

**Sources:** NASW Code of Ethics (2021), 1.17(c); NASW Code of Ethics (2021), 1.17(b)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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### 293. ETH-1560 (rev 2) · Best answer: C

After a client shouts during a video session, the social worker sends an email that evening saying services are over. The client still has active panic symptoms. The email offers no referral and no final session. Which term **MOST** likely describes the social worker's action?

**A.** Interruption of services
**B.** Boundary violation
**C.** Client abandonment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Interruption refers to a disruption caused by circumstances such as the worker's illness, relocation, unavailability or a technology failure, which the Code asks workers to manage by ensuring continuity (NASW Code 1.15). Here the worker made a deliberate decision to end care.

**B** **Incorrect** (trap: Wrong model or concept). A boundary violation is a harmful dual relationship or blurring of roles that exploits the client. Ending care abruptly is harmful, but the problem is leaving a client without services, not blurring roles.

**C** **Correct.** Abandonment means ending services a client still needs without adequate notice, transition or help arranging continued care (NASW Code 1.17(b)). Here a client with active symptoms is cut off by email, with no referral and no closing session. An outburst may call for a safety review, but it does not excuse ending care this way. It beats A because an interruption of services is a disruption caused by circumstances such as illness or relocation (1.15); this ending was the worker's deliberate choice.

**Takeaway:** Cutting off a client who still needs help, with no notice, transition or referral, is abandonment, whatever prompted it.

**Sources:** NASW Code of Ethics (2021), 1.17(b); NASW Code of Ethics (2021), 1.15
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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## 294. ETH-1563 (rev 2) · Best answer: B

A senior center's counseling program for older adults will lose its grant in 60 days. The board has appealed the funder's decision, and a ruling is expected in about 6 weeks. Program staff worry that telling clients now will cause some to drop out early. What is the **BEST** way for the social worker to proceed with current clients?

**A.** Wait for the appeal ruling before saying anything to clients
**B.** Tell clients now and begin planning options with each one
**C.** Give every client a list of community agencies at the last session

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Waiting avoids alarming clients if the grant is restored, but it gambles their continuity on the appeal. If the ruling is negative, about 2 weeks would remain for transfers, which is not prompt notice.

**B** **Correct.** Social workers who anticipate the termination or interruption of services notify clients promptly and seek transfer, referral or continuation based on each client's needs and preferences (NASW Code 1.17(e)). With 60 days left, telling clients now, including that an appeal is pending, gives each one time to plan. Early notice also protects continuity of care for clients who will need time to find and reach new providers (1.15). It beats A because a ruling 2 weeks before closure would leave too little time for orderly transfers if the appeal fails; honest notice about uncertainty beats a late surprise.

**C** **Incorrect** (trap: Misuses the technique). A generic list handed out at the final session is a late, one-size referral that ignores each client's needs and preferences. Transfer planning should be individual and start well before services end.

**Takeaway:** When a program ending is anticipated, give prompt, honest notice and plan transitions individually, even while the outcome is uncertain.

**Sources:** NASW Code of Ethics (2021), 1.17(e); NASW Code of Ethics (2021), 1.15
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 295. ETH-1567 (rev 1) · Best answer: C

What is the **MOST** appropriate way for a social worker in solo private practice to prepare for sudden death or incapacity?

**A.** Ask a family member to contact clients if needed
**B.** Tell clients at intake to seek another provider if needed
**C.** Name a colleague in a written plan to notify and refer clients

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Family members are not bound by professional confidentiality and may not know how to handle records or referrals. Having them contact clients risks revealing who is receiving services (NASW Code 1.07(t)).

**B** **Incorrect** (trap: Good but not best). Mentioning the possibility at intake is honest but does little when it happens. Clients would learn of the death or incapacity late, with no help to transfer and no one managing their records.

**C** **Correct.** Social workers make reasonable efforts to keep services going if they become unavailable through illness, disability or death (NASW Code 1.15). They also take precautions to protect confidentiality if they become incapacitated or die (1.07(t)). A written plan that names a qualified colleague, explains how to reach clients and records, and arranges referrals meets both duties; such a plan is often called a professional will. It beats B because a notice at intake leaves clients to find care alone in a crisis, with no one to inform them or manage their records.

**Takeaway:** Private practitioners plan ahead for death or incapacity by naming a qualified colleague to notify clients, secure records and arrange referrals.

**Sources:** NASW Code of Ethics (2021), 1.15; NASW Code of Ethics (2021), 1.07(t)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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### 296. ETH-1656 (rev 2) · Best answer: A

In the tenth session of trauma-focused work at a private practice, a client sits quietly for several minutes, then says, 'I think I'm done coming here,' and looks away. Sleep and work have improved. The previous session focused on the most painful memory so far. What should the social worker do **FIRST**?

**A.** Invite the client to say more about wanting to stop
**B.** Agree to end and plan a final session to review gains
**C.** Recommend continuing until the trauma processing is finished

**Why each option is right or wrong**

**A** **Correct.** A sudden wish to end deserves to be understood before it is acted on: it may reflect readiness, avoidance after a hard session, or a rupture the client has not named (Hepworth et al.). This remark is abrupt, follows the most painful session so far and comes with withdrawal, so inviting the client to say more comes first. A competent client may still end services (NASW Code 1.02, 1.17(a)); exploring keeps that choice informed rather than overriding it. It beats B because agreeing at once may close the door on an avoidance pattern or rupture that could be worked through.

**B** **Incorrect** (trap: Right action, wrong time). Honoring a client's decision to end, with a planned closing session, is right once the decision is clear (NASW Code 1.17(a)). Agreeing on the spot to an abrupt remark made after a painful session skips learning whether the client is ending from strength or retreating from something the work stirred up.

**C** **Incorrect** (trap: Overrides the client's choice). The worker may share an honest view once the client's reasons are known, but pressing a client to continue until the worker is satisfied overrides the client's judgment about their own care. Better sleep and work may also mean the client is ending from strength.

**Takeaway:** When a client's wish to end services is sudden or unexplained, explore it before agreeing or persuading; then honor an informed choice.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.17(a); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, termination: client-initiated endings
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 297. ETH-1669 (rev 2) · Best answer: C

A community agency will close its counseling program in 8 weeks. The director tells staff to wait until the final week to inform clients, so they do not leave early. A social worker meets with the director and explains that clients need time to plan, but the director refuses to change course. The agency's ethics committee meets in 2 weeks. What should the social worker do **NEXT**?

**A.** Tell clients now rather than wait for the committee
**B.** Follow the director's timeline and plan referrals quietly
**C.** Raise the director's timeline with the committee

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Prompt notice is the goal, and if the agency's channels fail in time, the worker may need to inform clients directly. Going around the director now skips a review that could change the policy for all clients in the program.

**B** **Incorrect** (trap: Misreads law or ethics). Planning referrals helps, but waiting until the final week leaves clients little time to process the ending or arrange care. Following an order that blocks prompt notice puts agency interests ahead of clients' needs.

**C** **Correct.** Clients facing the end of services should be told promptly so transfers can be arranged (NASW Code 1.17(e)). Social workers should not let an employer's orders interfere with ethical practice and should take reasonable steps to bring agency

practice in line with the Code (3.09(d)). The worker has already raised the issue with the director; the ethics committee is the next channel, and a meeting in 2 weeks still leaves about 6 weeks of notice if the policy changes. It beats A because acting alone against a direct order skips an available step that could fix the policy for every client.

**Takeaway:** When an employer blocks prompt notice of ending services, use the next internal channel before acting alone, keeping clients' needs central.

**Sources:** NASW Code of Ethics (2021), 1.17(e); NASW Code of Ethics (2021), 3.09(d)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 298. ETH-1675 (rev 1) · Best answer: B

A social worker at a community mental health agency learns they need surgery in 3 weeks, followed by a recovery period that could last several months. The worker carries 26 clients, several with recurrent depression, and wants to keep the medical details private. Colleagues at the agency have openings. How should the social worker **BEST** handle the coming absence with clients?

**A.** Wait to tell clients until the surgeon gives a firm medical timeline
**B.** Tell clients soon about a medical leave and review options with each
**C.** Share the diagnosis so clients understand why the worker will be away
**D.** Transfer every client to a colleague now, before the surgery date

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Waiting for an exact return date leaves clients little time to prepare and shortens the window for planning coverage. An uncertain length is itself information clients need. The notice can say the length is unknown and be updated later.

**B** **Correct.** Social workers who expect to interrupt services should notify clients promptly and seek transfer, referral or continuation based on each client's needs and preferences (NASW Code 1.17(e); 1.15). Three weeks allows time to talk with each client about interim coverage, transfer or pausing, and the worker can call it a medical leave without sharing details. It beats D because moving everyone at once ignores client preferences: some clients may want a colleague now, while others may prefer to wait, with coverage, for the worker's return.

**C** **Incorrect** (trap: Boundary slip or collusion). Clients need to know that the worker will be away and what their options are, not the worker's diagnosis. Sharing medical details shifts attention to the worker and can burden clients, especially those with depression, without improving their choices.

**D** **Incorrect** (trap: Hands off too soon). Arranging colleague coverage is part of the plan, but transferring every client now, without asking, removes the client's voice from the decision. Transfer fits when a client chooses it or when the absence becomes indefinite.

**Takeaway:** When services will be interrupted, notify clients promptly and plan continuation around each client's needs and preferences, without oversharing personal details.

**Sources:** NASW Code of Ethics (2021), 1.17(e); NASW Code of Ethics (2021), 1.15
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 299. ETH-1683 (rev 1) · Best answer: C

A client in private practice met every treatment goal for generalized anxiety 4 months ago, and symptom ratings have stayed low since. Recent sessions consist of friendly talk about the client's week. The client pays out of pocket and has explained, 'I just like having this hour.' The social worker's practice income has dropped this year. What should the social worker do **FIRST**?

**A.** Pick a date for the last session and inform the client
**B.** Continue weekly sessions, since the client values them
**C.** Raise ending with the client and review the gains made

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). A final date belongs in the plan, but choosing it before discussing ending with the client skips the review of gains and the client's reactions. The date should come out of that conversation.

**B** **Incorrect** (trap: Boundary slip or collusion). The client's enjoyment is real, but the sessions no longer treat anything, and the worker's lost income makes continuing a self-interested choice. Ongoing sessions would fit if new treatment goals emerged.

**C** **Correct.** Services should end when they are no longer required or no longer serve the client's needs (NASW Code 1.17(a)). Goals were met months ago, and sessions have drifted into social time. The worker's falling income is a conflict of interest that makes continuing tempting, so the worker acts on the clinical picture (1.06(a)). Termination starts with a conversation that reviews gains and plans the ending together. It beats A because setting a date first turns a shared process into an announcement.

**Takeaway:** When goals are met and sessions no longer serve treatment needs, begin planned termination, especially when the worker benefits from continuing.

**Sources:** NASW Code of Ethics (2021), 1.17(a); NASW Code of Ethics (2021), 1.06(a)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 300. ETH-1688 (rev 2) · Best answer: B

An outpatient client with bipolar I disorder is admitted to a state psychiatric hospital and is expected to stay about 6 more weeks. The client has signed a release for the inpatient team. Agency policy automatically closes cases after 30 days without contact, and day 30 is next week. What is the social worker's **MOST** appropriate action?

**A.** Close the case under the policy and mail a letter inviting return
**B.** Ask for an exception that keeps the case open through discharge
**C.** Call the client weekly to keep the case from closing

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Automatic closure fits clients who stop coming without explanation. Here the absence has a known cause and an expected end, and closing would cut the client off just before a high-risk move out of the hospital.

**B** **Correct.** Social workers make reasonable efforts to keep services going when illness interrupts them, and they work to keep agency policies from interfering with ethical practice (NASW Code 1.15; 3.09(d)). A hospitalization is not a dropout. Asking for an exception, then using the signed release to plan discharge follow-up with the hospital, keeps care continuous at a high-risk transition. It beats C because calls timed to reset the closure clock work around the policy instead of addressing it, and they press contact on a client who may be too ill to engage.

**C** **Incorrect** (trap: Good but not best). Keeping in touch during a hospital stay can help, but calls arranged to stop the closure clock serve the policy rather than the client, who may be too ill to engage. Raising the conflict openly fixes the rule for this client and others who are hospitalized.

**Takeaway:** When illness interrupts services, protect continuity: seek exceptions to rigid closure rules and coordinate the care transition rather than closing the case.

**Sources:** NASW Code of Ethics (2021), 1.15; NASW Code of Ethics (2021), 3.09(d)
**Area** I Values and Ethics · **Sub-area** I.B Ethical Service Delivery · **Topic** I.B.9 Law and ethics of ending services · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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# I.C Diversity and Social Justice

### 301. ETH-0009 (rev 1) · Best answer: C

An agency begins requiring government-issued photo identification at intake. Over the next month, a social worker notices that several unhoused clients and clients who recently immigrated leave without being served. What should the social worker do **FIRST**?

**A.** Waive the requirement for affected clients without asking approval
**B.** Refer clients without identification to another agency
**C.** Document who is turned away and raise it with agency leaders

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Quietly ignoring the policy may feel just, but it bypasses the agency's channels and leaves the barrier in place for everyone else. Social workers first try to change harmful policies through appropriate processes.

**B** **Incorrect** (trap: Hands off too soon). Referring people elsewhere accepts the barrier and pushes its cost onto the clients it excludes. It treats a structural problem as an individual one and leaves the policy unexamined.

**C** **Correct.** Anti-oppressive practice looks past individual cases to policies that block access for marginalized groups, and it holds institutions accountable (NASW Code 1.05(c)). The first step is to gather and document evidence of the policy's effect, then use agency channels to seek change (NASW Code 3.09(b)). It beats A because breaking the rule on one's own helps a few clients while leaving the policy in place, and it

exposes clients and the social worker to risk. Documented cases make a stronger case to leadership than anecdotes.

**Takeaway:** When a policy blocks access for marginalized groups, document its impact and advocate for change through organizational channels.

**Sources:** NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 3.09(b); NASW Code of Ethics (2021), 6.04(b)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 302. ETH-0115 (rev 1) · Best answer: C

An outpatient clinic closes the case of any client who misses 2 appointments in a row. A social worker reviews closure data and finds that most closed cases involve Black clients from a neighborhood whose bus route was recently cut. What is the **BEST** way for the social worker to address this pattern?

**A.** Help each affected client appeal the closure of their case
**B.** Keep affected clients' cases open without telling managers
**C.** Present the data to leaders and propose changing the rule
**D.** Complete training on implicit bias in clinical decisions

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Helping clients appeal is useful advocacy for those already harmed, and it can accompany policy work. On its own, it leaves the rule in place, so new closures keep happening along the same racial lines.

**B** **Incorrect** (trap: Misreads law or ethics). Quietly ignoring the rule protects a few clients but bypasses the agency and cannot last. It leaves the unfair policy unchanged for other workers' clients and puts the social worker in an ethics conflict with the employer.

**C** **Correct.** Anti-racist practice looks beyond individual cases to policies that produce unequal outcomes, and the Code calls on social workers to hold institutions accountable and act against discrimination (NASW Code 1.05(c), 6.04(d)). A neutral-looking rule is falling hardest on Black clients who lost transit access. Bringing the data to leaders with a proposed change addresses the cause through proper channels. It beats A because appeals help one client at a time, while the rule keeps closing new cases; the best response targets the policy itself.

**D** **Incorrect** (trap: Good but not best). Examining one's own bias is part of anti-racist practice, but the disparity here comes from a closure rule and a transit cut, not from one worker's decisions. Training alone leaves the policy unchanged.

**Takeaway:** When a neutral rule produces racially unequal outcomes, bring the evidence to agency leaders and advocate changing the rule.

**Sources:** NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 6.04(d)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 303. ETH-1559 (rev 1) · Best answer: A

A social worker chairs the quality committee of a community crisis service. A chart audit shows that Black youth receive police-involved crisis responses about twice as often as white youth with similar risk ratings. Staff say each decision followed protocol. What is the **BEST** response by the committee chair?

**A.** Lead a review of the dispatch process with staff and youth families
**B.** Require every staff member to complete implicit bias training
**C.** Gather another year of data before drawing any conclusions
**D.** Remind staff that the protocol applies equally to every youth

#### Why each option is right or wrong

**A** **Correct.** Anti-racist practice examines how institutional routines, not only individual attitudes, produce unequal outcomes. The Code calls on social workers to act against racism and to hold institutions accountable (NASW Code 1.05(b), 1.05(c); 6.04(d)). A gap that persists when every decision followed protocol points to the process itself. Reviewing dispatch criteria with staff and the families affected shares power and targets the source. It beats B because bias training addresses individual attitudes but leaves an unexamined process in place.

**B** **Incorrect** (trap: Good but not best). Implicit bias training can help individual staff and may be part of the plan. By itself it treats the gap as a matter of personal attitudes and leaves the dispatch criteria and routines that produced it unexamined.

**C** **Incorrect** (trap: Ignores risk). The audit already compares youth with similar risk ratings and shows a large gap. Waiting a year lets police-involved responses continue unequally; ongoing monitoring belongs alongside changes, not in place of them.

**D** **Incorrect** (trap: Wrong model or concept). Stressing equal application assumes that a neutral rule produces fair results. Here a protocol applied as written is yielding unequal outcomes, which is exactly what an anti-racist review must examine.

**Takeaway:** When a neutral-seeming process yields racial disparities, examine and change the process with those affected, not only individual attitudes.

**Sources:** NASW Code of Ethics (2021), 1.05(b); NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 6.04(d)
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### 304. ETH-1575 (rev 1) · Best answer: C

A social worker links each client's presenting problem to structural power, shares decisions with clients, and examines how the worker's own social position shapes the work. Which framework does this approach reflect?

**A.** Strengths perspective
**B.** Ecological systems perspective
**C.** Anti-oppressive practice
**D.** Person-centered approach

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). The strengths perspective centers clients' capacities and resources. It shares respect for clients, but it does not tie problems to structural power or ask the worker to examine their own social position, which define this description.

**B** **Incorrect** (trap: Wrong model or concept). The ecological perspective looks at person-environment fit and the exchanges between systems. It links problems to context, but it does not analyze power and oppression or require the worker to examine their own privilege.

**C** **Correct.** Anti-oppressive practice locates clients' difficulties within structures of power, works to share power with clients, and requires critical reflection on the worker's own social position (Dominelli, 2002). All three features in the description match. It beats B because the ecological perspective links problems to environmental systems but does not center power and oppression or ask workers to examine their own privilege. The Code likewise calls for critical self-reflection and recognizing clients as experts on their own culture (NASW Code 1.05(c)).

**D** **Incorrect** (trap: Wrong model or concept). The person-centered approach stresses empathy, genuineness and unconditional positive regard within the relationship. It is collaborative, but it does not connect problems to structural power or the worker's social position.

**Takeaway:** Anti-oppressive practice ties problems to structural power, shares power with clients, and requires reflection on the worker's own social position.

**Sources:** Dominelli, L. (2002). Anti-Oppressive Social Work Theory and Practice. Palgrave Macmillan, anti-oppressive practice: definition and principles; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 305. ETH-1578 (rev 1) · Best answer: A

During a child welfare team meeting, staff are deciding whether a family can keep a safety plan at home or needs a placement. A colleague says, 'Families from that housing complex never follow through.' Others nod, and the supervisor moves toward a vote. The family's record shows they have met each plan task so far. What should the social worker do **FIRST**?

**A.** Name the generalization and redirect to the case facts
**B.** Speak with the colleague privately after the meeting
**C.** Report the colleague's remark to the program director

**Why each option is right or wrong**

**A** **Correct.** Anti-racist and anti-oppressive practice calls for challenging stereotypes when they shape decisions about clients; the Code bars derogatory talk about clients and condoning discrimination (NASW Code 1.12; 4.02). Here a group stereotype is steering a placement decision in real time, and the family's own record contradicts it. Naming the generalization and returning the team to the evidence protects the family now. It beats B because a private talk later may help the colleague, but by then the vote would already rest on the stereotype.

**B** **Incorrect** (trap: Right action, wrong time). A private conversation is a respectful way to address a colleague and can follow. Waiting until after the meeting lets a stereotype shape a placement vote that is happening now.

**C** **Incorrect** (trap: Hands off too soon). Going to the director skips the chance to correct the decision in the room and bypasses a direct step. A formal report fits if the pattern continues after it has been addressed directly.

**Takeaway:** When a stereotype is shaping a decision about a client in the moment, challenge it then and redirect to the evidence.

**Sources:** NASW Code of Ethics (2021), 1.12; NASW Code of Ethics (2021), 4.02
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 306. ETH-1581 (rev 2) · Best answer: B

In supervision, a supervisee calls a client who has missed four group sessions 'noncompliant' and proposes closing the case. The client works night shifts, relies on two buses, and asked last month whether the group could meet at another time. What is the **BEST** action by the supervising social worker?

**A.** Approve closing the case under the attendance policy
**B.** Guide the supervisee to plan access options with the client
**C.** Suggest the supervisee assess the client's readiness to change

#### Why each option is right or wrong

**A** **Incorrect** (trap: Hands off too soon). Closing the case treats access barriers as noncompliance and ends service for a client who asked for a workable time. Closure under policy fits after reasonable efforts to remove barriers have failed.

**B** **Correct.** Anti-oppressive practice looks for structural barriers before labeling a client unmotivated, and the Code calls on social workers to expand choice and opportunity for oppressed and disadvantaged people (NASW Code 6.04(b)). The client works nights, depends on two buses and has already asked for another meeting time, so the absences point to access, not reluctance. Guiding the supervisee to work out schedule, format or transport options with the client addresses the real barrier and models a structural lens (Dominelli, 2002). It beats C because a readiness assessment still locates the problem in the client's motivation when the facts point to access.

**C** **Incorrect** (trap: Skips assessment). Assessing readiness is useful when ambivalence is the issue. Here the client asked for a different time, and the schedule and transport facts point to access, so this step misreads the problem.

**Takeaway:** Rule out structural barriers such as work hours and transport before labeling a client noncompliant or unmotivated.

**Sources:** NASW Code of Ethics (2021), 6.04(b); Dominelli, L. (2002). Anti-Oppressive Social Work Theory and Practice. Palgrave Macmillan, structural analysis of service barriers
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 307. ETH-1584 (rev 2) · Best answer: D

A social worker at a regional agency is asked to lead a needs assessment for a neighborhood the agency has served poorly. Past agency programs there drew few participants, and local leaders have told the agency, 'You always decide for us.' Agency leadership wants results within 6 months. What is the **BEST** way for the social worker to design the assessment?

**A.** Use a validated national survey to ensure comparable data
**B.** Analyze the agency's service records to find unmet needs
**C.** Hire an outside research firm to run neutral focus groups
**D.** Form a community advisory group to shape questions and methods

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Validated instruments add rigor and comparison, and an advisory group might choose to include one. Selecting it alone keeps the agency defining the neighborhood's needs, repeating the pattern local leaders named.

**B** **Incorrect** (trap: Good but not best). Agency records show the needs of people who already use services. They miss the many people who stayed away, who are the group the assessment most needs to reach.

**C** **Incorrect** (trap: Hands off too soon). An outside firm may seem neutral, but it moves control to another set of outsiders. People in the neighborhood still have no say in what is asked or how results are used.

**D** **Correct.** Anti-oppressive practice shares power with the people affected, and participatory approaches make community members partners in defining questions, methods and the use of results (Israel et al., 1998). Local leaders say the agency decides for them and past programs went unused, so involving the community in the design addresses both the power imbalance and the likely cause of low use. An advisory group shapes data collection rather than replacing it, so the 6-month window holds. It beats A because a national survey adds rigor but leaves decisions about what matters with the agency (NASW Code 6.02).

**Takeaway:** Anti-oppressive needs assessment shares design power with the affected community, especially where people report being decided for.

**Sources:** Israel, B. A., Schulz, A. J., Parker, E. A., & Becker, A. B. (1998). Review of community-based research: Assessing partnership approaches to improve public health. Annual Review of Public Health, 19, 173-202, whole article; NASW Code of Ethics (2021), 6.02

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### 308. ETH-1587 (rev 1) · Best answer: A

A social worker runs a reentry group at a county jail using a curriculum built around personal choices. In one session, members say that heavy policing of their neighborhoods and lack of jobs also led to their arrests. Several members stop taking part when the social worker returns to the workbook. What is the **BEST** response by the social worker?

**A.** Discuss structural factors alongside personal choices in the session
**B.** Validate the members' points, then return to the planned module
**C.** Record the concerns to share with curriculum developers later
**D.** Keep the focus on personal choices members control after release

**Why each option is right or wrong**

**A** **Correct.** Anti-oppressive practice links personal troubles to structural conditions while still supporting individual agency (Dominelli, 2002). Members have named policing and job loss, and they disengage when these are set aside. Bringing structural factors into the discussion alongside personal choices takes their experience seriously and keeps them engaged in the work. It beats B because a brief validation followed by a return to the workbook repeats the very move that led members to withdraw (NASW Code 1.05(b)).

**B** **Incorrect** (trap: Good but not best). Validation is a good start, but returning straight to the workbook signals that structural concerns don't belong in the group. Members already withdrew when that happened.

**C** **Incorrect** (trap: Right action, wrong time). Feedback to curriculum developers is worth giving and can follow. It does nothing for the members who are disengaging in the session now.

**D** **Incorrect** (trap: Wrong model or concept). Personal agency matters for reentry, and the curriculum stresses it. Focusing solely on individual choices dismisses the structural factors members named and can make their experience sound like excuse-making.

**Takeaway:** Anti-oppressive group work connects members' personal choices to the structural conditions they name rather than individualizing every problem.

**Sources:** Dominelli, L. (2002). Anti-Oppressive Social Work Theory and Practice. Palgrave Macmillan, structural and personal dimensions of oppression; NASW Code of Ethics (2021), 1.05(b)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 309. ETH-1590 (rev 1) · Best answer: D

At a domestic violence program, a social worker is safety planning with an adult client whose partner has made threats. The client says calling the police is off the table because officers injured a relative during a past call. The client names a cousin, a neighbor and a church member who would help. What is the **MOST** appropriate action by the social worker?

**A.** Encourage the client to call police in an emergency despite past harm
**B.** Explore the client's feelings about the relative's injury further
**C.** Refer the client to a program with police liaison advocates
**D.** Build the plan around the people and options the client trusts

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Police response is an option to discuss, and the client may choose it. Urging it over the client's stated, experience-based reasons overrides self-determination and makes the plan less likely to be followed.

**B** **Incorrect** (trap: Good but not best). The client has already explained the reason clearly. More exploration of the past event delays planning while a threat is active; trauma work can come later if the client wants it.

**C** **Incorrect** (trap: Hands off too soon). Referring out moves the client away from a program already able to plan with them. Police-linked advocacy also centers the option the client has ruled out.

**D** **Correct.** Anti-oppressive safety planning starts from the client's own analysis of risk, including well-founded distrust of police where communities have been harmed, and builds on supports the client identifies (NASW Code 1.02; 1.05(c)). The client has named three people who would help and a clear reason to avoid police. Building the plan around them yields a plan the client will use, while the worker can still share every emergency option. It beats A because pressing police contact overrides a competent client's informed judgment and may make the plan less likely to be used.

**Takeaway:** Safety plans build on the supports a client trusts, including community alternatives when police contact feels unsafe to the client.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 310. ETH-1593 (rev 1) · Best answer: C

A hospice social worker meets with the adult children of a Black patient with advanced cancer. The family declines a hospice admission, saying doctors 'gave up on' a relative decades ago because of race. A team member describes the family as 'in denial.' What is the **MOST** helpful response by the social worker?

**A.** Review hospice benefits and outcome data with the family
**B.** Document the family's denial and revisit hospice next week
**C.** Acknowledge the past harm and ask what would build trust

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Clear information matters and can follow once trust grows. Leading with data treats the decision as a knowledge gap and passes over the experience of racism the family just described.

**B** **Incorrect** (trap: Wrong model or concept). Labeling the family 'in denial' pathologizes a reasonable response to past discrimination. It also delays the conversation the family needs now about their experience and goals.

**C** **Correct.** Anti-racist practice recognizes that mistrust of health care among Black families often rests on real experiences of racism, not denial, and treats families as experts on their own experience (NASW Code 1.05(c)). The family names a specific past harm. Acknowledging it and asking what would help them trust the team respects that experience and keeps decisions with the family. It beats A because offering data

before addressing the mistrust answers a question the family did not ask and can feel like pressure.

**Takeaway:** Treat mistrust rooted in experiences of racism as legitimate; acknowledge it before offering information or recommendations.

**Sources:** NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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## 311. ETH-1595 (rev 3) · Best answer: B

A hospital social worker tracks discharges for a year and finds that older patients from a mostly Black zip code go to lower-rated nursing facilities more often than other patients with the same insurance. The social worker shares the data with the unit manager, who says to drop it. What is the **MOST** effective way for the social worker to address the pattern?

**A.** Steer each of the worker's own patients to higher-rated facilities
**B.** Present the findings to the quality improvement committee
**C.** File a complaint with the state civil rights office now

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Helping individual patients reach better facilities is worthwhile and can continue. As the sole response, it leaves the institutional pattern in place for patients served by other staff.

**B** **Correct.** When an agency practice disadvantages clients by race and a first internal step fails, the Code calls for working to align employer practices with ethical standards and for holding institutions accountable (NASW Code 3.09(d); 1.05(c); 6.04(d)). The social worker has solid data and the manager declined to act, so the effective step is the hospital committee charged with quality of care, which can change the pattern for every patient. It beats A because steering one's own patients helps a few people but leaves the discharge pattern in place for everyone else.

**C** **Incorrect** (trap: Right action, wrong time). An outside complaint may become appropriate if internal channels fail. The hospital's quality committee, charged with exactly this kind of pattern, has not yet seen the data, and going outside first skips a channel that can change practice directly.

**Takeaway:** If a manager dismisses evidence of racial disparity, take it to the next organizational channel rather than settling for individual workarounds.

**Sources:** NASW Code of Ethics (2021), 3.09(d); NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 6.04(d)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 312. ETH-1680 (rev 1) · Best answer: A

A social worker who manages an outpatient program reviews caseloads and finds that the program's two Black staff social workers carry nearly all of the Black clients. They are also asked to lead every diversity training, with no change in workload. One of them recently said, 'I'm running on empty.' What should the manager do **FIRST**?

**A.** Meet with the two social workers about the pattern and their workload
**B.** Spread new Black clients evenly across the whole staff
**C.** Add a stipend for the extra diversity training workload
**D.** Suggest the tired staff member use the employee assistance program

**Why each option is right or wrong**

**A** **Correct.** Social workers act to prevent discrimination in work assignments, and anti-racist practice examines how routine practices place unequal burdens by race (NASW Code 3.09(e); 1.05(c)). Before redesigning anything, the manager learns from the two social workers how the pattern formed, which assignments came from client requests and what relief they want. It beats B because spreading cases evenly could override clients who asked for a Black social worker, and it would be decided without the people carrying the burden.

**B** **Incorrect** (trap: Skips assessment). Changing how cases are assigned may be part of the fix. Done now, without knowing which assignments reflect client requests, it could take away a choice some Black clients made and ignore what the two social workers want.

**C** **Incorrect** (trap: Good but not best). Paying for added duties can be fair and may be part of the solution. On its own, it treats the uneven load as acceptable once paid and skips asking the two social workers what needs to change.

**D** **Incorrect** (trap: Hands off too soon). Offering support is kind, but sending one worker to the EAP frames a structural assignment problem as personal stress. The pattern stays in place for both workers.

**Takeaway:** When work assignments burden staff of color unequally, gather the affected staff's input before redesigning, and treat it as an organizational issue, not personal stress.

**Sources:** NASW Code of Ethics (2021), 3.09(e); NASW Code of Ethics (2021), 1.05(c)
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### 313. ETH-1685 (rev 1) · Best answer: A

After a client complains about hearing staff speak Spanish in the break room, a clinic director issues a rule that staff must speak only English at all times on the premises, including breaks. Several bilingual staff tell a social worker on the clinic's equity committee that they feel targeted. What is the social worker's **MOST** appropriate action?

**A.** Urge the director to replace the rule with one tied to client needs
**B.** Remind bilingual staff to follow the director's rule during review
**C.** File a discrimination complaint with a federal agency right away

**Why each option is right or wrong**

**A** **Correct.** Social workers act to prevent and eliminate discrimination in their organization's employment practices and hold institutions accountable (NASW Code 3.09(e); 1.05(c)). A rule requiring English at all times, breaks included, burdens staff by national origin; federal guidelines presume such blanket rules discriminatory, while narrower rules need a business necessity (29 CFR 1606.7). Raising this with the director and proposing a rule tied to real client-service needs uses the proper internal channel. It beats C because an outside complaint skips a direct route that is likely to work.

**B** **Incorrect** (trap: Misreads law or ethics). Asking staff to comply keeps a likely discriminatory rule in force and places the social worker on its side. Social workers should not condone discrimination (NASW Code 4.02). Following a narrower, justified rule could come later.

**C** **Incorrect** (trap: Skips assessment). Staff may file outside complaints, and that route becomes appropriate if internal efforts fail. Filing at once, before raising the concern with the director, skips the internal channel the committee role offers.

**Takeaway:** Blanket workplace rules that burden staff by language or national origin call for internal advocacy to replace them with narrowly justified policies.

**Sources:** NASW Code of Ethics (2021), 3.09(e); NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 4.02; EEOC Guidelines on Discrimination Because of National Origin, 29 CFR 1606.7
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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## 314. ETH-1691 (rev 1) · Best answer: C

A Black engineer in CBT for depression is the only Black member of a work team. The client has documented being passed over for two promotions and hearing a manager use a racial slur. The client's thought record reads: 'They'll never promote someone like me, so I'm worthless.' How should the social worker **BEST** approach this thought?

**A.** Challenge the promotion prediction as a fortune-telling distortion
**B.** Shift the sessions toward filing a formal discrimination complaint
**C.** Accept the bias as real and examine the self-worth conclusion
**D.** Set the thought record aside until the workplace problem resolves

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Fortune-telling is a real cognitive distortion, but this prediction rests on documented events. Treating it as faulty thinking invalidates the client's experience of racism and can rupture the alliance. Challenging predictions fits when the evidence does not support them.

**B** **Incorrect** (trap: Overrides the client's choice). Supporting a complaint may become part of the work if the client wants it, and the worker can raise the option. Steering sessions there now sets the depression aside and makes the decision for the client.

**C** **Correct.** Culturally responsive CBT separates accurate perceptions of oppression from the unhelpful conclusions a client draws from them, validating the first and working on the second (Hays, 2009). The promotions and the slur are documented, so the first half of the thought reflects real bias; the self-judgment is where cognitive work helps. This

keeps the problem located in workplace racism, not the client's thinking (NASW Code 1.05(b)). It beats A because labeling a well-founded expectation a distortion tells the client that noticing racism is the problem.

**D** **Incorrect** (trap: Good but not best). Waiting for the workplace to change leaves the self-blame untreated, even though it can be worked on now. The client's depression needs attention whether or not the bias stops.

**Takeaway:** In CBT with clients facing discrimination, validate accurate perceptions of bias and target the self-blaming conclusions drawn from them.

**Sources:** Hays, P. A. (2009). Integrating evidence-based practice, cognitive-behavior therapy, and multicultural therapy: Ten steps for culturally competent practice. Professional Psychology: Research and Practice, 40(4), 354-360, whole article; NASW Code of Ethics (2021), 1.05(b)

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 315. ETH-1695 (rev 1) · Best answer: B

Which term names the process in which oppressed people come to see how social and economic forces shape their lives, and then act together to change them?

**A.** Empowerment

**B.** Critical consciousness

**C.** Cultural humility

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Empowerment is the broader process of gaining personal, interpersonal and political power so people can improve their situations. Critical consciousness is one part of it: seeing how social structures shape personal problems.

**B** **Correct.** Critical consciousness, Freire's conscientizacao, is learning to perceive social, political and economic contradictions and to take action against oppressive conditions (Freire, 1970). It pairs awareness of structural forces with collective action, a foundation of anti-oppressive practice. It beats A because empowerment is the broader process of gaining personal, interpersonal and political power, and developing critical consciousness is one of its components (Gutierrez, 1990).

**C** **Incorrect** (trap: Wrong model or concept). Cultural humility describes a practitioner's stance of self-reflection and learning from clients. It concerns how the worker approaches difference, not how oppressed people come to analyze their own conditions.

**Takeaway:** Critical consciousness links personal problems to social structures and leads to collective action; it is a core process in empowerment and anti-oppressive practice.

**Sources:** Freire, P. (1970). Pedagogy of the Oppressed. Herder and Herder, conscientizacao (critical consciousness); Gutierrez, L. M. (1990). Working with women of color: An empowerment perspective. Social Work, 35(2), 149-153, empowerment process: critical consciousness

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.1 Practicing from anti-oppressive, anti-racist frameworks · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 316. ETH-1626 (rev 2) · Best answer: C

A community mental health agency receives 15 tablets with a year of free data service to lend to clients for video sessions. About 60 clients ask for one. Staff propose a first-come, first-served sign-up form on the agency website. The social worker on the planning group knows that many clients who lack internet at home have been missing sessions. Which way of allocating the tablets is **MOST** fair?

**A.** Offer them through the website form in sign-up order
**B.** Hold a random drawing among the clients who ask
**C.** Give priority to clients without a device or data plan
**D.** Let each worker choose the clients who seem most deserving

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). First come, first served sounds neutral, but a website form favors clients who are already online, which defeats the program's purpose. Sign-up order can be fair when every applicant has the same means to apply.

**B** **Incorrect** (trap: Good but not best). A lottery is transparent and gives each applicant an equal chance, so it is fairer than a race to sign up. Equal chances still ignore need, so tablets could go to clients who already attend video sessions without trouble.

**C** **Correct.** Distributive justice asks that scarce goods go first to those with the greatest need; Rawls's difference principle holds that inequalities are justified only when they benefit the least advantaged (Rawls, 1971). The tablets exist to close a digital gap, and clients without devices or data are the ones missing care. It beats B because a lottery treats applicants equally but would hand some tablets to clients who already have access. The Code also directs attention to differences in clients' access to electronic services (NASW Code 1.05(e)).

**D** **Incorrect** (trap: Misreads law or ethics). Personal nominations invite bias and inconsistency, and clients cannot see how decisions are made. Worker input is useful for confirming need, but it should apply shared, stated criteria rather than individual judgments about who deserves help.

**Takeaway:** When allocating scarce resources, use clear criteria that give priority to those with the greatest need, not first access or equal chance.

**Sources:** Rawls, J. (1971). A Theory of Justice. Harvard University Press, difference principle; NASW Code of Ethics (2021), 1.05(e)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 1 of 3

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### 317. ETH-1631 (rev 1) · Best answer: C

A school district gives every student the same laptop. It also gives home hotspots and extra tech support to students whose families lack internet service. Which justice principle does the added support for some students reflect?

**A.** Equality
**B.** Universal design
**C.** Equity

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Equality describes the first step, an identical laptop for every student. The hotspots go only to students who lack internet, so the added support departs from equal shares in order to meet need.

**B** **Incorrect** (trap: Wrong model or concept). Universal design builds products or settings that everyone can use from the start, such as captioned videos for all. Hotspots given only to families without service are a targeted addition, not a universal design feature.

**C** **Correct.** Equity means distributing resources according to need so that people can reach comparable outcomes; equality means giving everyone the same thing. The identical laptop is equality, and the hotspots and support for students without home internet are matched to need, which is equity. It beats A because identical laptops alone would leave unconnected students unable to use them. The Code calls for expanding opportunity with special regard for disadvantaged groups (NASW Code 6.04(b)), and social work education names equity as a core competency (CSWE, 2022).

**Takeaway:** Equality gives everyone the same; equity distributes resources by need so that people can reach comparable outcomes.

**Sources:** NASW Code of Ethics (2021), 6.04(b); Council on Social Work Education (2022). Educational Policy and Accreditation Standards, Competency 3: Engage Anti-Racism, Diversity, Equity, and Inclusion in Practice
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 318. ETH-1633 (rev 3) · Best answer: A

After a budget cut eliminated a paid peer specialist position, a recovery agency asks 3 clients in long-term recovery to serve as volunteer peer mentors for 20 hours a week each. They would do the duties the peer specialist once did. The clients are eager to help. The social worker sits on the planning committee. What is the **BEST** recommendation?

**A.** Seek funds to pay the mentors a fair wage
**B.** Accept the clients' offer to serve as volunteers
**C.** Thank the mentors with gift cards each month

**Why each option is right or wrong**

**A** **Correct.** The Code's Social Justice principle directs change efforts at poverty and toward equal access to resources and opportunity (NASW Code, Ethical Principles: Social Justice), and the Code bars taking unfair advantage of any professional relationship (1.06(b)); fair pay for work is part of economic justice. The clients would fill a staff job at 20 hours a week without pay, and their status as clients makes it hard to refuse or bargain. Proposing funding for paid roles treats their lived expertise as valuable labor. It beats B because willingness does not make unpaid replacement of a paid job fair, and the clients' reliance on the agency weakens the freedom of their offer.

**B** **Incorrect** (trap: Boundary slip or collusion). Honoring clients' wish to give back has value, and brief, optional volunteering can support recovery. Here the role replaces a paid job at 20 hours a week, and clients may feel unable to say no to the agency that serves them.

**C** **Incorrect** (trap: Good but not best). Gift cards show appreciation but fall far short of the value of a staff role, so the agency still gains a job's worth of work for little cost. Token thanks fit occasional volunteering, not ongoing staff duties.

**Takeaway:** Economic justice means fair pay for work; agencies should not fill paid roles with unpaid clients whose reliance limits their freedom to refuse.

**Sources:** NASW Code of Ethics (2021), 1.06(b); NASW Code of Ethics (2021), Ethical Principles: Social Justice
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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### 319. ETH-1637 (rev 2) · Best answer: D

A city plans a new diesel bus depot next to a low-income neighborhood. The only public hearing is held at 10 a.m. on a weekday, in English, with notice posted only on the city website. Residents say they learned of the plan after the vote. Which aspect of environmental justice does the hearing arrangement fail?

**A.** Distributive justice
**B.** Restorative justice
**C.** Intergenerational justice
**D.** Procedural justice

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Distributive justice concerns how environmental burdens and benefits are shared. Placing a diesel depot beside a low-income neighborhood may raise that concern too, but the hearing's timing, language and notice are about who could take part.

**B** **Incorrect** (trap: Wrong model or concept). Restorative justice repairs harm after it occurs by bringing affected people together around needs and obligations. Nothing here concerns repairing a past harm; the issue is exclusion from a decision.

**C** **Incorrect** (trap: Wrong model or concept). Intergenerational justice concerns duties to future generations, such as keeping air and land healthy for people not yet born. The hearing problem is about which of today's residents could take part in the decision, not about obligations to the future.

**D** **Correct.** Environmental justice has a distributive side, who bears environmental burdens, and a participatory side, who has a meaningful voice in decisions (Schlosberg, 2007). The 1991 Principles of Environmental Justice call for affected communities to take part as equal partners at every stage of decision-making (Principle 7). A weekday-morning, English-only hearing announced only online shut residents out, which is a failure of procedural justice. It beats A, the strongest competitor, because the depot's location may also be unfair, but the question asks about the hearing, which concerns participation rather than the share of burden.

**Takeaway:** Environmental justice includes fair sharing of burdens and meaningful participation; inaccessible hearings violate the participation, or procedural, side.

**Sources:** Schlosberg, D. (2007). Defining Environmental Justice: Theories, Movements, and Nature. Oxford University Press, distribution, recognition and participation; Principles of Environmental Justice (1991). First National People of Color Environmental Leadership Summit, Principle 7
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 320. ETH-1640 (rev 1) · Best answer: C

At a pediatric clinic, a social worker helps a parent whose 7-year-old's asthma keeps worsening. The apartment has visible mold and roaches. With the social worker's help, the parent sent the landlord a written repair request and a doctor's letter 6 weeks ago. The landlord has not responded and has hinted that the lease may not be renewed. What should the social worker do **NEXT**?

- **A.** Ask the doctor to strengthen the child's asthma treatment plan
- **B.** Help the parent search for a different apartment
- **C.** Connect the parent with a tenant legal aid program
- **D.** Organize other tenants to confront the landlord together

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Medical treatment matters, and the doctor has already documented the link to housing. Adjusting medicines while the child stays exposed to mold and pests treats the symptoms and leaves the environmental cause in place.

**B** **Incorrect** (trap: Good but not best). Moving can help if the family wants it, but it is costly, may not be possible, and shifts the burden onto the family. The landlord is responsible for repairs, so learning the family's rights should come before giving up the home.

**C** **Correct.** Social work's commitment to environmental justice (CSWE, 2022) means treating unhealthy housing as a structural cause of illness, not only a medical problem. The written request with a doctor's letter has failed, and the landlord's hint about the lease raises legal questions beyond the social worker's expertise. A tenant legal aid program can explain repair rights and any protection against retaliation (NASW Code 1.16(a)). It beats D because collective action may come later, but organizing before the parent knows their rights could expose the family to the retaliation they fear.

**D** **Incorrect** (trap: Right action, wrong time). Tenant organizing can win building-wide repairs and is a strong environmental justice tool. It is premature now: the parent fears losing the lease and has not chosen collective action, so legal advice comes first.

**Takeaway:** When unhealthy housing harms a client and direct requests fail, link the family to legal help on tenant rights before riskier collective steps.

**Sources:** Council on Social Work Education (2022). Educational Policy and Accreditation Standards, Competency 2: Advance Human Rights and Social, Racial, Economic, and Environmental Justice; NASW Code of Ethics (2021), 1.16(a)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 321. ETH-1643 (rev 2) · Best answer: C

A family services agency director says, 'We treat everyone the same, so we don't collect race or ethnicity data. Asking would only make clients uncomfortable.' The social worker leads the agency's quality improvement work. What is the **MOST** appropriate response?

**A.** Offer staff implicit bias training instead of collecting data
**B.** Collect race data just from clients who report unfair treatment
**C.** Propose collecting self-reported race data to find gaps

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Bias training is a common response and may help staff reflect, which makes it appealing when data collection meets resistance. Training alone cannot show whether outcomes differ by group or whether anything changed afterward; finding and tracking gaps takes data broken down by race and ethnicity.

**B** **Incorrect** (trap: Misuses the technique). Complaint-based data reflect who feels safe to speak up, not how services work for each group. Disparities in waitlists, referrals or outcomes often go unreported and only appear in agency-wide data.

**C** **Correct.** Racial justice requires seeing disparities before fixing them, and disparities in care are found by collecting quality data broken down by race, ethnicity and language (Institute of Medicine, 2009). A 'treat everyone the same' policy without data cannot show whether outcomes differ by group. Voluntary self-report respects clients' comfort because they may decline to answer. It beats B because collecting data only from clients who complain captures a biased slice and misses patterns no one reports. The Code asks social workers to hold institutions accountable for culturally responsive practice (NASW Code 1.05(c)).

**Takeaway:** Colorblind policies hide disparities; voluntary, self-reported race and ethnicity data let agencies find and correct unequal outcomes.

**Sources:** Institute of Medicine (2009). Race, Ethnicity, and Language Data: Standardization for Health Care Quality Improvement. National Academies Press, Summary; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 322. ETH-1645 (rev 2) · Best answer: D

A social worker at a housing program learns that a landlord refused to show a vacant apartment to a client after hearing the client has three children, saying the unit 'isn't right for kids.' The client is upset that other families may be turned away too, but does not want to file a complaint, saying the housing search is exhausting enough. Which response is **BEST**?

**A.** Respect the decision and focus on the housing search
**B.** File a fair housing complaint on the client's behalf
**C.** Encourage the client to reconsider filing a complaint
**D.** With permission, report the practice to a fair housing group

**Why each option is right or wrong**

- **A** **Incorrect** (trap: Good but not best). Supporting the client's search and choice is necessary, and it may be all the client wants at first. Stopping there ignores the client's stated worry about other families and misses a way to address the landlord's practice without burdening the client.
- **B** **Incorrect** (trap: Overrides the client's choice). Filing without consent overrides a competent client's decision and discloses private information. A complaint can be powerful when the client chooses it; here the client has declined.
- **C** **Incorrect** (trap: Too directive). Giving information about filing is fine, but urging the client after a clear, reasoned refusal adds pressure during a stressful search. The client can revisit the choice later.
- **D** **Correct.** Refusing to show housing because a family has children is familial status discrimination under the Fair Housing Act (42 U.S.C. 3604(a)). The client's choice not to file is respected (NASW Code 1.02), and the Code also calls for social action so that all people have equal access to the resources they need (NASW Code 6.04(a)). Sharing the landlord's practice with a fair housing organization, with the client's permission, can prompt testing or outreach without making the client a complainant. It beats A because focusing on the search alone honors autonomy but leaves a discriminatory practice in place for the next family, the very concern the client raised.

**Takeaway:** When a client declines to act on discrimination, respect the choice and, with permission, use channels that address the practice without burdening the client.

**Sources:** Fair Housing Act, 42 U.S.C. 3604(a); NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 6.04(a)

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 323. ETH-1704 (rev 2) · Best answer: A

At a family services agency, three bilingual case managers who completed the agency's interpreter training are often pulled from their own caseloads to interpret for colleagues. They carry full caseloads with no added pay, and one was marked down for late notes. They say they want to keep interpreting for families they know but need the work recognized. The social worker chairs the equity committee. What is the **BEST** action?

- **A.** Propose crediting interpreting time in workload and pay
- **B.** Ask the supervisor to revise the late-notes rating
- **C.** Arrange contract interpreters so bilingual staff stop interpreting
- **D.** Encourage the case managers to file a grievance themselves

**Why each option is right or wrong**

**A** **Correct.** Social workers act to prevent and eliminate discrimination in an employing organization's work assignments (NASW Code 3.09(e)) and challenge social injustice, including discrimination (NASW Code, Ethical Principles: Social Justice). Here bilingual staff carry unpaid extra duties and are penalized for the delays those duties cause, an inequity built into the workload system. Because the case managers want to keep interpreting, crediting that time in caseload size and pay corrects the inequity while keeping families' access to trusted interpreters. It beats C because contract interpreters would end the burden only by overriding the staff's stated wish.

**B** **Incorrect** (trap: Good but not best). The rating is unfair and should be revisited, but changing one evaluation fixes a symptom. The workload system that keeps adding unpaid interpreting would produce the same result again.

**C** **Incorrect** (trap: Good but not best). Contract interpreters are a sound choice when staff do not want the role or lack interpreter training. These case managers are trained and want to keep interpreting, so removing the duty sidesteps their preference rather than correcting the pay and workload gap.

**D** **Incorrect** (trap: Hands off too soon). Staff can file grievances, but the social worker chairs the committee charged with equity issues. Sending the problem back to the affected staff shifts the burden onto them when the worker has a direct channel to propose a fix.

**Takeaway:** When some staff carry unpaid extra duties because of their language or identity, advocate to recognize that work in workload and pay.

**Sources:** NASW Code of Ethics (2021), 3.09(e); NASW Code of Ethics (2021), Ethical Principles: Social Justice
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 324. ETH-1710 (rev 1) · Best answer: B

Which policy approach sets one shared goal for an entire population and then designs different strategies for each group based on the barriers it faces?

**A.** Universal provision
**B.** Targeted universalism
**C.** Means-tested targeting

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Universal provision offers the same program to everyone. It shares the population-wide reach but not the tailored strategies, so groups facing greater barriers may still fall short of the goal.

**B** **Correct.** Targeted universalism sets a universal goal, then uses targeted strategies matched to how different groups are situated so that all can reach it (powell, Menendian & Ake, 2019). The two features in the question, one shared goal and group-specific strategies, both point here. It beats C because means-tested programs direct help only to people below an income line and set no goal for the whole population.

**C** **Incorrect** (trap: Wrong model or concept). Means-tested programs serve only people who pass an income or asset test. They target help, but they lack the shared, population-wide goal that defines this approach.

**Takeaway:** Targeted universalism pairs one goal for everyone with strategies tailored to each group's situation, so differently placed groups can all reach it.

**Sources:** powell, j. a., Menendian, S., & Ake, W. (2019). Targeted Universalism: Policy & Practice. Haas Institute for a Fair and Inclusive Society, Defining targeted universalism

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 325. ETH-1718 (rev 3) · Best answer: A

A client works 38 hours a week at two jobs, supports two children, and falls behind on rent each winter. The social worker and client have reviewed the household budget together and found spending limited to basics. The client has never filed a tax return because the income seemed too low to matter. Which step is **MOST** helpful?

**A.** Help the client file to claim refundable tax credits

**B.** Help the client apply for emergency rental assistance

**C.** Review the client's spending for further savings

#### Why each option is right or wrong

**A** **Correct.** The Code's Social Justice principle aims change efforts at poverty and other structural injustice, not only personal habits (NASW Code, Ethical Principles: Social Justice). Low-wage working families who do not file can miss the refundable earned income tax credit, which pays out even when no tax is owed (26 U.S.C. 32). The budget review has shown that spending is limited to basics, so the gap is income. Helping the client file, for example through free tax preparation, can bring a sizable yearly payment. It beats B because emergency rental aid covers one shortfall, while refundable credits add income every year the client files, which addresses a gap that returns each winter.

**B** **Incorrect** (trap: Good but not best). Emergency rental aid can prevent an eviction and fits when the client faces one now. It covers a single shortfall, though, and the gap returns every winter because basics exceed income, so it leaves the recurring cause in place.

**C** **Incorrect** (trap: Right action, wrong time). A spending review was the right first step, and it is done. Repeating it adds little when basics alone exceed income, and it can signal blame for a structural problem.

**Takeaway:** When a budget review shows basics exceed income, pursue income supports such as unclaimed tax credits rather than personal spending changes.

**Sources:** Internal Revenue Code, 26 U.S.C. 32 (earned income tax credit); NASW Code of Ethics (2021), Ethical Principles: Social Justice

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.2 Principles of social, racial, environmental, economic justice · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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### 326. ETH-1600 (rev 1) · Best answer: C

A 34-year-old bisexual client in a small town meets with a social worker about low mood and poor sleep. The client keeps a same-sex relationship hidden from coworkers, plans each conversation to avoid slips, and expects to be shunned if found out. The client adds, 'Part of me still thinks something is wrong with me.' Which concept **MOST** likely explains this pattern?

**A.** Acculturative stress
**B.** Stereotype threat
**C.** Minority stress
**D.** Identity foreclosure

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Acculturative stress comes from adapting to a new culture after migration, such as language barriers and clashing values. Nothing here involves migration; the strain comes from stigma about sexual orientation in the client's own community.

**B** **Incorrect** (trap: Wrong model or concept). Stereotype threat is worry about confirming a negative group stereotype, which can lower performance on a task in the moment. It would fit a test or job review; it does not explain ongoing concealment, expected rejection and self-stigma.

**C** **Correct.** The minority stress model holds that stigma adds chronic strain for sexual minority people through outside events and through internal processes: expecting rejection, concealing one's identity and internalized stigma (Meyer, 2003). This client shows all three: constant vigilance to keep the relationship hidden, expected shunning, and the belief that 'something is wrong with me,' along with low mood. It beats B because stereotype threat is a short-lived performance effect when a stereotype becomes salient, not a lasting pattern of concealment and self-stigma.

**D** **Incorrect** (trap: Wrong model or concept). Identity foreclosure means committing to an identity, often one handed down by family, without exploring alternatives. This client has a clear sense of orientation; the struggle is the cost of others' stigma, not a lack of exploration.

**Takeaway:** Hiding identity, expecting rejection and internalized stigma are core minority stress processes that can harm sexual minority clients' mental health.

**Sources:** Meyer, I. H. (2003). Prejudice, social stress, and mental health in lesbian, gay, and bisexual populations: Conceptual issues and research evidence. Psychological Bulletin, 129(5), 674-697, whole article
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

### 327. ETH-1602 (rev 1) · Best answer: A

A 79-year-old resident of an assisted living facility tells the social worker that the 'friend' who visits daily is the resident's same-sex partner of 35 years. Staff have left the partner out of care-planning meetings because the partner is not family. The resident says, 'When I was young, people lost their jobs over this.' What should the social worker do **FIRST**?

**A.** Ask the resident how to include the partner and what to share
**B.** Tell staff about the relationship so the partner is included
**C.** Help the resident name the partner as health care agent
**D.** Encourage the resident to tell staff for better support

**Why each option is right or wrong**

**A** **Correct.** Clients control disclosure of their sexual orientation, and many older lesbian and gay adults learned to hide relationships after decades of legal and social penalties. The resident has shared the relationship with the social worker but has not said what staff should know or what role the partner should have. Learning those wishes comes before any action (NASW Code 1.02; 1.05(d)). It beats C because naming a health care agent may be a sound next step, but choosing it now assumes the resident's goals and leaves the exclusion from care planning unaddressed.

**B** **Incorrect** (trap: Misreads law or ethics). Including the partner is the right goal, but telling staff about the relationship without the resident's consent takes a disclosure decision away from the resident. Advocacy with staff fits once the resident decides what may be shared.

**C** **Incorrect** (trap: Right action, wrong time). Naming the partner as health care agent can protect the couple's role and is often worth raising. Choosing it first presumes the resident's goals; the social worker should first learn how the resident wants the partner included.

**D** **Incorrect** (trap: Too directive). Coming out may bring support, but urging it imposes the social worker's view on a resident whose caution comes from real past penalties. The resident decides whether and when to disclose.

**Takeaway:** Before advocating for an LGBT client's partner, learn the client's wishes; the client controls whether and how their identity is shared.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.05(d)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.3 Culture, race, gender, sexuality, intersectionality shaping identity and behavior · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 328. ETH-1605 (rev 2) · Best answer: B

A 27-year-old client in private practice says, 'I'm gay, but my faith is everything to me. I want you to help me get rid of these same-sex attractions.' The client reports guilt and growing isolation from both church friends and gay friends. What is the **MOST** appropriate response by the social worker?

**A.** Agree to work with the client on reducing same-sex attractions
**B.** Decline change work and offer to explore faith and identity
**C.** Encourage the client to leave the faith community for now

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Self-determination is a core value, but it does not require providing a practice the profession opposes as ineffective and potentially harmful. Agreeing would also reinforce the client's belief that the orientation itself is the problem.

**B** **Correct.** NASW opposes sexual orientation change efforts, including referral to them, and an APA task force found no reliable evidence that they work, with reports of harm (NASW, 2015; APA, 2009). Self-determination does not oblige a social worker to provide a harmful practice. The affirming response is to say so honestly and offer help with the real distress: the conflict between faith and identity, guilt and isolation. It beats C because steering the client away from the congregation imposes the worker's values on a client who calls faith central (NASW Code 1.05(c)).

**C** **Incorrect** (trap: Too directive). Some clients do find new communities, but this client has said faith is central. Urging distance from the congregation puts the worker's values in place of the client's and could deepen isolation.

**Takeaway:** Decline sexual orientation change efforts even when requested; help the client work through faith-identity conflict without imposing an outcome.

**Sources:** NASW (2015). Sexual Orientation Change Efforts (SOCE) and Conversion Therapy with Lesbians, Gay Men, Bisexuals, and Transgender Persons (position statement), whole statement; American Psychological Association (2009). Report of the American Psychological Association Task Force on Appropriate Therapeutic Responses to Sexual Orientation, conclusions and recommendations; NASW Code of Ethics (2021), 1.05(c)
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### 329. ETH-1608 (rev 1) · Best answer: C

A college student meets with a campus social worker about stress over choosing a major. The student wants to study art, but the parents favor nursing. The student says, 'In my family, big decisions are made together. Going against my parents would bring shame on all of us, and I don't want that.' Which approach is **BEST**?

**A.** Help the student separate personal wishes from parents' wishes
**B.** Invite the family to a session to negotiate the choice
**C.** Help the student weigh options in light of family values

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Separating one's own wishes from others' is central to some Western models of maturity. Here it treats the student's stated value as a problem to fix and pushes an individualist goal the student did not choose.

**B** **Incorrect** (trap: Right action, wrong time). A joint session could help if the student wants it. Arranging it now skips the student's own thinking about how to approach the parents and may feel like pressure from the social worker.

**C** **Correct.** Many people hold an interdependent view of the self, in which identity and good decisions are rooted in relationships and family harmony, not only personal preference (Markus & Kitayama, 1991). The student states this value and does not want to bring shame on the family. Working within that frame, such as exploring paths

that honor both art and family, respects the client as the expert on their own culture (NASW Code 1.05(c)). It beats B because a family meeting may help later, but the student has not asked for one.

**Takeaway:** Treat family-centered decision-making as part of the client's cultural view of self, not dependence; help clients choose within their own values.

**Sources:** Markus, H. R., & Kitayama, S. (1991). Culture and the self: Implications for cognition, emotion, and motivation. Psychological Review, 98(2), 224-253, whole article; NASW Code of Ethics (2021), 1.05(c)
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### 330. ETH-1610 (rev 1) · Best answer: B

Six months ago, a store guard falsely accused a 16-year-old Black student of theft. Since then, the student, who attends a mostly white high school, has joined the Black Student Union, reads Black history late into the night, wears African-print clothing, and calls longtime white friends 'part of the problem.' The parent asks the school social worker what is happening. In Cross's model, which stage **MOST** likely fits?

**A.** Encounter
**B.** Immersion-Emersion
**C.** Internalization
**D.** Pre-encounter

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Encounter is the event or realization that shatters a person's earlier view of race, here the false accusation. The student has moved past that moment into active, intense identity work.

**B** **Correct.** In Cross's model of Black identity development, the Immersion-Emersion stage follows a racial encounter and brings intense engagement with Black culture and history, often with anger at or withdrawal from white people (Cross, 1991). The student's new group, reading, dress and sharp rejection of white friends fit this stage. It beats A because the false accusation 6 months ago was the encounter, the event that set off the change; the student's current behavior shows what followed. Seeing a developmental stage helps the social worker reassure the parent and support the student.

**C** **Incorrect** (trap: Wrong model or concept). Internalization brings a secure, settled Black identity with less anger and more openness to relationships across race. Calling longtime friends 'part of the problem' shows the student has not reached this calmer stage.

**D** **Incorrect** (trap: Wrong model or concept). Pre-encounter describes the period before race is salient, when a person may adopt the dominant culture's views or downplay race. The student's identity is now strongly focused on race.

**Takeaway:** Intense engagement with one's racial group and anger at the dominant group after a racial encounter fits Cross's Immersion-Emersion stage.

**Sources:** Cross, W. E., Jr. (1991). Shades of Black: Diversity in African-American Identity. Temple University Press, nigrescence stages

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### 331. ETH-1613 (rev 1) · Best answer: A

In a rural primary care clinic, a 52-year-old male farmer is referred to the social worker after telling the physician he is 'just worn out.' He has been irritable, drinking more beer at night and snapping at his family. When the social worker mentions depression, he says, 'I'm not some weak guy who needs to cry about it.' Which response is **MOST** helpful?

**A.** Talk about the problems in his terms, such as stress and sleep
**B.** Explain that depression is a medical illness like any other
**C.** Refer him to a substance use program for the drinking

#### Why each option is right or wrong

**A** **Correct.** Traditional masculine norms of self-reliance and emotional control shape how many men experience and show distress, often as irritability, drinking or fatigue, and make labels like 'depression' feel threatening (Addis & Mahalik, 2003). Starting with the client's own words, such as being worn out, sleep and stress, lowers that barrier and builds engagement while assessment continues. It beats B because correcting his view, though accurate, argues with him at the first sign of reluctance and can end engagement before it starts.

**B** **Incorrect** (trap: Too directive). Teaching about depression can reduce stigma later. Offered right after he rejects the label, it argues against his view of himself and may increase defensiveness.

**C** **Incorrect** (trap: Hands off too soon). The increased drinking needs assessment, and a substance use referral may follow. Referring now, before engaging him or assessing the pattern, hands off a client who has not yet agreed there is a problem.

**Takeaway:** Gender norms shape how distress is shown and named; engage clients in their own terms rather than pressing a label they reject.

**Sources:** Addis, M. E., & Mahalik, J. R. (2003). Men, masculinity, and the contexts of help seeking. American Psychologist, 58(1), 5-14, whole article

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### 332. ETH-1615 (rev 2) · Best answer: D

Which term says that race, gender, class and other identities combine to shape unique experiences of privilege and oppression?

**A.** Double jeopardy
**B.** Cultural pluralism
**C.** Minority stress
**D.** Intersectionality

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Double jeopardy describes facing two forms of discrimination at once, treated as additive. Intersectionality goes further, holding that combined identities create distinct experiences that adding single categories cannot capture.

**B** **Incorrect** (trap: Wrong model or concept). Cultural pluralism describes a society in which distinct cultural groups keep their identities while living together. It concerns how groups coexist, not how one person's identities interact.

**C** **Incorrect** (trap: Wrong model or concept). Minority stress refers to chronic strain from stigma tied to a minority identity, such as expecting rejection. It addresses the effects of a stigmatized identity, not how several identities combine.

**D** **Correct.** Intersectionality, a term introduced by legal scholar Kimberlé Crenshaw, holds that identities such as race, gender and class interact, so a Black woman's experience cannot be understood by adding race and gender separately (Crenshaw, 1989). The Code asks social workers to understand oppression across these dimensions (NASW Code 1.05(d)). It beats A because double jeopardy describes stacked, additive disadvantage, which is the single-axis sum that intersectionality says misses how identities shape each other.

**Takeaway:** Intersectionality: interacting identities create distinct experiences of privilege and oppression; assess them together, not one category at a time.

**Sources:** Crenshaw, K. (1989). Demarginalizing the intersection of race and sex: A Black feminist critique of antidiscrimination doctrine, feminist theory and antiracist politics. University of Chicago Legal Forum, 1989(1), 139-167, whole article; NASW Code of Ethics (2021), 1.05(d)
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## 333. ETH-1617 (rev 1) · Best answer: A

A 15-year-old came out as gay to the parents a month ago and is now withdrawn at home; a screening last week found no suicidal thoughts. In a family session, the parents say, 'We love our child, but our faith teaches this is wrong. We won't let these friends visit, and we pray for a change.' The teen looks down and says nothing. Which approach is **BEST**?

**A.** Show how specific accepting behaviors protect the teen's health
**B.** Ask the parents to put their religious beliefs aside for the teen's sake
**C.** See the teen individually and pause the family sessions

**Why each option is right or wrong**

**A** **Correct.** Family responses to a youth's sexual orientation shape the youth's well-being: young adults who recalled high family rejection in adolescence reported far higher rates of suicide attempts and depression (Ryan et al., 2009). These parents express love alongside rejecting behaviors, such as banning friends and praying for change. Helping them see how specific behaviors affect their child, and that support can coexist with faith, works with their love rather than against their beliefs (NASW Code 1.05(c)). It beats B because demanding they drop their faith sets up a contest that is likely to end engagement.

**B** **Incorrect** (trap: Too directive). The parents' religious values are part of their identity, and asking them to abandon those values sets up a contest the social worker cannot win. Change is more likely when the focus is on specific behaviors.

**C** **Incorrect** (trap: Good but not best). Individual support for the teen may be needed alongside family work. Pausing family sessions gives up the chance to change the rejecting behaviors at home, which drive much of the risk.

**Takeaway:** When families reject a youth's sexual orientation, link specific accepting or rejecting behaviors to the youth's health without demanding they abandon beliefs.

**Sources:** Ryan, C., Huebner, D., Diaz, R. M., & Sanchez, J. (2009). Family rejection as a predictor of negative health outcomes in white and Latino lesbian, gay, and bisexual young adults. Pediatrics, 123(1), 346-352, whole article; NASW Code of Ethics (2021), 1.05(c)

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### 334. ETH-1620 (rev 1) · Best answer: C

In a pediatric clinic, a Black parent tells the social worker about recent talks with an 11-year-old. The parent explained that some people may treat the child unfairly because of race, then practiced with the child how to stay calm and respond if stopped by police or followed in a store. Which ethnic-racial socialization practice is the parent **MOST** likely using?

**A.** Promotion of mistrust
**B.** Cultural socialization
**C.** Preparation for bias
**D.** Egalitarianism

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Promotion of mistrust emphasizes caution and distrust toward other racial groups and lacks coping guidance. This parent teaches concrete responses to unfair treatment, which marks a different practice.

**B** **Incorrect** (trap: Wrong model or concept). Cultural socialization teaches children about their heritage, history and traditions to build pride. The parent's talks center on unfair treatment and how to respond, not on heritage or cultural practices.

**C** **Correct.** Preparation for bias refers to parents making children aware of discrimination and teaching ways to cope with it (Hughes et al., 2006). This parent names possible unfair treatment because of race and rehearses calm, practical responses to a police stop or store surveillance. It beats A because promotion of mistrust stresses wariness of other groups without coping strategies, while this parent focuses on skills for handling unfair treatment safely. Recognizing the practice helps the social worker see protective parenting rather than fearfulness.

**D** **Incorrect** (trap: Wrong model or concept). Egalitarianism emphasizes individual qualities and equality across groups, often downplaying race. This parent's message centers race and how others may respond to it.

**Takeaway:** Teaching children that unfair treatment may happen because of race, and how to cope with it, is preparation for bias, a protective practice.

**Sources:** Hughes, D., Rodriguez, J., Smith, E. P., Johnson, D. J., Stevenson, H. C., & Spicer, P. (2006). Parents' ethnic-racial socialization practices: A review of research and directions for future study. Developmental Psychology, 42(5), 747-770, whole article
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### 335. ETH-1700 (rev 1) · Best answer: C

A 19-year-old college student tells a campus social worker, 'I'm probably gay, though part of me wishes I weren't.' The student has started going to a campus LGBTQ meeting, mainly to feel less alone, and finds some members welcoming and others distant. The student has told no one else. Using Cass's model, which stage **MOST** likely fits this student?

**A.** Identity comparison
**B.** Identity acceptance
**C.** Identity tolerance
**D.** Identity pride

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). In identity comparison the person first considers that they might be gay, often feels alienated, and may keep a heterosexual image or treat the feelings as temporary. This student has moved past 'might' to 'probably' and is already seeking contact with other LGBTQ people.

**B** **Incorrect** (trap: Wrong model or concept). Identity acceptance brings a clear 'I am gay,' more positive feelings, growing contact with the community and selective disclosure. This student still voices ambivalence and has told no one, so full acceptance has not yet arrived.

**C** **Correct.** Cass (1979) described identity tolerance as the stage when a person concludes they are probably gay and seeks out other gay people, largely to ease isolation, while still holding mixed feelings about the identity. The student's 'probably,' the wish that it were otherwise, joining a group mainly to feel less alone and telling no one else all fit. It beats B because acceptance involves a settled 'I am' and some disclosure, which this student has not reached. Stage models describe common patterns, and individuals may not move through them in order.

**D** **Incorrect** (trap: Wrong model or concept). Identity pride involves strong immersion in LGBTQ community, wide disclosure and often anger toward a heterosexual world. This student is still uncertain and private, the opposite of that stage's open, us-versus-them stance.

**Takeaway:** In Cass's model, identity tolerance means concluding one is probably gay and seeking LGBTQ contact to ease isolation, while ambivalence remains.

**Sources:** Cass, V. C. (1979). Homosexual identity formation: A theoretical model. Journal of Homosexuality, 4(3), 219-235, stages 2-5: comparison, tolerance, acceptance, pride
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### 336. ETH-1706 (rev 1) · Best answer: B

In a children's hospital, Deaf parents who use American Sign Language learn that their 10-month-old has profound hearing loss. After a full discussion with the audiologist, they decline cochlear implant surgery. They plan to raise the child in ASL within the Deaf community and have joined a family sign program. The surgeon asks the social worker to 'help them see what is best' and mentions neglect. What is the **MOST** appropriate response?

**A.** Encourage the parents to reconsider, citing spoken-language outcomes
**B.** Support the parents' decision and their plan for early ASL
**C.** Request an ethics consultation to resolve the disagreement
**D.** File a medical neglect report with the child welfare agency

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). Parents deserve accurate information about implant outcomes, and the audiologist has already provided it. Pressing them to change an informed choice treats the hearing world's norm as the only good outcome and dismisses the family's culture.

**B** **Correct.** Many Deaf people experience deafness as a cultural and linguistic identity, and ASL is a full language that deaf children acquire on a typical timeline when exposed early (Humphries et al., 2012). These parents made an informed choice and have already arranged early ASL exposure, which protects the child from language deprivation. Respecting culture as a strength and the family's informed decision follows NASW Code 1.05(a) and 1.02. It beats C because an ethics consultation fits a real impasse over a child's welfare; here the plan gives the child full early language access, so no override is in question.

**C** **Incorrect** (trap: Hands off too soon). Ethics consultation helps when a team and family face a true impasse over a child's welfare. The parents' plan provides full early language access through ASL, so no override is in question, and referring the matter signals that the family's choice is suspect.

**D** **Incorrect** (trap: Misreads law or ethics). Medical neglect involves failing to provide care needed to prevent serious harm. Cochlear implants are elective, and a plan for early, full exposure to ASL addresses language development, so a report would not fit the usual threshold and would damage trust.

**Takeaway:** Deafness can be a cultural and linguistic identity; support informed parental choices that secure early, full language access rather than pressing medical norms.

**Sources:** Humphries, T., Kushalnagar, P., Mathur, G., Napoli, D. J., Padden, C., Rathmann, C., & Smith, S. R. (2012). Language acquisition for deaf children: Reducing the harms of zero tolerance to the use of alternative approaches. Harm Reduction Journal, 9, 16, whole article; NASW Code of Ethics (2021), 1.05(a); NASW Code of Ethics (2021), 1.02

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### 337. ETH-1711 (rev 2) · Best answer: C

A 26-year-old gay Muslim client tells a social worker that the local LGBTQ center's events, often held in bars, feel foreign to their faith, while sermons at their mosque make them hide their sexuality. The client says clearly that they want to keep both their faith and their relationships, and asks where they might find belonging. The social worker had suggested the center's support group. What should the social worker do **NEXT**?

**A.** Urge the client to give the LGBTQ group a few more tries
**B.** Explore whether the client's faith reflects internalized homophobia
**C.** Help the client find LGBTQ groups within their faith, including online

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). The suggestion was reasonable at first, but the client has explained why that space feels foreign to their faith. Pressing it again asks the client to set aside one identity to fit a service.

**B** **Incorrect** (trap: Too directive). Exploring internalized stigma can help a client who rejects their own sexuality. This client wants both faith and relationships, so treating religious commitment as a possible symptom misreads the problem and can harm trust.

**C** **Correct.** Intersectionality holds that identities combine to create distinct experiences that cannot be served one identity at a time (Crenshaw, 1989; NASW Code 1.05(d)). The client wants to keep both faith and relationships and has explained why each single-identity space falls short. Linking the client with LGBTQ Muslim communities, including online networks, meets the stated goal of belonging as a whole person. It beats B because the client has described a mismatch of settings, not self-rejection, and probing for internalized homophobia would cast the client's faith as a symptom.

**Takeaway:** Serve clients with intersecting identities as whole people; when single-identity services fall short, seek communities that honor those identities together.

**Sources:** Crenshaw, K. (1989). Demarginalizing the intersection of race and sex: A Black feminist critique of antidiscrimination doctrine, feminist theory and antiracist politics. University of Chicago Legal Forum, 1989(1), 139-167, whole article; NASW Code of Ethics (2021), 1.05(d)
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### 338. ETH-1715 (rev 1) · Best answer: C

A 15-year-old with one Black parent and one white parent tells the school social worker that classmates keep saying the teen 'has to pick a side.' The teen calls themself mixed and feels close to both families' traditions. A teacher has suggested the teen would fit in better by identifying as Black, since that is how others see them. What is the **MOST** appropriate response?

**A.** Encourage the teen to identify as others perceive them
**B.** Explore which side of the family feels most like home
**C.** Affirm the teen's right to define their own identity

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Others' perceptions shape the bias a multiracial teen may face, and talking about that can help. Telling the teen to adopt that label lets outsiders define identity and dismisses the teen's own sense of self.

**B** **Incorrect** (trap: Misuses the technique). Identity exploration is valuable, but this question assumes the teen must choose one family's side, the same either-or framing the classmates impose. The teen has already said they feel close to both.

**C** **Correct.** Multiracial people have the right to name their own identity, which may differ from how others see them and may shift across settings (Root, 1996). The teen already describes an identity rooted in both families, so the worker's role is to affirm that self-definition and help the teen respond to pressure, recognizing clients as experts on their own culture (NASW Code 1.05(c)). It beats A because how others perceive the teen matters for preparing for bias, but it does not decide who the teen is; urging a single label repeats the classmates' pressure.

**Takeaway:** Multiracial clients may define their own identity, even when it differs from how others see them; affirm it rather than urging one label.

**Sources:** Root, M. P. P. (Ed.). (1996). The Multiracial Experience: Racial Borders as the New Frontier. Sage Publications, A bill of rights for racially mixed people; NASW Code of Ethics (2021), 1.05(c)
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### 339. ETH-1719 (rev 2) · Best answer: A

In a primary care clinic, a client with type 2 diabetes who uses insulin mentions a plan to fast during Ramadan next month. The physician, concerned about low blood sugar, asks the social worker to 'talk them out of it.' The social worker is meeting the client for the first time. What should the social worker do **FIRST**?

**A.** Ask the client to describe what fasting means to them
**B.** Arrange a pre-Ramadan medical visit to plan safer fasting
**C.** Help the client plan Ramadan meals before dawn and after sunset
**D.** Explain that the faith excuses people who are ill from fasting

#### Why each option is right or wrong

**A** **Correct.** Religion shapes health decisions (NASW Code 1.05(a)), and clients are the experts on their own culture (1.05(c)), so respecting a faith practice starts with learning what it means to the client. The social worker is meeting the client for the first time, the fast is a month away, and the physician wants persuasion. Asking about the fast first lets any safety plan be built with the practice rather than against it. It beats B because a medical visit to plan safer fasting is the right next step, but arranging it before hearing the client treats the practice as a problem to manage.

**B** **Incorrect** (trap: Right action, wrong time). Fasting while using insulin carries a real risk of low blood sugar, and guidelines call for risk stratification and medication adjustment before Ramadan (Hassanein et al., 2017). This visit should follow soon. Arranged before the social worker learns the client's goals for the fast, it can feel like the persuasion the physician asked for.

**C** **Incorrect** (trap: Right action, wrong time). Meal timing matters when fasting with diabetes, and meal planning may follow. It belongs after the client's goals are known and the physician has reviewed insulin, since meal plans alone cannot make the current regimen safe.

**D** **Incorrect** (trap: Outside the social work role). Many traditions excuse the ill from fasting, but whether that applies is for the client and their religious advisors to decide. A social worker who rules on it oversteps and may damage trust.

**Takeaway:** When a religious practice raises a health concern, first learn what it means to the client, then plan safety with the practice rather than against it.

**Sources:** Hassanein, M., et al. (2017). Diabetes and Ramadan: Practical guidelines. Diabetes Research and Clinical Practice, 126, 303-316, risk stratification; medication adjustment; NASW Code of Ethics (2021), 1.05(a); NASW Code of Ethics (2021), 1.05(c)

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## 340. ETH-0010 (rev 1) · Best answer: B

A client with limited English proficiency arrives for a mental health intake with their 15-year-old child, who offers to interpret. The client says they would prefer that the child interpret. What is the **MOST** appropriate action?

**A.** Proceed with the child interpreting, as the client prefers

**B.** Use a qualified telephone or video interpreter for the intake

**C.** Ask a bilingual receptionist to interpret during the intake session

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). A child interpreting a parent's mental health intake may be exposed to adult content, role reversal and pressure to filter what is said. Even with the client's preference, a minor is an inappropriate interpreter outside an emergency.

**B** **Correct.** A clinical intake requires accurate, confidential communication. Social workers use qualified interpreters for clients with language barriers (NASW Code 1.03(b); Cultural Competence Standard 9). Federal health-care rules bar relying on a minor child to interpret except briefly in an emergency (45 CFR 92.201(e)). A qualified remote interpreter is available now, so the intake can go ahead properly. It beats C because a bilingual staff member without interpreter training may miss clinical meaning, and the client never agreed to share this information with front-desk staff.

**C** **Incorrect** (trap: Good but not best). A bilingual receptionist is closer to adequate than a child, but staff without interpreter training may misinterpret clinical terms. It also widens who hears confidential information. A qualified interpreter is the standard.

**Takeaway:** Use qualified interpreters for clinical communication; never rely on a client's minor child except briefly in an emergency.

**Sources:** NASW Code of Ethics (2021), 1.03(b); NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 9: Language and Communication; Nondiscrimination in Health Programs and Activities (Section 1557 regulations), 45 CFR 92.201(e)

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### 341. ETH-1601 (rev 2) · Best answer: B

In a hospital emergency department, a social worker is asked to assess a patient with limited English who took extra pills. The patient's adult sibling, who is fluent in English, offers to interpret. A bilingual nurse whose language skills have not been assessed is on shift. A telephone interpreter is available at once, and an in-person interpreter can arrive in 2 hours. What should the social worker do **FIRST**?

**A.** Let the sibling interpret so the assessment can begin
**B.** Start the assessment with the phone interpreter
**C.** Wait for the in-person interpreter before assessing
**D.** Ask the bilingual nurse to interpret the interview

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). A relative may filter the patient's words, and the patient may hide suicidal thoughts or family conflict in front of a sibling. Federal rules allow an accompanying adult to interpret only as an emergency stopgap until a qualified interpreter arrives, or at the patient's own private request. Here the sibling offered, and a qualified interpreter is ready.

**B** **Correct.** Health programs must offer a qualified interpreter and may not rely on an accompanying adult or untested bilingual staff except in narrow cases, such as an emergency stopgap or the patient's own private request (45 CFR 92.201(c), (e)). A qualified telephone interpreter is ready now, so the risk assessment after a possible overdose can start at once without depending on family. It beats C because an in-person interpreter adds nuance, but a 2-hour delay in assessing suicide risk is unsafe when a qualified option exists now. Clear communication is also needed for the patient to understand and take part (NASW Code 1.03(b)).

**C** **Incorrect** (trap: Ignores risk). In-person interpreting captures tone and gesture and suits long or complex sessions. Waiting 2 hours to assess a patient after a possible overdose delays safety decisions when a qualified telephone interpreter can start now.

**D** **Incorrect** (trap: Misuses the technique). Bilingual staff can interpret when their skills have been assessed and they are trained for the role. An untested speaker may miss or soften key risk statements, so this is the right kind of resource used the wrong way.

**Takeaway:** When interpretation is urgent, use a qualified remote interpreter now rather than family, untested staff or a delayed in-person interpreter.

**Sources:** Section 1557 of the Affordable Care Act regulations (45 CFR Part 92), 45 CFR 92.201(c), (e); NASW Code of Ethics (2021), 1.03(b)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

### 342. ETH-1604 (rev 1) · Best answer: A

A home health social worker visits an older client who speaks Vietnamese to set up a schedule for home-delivered meals. The client's adult child offers to interpret. With the child out of the room, the social worker uses a telephone interpreter to explain that a free interpreter is available. The client clearly prefers the child, and the child agrees. What should the social worker do **NEXT**?

**A.** Proceed as the client chose and document the choice
**B.** Keep the telephone interpreter on the line for the visit
**C.** Explain again why a trained interpreter is better

**Why each option is right or wrong**

**A** **Correct.** Federal rules for health programs allow an accompanying adult to interpret when the client, told privately through a qualified interpreter, asks for it, the adult agrees, the choice is documented and reliance suits the situation (45 CFR 92.201(e)(2)). Each condition is met, and the topic is a routine meal schedule rather than sensitive clinical content. It beats B because insisting on the telephone interpreter overrides an informed, low-risk choice by a competent client (NASW Code 1.02). A qualified interpreter would be the better choice for sensitive or complex topics.

**B** **Incorrect** (trap: Overrides the client's choice). A qualified interpreter is the default and would be right if the client had not made an informed choice or the topic were sensitive. Here, overriding a competent client's private, informed preference on a routine matter is paternalistic.

**C** **Incorrect** (trap: Right action, wrong time). The client has already heard, privately and through a qualified interpreter, that a free interpreter is available. Repeating the case for it pressures the client and suggests the choice is not respected.

**Takeaway:** A competent client may choose an adult companion to interpret after a private offer of a free qualified interpreter, when the topic suits it.

**Sources:** Section 1557 of the Affordable Care Act regulations (45 CFR Part 92), 45 CFR 92.201(e)(2); NASW Code of Ethics (2021), 1.02
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 343. ETH-1607 (rev 3) · Best answer: A

A Deaf adult who grew up in another country begins services at a community mental health center. A qualified ASL interpreter attends the intake but reports that the client uses many home signs and signs from another country, and much is being missed. Twenty minutes of slower pacing and simpler questions have not closed the gap. What should the social worker do **NEXT**?

**A.** Add a Certified Deaf Interpreter to the team
**B.** Replace the interpreter with written notes in English
**C.** Invite a family member who knows the client's signs
**D.** Refer the client to a provider who signs fluently

**Why each option is right or wrong**

**A** **Correct.** A Certified Deaf Interpreter is a Deaf interpreter who teams with a hearing ASL interpreter when a client uses home signs, a foreign sign language or limited language, bringing native sign fluency and cultural knowledge (RID, 1997). The hearing interpreter has reported exactly these barriers, and simpler questions have not closed the gap. Effective communication is required (28 CFR 36.303(c)(1)). It beats D because a provider fluent in ASL would face the same nonstandard signs, and the client would lose an otherwise suitable service.

**B** **Incorrect** (trap: Misuses the technique). Written English depends on literacy in a second language, and many Deaf people, especially those schooled abroad, do not read English well enough for a clinical interview. Notes may suit a brief exchange, not an assessment.

**C** **Incorrect** (trap: Misreads law or ethics). A relative may understand the home signs, but family interpreters can filter content and compromise privacy, and the rules bar relying on an accompanying adult except in an emergency or at the client's own request (28 CFR 36.303(c)(3)). A qualified team can be arranged.

**D** **Incorrect** (trap: Hands off too soon). A signing provider can be ideal for many Deaf clients, but fluency in ASL does not solve home signs or a foreign sign language. Adding the right interpreter keeps the client in the chosen service.

**Takeaway:** When a Deaf client's signing is nonstandard or limited, team a Certified Deaf Interpreter with the ASL interpreter.

**Sources:** Registry of Interpreters for the Deaf (1997). Use of a Certified Deaf Interpreter (Standard Practice Paper), whole paper; ADA Title III regulations, 28 CFR 36.303(c)(1); ADA Title III regulations, 28 CFR 36.303(c)(3)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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## 344. ETH-1609 (rev 3) · Best answer: B

An autistic 26-year-old client at a community agency joins a job-search support group. After 2 sessions, the client tells the social worker that the bright lights and overlapping voices in the group room become overwhelming, so the client leaves early. The client still wants to be in the group. What is the **BEST** response by the social worker?

**A.** Coach the client to ask the group for lighting and noise changes
**B.** Arrange sensory changes, such as softer lighting and a break space
**C.** Offer individual sessions in place of the group

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Supporting self-advocacy is valuable, and the client may later choose to tell the group what helps. Making the client ask peers for changes, though, shifts the agency's duty to modify its own setting onto the client and requires a disclosure the client has not chosen.

**B** **Correct.** Strong reactions to sensory input are a recognized feature of autism (DSM-5-TR). Access means adjusting the environment so the client can take part, and agencies must make reasonable modifications when needed for equal access unless they would fundamentally alter the service (28 CFR 36.302(a)). The client still wants the group, and modest changes such as dimmer lights, turn-taking rules and a break

space keep that choice. It beats A because coaching the client to request changes from peers puts the agency's duty to make reasonable modifications on the client and requires a disclosure the client has not chosen.

**C** **Incorrect** (trap: Good but not best). Individual sessions can meet some needs, but the client chose the group for its peer contact. Moving the client out instead of fixing the room offers a separate, lesser service when a reasonable change would allow full participation.

**Takeaway:** For neurodivergent clients, adjust the environment so they can join the service they chose, rather than asking them to endure or leave it.

**Sources:** ADA Title III regulations, 28 CFR 36.302(a); DSM-5-TR (APA, 2022), Neurodevelopmental Disorders: Autism Spectrum Disorder
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 345. ETH-1611 (rev 2) · Best answer: A

At a hospital discharge meeting, a 40-year-old patient with cerebral palsy communicates by typing on a speech-generating device. Each time the team asks about plans for home, members turn to the patient's caregiver for the answer. The patient starts typing an answer but stops. What should the social worker do **FIRST**?

**A.** Ask the patient the question and wait for the typed reply
**B.** Ask the caregiver to answer, then check with the patient
**C.** Switch to yes-or-no questions to speed up the meeting

#### Why each option is right or wrong

**A** **Correct.** Effective communication with a person with a disability means communicating with that person through their own method, with time to use it (28 CFR 36.303(c)(1)). The patient is a competent adult making discharge decisions, yet the team keeps redirecting questions to the caregiver. Turning back to the patient and waiting for the typed answer restores the patient's voice in decisions about their own life (NASW Code 1.02). It beats B because checking with the patient after the caregiver speaks still treats the caregiver's answer as the main one.

**B** **Incorrect** (trap: Right action, wrong time). Caregivers often add useful detail, and their input fits after the patient has answered. Letting the caregiver speak first and then asking the patient to agree makes the patient a reviewer of their own plan.

**C** **Incorrect** (trap: Misuses the technique). Yes-or-no questions can help in some situations, but they narrow what a capable patient can say about plans for home. Using them to save time trades the patient's full answer for the team's convenience.

**Takeaway:** With clients who use alternative communication, speak directly to them and allow time for their reply rather than turning to companions.

**Sources:** ADA Title III regulations, 28 CFR 36.303(c)(1); NASW Code of Ethics (2021), 1.02
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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### 346. ETH-1614 (rev 1) · Best answer: B

A caregiver support group at a nonprofit agency meets in a second-floor room with no elevator. A new member who uses a wheelchair wants to join. The program director suggests that the social worker offer this member individual sessions instead. A ground-floor conference room is free at the group's meeting time. What should the social worker do?

**A.** Offer the member individual sessions as the director suggests
**B.** Move the group to the accessible downstairs room
**C.** Have the member join the group by video from downstairs

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Individual sessions may help some caregivers, but offering them in place of the group gives the member a separate service because of disability. The member wants the group, and an accessible room is free.

**B** **Correct.** Agencies open to the public must serve people with disabilities in the most integrated setting appropriate to their needs, and offering a separate service does not justify exclusion from the regular program (28 CFR 36.203). An accessible room is free at the same time, so moving the group is simple and lets the member take part fully. It beats C because joining by video from another room keeps the member apart from peers when an equal, in-person option is readily available.

**C** **Incorrect** (trap: Good but not best). Video can widen access when no accessible space exists. Here it would seat the member apart from the group when an accessible room is free, so it falls short of equal participation.

**Takeaway:** Serve clients with disabilities in the most integrated setting: fix access to the regular program rather than offering a separate service.

**Sources:** ADA Title III regulations, 28 CFR 36.203
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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### 347. ETH-1616 (rev 2) · Best answer: C

During a child welfare meeting with a parent who speaks Haitian Creole, the social worker notices that the agency interpreter is giving long replies the parent did not say. The interpreter then adds, in English, 'I told the parent to be grateful for the help.' What should the social worker do **FIRST**?

**A.** Ask a bilingual coworker to take over interpreting now
**B.** Report the interpreter to the language service afterward
**C.** Pause and restate the interpreter's role right away

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Acting now is right, which makes this tempting. A bilingual coworker who is not a qualified interpreter can add new errors and role confusion in a high-stakes meeting. If the agency interpreter cannot keep to the role once it is restated, a qualified replacement is the next step.

**B** **Incorrect** (trap: Right action, wrong time). Reporting the conduct may be appropriate after the meeting, especially if it continues. As a first step it lets the current meeting go on with inaccurate interpreting in a case with high stakes for the family.

**C** **Correct.** Interpreters should convey messages accurately and completely and should not counsel, advise or insert their own views (NCIHC, 2004). The interpreter is adding content and advising the parent in a meeting with high stakes, so the social worker should stop at once and restate the role: interpret everything said, add nothing, leave nothing out. It beats B because a report may follow, but waiting lets the meeting continue on distorted communication that could affect the parent's case and understanding (NASW Code 1.03(b)).

**Takeaway:** If an interpreter adds, omits or advises, stop and restate the interpreter's role at once; report the problem afterward if needed.

**Sources:** National Council on Interpreting in Health Care (2004). A National Code of Ethics for Interpreters in Health Care, ethical principles: accuracy; impartiality; NASW Code of Ethics (2021), 1.03(b)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 348. ETH-1618 (rev 2) · Best answer: C

A Deaf client at a nonprofit counseling agency asks for an ASL interpreter for weekly sessions. The program manager tells the social worker to use written notes because interpreters are costly. When the social worker lays out the client's request and the access concern, the manager repeats the decision. What should the social worker do **NEXT**?

**A.** Use written notes as directed and record the client's request
**B.** Ask the client to bring a relative who signs to sessions
**C.** Seek guidance from the agency's director or compliance officer
**D.** Refer the client to an agency that provides interpreters

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Recording the request is good practice, but following a directive that likely denies effective communication puts cost ahead of the client's access rights. Written notes suit brief, simple exchanges, not weekly counseling.

**B** **Incorrect** (trap: Misreads law or ethics). Agencies may not require a client to bring someone to interpret, and relatives raise privacy and accuracy concerns in counseling. Providing a qualified interpreter is the agency's responsibility.

**C** **Correct.** A private agency must provide auxiliary aids, including qualified interpreters, when needed for effective communication, and may not require the client to bring someone to interpret (28 CFR 36.303(b)(1), (c)). Weekly counseling is unlikely to work through written notes. Social workers should not let agency directives interfere with ethical practice (NASW Code 3.09(d)). The social worker has raised the issue with the manager without success, so the director or compliance officer is the next channel. It beats D because referring out shifts the agency's duty onto the client.

**D** **Incorrect** (trap: Hands off too soon). Referral would fit if the client wanted a program that specializes in serving Deaf clients. Here the client chose this agency, and referring out because of interpreter costs shifts the agency's legal duty onto the client.

**Takeaway:** When a manager refuses a required accommodation after it is raised, take the concern to the next level, such as the director or compliance officer.

**Sources:** ADA Title III regulations, 28 CFR 36.303(b)(1), (c); NASW Code of Ethics (2021), 3.09(d)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 349. ETH-1701 (rev 2) · Best answer: A

A client arrives at a private nonprofit counseling center with a dog that wears no vest. The client explains that the dog is trained to nudge them and lead them outside when a dissociative episode begins. The receptionist says animals cannot come in unless there is a doctor's letter and asks the social worker how to proceed. What should the social worker tell the receptionist?

**A.** Admit the dog without requiring any paperwork
**B.** Admit the dog once a provider letter is on file
**C.** Admit the dog but keep it in the waiting area during sessions

#### Why each option is right or wrong

**A** **Correct.** Under ADA Title III, a service animal is a dog individually trained to do work or perform tasks for a person with a disability, and psychiatric tasks count (28 CFR 36.104). Staff may ask whether the dog is needed because of a disability and what task it performs, but may not require certification or other documentation (28 CFR 36.302(c)(6)). The client has already named a trained task, interrupting dissociation, so the dog should be admitted. It beats C because a service animal may go with the client into every area open to clients, including the counseling room (28 CFR 36.302(c)(7)).

**B** **Incorrect** (trap: Misreads law or ethics). A provider letter is documentation, and the regulations bar requiring it for a service animal at a public accommodation. Letters can matter in other settings, such as housing requests for assistance animals, but not for entry to this center.

**C** **Incorrect** (trap: Misreads law or ethics). Keeping the dog out of the counseling room can seem a fair middle ground, but a service animal may accompany the client wherever clients are allowed to go (28 CFR 36.302(c)(7)). This dog's task, interrupting dissociation, may be needed most during sessions.

**Takeaway:** Admit a trained service dog, including one for a psychiatric disability; staff may ask about need and task but cannot demand documentation.

**Sources:** ADA Title III regulations, 28 CFR 36.302(c)(6); ADA Title III regulations, 28 CFR 36.104 (definition of service animal); ADA Title III regulations, 28 CFR 36.302(c)(7)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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### 350. ETH-1707 (rev 1) · Best answer: A

An autistic adult starts counseling at an agency whose secure online system supports both video and live written messaging. The client explains that speaking aloud under stress makes them lose words and that video eye contact leads to shutdown, but they write clearly and quickly. The supervisor prefers video so the social worker can observe affect. What is the **BEST** way to provide services?

**A.** Hold sessions through the system's text chat
**B.** Hold video sessions on the system with the camera off
**C.** Refer the client to an autism specialty program

**Why each option is right or wrong**

**A** **Correct.** Accessible practice adapts the service to how a neurodivergent client communicates best, treating this as a difference rather than a deficit. The client has named the barrier, live speech under stress, and a channel that works, writing, and the agency's secure system supports it. Technology standards call for assessing whether an electronic format suits each client, including people with differing abilities (Technology Standards 2.01, 2.05), and the ADA requires reasonable changes in practices (28 CFR 36.302(a)). It beats B because turning off the camera removes eye contact but still demands live speech.

**B** **Incorrect** (trap: Misuses the technique). Removing video helps with eye contact and might suit a client whose only difficulty was being seen. It still requires speaking aloud in real time, which the client says causes words to drop out under stress.

**C** **Incorrect** (trap: Hands off too soon). A specialty program helps when needs exceed the worker's skills. This client needs a change in format the agency already offers, so referring out turns a simple accommodation into a barrier to service.

**Takeaway:** Match the service format to a neurodivergent client's stated communication strengths when a secure option exists; accommodation is not a lesser service.

**Sources:** NASW, ASWB, CSWE & CSWA Standards for Technology in Social Work Practice (2017), Standard 2.01; Standard 2.05; ADA Title III regulations, 28 CFR 36.302(a)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 351. ETH-1713 (rev 1) · Best answer: B

A parent's child welfare case plan requires a 10-week parenting class run by a private nonprofit. The parent has a documented reading disability and turns in blank worksheets but takes an active part in discussions. The facilitator plans to report the parent as noncompliant, and the parent asks the caseworker, a social worker, for help. What should the social worker do **FIRST**?

**A.** Help the parent complete the worksheets after each class
**B.** Request oral or recorded worksheet responses
**C.** Find a different parenting program with less written work

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Extra help can support a parent with a reading disability and may serve as a fallback. It leaves the inaccessible format in place, adds to the parent's load, and can blur whether the answers show the parent's own learning.

**B** **Correct.** Programs open to the public must make reasonable changes in practices when needed to avoid excluding people with disabilities, unless the change would alter the nature of the service (28 CFR 36.302(a)). The parent's active discussion shows learning; the barrier is the written format. Asking for oral or recorded responses fixes the barrier at its source and lets the facilitator judge the parent's real progress. Disability is part of the diversity social workers must understand (NASW Code 1.05(d)). It beats A because worksheets finished with the caseworker's help leave the format unchanged and may raise doubts about whose work they are.

**C** **Incorrect** (trap: Right action, wrong time). Changing programs is a later step, needed only if the provider refuses a reasonable change. Starting there delays the case plan and leaves the current program's barrier in place for other parents.

**Takeaway:** When a disability blocks a required program's format, request a reasonable change in the format rather than workarounds or a new program.

**Sources:** ADA Title III regulations, 28 CFR 36.302(a); NASW Code of Ethics (2021), 1.05(d)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 3 of 3

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### 352. ETH-1720 (rev 2) · Best answer: C

A 70-year-old client with macular degeneration receives telehealth counseling from an agency. The client uses a screen reader but cannot read the agency's scanned consent forms and treatment plans, so an adult child reads them aloud. The client says they want to review these papers privately and at their own pace, and that Word documents work well with the screen reader. What is the **MOST** appropriate response?

**A.** Read each document aloud to the client during sessions
**B.** Email the scanned forms so the client can enlarge them
**C.** Send the documents as accessible electronic files

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Reading documents aloud is a reasonable short-term step and keeps family out of private matters. It still ties review to session time and the worker's pace, when the client asked to read independently.

**B** **Incorrect** (trap: Misuses the technique). Emailing seems to give private, self-paced access, but scanned pages are images that a screen reader cannot read, and enlarging them does not help a client who relies on audio output. The format, not the delivery route, is the barrier the client named.

**C** **Correct.** Auxiliary aids must be effective, timely and provided in a way that protects the privacy and independence of the person with a disability, and the provider should consult the person about what works (28 CFR 36.303(c)(1)(ii)). The client named both the goal, private review at their own pace, and a format that works. Sending accessible electronic files meets both. It beats A because reading aloud in session keeps family out but still prevents the client from reviewing the papers independently and on their own schedule.

**Takeaway:** Provide documents in the accessible format the client names, protecting privacy and independence, rather than relying on others to read for them.

**Sources:** ADA Title III regulations, 28 CFR 36.303(c)(1)(ii)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.4 Access: language, interpretation, ASL, physical, cultural, neurodiversity · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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## 353. ETH-0117 (rev 1) · Best answer: B

A child welfare social worker is building a case plan with a parent whose children are in foster care. The parent agrees to every proposed goal without comment. Earlier, the parent described distrust of the agency after relatives lost custody of their children. What is the **MOST** appropriate response by the social worker?

**A.** Finalize the plan, since the parent agreed to each goal
**B.** Acknowledge the agency's authority and ask the parent's own goals
**C.** Tell the parent they seem to be holding back opinions
**D.** Reassure the parent that the agency's aim is reunification

#### Why each option is right or wrong

**A** **Incorrect** (trap: Skips assessment). Treating silent agreement as commitment assumes the plan reflects the parent's wishes. Given the agency's power and the parent's stated distrust, the agreement may be deference, and the plan may fail later.

**B** **Correct.** A parent in child welfare faces real power: the agency can affect custody. Agreement without comment may reflect that power and past distrust rather than true buy-in. Naming the agency's authority openly and inviting the parent's own goals shares power and makes honest input safer (NASW Code 1.02, 1.05(c)). It beats D because reassurance about reunification is kind and true, but it leaves the power gap unspoken and still does not draw out the parent's views.

**C** **Incorrect** (trap: Too directive). Pointing out that the parent seems to be holding back interprets the behavior before trust exists. It can feel like an accusation from someone with power over the family and may deepen the silence.

**D** **Incorrect** (trap: Good but not best). Reassurance is accurate and well-meant, but it speaks for the agency without addressing the power imbalance. The parent still has no clear invitation to disagree or add goals.

**Takeaway:** When a client defers to a worker with authority, name the power openly and invite the client's own goals before finalizing plans.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 354. ETH-1576 (rev 2) · Best answer: B

A Black client in outpatient treatment for depression asks the white social worker about working with a Black social worker at the clinic instead. Invited to say more, the client says the work has helped but that they want to talk about racism at work without explaining it first. A Black colleague has openings. What should the social worker do **NEXT**?

**A.** Explore whether the request reflects discomfort with the current work
**B.** Arrange the transfer and offer a joint handoff session
**C.** Propose a few more sessions to work through the concern together
**D.** Reassure the client that the worker is willing to learn

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). The social worker has already asked, and the client gave a clear reason tied to the burden of explaining racism. Probing again for hidden discomfort treats an informed choice as a symptom. More exploration fits when the reason is unclear.

**B** **Correct.** Clients have the right to make informed choices about their care, and asking for a provider who shares one's racial identity is a legitimate choice that can lift the burden of explaining racism (NASW Code 1.02; 1.05(c)). The client has explained the reason, values the work, and a colleague is available, so the next step is an orderly, warm transfer (NASW Code 1.16(b)). It beats C because pressing for more sessions puts the worker's wish for continuity ahead of a clear client choice; the joint handoff protects continuity instead.

**C** **Incorrect** (trap: Overrides the client's choice). Continuity matters, but the client has made an informed choice and a provider is available. Asking for more sessions first puts the worker's preference ahead of the client's. A short transition is appropriate if the client wants one.

**D** **Incorrect** (trap: Good but not best). Offering to learn is well meant, but it asks the client to keep doing the explaining they want to avoid. It centers the worker's growth rather than the client's stated need.

**Takeaway:** When a client makes an informed request for a provider who shares their identity, honor it with an orderly, warm transfer.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 1.16(b)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

### 355. ETH-1579 (rev 2) · Best answer: A

A 16-year-old Mexican American client tells a social worker that darker-skinned relatives are 'less smart' and that lighter skin makes people 'more respectable.' The client avoids the sun to stay light and calls classmates with Indigenous features 'low class.' These views echo messages the client has heard from family and media. Which concept **MOST** likely explains the client's beliefs?

**A.** Internalized racial oppression
**B.** Acculturative stress
**C.** Assimilation
**D.** Stereotype threat

#### Why each option is right or wrong

**A** **Correct.** Internalized racial oppression is the acceptance, by members of a targeted group, of the dominant society's devaluing messages and racial hierarchy (Pyke, 2010). The client has taken in the idea that lighter skin and European features mean worth and applies it to family, peers and self. It beats C because assimilation describes trading heritage culture for the dominant culture, while this client has absorbed a racial hierarchy that devalues darker skin and Indigenous features. Pyke frames this as a product of structural racism, not a personal weakness.

**B** **Incorrect** (trap: Wrong model or concept). Acculturative stress is strain from adapting to a new or dominant culture, such as conflict between home and school norms. The client's statements show acceptance of a skin-color hierarchy rather than distress about adjusting.

**C** **Incorrect** (trap: Wrong model or concept). In Berry's acculturation model, assimilation means adopting the dominant culture while letting go of one's heritage culture (Berry, 1997). The client's statements rank people's worth by skin color, including relatives and self, rather than describing a shift in cultural practices or identity.

**D** **Incorrect** (trap: Wrong model or concept). Stereotype threat is anxiety about confirming a negative stereotype that can lower performance in evaluative settings. The client is endorsing the stereotypes, not fearing being judged by them.

**Takeaway:** Members of targeted groups may absorb the dominant society's devaluing messages about their own group; this reflects structural racism, not personal failing.

**Sources:** Pyke, K. D. (2010). What is internalized racial oppression and why don't we study it? Acknowledging racism's hidden injuries. Sociological Perspectives, 53(4), 551-572, whole article; Berry, J. W. (1997). Immigration, acculturation, and adaptation. Applied Psychology: An International Review, 46(1), 5-34, acculturation strategies: assimilation

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

### 356. ETH-1582 (rev 1) · Best answer: C

At a first session in private practice, a Black client describes tension with neighbors since moving to a mostly white suburb. The client pauses, looks at the white social worker, and says, 'I don't know if you'd get this part.' What is the social worker's **BEST** response?

**A.** Assure the client of the worker's training in diverse cultures
**B.** Note the comment and return to it once rapport is stronger
**C.** Name the racial difference and ask how it may affect the work

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Citing training defends the worker's competence instead of engaging the client's concern. It also implies that cultural knowledge can stand in for hearing the client as the expert on their own experience.

**B** **Incorrect** (trap: Right action, wrong time). Rapport matters, but the client raised the issue now. Setting it aside signals that race is off limits and leaves the doubt unanswered when the client was most open to discussing it.

**C** **Correct.** Broaching means the social worker openly raises race and culture, including differences between worker and client, and invites the client to discuss how they matter (Day-Vines et al., 2007). The client has signaled doubt tied to the racial difference, so naming it directly shows the topic is welcome and the worker can handle it. It beats B because waiting for more rapport leaves the doubt unanswered at the moment it was raised, and avoidance can erode trust. It also honors the client as the expert on their experience (NASW Code 1.05(c)).

**Takeaway:** When racial difference surfaces in the relationship, raise it openly and invite the client's view of its effect on the work.

**Sources:** Day-Vines, N. L., et al. (2007). Broaching the subjects of race, ethnicity, and culture during the counseling process. Journal of Counseling & Development, 85(4), 401-409, whole article; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 357. ETH-1585 (rev 1) · Best answer: B

A social worker leads a support group for older adult caregivers at a senior center. In the third meeting, white members do most of the talking, and one member twice talks over a Black member to correct that member's story about a clinic visit. The Black member and two Latino members have gone quiet. What is the **MOST** appropriate action by the social worker?

**A.** Check in privately with the quieter members after the meeting
**B.** Name the pattern and set a norm that each member is heard
**C.** Let the group develop its own norms over the next sessions

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Private check-ins show care and may help later. Used alone, they ask the members being silenced to manage the problem and leave the dominant pattern in place for the next meeting.

**B** **Correct.** Group leaders shape norms, and early sessions are when norms take hold (Yalom & Leszcz, 2020). A pattern in which members of color are talked over and fall

silent reproduces racial power differences inside the group. Naming the pattern and setting a norm that each member is heard addresses it openly and protects equal voice (NASW Code 1.05(b)). It beats A because private check-ins may comfort the quieter members but place the burden on them and leave the group's norm unchanged.

**C** **Incorrect** (trap: Wrong model or concept). Groups do develop norms, but leaders actively shape them, especially early. Waiting lets an unequal pattern harden and signals that talking over members of color is acceptable.

**Takeaway:** When group interaction reproduces racial power differences, the leader names the pattern and sets norms for equal voice.

**Sources:** Yalom & Leszcz (2020). The Theory and Practice of Group Psychotherapy (6th ed.), the therapist's basic tasks: culture building and norms; NASW Code of Ethics (2021), 1.05(b)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 358. ETH-1588 (rev 1) · Best answer: C

A white social worker goes with a Black client to a benefits office to appeal a denial. The caseworker there speaks to the social worker alone and brushes off the client's answers. The client prepared the appeal and wants to present it. What is the **BEST** action by the social worker?

**A.** Present the appeal so the caseworker will listen and decide
**B.** File a complaint about the caseworker's conduct after the visit
**C.** Turn each question back to the client and stay present

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). Presenting the case might speed a decision, but it overrides the client's wish to speak and confirms the caseworker's pattern of treating the white professional as the real authority.

**B** **Incorrect** (trap: Right action, wrong time). A complaint may be appropriate later if the client wants one. It does not change what happens in the room, where the client is being sidelined now.

**C** **Correct.** Racial and professional privilege can lead institutions to defer to a white worker over a Black client. Empowerment-focused advocacy uses that access to make room for clients to speak for themselves rather than taking over (NASW Cultural Competence Standards, Standard 6; NASW Code 1.02). The client prepared the appeal and wants to present it, so turning questions back to the client while staying present honors that choice and counters the caseworker's deference. It beats A because speaking for the client may win the appeal but reinforces the dynamic that sidelined the client.

**Takeaway:** Use racial and professional privilege to amplify the client's voice, not to replace it.

**Sources:** NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 6: Empowerment and Advocacy; NASW Code of Ethics (2021), 1.02
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 359. ETH-1592 (rev 1) · Best answer: B

A white supervisee describes a Black client who wants help finding a better-paying job. The supervisee keeps steering sessions toward anger management, saying, 'I know what this client really needs.' The client has missed the last two sessions. What is the **MOST** appropriate response by the supervising social worker?

**A.** Reassign the client to a social worker with more experience
**B.** Help the supervisee reflect on privilege and power in this pattern
**C.** Direct the supervisee to follow the client's stated goals

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). Reassignment removes the learning opportunity and may disrupt the client's care. It fits if the supervisee cannot change course after supervision, not as a first response.

**B** **Correct.** Cultural competence standards ask social workers to recognize their own privilege and power and how these shape their work, and supervision supports that self-awareness (NASW Cultural Competence Standards, Standard 2; NASW Code 1.05(c)). The supervisee is replacing the client's goal with an assumption about what a Black client needs, and the client is pulling away. Helping the supervisee examine that pattern addresses its source and prepares them to follow the client's lead. It beats C because an order fixes this case but leaves the assumption unexamined for the next client.

**C** **Incorrect** (trap: Too directive). Returning to the client's goals is necessary, and the supervisor may say so. Giving the instruction alone leaves the supervisee's assumption about what the client 'really needs' unexamined.

**Takeaway:** Supervisors help supervisees examine how privilege and role power lead them to override clients' goals, not just correct one case.

**Sources:** NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 2: Self-Awareness; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 3 of 3

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### 360. ETH-1676 (rev 2) · Best answer: C

A white clinical supervisor notices that a Black supervisee agrees with every plan in supervision but later handles several family cases differently, with good results. When the supervisor asks about it, the supervisee says, 'I didn't think disagreeing with you was an option here.' What is the supervisor's **BEST** response?

**A.** Remind the supervisee to voice disagreement instead of bypassing plans
**B.** Offer to arrange supervision with a Black colleague instead
**C.** Name the power and racial difference and invite open disagreement

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Revisiting plans rather than bypassing them is the right norm. Said now, though, it answers the supervisee's disclosure with a rule and puts the whole burden on the supervisee. It fits after the supervisor has made disagreement clearly safe.

**B** **Incorrect** (trap: Hands off too soon). Moving the supervisee to a Black supervisor treats a dynamic the supervisor helps create as the supervisee's problem to escape. It also assumes the supervisee wants a race-matched supervisor, which the supervisee has not said.

**C** **Correct.** Supervisors hold evaluative power, and a racial difference can make that power harder for a supervisee to challenge. Naming both openly and inviting disagreement sets clear, culturally sensitive boundaries and makes room for honest clinical judgment (NASW Code 3.01(b); 1.05(c)). The comment shows the problem lies in the relationship, not in clinical skill, since the departures worked. It beats A because a reminder, though fair, leaves the supervisor's role in the silence unspoken and keeps the burden on the supervisee.

**Takeaway:** Supervisors address how their power and racial differences can silence supervisees by naming them and inviting disagreement, rather than enforcing compliance.

**Sources:** NASW Code of Ethics (2021), 3.01(b); NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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### 361. ETH-1682 (rev 1) · Best answer: B

After one session with a Black social worker, a white client at a community mental health center asks the intake supervisor for a new worker. The client says, 'Nothing personal, but I'd do better with someone more like me.' The supervisor has already talked with the social worker, who is comfortable continuing. What is the supervisor's **BEST** response?

**A.** Reassign the client to a white social worker as requested
**B.** Meet with the client about the request and affirm the assignment
**C.** Ask the social worker again how continuing would feel

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Honoring client preferences is usually good practice, but this request asks the agency to remove a qualified worker because of race. Granting it would make the agency part of the discrimination. Reassignment could be reconsidered if a specific clinical problem unrelated to race emerges.

**B** **Correct.** Social workers should not facilitate discrimination based on race, including in an agency's work assignments (NASW Code 4.02; 3.09(e)). Granting this request would let a preference about the worker's race decide staffing. Meeting with the client lets the supervisor hear any real clinical concern while explaining that assignments rest on qualifications and fit. It beats A because self-determination covers a client's goals and choices in treatment; it does not require the agency to discriminate against its staff.

**C** **Incorrect** (trap: Good but not best). Checking the worker's wishes was the right step, and it has been done: the worker wants to continue. Asking again shifts the burden back to the worker and delays the response the client needs to hear.

**Takeaway:** Agencies should not honor a client's request to remove a staff member because of race; address the request directly while protecting the assignment.

**Sources:** NASW Code of Ethics (2021), 4.02; NASW Code of Ethics (2021), 3.09(e)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 362. ETH-1687 (rev 1) · Best answer: A

In an employee assistance session, a white client says a workplace diversity training called them 'privileged.' The client says angrily, 'I grew up poor and worked for everything I have. Nobody handed me anything.' This is the first time the client has raised the topic. What is the social worker's **MOST** appropriate response?

**A.** Explore what the training's label stirred up for the client
**B.** Explain that racial privilege can exist alongside economic hardship
**C.** Agree that the training should have focused on class instead

**Why each option is right or wrong**

**A** **Correct.** When a client reacts defensively to the idea of racial privilege, exploring the reaction comes before teaching the concept. The anger likely carries hurt about the client's own hardship, and feeling heard makes later reflection on privilege possible (NASW Code 1.05(c)). It beats B because B's point is accurate, since advantage tied to race can coexist with economic struggle (McIntosh, 1989), but offered as the first response it reads as a correction and is likely to deepen the defensiveness.

**B** **Incorrect** (trap: Too directive). The point is accurate: race-based advantage can coexist with real economic hardship (McIntosh, 1989). As the first response to an angry client, though, it reads as a correction and is likely to harden the defensiveness. It fits once the client feels heard.

**C** **Incorrect** (trap: Boundary slip or collusion). Agreeing that class matters more sides with the defensiveness and implies racial privilege is not real. It may feel supportive, but it closes off reflection the client could benefit from.

**Takeaway:** When a client reacts defensively to discussions of racial privilege, explore the reaction before teaching the concept.

**Sources:** NASW Code of Ethics (2021), 1.05(c); McIntosh, P. (1989). White privilege: Unpacking the invisible knapsack. Peace and Freedom, July/August, 10-12, whole article
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 363. ETH-1693 (rev 2) · Best answer: C

A client who resettled in the United States 8 months ago brings a small gift to every home visit. The client presses each gift into the social worker's hands at the door and seems anxious when the worker hesitates. The two have not yet talked about the gifts. What response is **MOST** helpful?

**A.** Explain that services continue with or without gifts
**B.** Accept each gift to honor the client's custom
**C.** Ask the client what the gifts are meant to express
**D.** Decline the gifts, citing the agency's no-gifts policy

**Why each option is right or wrong**

**A** **Incorrect** (trap: Skips assessment). This reassurance is exactly right if the client fears that benefits depend on gifts, a fear common among people who have dealt with officials who expected them. Offered before the client explains anything, it guesses at the motive and may confuse or embarrass a client who gives out of courtesy or gratitude.

**B** **Incorrect** (trap: Boundary slip or collusion). Small gifts with cultural meaning can sometimes be accepted. Accepting every gift without knowing why it is given, though, could reinforce an exchange the client feels obliged to keep up, especially if the client believes help depends on it.

**C** **Correct.** A gift's meaning shapes the right response: it may be courtesy, gratitude or a sign that the client fears services depend on pleasing the worker (Reamer, 2020). The client's anxiety at the worker's hesitation suggests something important rides on the gifts, but nothing yet shows what. Asking openly lets the client explain and treats the client as the expert on their own experience and culture (NASW Code 1.05(c)). It beats A because reassuring that services continue answers a fear the client has not voiced and may miss the real meaning.

**D** **Incorrect** (trap: Good but not best). Citing policy may be accurate, but it ignores the client's evident anxiety and may sound like a warning. Any limit on gifts lands better once the worker understands what the gifts mean to the client.

**Takeaway:** Before accepting or declining a client's repeated gifts, learn what they mean to the client; the meaning shapes the response.

**Sources:** NASW Code of Ethics (2021), 1.05(c); Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, gifts and favors
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.5 Power differentials, racial privilege, internalized inferiority/superiority · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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### 364. ETH-0011 (rev 1) · Best answer: A

In the third session, a Black client tells the social worker that an earlier comment, 'You're so articulate,' felt like a racial stereotype. The social worker listens without interrupting until the client finishes. What should the social worker do **NEXT**?

**A.** Acknowledge the impact of the comment and apologize
**B.** Explain that the comment was meant as a compliment
**C.** Offer to transfer the client to a different social worker

**Why each option is right or wrong**

**A** **Correct.** When a client names a microaggression, repair starts with taking responsibility for the impact rather than defending intent. Acknowledging the harm and apologizing models accountability and protects the alliance (Sue et al., 2007; NASW Code 1.05(c)). The social worker can then invite more feedback and reflect on their own bias outside the session. It beats B because explaining good intentions shifts attention to the social worker's feelings and implies the client misread the comment, which compounds the harm.

**B** **Incorrect** (trap: Good but not best). Explaining intent centers the social worker and tells the client their perception is wrong. Praise that implies surprise at a Black

person's verbal skill is a recognized microaggression, whatever the intent (Sue et al., 2007).

**C** **Incorrect** (trap: Hands off too soon). Offering a transfer avoids the repair work and can signal that the relationship cannot survive honest feedback. A transfer can be discussed later if the client wants one, but it is a poor first response to feedback.

**Takeaway:** When a client names a microaggression, acknowledge the impact and take responsibility first; don't defend intent.

**Sources:** Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2007). Racial microaggressions in everyday life: Implications for clinical practice. American Psychologist, 62(4), 271-286, whole article; NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 365. ETH-0118 (rev 1) · Best answer: C

At intake, a social worker asked a client with a Spanish surname whether they needed an interpreter. The client, who grew up in the United States and speaks only English, said no and was quiet for the rest of the session. In supervision afterward, the social worker identifies the question as an assumption based on the client's name. What is the social worker's **BEST** action?

**A.** Offer the client a transfer to a bilingual colleague at the agency
**B.** Wait to see if the client brings up the question
**C.** Acknowledge the question's impact with the client next session

#### Why each option is right or wrong

**A** **Incorrect** (trap: Hands off too soon). Offering a transfer assumes the client wants a worker who shares a presumed identity, repeating the original assumption. It also avoids the repair the relationship needs. A transfer fits if the client asks for one.

**B** **Incorrect** (trap: Good but not best). Waiting leaves the client to raise a painful moment with the person who caused it, which many clients will not do. The rupture may lead to dropout before the client ever brings it up.

**C** **Correct.** A microaggression can harm the alliance whether or not it was intended, and the social worker, not the client, is responsible for repair (Sue et al., 2007). The client's withdrawal after the question signals a rupture. Raising it directly, owning the assumption and inviting the client's reaction practices cultural humility and self-correction (NASW Code 1.05(c)). It beats B because waiting puts the burden on the client, who may simply not return; open repair of ruptures supports a stronger alliance (Safran & Muran, 2000).

**Takeaway:** After a microaggression, the social worker initiates repair: acknowledge the impact, take responsibility and invite the client's response.

**Sources:** Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2007). Racial microaggressions in everyday life: Implications for clinical practice. American Psychologist, 62(4), 271-286, whole article; NASW Code of Ethics (2021), 1.05(c); Safran, J. D., & Muran, J. C. (2000). Negotiating the Therapeutic Alliance: A Relational Treatment Guide. Guilford Press, rupture and repair
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 366. ETH-1577 (rev 1) · Best answer: D

A hospital social worker meets with a Black adult patient admitted during a sickle cell pain crisis. The patient says pain medicine has been delayed and that a nurse called the patient 'drug-seeking.' The chart shows pain scores at the top of the scale for 6 hours. What should the social worker do **FIRST**?

**A.** Help the patient file a grievance with patient relations
**B.** Explain how the unit decides when to give pain medicine
**C.** Coach the patient to describe the pain calmly to staff
**D.** Bring the untreated pain to the care team now

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). A grievance is a valid option if the patient wants one, and the social worker can help with it later. Filing first leaves severe pain untreated while the complaint is reviewed.

**B** **Incorrect** (trap: Good but not best). Explaining unit procedures defends the delay rather than addressing it. The patient's report and the chart point to a treatment gap that needs action, not an explanation of policy.

**C** **Incorrect** (trap: Too directive). Coaching the patient to stay calm shifts responsibility for the problem onto the patient. Self-advocacy support can come later, but the care team, not the patient, needs to act on the untreated pain.

**D** **Correct.** Implicit bias can lead clinicians to underrate and undertreat pain in Black patients; in one study, medical trainees who held false beliefs about biological differences rated a Black patient's pain lower (Hoffman et al., 2016). With top-of-scale pain for hours and a stigmatizing label in play, the first step is to advocate with the team so that pain is reassessed and treated (NASW Code 1.05(b)). It beats A because a grievance documents the problem but leaves the patient in pain during review; it can follow if the patient wants it.

**Takeaway:** When bias may be driving undertreatment, advocate within the care team for prompt reassessment before pursuing formal complaints.

**Sources:** Hoffman, K. M., Trawalter, S., Axt, J. R., & Oliver, M. N. (2016). Racial bias in pain assessment and treatment recommendations, and false beliefs about biological differences between blacks and whites. PNAS, 113(16), 4296-4301, whole article; NASW Code of Ethics (2021), 1.05(b)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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### 367. ETH-1580 (rev 2) · Best answer: C

In a telehealth session, a Black client describes months of coworkers touching the client's hair, asking whether the client 'got the job through a program,' and joking about the client's 'attitude.' The client now sleeps poorly and dreads Mondays. The client asks, 'Am I being too sensitive?' What is the **MOST** appropriate response by the social worker?

**A.** Help the client test whether the coworkers really meant harm
**B.** Encourage the client to report each incident to human resources
**C.** Affirm that repeated slights can cause real stress and harm

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Weighing the evidence for a thought helps with distorted beliefs, but the client is describing a repeated, real pattern. Testing the coworkers' intent implies the client misread events and can deepen the harm.

**B** **Incorrect** (trap: Too directive). Reporting may become a good option, and the social worker can help the client weigh it. Advising it now, before validating the experience or learning the client's goals, is premature and directive.

**C** **Correct.** Microaggressions are brief, everyday slights that send demeaning racial messages, whether intended or not, and their cumulative effect can harm well-being (Sue et al., 2007). The client describes a repeated pattern with clear effects on sleep and mood and asks whether the reaction is valid. Affirming the harm validates the experience and opens the way to assessment and coping. It beats A because questioning the coworkers' intent recasts an accurate perception as a thinking error, which can itself feel dismissive (NASW Code 1.05(b)).

**Takeaway:** When a client describes repeated microaggressions, validate the harm first; do not treat accurate perceptions of bias as distortions.

**Sources:** Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2007). Racial microaggressions in everyday life: Implications for clinical practice. American Psychologist, 62(4), 271-286, whole article; NASW Code of Ethics (2021), 1.05(b)

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## 368. ETH-1583 (rev 2) · Best answer: A

A client tells a social worker about being the sole Asian American employee on a team and being left out of meetings. A friend responded, 'I don't see race. People are just people.' Under Sue and colleagues' taxonomy, the friend's comment is **MOST** likely which type of microaggression?

**A.** Microinvalidation

**B.** Microinsult

**C.** Microassault

**Why each option is right or wrong**

**A** **Correct.** Sue et al. (2007) describe microinvalidations as messages that exclude, negate or nullify the thoughts, feelings or experiences of people of color; claiming not to see race is a central example. The friend's comment denied that race could have shaped the client's exclusion from meetings. It beats B because microinsults convey rudeness and demean a person's racial heritage, such as implying someone was hired because of race; the friend's comment erases the racial experience rather than demeaning it.

**B** **Incorrect** (trap: Wrong model or concept). A microinsult subtly demeans a person's racial identity, such as implying someone got a job through a quota. The friend did not demean the client's heritage but denied that race mattered at all.

**C** **Incorrect** (trap: Wrong model or concept). A microassault is an explicit, usually deliberate racial attack, such as a slur or purposeful exclusion. The friend's comment was not openly hostile and was likely meant to be supportive.

**Takeaway:** Color-blind statements that deny the role of race are microinvalidations: they negate the racial experiences of people of color.

**Sources:** Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2007). Racial microaggressions in everyday life: Implications for clinical practice. American Psychologist, 62(4), 271-286, taxonomy of racial microaggressions

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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## 369. ETH-1586 (rev 2) · Best answer: C

A school social worker reviews a year of the worker's own referral records. Black students were referred for behavior evaluations at about twice the rate of white students with similar incident reports. The worker has already flagged the five most recent referrals. What should the social worker do **NEXT**?

**A.** Re-review the flagged referrals alone and withdraw biased ones

**B.** Take an online implicit association test to gauge the bias

**C.** Review the referrals and the pattern with a supervisor

**D.** Raise the referral threshold for Black students going forward

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Rechecking one's own decisions is a sound instinct, but implicit bias works outside awareness, so a solo review may miss it. Withdrawing referrals without an outside check could also deny evaluations to students who need them.

**B** **Incorrect** (trap: Right action, wrong time). A self-test can raise awareness and may belong in a later plan for growth. It measures associations rather than correcting decisions already affecting students, so reviewing the flagged referrals with an outside check comes first.

**C** **Correct.** The Code calls for critical self-reflection and self-correction about bias, and for seeking colleagues' counsel when it serves clients (NASW Code 1.05(c); 2.05(a)). Implicit bias operates outside awareness, so a worker reviewing their own decisions alone may repeat it. The worker has found a consistent disparity and flagged recent cases; reviewing them with a supervisor adds an outside check, helps correct affected referrals and builds a plan to prevent recurrence. It beats A because withdrawing referrals on one's own relies on the same judgment that produced the pattern.

**D** **Incorrect** (trap: Misreads law or ethics). A different standard by race replaces one disparity with another and may harm Black students who need evaluation. The remedy is a consistent, criteria-based process reviewed with others.

**Takeaway:** When a worker finds bias in their own decisions, an outside review with a supervisor or consultant should guide the correction.

**Sources:** NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 2.05(a)

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### 370. ETH-1589 (rev 1) · Best answer: B

In a residential treatment group for adolescents, one member says to an Asian American member, 'Your English is really good for someone like you.' The Asian American member laughs and looks down. Other members glance at the social worker leading the group. What is the social worker's **BEST** response?

**A.** Ask the Asian American member to share how the comment felt
**B.** Pause the group and name the comment as a stereotype
**C.** Talk with the member who made the comment after group
**D.** Continue the discussion, since the member laughed it off

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misuses the technique). Inviting feelings is often useful, but asking the targeted member to explain the hurt in front of peers puts the burden of response on that member. Support for the member can follow privately once the leader has addressed the comment.

**B** **Correct.** Microinterventions by a leader or bystander include making the hidden message visible and interrupting it, which protects the target and sets group norms (Sue et al., 2019). The member's laugh and lowered gaze, and the group's glances, show the comment landed. Pausing to name it as a stereotype that the member is foreign addresses the harm where it happened and shows the group how such comments are handled. It beats C because a private talk can educate the speaker but leaves the target and the group without a response.

**C** **Incorrect** (trap: Right action, wrong time). A private conversation with the speaker can help and may follow. On its own it leaves the targeted member and the group with silence, which can read as approval.

**D** **Incorrect** (trap: Good but not best). Laughing it off is a common way targets cope with a microaggression, not a sign it caused no harm. Moving on leaves the stereotype unchallenged in front of the group.

**Takeaway:** When a microaggression occurs in a group, the leader interrupts it in the moment rather than leaving the target to respond.

**Sources:** Sue, D. W., Alsaidi, S., Awad, M. N., Glaeser, E., Calle, C. Z., & Mendez, N. (2019). Disarming racial microaggressions: Microintervention strategies for targets, White allies, and bystanders. American Psychologist, 74(1), 128-142, microintervention strategies: make the invisible visible
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 371. ETH-1591 (rev 3) · Best answer: A

On an inpatient psychiatric unit, a Black adult patient is admitted with psychotic symptoms after a week without sleep. The patient is guarded with staff and tells the social worker about being restrained during a past emergency visit while a white patient who was shouting was not. In rounds, the team cites the patient's distrust of staff as paranoia that supports a schizophrenia diagnosis. What should the social worker do **FIRST**?

**A.** Point out that the distrust may reflect experiences of racism
**B.** Record the restraint account in the psychosocial assessment
**C.** Question the patient further to see if the distrust is a fixed belief

**Why each option is right or wrong**

**A** **Correct.** Implicit bias can lead clinicians to read a Black patient's distrust, often rooted in real experiences of discrimination, as a symptom. Black patients receive psychotic disorder diagnoses at three to four times the rate of white patients, and reviews link the gap in part to clinician bias and to distrust that colors clinical judgment (Schwartz & Blankenship, 2014). The patient describes a specific, plausible experience of unequal treatment, so naming this context in rounds, before the diagnosis is set, lets the team weigh the distrust accurately (NASW Code 1.05(b)). It beats C because probing a credible account as a possible delusion repeats the bias at issue.

**B** **Incorrect** (trap: Right action, wrong time). Documenting the account matters and should happen, but it can follow. A note the team may not read before the diagnosis is set leaves a possibly biased reading of the distrust in place.

**C** **Incorrect** (trap: Misuses the technique). Checking whether a belief is fixed and implausible is part of assessing possible delusions. This account is specific, plausible and tied to a real event, so testing it as a symptom treats a report of unequal treatment as pathology, the very error at issue.

**Takeaway:** When a client's distrust may stem from experiences of racism, raise that context with the team before it is counted as a psychotic symptom.

**Sources:** Schwartz, R. C., & Blankenship, D. M. (2014). Racial disparities in psychotic disorder diagnosis: A review of empirical literature. World Journal of Psychiatry, 4(4), 133-140, whole article; NASW Code of Ethics (2021), 1.05(b)

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 372. ETH-1594 (rev 2) · Best answer: A

Which statement about implicit bias is **MOST** accurate?

**A.** It can shape judgments even in people with egalitarian beliefs
**B.** It is prejudice that people knowingly hide from others
**C.** It can be lastingly reduced by a brief training session

**Why each option is right or wrong**

**A** **Correct.** Implicit bias refers to associations that operate outside conscious awareness and can influence judgment and behavior, even in people who sincerely hold egalitarian views (Greenwald & Banaji, 1995). This is why good intentions alone do not prevent biased decisions and why structured, reviewed processes matter. It beats C because a large test of brief interventions found that shifts in implicit preferences faded within hours to days (Lai et al., 2016).

**B** **Incorrect** (trap: Wrong model or concept). Prejudice that a person is aware of and conceals is explicit bias. Implicit bias operates outside awareness, so people may not know they hold it.

**C** **Incorrect** (trap: Wrong model or concept). Brief interventions can shift measured implicit preferences right away, but research found the effects faded within days. Lasting change relies on sustained effort and structured decision processes.

**Takeaway:** Implicit bias operates outside awareness and can affect decisions despite good intentions; brief training does not durably remove it.

**Sources:** Greenwald, A. G., & Banaji, M. R. (1995). Implicit social cognition: Attitudes, self-esteem, and stereotypes. Psychological Review, 102(1), 4-27, whole article; Lai, C. K., et al. (2016). Reducing implicit racial preferences: II. Intervention effectiveness across time. Journal of Experimental Psychology: General, 145(8), 1001-1016, whole article

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** MOST · **Cognitive** recall · **Difficulty** 1 of 3

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## 373. ETH-1606 (rev 1) · Best answer: D

A 68-year-old transgender woman on a hospital medical unit has stopped eating her meals and refuses physical therapy. She tells the social worker that staff keep calling her 'sir' and using her former name, which is printed on her wristband. She asks the social worker for help and agrees that her care team can be told. What should the social worker do **NEXT**?

**A.** Ask the patient to describe how the comments affect her

**B.** Request a psychiatric consult for her refusals and poor intake

**C.** Coach the patient to correct staff each time it happens

**D.** Ask the unit to update her name and pronouns and brief staff

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Exploring impact is often a good early step. Here the patient has already described the harm, tied it to her withdrawal and asked for help, so more exploration delays the change she requested.

**B** **Incorrect** (trap: Skips assessment). Refusals and poor intake deserve attention, and a psychiatric consult would fit if they persisted after the cause was addressed. Treating them first as a mental health problem ignores the clear cause she has named.

**C** **Incorrect** (trap: Good but not best). Self-advocacy skills can help, but asking a hospitalized patient to correct staff every time places the work of change on the person being harmed. The hospital is responsible for fixing its records and practices.

**D** **Correct.** Misgendering and use of a former name are microaggressions that signal a person's identity is not respected, and they can drive patients away from care (Nadal et al., 2012). The patient has named the harm, linked it to her withdrawal and agreed that the team may be told, so the next step is to fix the source: the record and staff practice. Holding institutions accountable is part of the social worker's role (NASW Code 1.05(c)). It beats C because coaching leaves the burden of repeated correction on the patient.

**Takeaway:** When staff misgender a patient, fix the source by correcting records and staff practice rather than leaving correction to the patient.

**Sources:** Nadal, K. L., Skolnik, A., & Wong, Y. (2012). Interpersonal and systemic microaggressions toward transgender people: Implications for counseling. Journal of LGBT Issues in Counseling, 6(1), 55-82, whole article; NASW Code of Ethics (2021), 1.05(c)

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 374. ETH-1678 (rev 1) · Best answer: C

At the social worker's invitation, a 15-year-old Black adoptee in a family session describes being followed through a store by a security guard. The white adoptive parent responds, 'You're overthinking it. In this family, we don't see color.' The teen looks away and stops talking. What should the social worker do **NEXT**?

**A.** Meet with the parent alone later to address the color-blind view
**B.** Help the teen plan ways to respond calmly to store security
**C.** Slow the exchange and help the parent hear the teen's experience

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Coaching the parent privately about color-blind parenting may be valuable, and it can follow. Waiting until then leaves the teen's account dismissed in front of the worker, which can confirm the teen's choice to go silent.

**B** **Incorrect** (trap: Good but not best). Planning for encounters with security may matter later, if the teen wants it. Offered now, it skips past the parent's dismissal and implies the teen must manage the problem alone, which can feel like a second invalidation.

**C** **Correct.** A color-blind reply to a report of racial profiling is a microinvalidation: it dismisses the person's experience of race, and from a parent it can teach a child to stop sharing (Sue et al., 2007). The teen's withdrawal shows the harm is happening now, so the worker steps in, slowing things down and helping the parent listen without correcting (NASW Code 1.05(b)). It beats A because a later private meeting may help the parent, but the teen would leave this session having learned that raising race shuts the family down.

**Takeaway:** When a family member dismisses a client's experience of racism in session, intervene in the moment so the client's account is heard.

**Sources:** Sue, D. W., Capodilupo, C. M., Torino, G. C., Bucceri, J. M., Holder, A. M. B., Nadal, K. L., & Esquilin, M. (2007). Racial microaggressions in everyday life: Implications for clinical practice. American Psychologist, 62(4), 271-286, microinvalidation: color blindness; NASW Code of Ethics (2021), 1.05(b)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 375. ETH-1684 (rev 1) · Best answer: D

A first-generation college student meets with a campus social worker after failing a math placement exam. The student earned A's in high school math and scores well on practice tests at home. Just before the exam, an advisor remarked that students 'from your neighborhood' usually need remedial math. The student says their mind went blank. Which concept **MOST** likely explains the poor score?

**A.** Impostor phenomenon
**B.** Learned helplessness
**C.** Social anxiety disorder
**D.** Stereotype threat

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Impostor feelings involve doubting one's own competence despite clear success and fearing exposure as a fraud. Nothing suggests a long-standing sense of fraud, and the failure followed a specific remark about the student's group.

**B** **Incorrect** (trap: Wrong model or concept). Learned helplessness follows repeated uncontrollable failures that lead a person to stop trying. This student has a record of success and was trying hard; the drop was sudden and tied to the exam setting.

**C** **Incorrect** (trap: Near-miss diagnosis). Social anxiety disorder centers on fear of scrutiny across social situations, typically lasting 6 months or more (DSM-5-TR). One exam-day episode after a biased remark does not support it.

**D** **Correct.** Stereotype threat is the risk of confirming a negative stereotype about one's group; when the stereotype is made salient just before a test, that concern can lower performance (Steele & Aronson, 1995). The advisor's remark cast the student's group as weak in math right before the exam, and the student does well when no such cue is present. It beats A because impostor feelings are a lasting sense of being a fraud despite success, not a drop tied to a group stereotype raised at test time.

**Takeaway:** A negative group stereotype raised before a task can depress performance; recognize stereotype threat instead of attributing the result to ability.

**Sources:** Steele, C. M., & Aronson, J. (1995). Stereotype threat and the intellectual test performance of African Americans. Journal of Personality and Social Psychology, 69(5), 797-811, whole article; DSM-5-TR (APA, 2022), Anxiety Disorders: Social Anxiety Disorder

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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## 376. ETH-1689 (rev 1) · Best answer: C

A social worker notices that the clinic's intake coordinator routes every Spanish-speaking caller to case management only, noting 'not suited for therapy,' even when callers ask for counseling. The clinic has bilingual therapists with openings. The social worker raised the pattern privately with the coordinator, who replied, 'It's what works for those clients.' What should the social worker do **NEXT**?

**A.** Talk with the coordinator again about the routing pattern

**B.** Contact the affected callers to offer clinic therapy directly

**C.** Bring the routing pattern to the clinic's program director

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Talking with the colleague is the right first step, and it has been taken. The coordinator dismissed the concern, so repeating it delays action while more callers are steered away from therapy.

**B** **Incorrect** (trap: Good but not best). Reaching out to affected callers may become part of the fix, but doing it alone bypasses intake and leaves the biased routing in place for future callers. The program director can correct the process and arrange outreach.

**C** **Correct.** When a colleague acts unethically, social workers first discuss it with the colleague when that is likely to help, then take action through appropriate formal channels (NASW Code 2.10(c)-(d)). Routing callers by language rather than request

denies Spanish speakers the therapy others receive, a biased practice social workers should not condone (4.02). The private talk failed, so the next step is the program director, who oversees intake. It beats A because a second talk repeats a step the coordinator already rejected.

**Takeaway:** If a colleague's biased practice continues after a direct conversation, take it to the appropriate formal channel, such as the program director.

**Sources:** NASW Code of Ethics (2021), 2.10(c); NASW Code of Ethics (2021), 2.10(d); NASW Code of Ethics (2021), 4.02

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 1 of 3

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### 377. ETH-1694 (rev 1) · Best answer: C

A school social worker notices that a 9-year-old who recently arrived from Nigeria has started asking classmates to call them 'Sam' instead of their Yoruba name. Since the first week, the teacher has shortened the name, saying it is 'too hard to say.' The child seems quieter in class. What should the social worker do **FIRST**?

**A.** Ask the teacher to use the child's full Yoruba name

**B.** Lead a class lesson on names and their meanings

**C.** Ask the child privately about the name they want and why

**D.** Encourage the child to correct the teacher's pronunciation

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Talking with the teacher is likely needed, and soon. Asking for the full name before hearing from the child risks overriding the child's own preference, which may have been chosen freely.

**B** **Incorrect** (trap: Good but not best). A lesson that values names can improve the class climate later. Done first, it may spotlight a child who is already withdrawing, without knowing what the child wants.

**C** **Correct.** Repeatedly shortening a student's name because it seems hard to say is a racial microaggression that can teach children their identity is a burden (Kohli & Solórzano, 2012). Still, the new nickname could be a free choice or a response to that message, and only the child can say which. Asking privately treats the child as the expert on their own identity (NASW Code 1.05(c)). It beats A because pressing the teacher to use the full name could override the child's choice; that step should follow what the child says.

**D** **Incorrect** (trap: Too directive). Asking a quiet 9-year-old to correct an adult puts the burden of the teacher's behavior on the child and may add stress. Self-advocacy coaching can come later, with support, if the child wants it.

**Takeaway:** When a microaggression may have shaped a child's choices, ask the child what they want before acting on their behalf.

**Sources:** Kohli, R., & Solórzano, D. G. (2012). Teachers, please learn our names!: Racial microaggressions and the K-12 classroom. Race Ethnicity and Education, 15(4), 441-462, whole article; NASW Code of Ethics (2021), 1.05(c)

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.6 Privilege, implicit bias, microaggressions and their client impact · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 378. ETH-1625 (rev 1) · Best answer: B

At a drop-in center for youth experiencing homelessness, a 16-year-old who left home after coming out as gay tells the social worker about staying with adults who offer a bed in exchange for sex. The youth says, 'Nobody makes me. It's my choice.' No third party arranges the meetings. Under federal law, how is this situation classified?

**A.** Survival sex, since no one else arranges or profits

**B.** Sex trafficking of a minor

**C.** Trafficking if force or coercion can be shown

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Survival sex describes trading sex to meet basic needs such as food or shelter. It names the youth's motive, not the legal status. Federal law needs no third-party profiteer: an adult who obtains or patronizes a minor for a sex act in exchange for a bed is trafficking.

**B** **Correct.** Federal law defines a commercial sex act as any sex act exchanged for anything of value. It treats a person under 18 induced into such an act as a victim of a severe form of trafficking, with no need to prove force, fraud or coercion (22 U.S.C. 7102(4), (11)(A)). A bed is something of value, and the youth is 16. It beats A because 'survival sex' describes why the youth agrees, not how the law classifies it. Treating the exchange as a choice can leave a marginalized youth without protection, and the Code calls for action against exploitation (NASW Code 6.04(d)).

**C** **Incorrect** (trap: Misreads law or ethics). Force, fraud or coercion must be shown when the person in commercial sex is an adult. For anyone under 18, federal law drops that element, so the youth's description of the exchange as a personal choice does not change the classification.

**Takeaway:** A minor who trades sex for anything of value, including a place to sleep, is a trafficking victim under federal law; no force must be shown.

**Sources:** Trafficking Victims Protection Act of 2000, 22 U.S.C. 7102(4), (11)(A); NASW Code of Ethics (2021), 6.04(d)

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 379. ETH-1629 (rev 2) · Best answer: B

A social worker at an urban domestic violence program completes intake with a client who identifies as Diné (Navajo) and as Mexican American. The agency database allows more than one race, but staff usually copy the single race on the police report, which lists 'White.' The program director is preparing a report on violence against Indigenous clients. What is the **BEST** way to complete the race entry?

**A.** Copy the race listed on the police report

**B.** Enter each race the client names, plus tribal nation

**C.** Record the Native identity alone so the report counts the client

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Consistency with outside records sounds efficient, but copying a police entry repeats a known source of error. Police and agency records often misclassify Native people, which undercounts the violence they face and weakens the case for services and funding.

**B** **Correct.** Clients are the experts on their own identity (NASW Code 1.05(c)), and misclassification hides violence against Indigenous people. A national review of cases of missing and murdered Indigenous women found records that listed Native victims as White and police systems that could not search by Native identity (Urban Indian Health Institute, 2018). Recording each identity the client reports, with the tribal nation, keeps the record accurate and lets the director's report count this client. It beats C because recording the Native identity alone serves the report but overrides how the client identifies; self-identification, recorded in full, serves both accuracy and the count.

**C** **Incorrect** (trap: Overrides the client's choice). Counting Indigenous clients accurately is the right goal, which makes this tempting. Choosing which identity to record, though, puts the agency's purpose ahead of the client's self-identification and erases the Mexican American identity the client also named; an accurate count comes from recording all of it.

**Takeaway:** Record race, ethnicity and tribal affiliation as clients self-identify; misclassifying Indigenous people hides the violence they experience.

**Sources:** Urban Indian Health Institute (2018). Missing and Murdered Indigenous Women & Girls: A snapshot of data from 71 urban cities in the United States, Findings: racial misclassification; Recommendations; NASW Code of Ethics (2021), 1.05(c)

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 380. ETH-1632 (rev 1) · Best answer: D

A university team plans to study blood samples that members of a tribal community gave years ago for diabetes research. The new study would analyze the samples for population ancestry. The samples are de-identified, and the university's review board has ruled that new consent is not required. The social worker on the team knows the community has had samples used in ways members never agreed to. What is the **MOST** appropriate step?

**A.** Proceed, as the review board has approved the plan

**B.** Contact the original donors to ask for individual consent

**C.** Share study results with tribal leaders once analysis ends

**D.** Seek the tribal government's approval before any reuse

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Review board approval meets the regulatory minimum, and de-identified data often fall outside consent rules. Legal permission does not settle the ethical question here, where reuse for ancestry analysis mirrors past harm and the tribe has its own authority over research on its people.

**B** **Incorrect** (trap: Good but not best). Asking donors is more respectful than proceeding without notice, but ancestry findings affect the whole community, not only the

individual donors. Tribal government approval comes first; individual consent can follow if the tribe agrees to the study.

**C** **Incorrect** (trap: Right action, wrong time). Returning results to the community is good practice in community-engaged research. Sharing them only after the analysis presents a finished decision and repeats the pattern of acting first and informing the community later.

**D** **Correct.** Research exploitation has lasting effects on Indigenous communities. In the Havasupai case, blood given for diabetes research was later used for other studies, including ancestry work, without the tribe's agreement, and the dispute ended in a 2010 settlement (Mello & Wolf, 2010). Tribal nations are sovereign governments and may set their own research approval rules. It beats B because individual consent, though respectful, bypasses the tribe's collective authority over research about shared ancestry. The Code asks researchers to weigh consequences and follow guidelines that protect participants (NASW Code 5.02(d)).

**Takeaway:** When research touches a community harmed by past research exploitation, seek collective approval, such as tribal government consent, beyond minimum legal review.

**Sources:** Mello, M. M., & Wolf, L. E. (2010). The Havasupai Indian tribe case: Lessons for research involving stored biologic samples. New England Journal of Medicine, 363(3), 204-207, whole article; NASW Code of Ethics (2021), 5.02(d)

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 381. ETH-1636 (rev 2) · Best answer: A

At an urban Indian health center, a family tells the social worker that their 19-year-old relative has not been seen or heard from in 2 days, which is out of character. When they called police, an officer said young adults often leave on their own and told them to call back in 3 days. The family asks for help. What should the social worker do **FIRST**?

**A.** Help the family file a missing person report today
**B.** Help the family post a missing notice on social media
**C.** Advise the family to wait 3 days while calling friends
**D.** Help the family call area hospitals and jails

#### Why each option is right or wrong

**A** **Correct.** Federal law requires states to ensure that police impose no waiting period on reports of missing persons under 21 and that reports enter national databases promptly (34 U.S.C. 41307, 41308). Indigenous people who go missing are often dismissed as runaways or left out of police records, which delays searches (Urban Indian Health Institute, 2018). The 19-year-old's silence is out of character, so the report should be taken now. It beats D because calling hospitals and jails is a sensible step that can run alongside, but only an official report starts the database entry and the police search.

**B** **Incorrect** (trap: Right action, wrong time). Community alerts are a common and useful tool, and Native families often lead their own searches. They should follow, not replace, an official report, which starts database entries and police duties.

**C** **Incorrect** (trap: Ignores risk). No waiting period applies to a missing person this age, and delay loses time when early action matters most. Accepting the officer's advice repeats a pattern that leaves Indigenous missing persons cases unrecorded.

**D** **Incorrect** (trap: Right action, wrong time). Checking hospitals and jails is a common early step, and it may quickly locate the young person. Doing it before the report, though, lets another day pass without the database entry and police duties that federal law says should start without a waiting period.

**Takeaway:** When a young person goes missing, press for an immediate police report; no waiting period applies, and delays fall hardest on Indigenous families.

**Sources:** National Child Search Assistance Act of 1990 (as amended), 34 U.S.C. 41307, 41308; Urban Indian Health Institute (2018). Missing and Murdered Indigenous Women & Girls: A snapshot of data from 71 urban cities in the United States, Findings: underreporting in law enforcement records
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 382. ETH-1639 (rev 1) · Best answer: A

A social worker at a residential substance use program is writing an intake summary for a 24-year-old client. The client describes being made to sell sex by an older partner starting at age 16. An old court record lists 'juvenile prostitution arrest, age 16.' What is the **MOST** appropriate way to document this history?

**A.** Describe it as sexual exploitation that began at age 16
**B.** Copy the court record's wording for consistency
**C.** Leave the history out to protect the client from future stigma

#### Why each option is right or wrong

**A** **Correct.** Under federal law, a minor induced into commercial sex is a trafficking victim, whatever an old charge said (22 U.S.C. 7102(11)(A)). The Code calls for accurate, respectful language in all communication about clients (NASW Code 1.12). Describing the history as exploitation that began at 16 is accurate and nonstigmatizing, and it flags trauma that matters for treatment. It beats C because the history bears on substance use and trauma care; leaving it out gives up accuracy to avoid a stigma that better wording already prevents.

**B** **Incorrect** (trap: Misreads law or ethics). Matching outside records can seem objective, but the court label treats a 16-year-old trafficking victim as an offender. Copying it carries a stigmatizing frame, at odds with federal law's view of minors as victims, into the treatment record.

**C** **Incorrect** (trap: Good but not best). Protecting clients from stigma is a valid goal, and records should hold only relevant details. This history bears directly on trauma and substance use treatment, so it belongs in the record in respectful terms.

**Takeaway:** Document the exploitation of a minor as victimization in accurate, respectful terms, not with stigmatizing labels carried over from old records.

**Sources:** Trafficking Victims Protection Act of 2000, 22 U.S.C. 7102(11)(A); NASW Code of Ethics (2021), 1.12
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 383. ETH-1642 (rev 1) · Best answer: A

A school social worker meets with a 16-year-old who arrived in the country alone and now lives with an adult sponsor. The teen falls asleep in class. After gentle questions, the teen explains working nights at a factory 6 days a week to repay a large travel debt to the sponsor, who keeps the teen's identity papers. The teen adds, 'In my country, young people help the family.' What should the social worker do **NEXT**?

**A.** Contact the child abuse hotline about suspected trafficking
**B.** Meet with the sponsor to discuss cutting the work hours
**C.** Ask the teen more about family expectations back home
**D.** Treat the work as a cultural norm and adjust school supports

**Why each option is right or wrong**

**A** **Correct.** A minor working long night hours to repay a debt to a sponsor who holds the teen's identity papers shows recognized signs of labor trafficking; debt bondage is named in the federal definition (22 U.S.C. 7102(7), (11)(B)). Federal law treats a child identified as a trafficking victim as a victim of child abuse (42 U.S.C. 5106g(b)(1)), and reasonable suspicion calls for a report to the child abuse hotline run by child protective services. It beats C because the teen has already described the work and its cultural meaning; family values about helping do not explain withheld papers and a debt owed to the sponsor.

**B** **Incorrect** (trap: Ignores risk). Talking with caregivers is often part of addressing a teen's overwork. Here the sponsor holds the debt and the papers, so alerting the sponsor could lead to threats, moving the teen or hiding evidence before child protective services can act.

**C** **Incorrect** (trap: Right action, wrong time). Exploring cultural meaning is respectful when facts are unclear. The teen has already described the debt, the hours and who holds the papers, so more questions repeat a finished step, delay protection and can feel like disbelief.

**D** **Incorrect** (trap: Skips assessment). Many families expect teens to contribute income, and that alone is not abuse. A debt owed to the sponsor and withheld identity papers go beyond a cultural norm and point to exploitation of a youth whose migration status adds vulnerability.

**Takeaway:** A working minor who owes a debt to a caregiver who holds the minor's papers shows trafficking signs; report to child protection without alerting the caregiver.

**Sources:** Trafficking Victims Protection Act of 2000, 22 U.S.C. 7102(7), (11)(B); Child Abuse Prevention and Treatment Act, 42 U.S.C. 5106g(b)(1)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

### 384. ETH-1703 (rev 1) · Best answer: B

A community health center serving mainly Black, Latina and Native clients on Medicaid sets a quarterly target for long-acting contraceptive placements, such as implants and IUDs. Staff who meet it are praised at meetings. A client tells the social worker that a staff member kept pushing an implant after the client asked for pills and explained that a grandparent was sterilized without consent. The client now has the pills. What is the **BEST** action for the social worker?

**A.** Report the staff member's conduct to the clinic manager
**B.** Ask clinic leaders to drop the target for these methods
**C.** Train staff to counsel neutrally while keeping the target

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Raising the staff member's conduct is reasonable, and the manager should know. Stopping there treats the problem as one person's lapse, while the target that rewards placements keeps pressuring every staff member and client.

**B** **Correct.** Given the history of coerced sterilization and contraception aimed at women of color and low-income women, treating long-acting placements as the measure of success pressures staff to steer clients and erodes trust (Gómez, Fuentes & Allina, 2014). This client's experience shows the pressure is already shaping counseling. Social workers act against institutional practices that harm oppressed groups and hold institutions accountable (NASW Code 1.05(b), 1.05(c)). Dropping the target removes the cause and opens the way to measures of choice-centered counseling. It beats C because retraining staff while the reward for placements remains leaves in place the incentive that drove the pushing.

**C** **Incorrect** (trap: Good but not best). Full, neutral counseling is the right standard, and training can support it. While the target and its praise stay, staff still face pressure to steer clients, so training alone leaves the main cause untouched.

**Takeaway:** Targets or rewards for specific contraceptive methods invite coercion, especially where reproductive exploitation has a history; advocate for choice-centered measures instead.

**Sources:** Gómez, A. M., Fuentes, L., & Allina, A. (2014). Women or LARC first? Reproductive autonomy and the promotion of long-acting reversible contraceptive methods. Perspectives on Sexual and Reproductive Health, 46(3), 171-175, whole article; NASW Code of Ethics (2021), 1.05(b); NASW Code of Ethics (2021), 1.05(c)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

### 385. ETH-1709 (rev 1) · Best answer: C

A residential substance use program serves several Native clients. A non-Native staff member began leading smudging and a 'spirit animal' exercise for all residents after watching online videos. Native residents tell the director the activities mock their ceremonies, though two of them want traditional healing as part of treatment. The director stops the activities and asks the social worker for advice. What should the social worker recommend **FIRST**?

**A.** Ban Native cultural practices from the program
**B.** Ask the Native residents to lead smudging for the whole group
**C.** Seek guidance from local tribal leaders on these practices
**D.** Have staff complete training on Native spiritual traditions

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). A ban ends the harmful activity but also denies the residents who asked for traditional healing. Respect for culture means offering these practices properly, not removing them.

**B** **Incorrect** (trap: Boundary slip or collusion). Native residents are experts on their own experience, but leading ceremonies for the program shifts staff work onto clients and may not match who is authorized to lead a ceremony. The program owes the community this effort, not the residents.

**C** **Correct.** Cultural humility treats a community as the authority on its own practices and holds institutions accountable for culturally responsive services (NASW Code 1.05(c)). Ceremonies taken up by outsiders without permission repeat a long pattern of appropriation that the residents named as harmful. Asking local tribal leaders how, and by whom, traditional healing can be offered respects that authority and lets the program meet the two residents' requests through community resources (NASW Cultural Competence Standard 5). It beats D because staff training is a useful later step, but its content and trainers should come from the community.

**D** **Incorrect** (trap: Right action, wrong time). Training can help staff understand Native traditions and avoid repeating the harm. Choosing a training before speaking with tribal leaders risks more outsider-designed content; it fits after consultation, ideally led or approved by the community.

**Takeaway:** When a program misuses another culture's ceremonies, consult that community's cultural authorities before offering or redesigning culturally based practices.

**Sources:** NASW Code of Ethics (2021), 1.05(c); NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 5: Service Delivery
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 386. ETH-1716 (rev 1) · Best answer: B

The Missing and Murdered Indigenous Women and Relatives (MMIW) movement chiefly calls attention to which problem?

**A.** Loss of Native languages caused by past boarding school policies
**B.** Disappearances and killings met by weak official response
**C.** Overrepresentation of Native children in foster care

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Boarding schools did suppress Native languages and are part of the historical trauma many Native communities carry. That history is distinct from the MMIW movement, which focuses on current violence and the official response to it.

**B** **Correct.** The MMIW movement, also called MMIWR or MMIP to include relatives and people, highlights the high rates at which Indigenous women and girls go missing or are killed, along with underreporting, racial misclassification and weak investigation (Urban Indian Health Institute, 2018). Social workers serving Native communities need this context to act quickly on missing-person concerns and to understand community grief and mistrust. It beats C because child welfare overrepresentation is a related injustice that the Indian Child Welfare Act addresses, not the focus of MMIW.

**C** **Incorrect** (trap: Wrong model or concept). Native children are overrepresented in foster care, a problem the Indian Child Welfare Act was passed to address (25 U.S.C. 1901(4)). The MMIW movement focuses instead on violence against Native women and relatives.

**Takeaway:** MMIW highlights disproportionate violence against Indigenous women and relatives and gaps in official response, context for urgent, culturally informed action.

**Sources:** Urban Indian Health Institute (2018). Missing and Murdered Indigenous Women & Girls: A snapshot of data from 71 urban cities in the United States, Findings: underreporting in law enforcement records; racial misclassification; Indian Child Welfare Act of 1978, 25 U.S.C. 1901(4)

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.7 Exploitation's cultural impact on marginalized communities (trafficking, MMIW) · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 387. ETH-1627 (rev 1) · Best answer: A

A middle school uses restorative circles after harm. A 13-year-old painted insults over a classmate's mural and has admitted it. The classmate tells the school social worker, 'I don't want to sit in a room with them.' The principal says the circle is school policy and should go ahead. What is the **MOST** appropriate response?

**A.** Meet with the classmate alone to learn what repair they want

**B.** Encourage the classmate to attend, as meeting aids healing

**C.** Hold the circle with a staff member speaking for the classmate

**Why each option is right or wrong**

**A** **Correct.** Restorative justice centers the needs of the person harmed and relies on voluntary participation (Zehr, 2015). The classmate has clearly declined to meet, and the choice belongs to them (NASW Code 1.02). Meeting separately keeps the work restorative: the social worker learns what the classmate needs, such as a repainted mural or a written apology, and can carry that to the student who caused harm. It beats C because a stand-in speaks for the classmate before anyone knows what the classmate wants, and it still runs the meeting the classmate refused.

**B** **Incorrect** (trap: Overrides the client's choice). Joint meetings can be powerful, and some harmed people later choose them. Urging the classmate to attend after a clear refusal turns a voluntary process into pressure, which can deepen the harm. The option can be revisited if the classmate raises it.

**C** **Incorrect** (trap: Misuses the technique). Surrogates and indirect formats are used in restorative work, but only after the person harmed has said what they need and agreed to the format. Running the circle now satisfies the policy while sidelining the classmate's voice.

**Takeaway:** Restorative processes are voluntary for those harmed; when they decline to meet, use separate or indirect steps built around their needs.

**Sources:** Zehr, H. (2015). The Little Book of Restorative Justice (revised and updated ed.). Good Books, Principles: harms and needs; voluntary participation; NASW Code of Ethics (2021), 1.02
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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### 388. ETH-1630 (rev 1) · Best answer: A

Which question is central to a restorative justice response to wrongdoing?

**A.** Who was harmed, and what do they need?
**B.** Which rule was broken, and what penalty fits?
**C.** How can both sides compromise and move on?

#### Why each option is right or wrong

**A** **Correct.** Zehr frames restorative justice around harms, needs and obligations: who has been hurt, what they need, and whose obligation it is to repair the harm (Zehr, 2015). The process engages those with a stake in the harm, starting with the person harmed. It beats C because restorative justice is not mediation between equal parties; the person who caused harm must accept responsibility, so a mutual compromise misstates its moral footing. B is the question of the retributive model that restorative justice was built to contrast with.

**B** **Incorrect** (trap: Wrong model or concept). This is the core question of a retributive, criminal-justice frame: identify the violation and assign a proportionate punishment. Restorative approaches may run alongside the legal system, but they ask about harm and repair instead.

**C** **Incorrect** (trap: Wrong model or concept). Compromise suits disputes in which both parties share fault, such as a neighbor conflict in mediation. In restorative justice one party has caused harm and must own it, so splitting the difference would minimize the wrong.

**Takeaway:** Restorative justice asks who was harmed, what they need and who must repair it, rather than which rule was broken and what punishment fits.

**Sources:** Zehr, H. (2015). The Little Book of Restorative Justice (revised and updated ed.). Good Books, Restorative justice defined; guiding questions
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 389. ETH-1634 (rev 1) · Best answer: B

A juvenile diversion program refers a 15-year-old for a restorative conference with a store owner whose shop window was broken. At the preparation meeting, the youth tells the social worker, 'I wasn't even there. They just blame me because of my friends.' The store owner is eager to meet. What is the **BEST** way to proceed?

**A.** Hold the conference so the youth hears the owner's losses
**B.** Put the conference on hold while the youth denies involvement
**C.** Let the store owner decide whether the conference goes ahead
**D.** Urge the youth to admit the act to stay in diversion

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Hearing the impact can prompt accountability when a youth minimizes harm. A flat denial is different: the meeting could force an innocent youth to absorb blame, and the owner may be hurt again by hearing a denial in person.

**B** **Correct.** A restorative conference requires the person who caused harm to accept at least some responsibility; without that, the meeting risks hurting the harmed person again and pressuring a youth who may be innocent (Zehr, 2015). The youth flatly denies being there, so preparation shows the conference is not ready. Pausing it and returning the question to the referral source protects both parties. It beats C because the owner's willingness is necessary but cannot supply the responsibility the process needs. Pressing for an admission would also undercut the youth's free choice (NASW Code 1.02).

**C** **Incorrect** (trap: Good but not best). The harmed person's needs are central, and the owner's willingness is required for a conference. It is not enough on its own: the youth must also accept responsibility, so the owner's eagerness cannot decide the question.

**D** **Incorrect** (trap: Overrides the client's choice). Tying diversion to a confession coerces the youth and can produce a false admission. Responsibility in restorative work must be freely accepted; if the youth maintains innocence, the case returns to the usual process.

**Takeaway:** A restorative conference needs the wrongdoer's acknowledgment of responsibility; when the person denies the act, pause rather than proceed or pressure.

**Sources:** Zehr, H. (2015). The Little Book of Restorative Justice (revised and updated ed.). Good Books, Restorative justice is not mediation; offender responsibility; NASW Code of Ethics (2021), 1.02
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

### 390. ETH-1638 (rev 1) · Best answer: B

A county child welfare agency and two tribal nations agree to a joint truth-and-reconciliation effort about decades of Native children placed in non-Native foster homes. Agency leaders want to open with a formal apology and a list of reforms drafted by agency staff. The social worker on the planning group is asked for input. Which opening step is **MOST** consistent with this kind of effort?

**A.** Release the agency's apology and reform list at the first session
**B.** Gather statements from affected families, with tribes co-leading
**C.** Have agency staff review old case files to find errors

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Acknowledgment and apology are part of reconciliation and may come early in some efforts. Issued before affected families have spoken, an apology and agency-written reforms define the harm from the agency's view and risk seeming hollow.

**B** **Correct.** Truth and reconciliation work starts by hearing and documenting the experiences of those harmed, under shared leadership with the affected nations; reconciliation and reform follow from that truth. The Maine Wabanaki-State Child Welfare Truth and Reconciliation Commission gathered statements from Wabanaki people and child welfare staff before making recommendations (Maine Wabanaki-State TRC, 2015). It beats A because an apology and reforms written before families speak let the agency define both the harm and the remedy. Canada's Commission likewise called for child welfare workers to be trained in the history it documented (TRC of Canada, 2015, Call 1(iii)).

**C** **Incorrect** (trap: Good but not best). File review can document patterns and support truth-telling. Done alone by agency staff, it keeps the agency in control of the account and leaves out the families and tribal nations who lived the history.

**Takeaway:** Truth and reconciliation begins with hearing those harmed, under shared leadership; apologies and reforms should follow from that truth.

**Sources:** Maine Wabanaki-State Child Welfare Truth and Reconciliation Commission (2015). Beyond the Mandate: Continuing the Conversation, statement gathering and recommendations; Truth and Reconciliation Commission of Canada (2015). Calls to Action, Call to Action 1(iii)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 391. ETH-1644 (rev 1) · Best answer: B

A 68-year-old client at a county behavioral health clinic says that, as a child, county caseworkers removed the client from a Native family and placed the client with strangers, and the client's siblings were never found. The client has described this in detail over two sessions. Today the client asks the social worker, 'Does the county admit it was wrong?' What should the social worker do **FIRST**?

**A.** Explain how child welfare policies have changed since that time
**B.** Acknowledge that removals like this caused deep, lasting harm
**C.** Invite the client to say more about what the removal cost
**D.** Refer the question to county leaders for an official reply

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Changes such as the Indian Child Welfare Act are worth sharing, and the client may want to hear them. Offered first, they can sound like a defense of the agency and leave the question about wrongdoing unanswered.

**B** **Correct.** Truth and reconciliation starts with honest acknowledgment of harm by the institutions involved. Congress passed the Indian Child Welfare Act after finding that many Native children were removed by nontribal agencies, often without good cause, and placed in non-Native homes (25 U.S.C. 1901(4)). The client has already told the story in detail and now asks a direct question, so a plain acknowledgment of the harm comes first; NASW has likewise apologized for the profession's role in harming people of color (NASW, 2021). It beats A because describing reforms first sounds defensive and sidesteps the question.

**C** **Incorrect** (trap: Good but not best). Inviting more of the story is useful when a client has not yet been heard. This client has already described the loss over two sessions and now asks a direct question, so more exploration sidesteps it.

**D** **Incorrect** (trap: Hands off too soon). County leaders may give an official position later, and the client can be told how to ask. Handing off the question now leaves the client without an honest answer from the person in the room.

**Takeaway:** When a client asks whether an institution's past harm is admitted, answer honestly and acknowledge the harm before explaining reforms.

**Sources:** Indian Child Welfare Act of 1978, 25 U.S.C. 1901(4); NASW (2021). Undoing Racism Through Social Work: NASW Report to the Profession on Racial Justice Priorities and Action, apology
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 392. ETH-1705 (rev 1) · Best answer: C

At a county behavioral health clinic, a staff member made a mocking comment about a colleague's tribal ceremony during a team meeting. The staff member has admitted it and wants to make amends, and the colleague has agreed to a restorative circle. The clinic asks the social worker, a trained facilitator, to lead it. What should the social worker do **FIRST**?

**A.** Convene the circle and open with shared ground rules
**B.** Ask the staff member to write an apology to the colleague
**C.** Meet separately with each person to prepare

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Shared agreements are a standard opening for a circle. Setting them before meeting each party alone skips the step that tests readiness and lets the harmed colleague shape the process.

**B** **Incorrect** (trap: Right action, wrong time). A written apology may become part of the repair, but deciding its form before hearing from the colleague puts the outcome ahead of the harmed person's needs. Agreements should come out of the restorative process.

**C** **Correct.** Restorative processes depend on preparation: the facilitator meets each party separately before any joint meeting to hear their experience, explain the process, check readiness and confirm that participation is voluntary (Office for Victims of

Crime, 2000; Zehr, 2015). The colleague harmed by a remark about a sacred practice needs a chance to say what repair would mean before facing the staff member. It beats A because ground rules matter once people are in the room, but opening a circle without preparation risks re-harming the colleague.

**Takeaway:** Before any restorative circle, meet separately with the harmed person and the person responsible to prepare, check readiness and confirm voluntary participation.

**Sources:** Office for Victims of Crime (2000). The Restorative Justice and Mediation Collection: Executive Summary (OVC Bulletin), Preparation: separate in-person meetings with each party; Zehr, H. (2015). The Little Book of Restorative Justice (revised and updated ed.). Good Books, Principles: harms and needs; voluntary participation
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 393. ETH-1714 (rev 2) · Best answer: A

In a youth diversion conference, a 16-year-old who vandalized a community garden hears from gardeners about the damage. Elders and the youth's coach say the act was wrong and hurtful, then describe the youth's strengths and invite the youth to help replant. The gardeners then thank the youth and ask them to join the next community workday. Which concept **MOST** likely describes this approach?

**A.** Reintegrative shaming
**B.** Stigmatizing shaming
**C.** Labeling
**D.** Deterrence

#### Why each option is right or wrong

**A** **Correct.** Braithwaite (1989) described reintegrative shaming as disapproval of the act from people the offender respects, followed by gestures of forgiveness and reacceptance. Here elders and the coach name the harm, affirm the youth's worth, offer a way to repair it and close by inviting the youth into the group's work, which separates the act from the person. It beats B because stigmatizing shaming condemns the person as well as the act and offers no path back, which tends to push young people toward others who reject the rules.

**B** **Incorrect** (trap: Wrong model or concept). Stigmatizing shaming marks the person as bad and leaves them outcast, and Braithwaite linked it to more offending. This conference condemns the act while affirming the youth and ends with an invitation to join in, the opposite pattern.

**C** **Incorrect** (trap: Wrong model or concept). Labeling theory explains how official labels such as 'delinquent' can push a young person toward a deviant identity. The conference avoids attaching a lasting label, so labeling describes a risk the process is designed to prevent.

**D** **Incorrect** (trap: Wrong model or concept). Deterrence aims to prevent offending through the threat of punishment. This conference relies on relationships, repair and acceptance rather than fear of penalties.

**Takeaway:** Reintegrative shaming disapproves of the harmful act while affirming the person and offering reacceptance; stigmatizing shaming rejects the person.

**Sources:** Braithwaite, J. (1989). Crime, Shame and Reintegration. Cambridge University Press, reintegrative versus stigmatizing shaming
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.8 Restorative practice, truth and reconciliation with historically oppressed groups · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 394. ETH-1603 (rev 1) · Best answer: C

At a family shelter, a client who fled political violence and arrived in the country 10 months ago tells the social worker, 'A friend said there is a time limit to ask for protection here. Should I apply for asylum?' The client has not met with any lawyer. What should the social worker do **FIRST**?

**A.** Explore the violence the client faced before leaving
**B.** Help the client write a statement for the asylum form
**C.** Arrange a prompt meeting with an immigration legal provider

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Understanding the client's experiences will matter for support and later for any legal claim. Opening a detailed trauma history now delays a time-sensitive legal step and may push the client to retell painful events before a lawyer can guide what is needed.

**B** **Incorrect** (trap: Outside the social work role). Drafting the asylum statement is legal work that shapes a claim and its credibility. Social workers may later support a client through the process, but preparing filings belongs with an immigration attorney or accredited representative.

**C** **Correct.** Whether and how to seek asylum is a legal question with strict filing rules, including a general one-year deadline after arrival (8 U.S.C. 1158(a)(2)(B)). Social workers do not give legal advice; they refer when another professional's specialized knowledge is needed (NASW Code 1.16(a)). With about 2 months left and no lawyer involved, a prompt link to an immigration attorney or accredited representative protects the client's options. It beats A because trauma-informed support matters but can continue alongside legal help and should not delay a time-limited decision.

**Takeaway:** Time-sensitive immigration questions go promptly to an immigration attorney or accredited representative; social workers support but do not give legal advice.

**Sources:** Immigration and Nationality Act, 8 U.S.C. 1158(a)(2)(B); NASW Code of Ethics (2021), 1.16(a)
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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### 395. ETH-1612 (rev 1) · Best answer: C

At a community health center, a social worker completes an intake with an adult client who was resettled as a refugee 3 months ago after 6 years in a refugee camp. The client reports poor sleep, headaches and worry about a sibling still abroad. What is the **MOST** appropriate way for the social worker to structure the assessment?

**A.** Focus on current resettlement needs such as housing and work
**B.** Begin with detailed processing of the most painful war memories
**C.** Ask about life before leaving, in transit and since arriving

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Housing, work and benefits are pressing and belong in the plan. Assessing only the present, though, misses how years in a camp and losses before leaving may drive the client's symptoms.

**B** **Incorrect** (trap: Right action, wrong time). Trauma processing can help later with a trained provider and a stable client. Starting an intake with detailed exposure to the worst memories, before safety and trust exist, can overwhelm a newly resettled client.

**C** **Correct.** A stage-of-migration framework assesses experiences before departure, during transit and after arrival, because stressors and strengths from each stage shape current functioning (Drachman, 1992). This client's history spans the home country, 6 years in a camp and 3 months of resettlement, plus ongoing worry about a sibling abroad, so the symptoms may link to any stage. It beats A because current needs are urgent and must be met, but focusing only on them misses trauma, losses and strengths from earlier stages.

**Takeaway:** Assess refugees and immigrants across migration stages, before departure, in transit and after arrival, since each can shape current functioning.

**Sources:** Drachman, D. (1992). A stage-of-migration framework for service to immigrant populations. Social Work, 37(1), 68-72, whole article
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 396. ETH-1619 (rev 2) · Best answer: B

A 14-year-old joined a parent in the United States 4 months ago after living 7 years with a grandparent in the home country while the parent worked here. The school social worker learns the teen is withdrawn, cries about the grandparent and refuses to follow the parent's rules. The parent calls the teen ungrateful. Which approach is **BEST**?

**A.** Help the parent set firmer rules with clear consequences
**B.** Help both see the conflict as grief and strain from long separation
**C.** Refer the teen for evaluation of oppositional defiant disorder

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Clear rules matter, and structure can help once the relationship is stronger. Leading with firmer consequences treats the conflict as simple misbehavior and can deepen the teen's sense of loss and resentment.

**B** **Correct.** Many immigrant youth go through long separations from parents during migration, and reunification often brings grief for the caregiver left behind, distance

from a parent who feels like a stranger, and conflict over authority (Suarez-Orozco et al., 2011). The teen's crying for the grandparent, withdrawal and refusal of rules 4 months after arrival fit this pattern. Helping parent and teen understand it and rebuild their bond addresses the cause. It beats A because firmer rules may intensify the power struggle while the teen still grieves and does not yet trust the parent's authority.

**C** **Incorrect** (trap: Skips assessment). Defiance that began with a major family transition and comes with grief points first to adjustment and loss. A diagnostic referral now risks labeling an expected reaction as a disorder.

**Takeaway:** After long migration-related separations, reunified families often face grief and authority conflict; address the separation, not just the behavior.

**Sources:** Suarez-Orozco, C., Bang, H. J., & Kim, H. Y. (2011). I felt like my heart was staying behind: Psychological implications of family separations and reunifications for immigrant youth. Journal of Adolescent Research, 26(2), 222-257, whole article

**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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### 397. ETH-1628 (rev 2) · Best answer: A

At a refugee resettlement agency, a client who arrived 8 months ago is behind on the electric bill. While going over the household budget, the client mentions sending about a third of each paycheck to parents and siblings in a refugee camp, then stops and watches the social worker's reaction. What should the social worker do **FIRST**?

**A.** Ask the client about the money sent home and what it means

**B.** Advise pausing the transfers until the electric bill is current

**C.** Build a budget that treats the transfers as a fixed expense

#### Why each option is right or wrong

**A** **Correct.** Many immigrants keep close financial and emotional ties with family abroad, and money sent home is often an obligation rather than a spending choice (Falicov, 2007). The client has just raised the transfers and paused, watching for a reaction, and the worker does not yet know what the money means to the client or the family. Asking openly treats the client as the expert on their own culture (NASW Code 1.05(c)) and shapes the budget that follows. It beats C because treating the transfers as fixed may well be right, but deciding it before the client explains assumes the meaning instead of learning it.

**B** **Incorrect** (trap: Too directive). Pausing the transfers might clear the bill fastest, but it imposes the worker's priorities before the worker even knows what the money does for the family. Whether to change the amount is the client's decision to make after weighing options (NASW Code 1.02).

**C** **Incorrect** (trap: Right action, wrong time). Building the budget around the transfers is often the right plan when they are a family obligation, and it may follow. Setting it up before the client explains what the money means skips the client's own account, which should shape the plan.

**Takeaway:** Ask what money sent to family abroad means before budgeting; then plan around it rather than asking the client to cut it.

**Sources:** Falicov, C. J. (2007). Working with transnational immigrants: Expanding meanings of family, community, and culture. Family Process, 46(2), 157-171, whole article; NASW Code of Ethics (2021), 1.05(c); NASW Code of Ethics (2021), 1.02
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 398. ETH-1635 (rev 2) · Best answer: C

A community counseling agency's intake form asks every new client for immigration status. No funder or program rule requires it, and staff say it is just for agency records. Since local enforcement actions made the news, several clients have left the question blank or ended intake early. What is the **BEST** recommendation for the social worker on the quality committee?

**A.** Keep the question but label it optional
**B.** Add a note on the form assuring clients of privacy
**C.** Take the question off the form

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Marking a question optional reduces pressure and works for some demographic items. For immigration status, the question itself can frighten clients in mixed-status families, and the agency would still hold sensitive answers it has no use for.

**B** **Incorrect** (trap: Misreads law or ethics). Reassurance helps, but confidentiality has legal limits, and a promise cannot fully protect records the agency should not keep. Collecting sensitive status data with no service purpose remains the core problem.

**C** **Correct.** The Code bars soliciting private information from clients except for compelling professional reasons (NASW Code 1.07(a)). Immigration status is sensitive, no program requires it, and asking everyone is now driving clients away. Removing the question protects privacy and access; status can be asked later, with an explanation, only if a specific benefit requires it. It beats A because an optional question still signals that the agency collects status, which is what deters fearful clients. The Code also names immigration status among the dimensions of oppression social workers must understand (NASW Code 1.05(d)).

**Takeaway:** Ask about immigration status only when a specific service or eligibility rule requires it; unneeded questions deter fearful clients.

**Sources:** NASW Code of Ethics (2021), 1.07(a); NASW Code of Ethics (2021), 1.05(d)
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### 399. ETH-1641 (rev 1) · Best answer: B

Which term describes a person who entered the United States on a student visa and is now applying for protection from persecution in the home country?

**A.** Refugee
**B.** Asylum seeker
**C.** Temporary protected status holder

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Refugee status is granted abroad through overseas processing before the person travels to the United States. Someone who applies after arriving is seeking asylum, even though the persecution standard is the same.

**B** **Correct.** An asylum seeker requests protection from persecution from inside the United States or at a port of entry; a refugee is screened and approved abroad before arrival (8 U.S.C. 1158; 8 U.S.C. 1157). Both rest on the same definition of persecution (8 U.S.C. 1101(a)(42)), but the status shapes services: refugees arrive with resettlement agency support, while asylum seekers awaiting a decision generally lack it. It beats A because the person applied after arriving, which makes them an asylum seeker rather than a refugee.

**C** **Incorrect** (trap: Wrong model or concept). Temporary protected status covers nationals of countries designated because of armed conflict or disaster, whatever their personal history of persecution. It is a separate humanitarian status, not the label for a pending persecution claim.

**Takeaway:** Refugees are approved abroad before arrival; asylum seekers apply from inside the country, often without resettlement supports while they wait.

**Sources:** Immigration and Nationality Act, 8 U.S.C. 1101(a)(42), 1157, 1158
**Area** I Values and Ethics · **Sub-area** I.C Diversity and Social Justice · **Topic** I.C.9 Globalization, immigration, refugee status effects on clients and services · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 400. ETH-1702 (rev 2) · Best answer: D

At a school team meeting, a teacher proposes testing a 12-year-old refugee student for intellectual disability because the student reads at an early-elementary level in English. Records show about 3 years of schooling in a refugee camp, taught in Arabic. The student solves math puzzles quickly and teaches classmates complex games. What is the **BEST** recommendation for the school social worker to make?

**A.** Proceed with the school's standard intelligence tests in English
**B.** Delay any evaluation until the student is fluent in English
**C.** Place the student in a self-contained class while testing proceeds
**D.** Seek native-language testing that accounts for lost instruction

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Standard English-language intelligence tests would measure exposure to English and to schooling more than ability. Results could mislabel a student whose gaps reflect lost instruction, which the rules do not allow as a basis for eligibility.

**B** **Incorrect** (trap: Good but not best). Waiting for English fluency can take years and may leave a real disability unaddressed. The better path is an evaluation in the student's language that accounts for interrupted schooling, alongside strong language instruction.

**C** **Incorrect** (trap: Skips assessment). Moving the student to a self-contained class before any evaluation assumes a disability and removes the student from peers and grade-level instruction. Placement decisions follow evaluation, not the other way around.

**D** **Correct.** Federal special education rules require assessments given in the child's native language and bar a disability finding when the determining factor is lack of appropriate instruction or limited English proficiency (34 CFR 300.304(c)(1); 300.306(b)). This student had about 3 years of interrupted camp schooling in Arabic, and quick math reasoning and complex play suggest strong learning ability. An evaluation in the student's language that weighs this history guards against mislabeling while still checking for a real disability. It beats B because language status alone is no reason to postpone, and postponing can leave a true need unmet.

**Takeaway:** For refugee students with interrupted schooling, seek evaluations in their language that weigh lost instruction before labeling a disability.

**Sources:** Individuals with Disabilities Education Act regulations, 34 CFR 300.304(c)(1)(i)-(ii); Individuals with Disabilities Education Act regulations, 34 CFR 300.306(b)
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### 401. ETH-1708 (rev 2) · Best answer: D

A 68-year-old refugee from a rural farming region has lived in the United States for 3 years. The client works part-time and enjoys time with grandchildren. The client mourns that the family can no longer hold seasonal ceremonies for ancestors at the village shrine and dreams that the ancestors are hungry and calling. The client says, 'I left them with no one to care for them.' Which concept **MOST** likely describes this distress?

**A.** Prolonged grief disorder
**B.** Posttraumatic stress disorder
**C.** Acculturative stress
**D.** Cultural bereavement

#### Why each option is right or wrong

**A** **Incorrect** (trap: Near-miss diagnosis). Prolonged grief disorder follows the death of someone close at least 12 months earlier, with near-daily yearning or preoccupation and significant distress or impairment (DSM-5-TR). This client's loss is of ceremonies and ancestral duties, and work and family life continue.

**B** **Incorrect** (trap: Near-miss diagnosis). PTSD requires exposure to a traumatic event plus intrusion, avoidance, negative changes in mood and thinking, and arousal symptoms. These dreams express guilt about unmet duties to ancestors rather than reliving a traumatic event, and daily functioning is intact.

**C** **Incorrect** (trap: Wrong model or concept). Acculturative stress comes from the strain of adapting to a new culture, such as language barriers or discrimination. This client's

pain points backward, toward lost practices and obligations, rather than toward difficulty adjusting to life here.

**D** **Correct.** Cultural bereavement describes a refugee's grief over the loss of homeland, community and cultural practices, including unmet obligations to ancestors, and it can be a meaningful, even constructive, response rather than a disorder (Eisenbruch, 1991). The distress centers on lost ceremonies and ancestors left without care, while work and family life continue. It beats A because prolonged grief disorder follows the death of a close person, with intense yearning and impairment; this grief is for a lost way of life and its duties.

**Takeaway:** Refugees' grief for lost homeland, rituals and ancestral duties may be cultural bereavement, a meaningful response, not automatically a psychiatric disorder.

**Sources:** Eisenbruch, M. (1991). From post-traumatic stress disorder to cultural bereavement: Diagnosis of Southeast Asian refugees. Social Science & Medicine, 33(6), 673-680, whole article; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder

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### 402. ETH-1712 (rev 2) · Best answer: A

At a community health center, an undocumented parent says a neighbor was just detained by immigration agents. The parent's children, ages 6 and 9, are U.S. citizens. After spending most of the session on the arrest, the sleepless nights since and a call that morning to the family's immigration attorney, the parent asks, 'If they take me, who will pick up my kids?' What should the social worker do **NEXT**?

**A.** Help the parent make a written plan naming a caregiver

**B.** Invite the parent to say more about the fear of detention

**C.** Reassure the parent that parents of citizen children are rarely detained

#### Why each option is right or wrong

**A** **Correct.** Immigration enforcement fear weighs on children in mixed-status families whether or not a parent is detained (Dreby, 2012), and a concrete plan restores some control. The parent has already voiced the fear and has legal help for the immigration case; the question now is practical. Helping the parent name a trusted caregiver, arrange school pickup permission and gather key documents answers the request, while the attorney advises on any legal forms. This supports the parent's own goal (NASW Code 1.02). It beats B because exploring a fear the parent has already described delays the planning the parent asked for.

**B** **Incorrect** (trap: Right action, wrong time). Exploring fear is a good opening when a client first raises enforcement worries. This parent has spent most of the session on the fear and now asks a specific practical question, so more exploration repeats ground already covered.

**C** **Incorrect** (trap: Ignores risk). No social worker can predict enforcement actions, and parents of citizen children are detained. False reassurance may leave the children without a plan and undermines the parent's trust.

**Takeaway:** When mixed-status families fear enforcement, help parents make concrete child-care plans and leave legal questions to their attorney.

**Sources:** Dreby, J. (2012). The burden of deportation on children in Mexican immigrant families. Journal of Marriage and Family, 74(4), 829-845, whole article; NASW Code of Ethics (2021), 1.02
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